



TEXAS
Health and Human
Services

Texas Medicaid and CHIP Reference Guide

FIFTEENTH EDITION

TEXAS HEALTH AND HUMAN
SERVICES COMMISSION
2024

The information presented in this book is intended to provide a helpful reference for the subjects discussed. This book is not meant to be used, nor should it be used, as an official record of Texas Medicaid and CHIP policies. Readers should be aware that the information listed in this book, including any websites, may change.

Foreword

When I started working at the Health and Human Services Commission (HHSC) in 2007 as a policy analyst, I never imagined that I would work in Texas Medicaid and Children's Health Insurance Program (CHIP) for over 15 years or that I would go on to serve as the State Medicaid Director. The mission drew me in, and I continue to be proud of the work we do to better the lives of the Texans we serve.

Texas has one of the largest Medicaid programs in the country, serving millions of people across the state. To ensure that we continue to deliver quality, cost-effective services, Texas Medicaid and CHIP is focused on several main priorities. We ensure that members have timely access to the services they need, and we work hard to encourage provider participation in the program. To incentivize value, we are shifting away from a transactional model of health care to focus instead on quality of care and better health outcomes. Finally, we continue to strengthen partnerships in the managed care model to improve service delivery and enhance oversight.

HHSC has made significant strides toward modernizing systems, upgrading processes, and advancing services, but we still have room for growth. As this book goes to print, we are moving forward with key initiatives, including improvements to services in areas such as behavioral health and maternal health. Looking to the future, we are committed to continuous advancement — incremental gains will lead to significant impacts for our members as we respond to the ever-changing health care landscape.

We are pleased to bring you the 15th edition of the Texas Medicaid and CHIP Reference Guide. I want to highlight that this edition has a new appendix that features our Texas Medicaid and CHIP roadmaps. These maps capture where we have been and where we are heading in a creative, dynamic way. We create this resource to help people better understand our programs, and we hope it will be useful as a guide to the work we do at Texas Medicaid.

With gratitude,

A handwritten signature in dark ink, appearing to read "Emily Zalkovsky", with a long, sweeping horizontal line extending to the right.

Emily Zalkovsky
State Medicaid Director

The Data in this Guide

Below are details about the data referenced throughout the 15th edition. Information contained in this book was current as of August 2024, unless otherwise noted. Program and financial information may change after publication due to changes to federal and state regulations, the state of the economy, and other factors.

About	Details
Time period	All data is state fiscal year (SFY) 2023, unless otherwise noted. Income limits are effective as of March 1, 2024.
Caseload count	Caseload trends and numbers are based on the monthly average number of clients covered by Medicaid or CHIP. The unduplicated count, which is the total number of individual Texans who received Medicaid or CHIP services over a period of time, is noted if used. Total caseload numbers combine Medicaid and CHIP, unless otherwise noted. Caseloads in this book do not reflect the unwinding of continuous coverage due to the federal Public Health Emergency (PHE).
Cost information	Costs include both partial- and full-benefit clients, unless otherwise noted. Funds exclude Disproportionate Share Hospital (DSH), Uncompensated Care (UC) and Delivery System Reform Incentive Payment (DSRIP) funds, unless otherwise noted.
Sources	Health and Human Services Commission (HHSC) Financial Services, including Forecasting, provided the majority of data contained in this book. Their primary sources are: • Premiums Payable System data provides a summary of all Medicaid-enrolled clients each month. Both monthly Premiums Payable System files and final eight-month files, which contain all retroactive adjustments, are used in the analyses. • Expenditure information is obtained from the Texas Medicaid & Healthcare Partnership through the databases in the Vision 21 platform, which includes paid claims, managerial reporting of cash flow, provider and client information, and managed care encounter data. Expenditures include direct payments to physicians, hospitals and entities that provide ancillary services. Financial information is provided using form CMS-64, Quarterly Medicaid Statement of Expenditures for the Medical Assistance Program and form CMS-37, the Medicaid Program Budget Report. Additional financial information is provided by the Medicaid Statistical Information System. • Unpublished analyses conducted by HHSC Financial Services staff are also used to provide financial information. Alternative sources are noted throughout the book.

Helpful Websites

Website Name	Link	What You'll Find
Health and Human Services Commission (HHSC) - Medicaid and CHIP	HHS.Texas.gov/Services/Health/Medicaid-CHIP	The Medicaid and CHIP landing page provides access to descriptions about programs and services, resources for members (including managed care report cards, Service Delivery Area maps, etc.), topics for business partners and general information about Medicaid and CHIP, including enrollment statistics.
Medicaid and CHIP Current Caseload Data	HHS.Texas.gov/about/records-statistics/data-statistics/healthcare-statistics	HHSC posts Medicaid and CHIP caseload data to this page on a regular basis.
HHSC CFO Reference Guide	HHS.Texas.gov/sites/default/files/documents/cfo-reference-guide-2024.pdf	The HHSC CFO Reference Guide provides a detailed overview of agency finances. The following sections give insight into how HHSC puts together its budget: • Medicaid and CHIP Client Services • Non-Medicaid Client Services • Facility-Based Services • Other Agency Topics
Health Care Payment Learning and Action Network (HCP-LAN)	HCP-LAN.org	A website with resources focused on increasing the percentage of U.S. health care payments tied to quality and value, including the HCP-LAN Alternative Payment Model (APM) framework, a consensus framework for classifying APMs. HHSC uses this framework for its Texas Medicaid value-based payment requirements for managed care organizations (MCOs).
Office of the Ombudsman	HHS.Texas.gov/services/your-rights/hhs-office-ombudsman	The Office of the Ombudsman manages and resolves complaints submitted by Texans receiving Health and Human Services benefits, including Medicaid.
Office of the Inspector General, Texas Health and Human Services	OIG.hhs.texas.gov	The public, clients and providers may refer potential fraud, waste and abuse on this website or call the Office of Inspector General's (OIG) fraud hotline at 800-436-6184.

Website Name	Link	What You'll Find
Texas Healthcare Learning Collaborative (THLC) Portal	THLCPortal.com	A public reporting platform and a tool for contract oversight and MCO quality improvement efforts, including MCO report cards and other key performance data.
Your Texas Benefits	YourTexasBenefits.com	A self-service website where Texans can apply for HHSC programs and manage their benefits.
Uniform Managed Care Manual	HHS.Texas.gov/services/health/medicaid-chip/managed-care-contract-management/texas-medicaid-chip-uniform-managed-care-manual	Defines procedures that MCOs must follow to meet certain HHSC managed care contract requirements.
Vendor Drug Program (VDP)	TxVendorDrug.com	Information about VDP, including the Texas Formulary, the Preferred Drug List (PDL), prior authorizations, program enrollment and provider resources.

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Quick Facts

Quick facts about Medicaid and CHIP

Quick Facts About Medicaid and CHIP

General Information

Medicaid

A health care and long-term services and supports program for certain groups of low-income people

Serves children and their caretakers, pregnant women, and individuals over age 65 or those with disabilities

Funded through state funds, matched with uncapped federal dollars at a set percentage rate



CHIP

A similar program for children whose families earn too much to qualify for Medicaid

Serves children and the unborn children of pregnant women

Funded through state funds, matched with capped federal dollars at a set percentage rate

99% of total enrollment

5.7 million people enrolled in Medicaid in 2023

1% of total enrollment

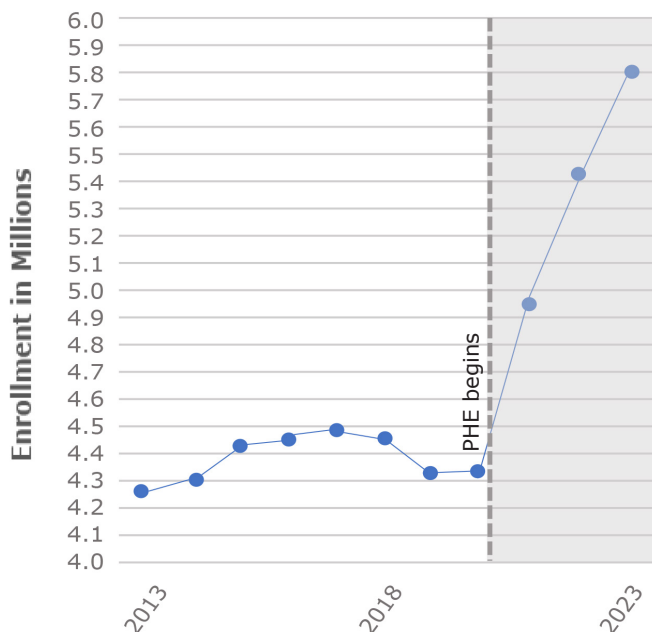
81K people enrolled in CHIP in 2023

Caseloads impacted, see below.

Snapshot of Texas Medicaid and CHIP Clients

Total Medicaid and CHIP Enrollment

Caseloads were impacted by the federal Public Health Emergency (PHE) (see Chapter 1, page 12).



19% of Texans covered



53% of Texas births covered by Medicaid*



51% of Texas children on Medicaid or CHIP



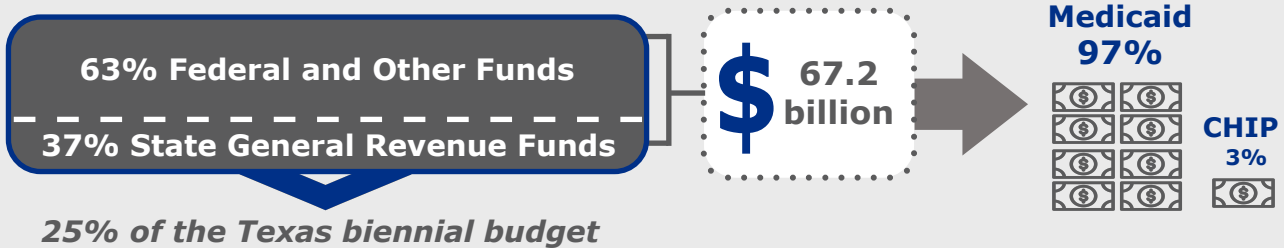
56% of nursing home residents covered by Medicaid

For a breakdown of Medicaid vs. CHIP enrollment by year see Appendix C, page 168.

*Data is from 2022.

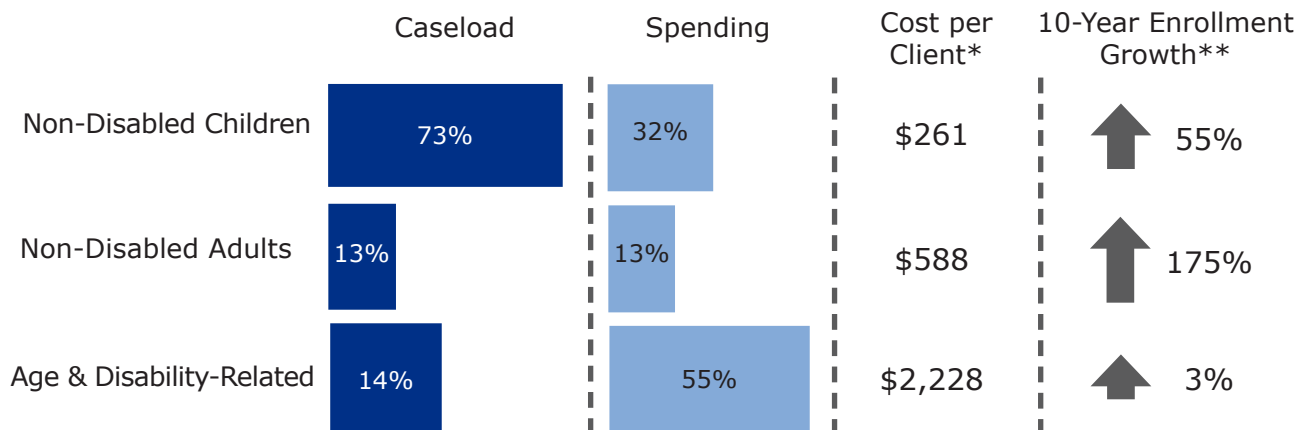
Quick Facts About Medicaid and CHIP

Biennial Funding



Spending

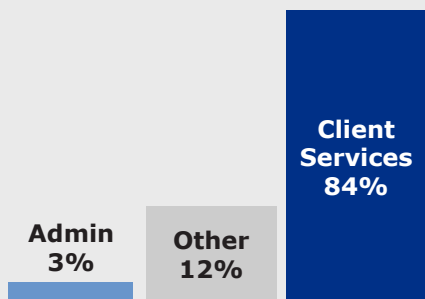
Trends in Caseload and Spending for Major Medicaid Client Categories



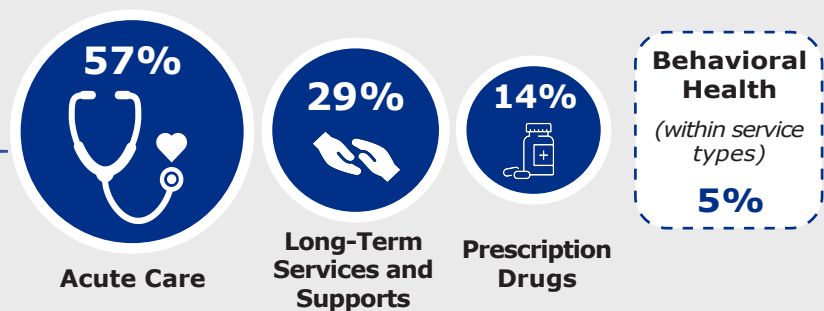
*Average cost per client per month is based on full-benefit clients only.

**Caseloads were impacted by the federal PHE declared in March 2020 (see Chapter 1, page 12).

Spending by Category



Spending by Service Type



Percentages are rounded.

Other: includes DSH, UC/PHP-CCP, DSRIP and supplemental payment programs.

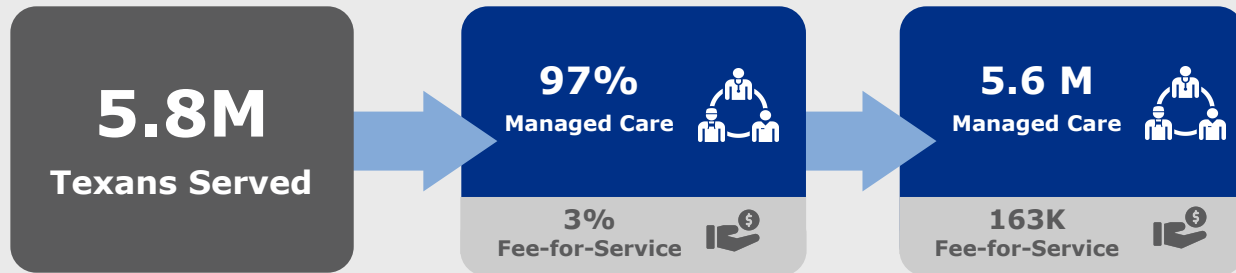
Acute Care: includes Medicare-related, MTP and Dental expenditures.

Prescription Drug: includes Clawback.

Behavioral Health: excludes DSH, UC, UC/PHP-CCP, DSRIP, Supplemental Payment Program and agency administration costs.

Quick Facts About Medicaid and CHIP

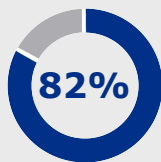
Service Delivery Model



Key Attributes of the Managed Care Service Delivery Model

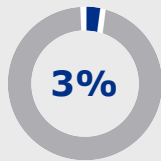
- ✓ Delivers services through managed care organizations (MCOs) that are paid a fixed amount per member enrolled per month
- ✓ Achieves value by incentivizing MCO improvements in quality of care and cost-effectiveness
- ✓ Serves as the member's "medical home" by providing comprehensive preventive and primary care

Managed Care Product Lines



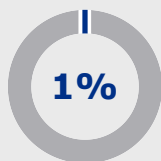
STAR

Children, pregnant and postpartum women, and some families



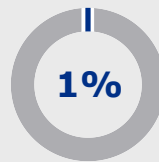
STAR Kids

Children and youth with disabilities



CHIP

Children and youth who don't qualify for Medicaid due to family income



STAR Health

Children who get Medicaid through the Department of Family and Protective Services and young adults previously in foster care



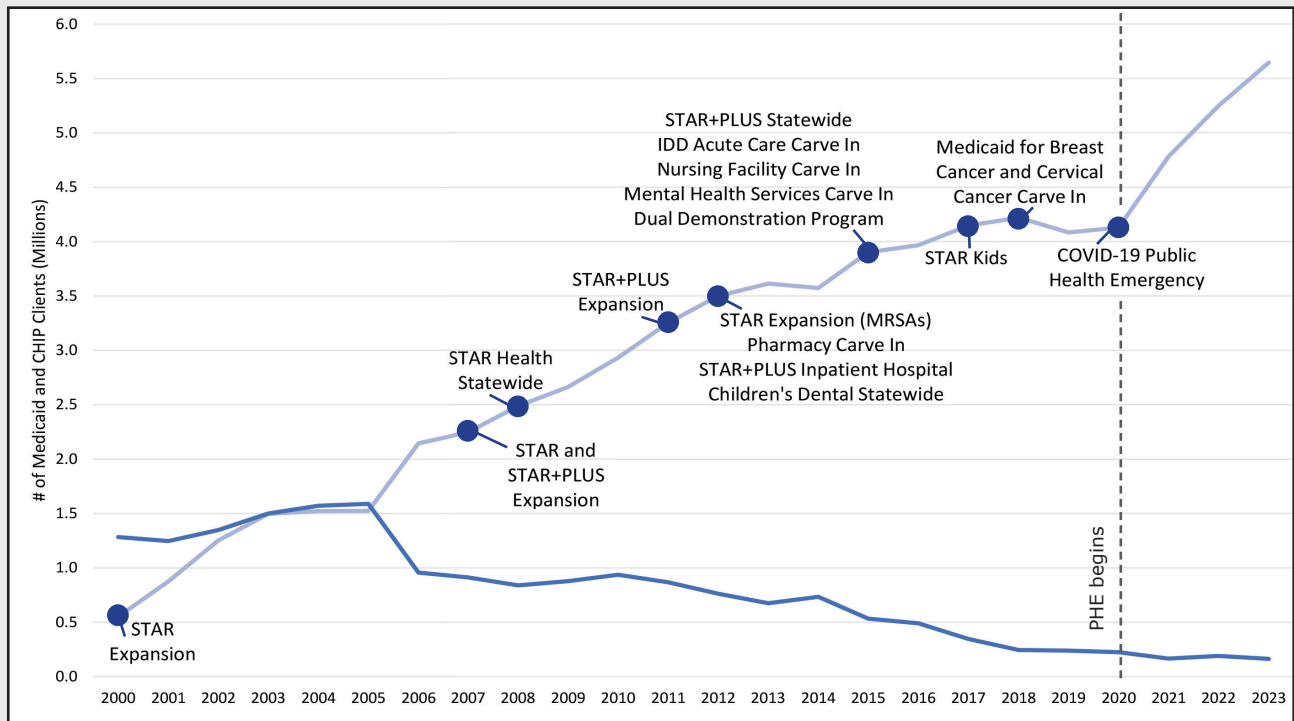
STAR+PLUS

Adults with a disability, people age 65 and older (including those dually eligible for Medicare and Medicaid), and women with breast or cervical cancer

CHIP includes CHIP Perinatal. Remaining percentage is FFS.

Quick Facts About Medicaid and CHIP

Managed Care Growth Over Time



Over the Past 10 Years

Texas Medicaid managed care enrollment has increased by 35%.

Preventative Care Improved



Childhood Behavioral Health

Mitigating long-term impacts of antipsychotics **↑31%**



Adult Behavioral Health

Members adhering to prescribed medications **↑13%**



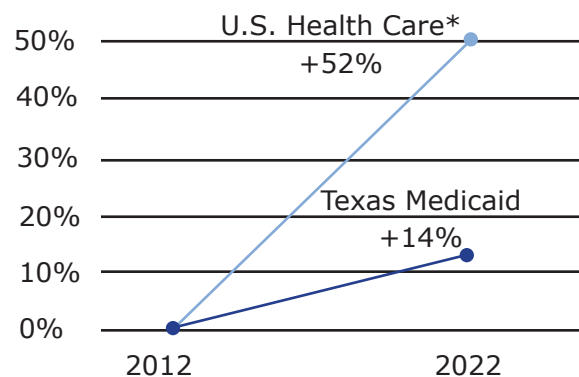
Chronic Conditions

Asthma management for adults and children **↑11%**

Cost Growth Contained

Increased enrollment and improved preventative care within managed care keeps Texas Medicaid costs contained — 38 percentage points lower than the U.S. national average

Cost per Person Increase



Data: STAR only, 2014 vs. 2022 (9 years). Percentages are estimates due to methodology changes that occurred over time.

Texas Medicaid is based on full-benefit clients.

*Source: CMS, Office of the Actuary — data is for CY12 to CY22.

Chapter 1

Who can get Medicaid or CHIP, and how can they get it?



At-a-Glance

Who can get Medicaid or CHIP?

Medicaid provides health care and long-term services and supports (LTSS) to low-income children and their families, pregnant women, former foster care youth, people with disabilities, and people age 65 and older.

The Children's Health Insurance Program (CHIP) provides health care to children who are not eligible for Medicaid based on their family's income. Texans who apply for benefits and do not qualify for Medicaid are automatically tested for CHIP eligibility.

How can Texans get Medicaid or CHIP?

Apply for Benefits



Eligibility Determination



Enrollment



Four ways to apply:

[YourTexasBenefits.com](https://www.yourtexasbenefits.com)

Local eligibility offices and community partners

2-1-1 phone service

Mail or fax

Eligibility is determined by:

Household income



Citizenship and residency



Program requirements



Most clients are enrolled in managed care, where they:

1. Choose a health plan.

2. Pick a primary care provider.

3. Receive a Your Texas Benefits Card and health plan ID card.

Residency and Citizenship

To qualify for Texas Medicaid, an applicant must:

- Live and intend to remain in Texas.
- Have a Social Security number (SSN) or apply for one.
- Be a citizen of the United States (U.S.) or meet alien status requirements.

All U.S. citizens and nationals are entitled to apply for and get Medicaid if they meet all eligibility requirements and can provide proof of citizenship.

Qualified Aliens

Most noncitizens cannot qualify for regular Medicaid or Children’s Health Insurance Program (CHIP) benefits. Qualified aliens are usually subject to limited eligibility or to a waiting period. The following categories do not have limited eligibility or a waiting period:

- Compact of Free Association citizens residing in the U.S.
- Veterans; active-duty members of the U.S. armed forces, including their spouses and dependent children; and Canadian-born American Indians.
- Lawful permanent residents (LPRs) who received Supplemental Security Income (SSI), Medicaid or both on Aug. 22, 1996, or can be credited with 40 qualifying quarters of Social Security coverage and have lawfully resided in the U.S. since Aug. 22, 1996.

Limited Eligibility

Some aliens may qualify for Medicaid for up to seven years. These include people who have been granted asylum, refugees, Afghan and Iraqi Special Immigrants, Cuban/Haitian entrants, Amerasians, and aliens whose deportations are being withheld. Victims of human trafficking are eligible for up to four years from date of entry or until the law enforcement extension expires.

Lawful Permanent Residents

LPRs are noncitizens who are lawfully authorized to live permanently within the U.S. LPRs who arrived on or after Aug. 22, 1996, are typically subject to a five-year waiting period.

Some qualified immigrant and non-immigrant alien children lawfully residing in the U.S. may qualify for Medicaid or CHIP regardless of their date of entry (see Appendix A, page 141, for the Children’s Health Insurance Program Reauthorization Act of 2009).

Emergency Medicaid

Undocumented aliens and other noncitizens who do not have a qualifying alien status for Medicaid may qualify for Emergency Medicaid coverage if all other eligibility requirements are met except for alien status. Undocumented aliens are not required to provide an SSN. If determined eligible, Medicaid only covers their care until the emergency medical condition is stabilized.

Financial Eligibility

Financial eligibility for Medicaid and CHIP is primarily based on how an applicant's household income compares to the U.S. Department of Health and Human Services' definition of the federal poverty level (FPL). The FPL is updated annually.

Under federal law, financial eligibility for most Medicaid and CHIP applicants must be determined using Modified Adjusted Gross Income (MAGI) methodologies. MAGI uses federal income tax rules to decide if an applicant qualifies based on how they plan to file their taxes and their countable income. Applicants do not have to file federal income taxes to apply or to be determined eligible for Medicaid.

Some groups are excluded from MAGI, including:

- People who are age 65 and older.
- People with disabilities.
- People receiving SSI.

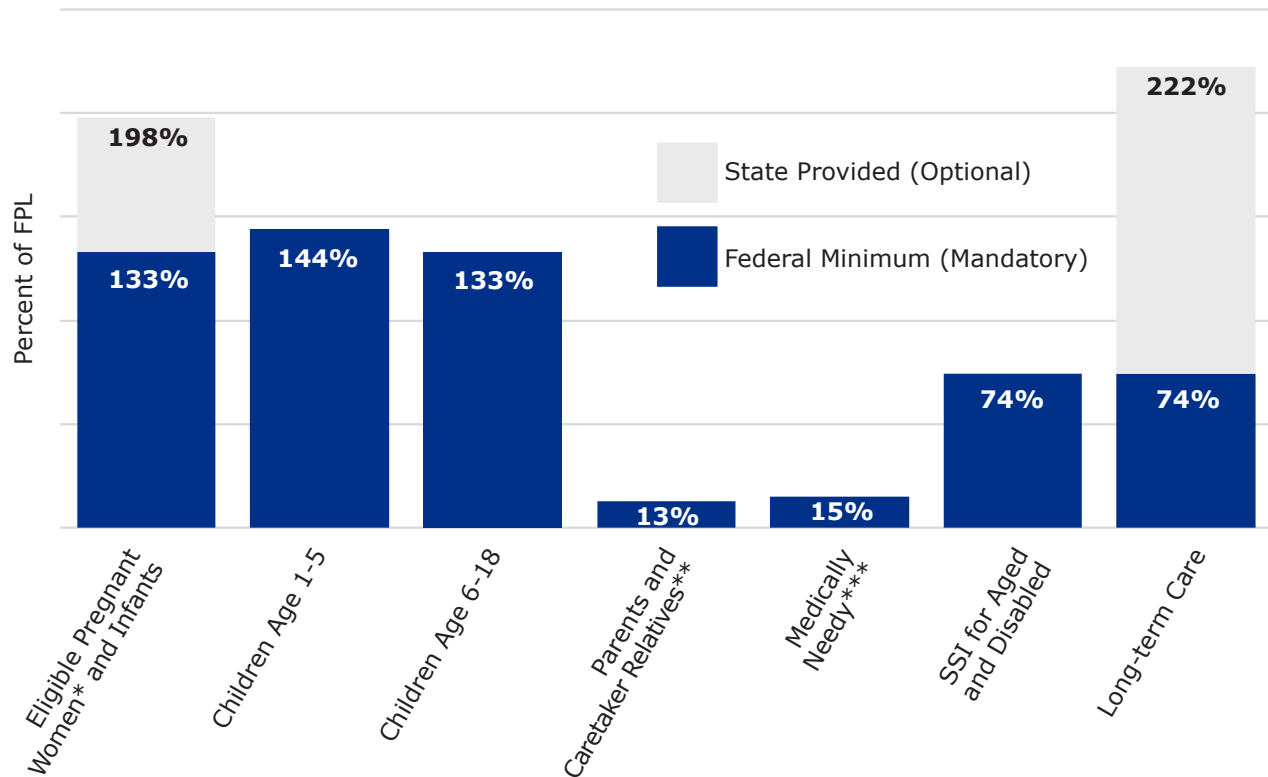
These groups must report and provide proof of the resources they own or are paying for, such as vehicles, checking or savings accounts, debit accounts, life insurance policies and property, including rental homes.

Mandatory vs. Optional Coverage

Federal law requires states to cover Medicaid-eligible groups up to a minimum percentage of the FPL and allows states the option to expand eligibility beyond federal standards.

The following chart depicts the 2024 Texas Medicaid income levels for the most common Medicaid eligibility categories. Mandatory levels identify the coverage required by the federal government. Optional levels show the coverage Texas has implemented.

Texas Medicaid Income Eligibility Levels for Selected Programs (as a Percent of the FPL)



*Through 12 months postpartum.

**For Parents and Caretaker Relatives, maximum monthly income limit in SFY24 was \$230 for a family of three, or about 13 percent of the FPL.

***For Medically Needy pregnant and postpartum women and children, the maximum monthly income limit in SFY24 was \$275 for a family of three, or about 15 percent of the FPL.

This figure reflects eligibility levels as of March 2024. In 2014, the Affordable Care Act required states to adjust income limits for pregnant women, children, and parents and caretaker relatives to account for MAGI changes.

Continuous Medicaid Coverage During the COVID-19 Public Health Emergency

COVID-19 Public Health Emergency

In response to the COVID-19 pandemic, the federal government declared a Public Health Emergency (PHE) in 2020 and required that all recipients be allowed to keep their Medicaid coverage regardless of eligibility — this was referred to as continuous coverage. As a result, the number of Texans receiving Medicaid increased from 3.9 million to about 6 million, from March 2020 through March 2023. Even though the PHE officially ended on May 11, 2023, federal legislation ended continuous coverage sooner.

Unwinding Continuous Coverage

At the end of 2022, Congress delinked the continuous coverage requirement from the PHE and allowed states to begin ending coverage as early as March 31, 2023. On April 1, 2023, Health and Human Services Commission (HHSC) initiated the “unwinding” process, which included a phased eligibility review of all Texans receiving Medicaid. The review started with people who were least likely to still be eligible, and by March 31, 2024, HHSC had initiated reviews for 100% of the active Medicaid recipients. For current caseload data, visit <https://www.hhs.texas.gov/about/records-statistics/data-statistics/healthcare-statistics>.

HHSC developed a multi-pronged communication plan and recruited 568 “ambassadors” to help inform recipients, providers, advocates and other stakeholders about the unwinding process. Recipients received information about the renewal process by mail, robocalls, emails and texts. HHSC implemented a federally approved waiver allowing managed care organizations (MCOs) to help recipients with their renewal applications. With funds appropriated by the 88th Legislature, HHSC also invested in recruitment and retention efforts for the eligibility workforce by hiring more than 1,000 eligibility staff and 400 call center staff.

After HHSC completed the 12-month unwinding efforts, about 51 percent of all eligibility renewals were approved, and 46 percent were denied. One-third of those were denied because they were no longer eligible, and two-thirds were denied due to procedural issues, such as failing to return the renewal form. People who were denied for Medicaid renewal were automatically referred to other coverage options, such as CHIP or the Federal Marketplace. As of April 10, 2024, and after continuous coverage, about 25 percent of those who are no longer eligible for Medicaid or CHIP now receive benefits through the Federal Marketplace.*

People who lost coverage can appeal by calling 2-1-1 or visiting a local eligibility office.

*The numbers included reflect the status of renewals from April 1, 2023 to April 10, 2024, as published in the April 2024 End of Continuous Coverage Dashboard. While renewals of all active Medicaid recipients were initiated by March 31, 2024, members were allowed 30 days to complete and return their renewal forms. Numbers do not add up to 100 percent due to 3 percent of the renewals being pended.

Timeline of Continuous Medicaid Coverage During the COVID-19 PHE

January 2020	March 2020	December 2022	March 2023	May 2023	March 2024
The U.S. Department of Health and Human Services declared a PHE in response to the COVID-19 pandemic.	Federal legislation required states to maintain Medicaid coverage for recipients, regardless of eligibility.	Federal legislation ended continuous Medicaid coverage and allowed states to begin a 12-month unwinding process on April 1, 2023.	The U.S. Department of Health and Human Services ended continuous Medicaid coverage on March 31, 2023.	The U.S. Department of Health and Human Services officially ended the PHE on May 11, 2023.	HHSC initiated eligibility reviews for all active Medicaid recipients by March 31, 2024.



Children and Youth

Children's Medicaid

Most Medicaid clients are children. To qualify for Children's Medicaid, a child must be age 18 or younger. Based on the 2023 Consolidated Appropriations Act, beginning Jan. 1, 2024, eligible children under the age of 19 receive 12 months of Medicaid or CHIP continuous eligibility coverage, regardless of changes in income.

Newborns (under 12 months) born to mothers getting Medicaid at the time of delivery are automatically eligible for Children's Medicaid and remain eligible through the month of their first birthday.

The table below shows the 2024 Children's Medicaid monthly income limits by household size for children age 6 through 18. Resources are not considered when determining eligibility for Children's Medicaid.

Monthly Income Limits by Household Size for Children's Medicaid	
Household Size (Adults Plus Children)	Household Income
1*	\$1,670
2	\$2,266
3	\$2,862
4	\$3,458
For each additional person, add:	\$597

*A family of one might be a child who does not live with a parent or other relative.

Former Foster Care Youth

Medicaid for Former Foster Care Children

Children who aged out of the foster care system at age 18 and were receiving federally funded Medicaid at the time may continue to be eligible for Medicaid through the end of the month of their 26th birthday. Income and resource limits do not apply.

Medicaid for Transitioning Foster Care Youth

Former foster care youth who were not receiving Medicaid when they aged out of foster care at age 18 may be eligible for Medicaid for Transitioning Foster Care Youth (MTFCY) through the end of the month of their 21st birthday.

To qualify, a person must meet residency and citizenship requirements and:

- Not have adequate health coverage.
- Be at or below 413 percent of the FPL (or \$5,184 per month for a household of one).

Resource limits do not apply to MTFCY. In addition, a person under an Interstate Compact on the Placement of Children agreement may be eligible for MTFCY if all other eligibility requirements are met.

Children's Health Insurance Program

CHIP covers children who do not qualify for Medicaid based on household income but do not have other health insurance. To qualify for CHIP, a child must be:

- Age 18 or younger.
- Uninsured for at least 90 days or have a "good cause" exemption.
- Living in a household with an income at or below 201 percent of the FPL.

Enrollment fees and copays are determined based on household income. Yearly enrollment fees are \$50 or less per family. Copays for both doctor visits and medicine range from \$0 to \$5 for lower-income families and \$20 to \$35 for higher-income families.

See the table below for the CHIP monthly income limits by household size. Resources are not considered when determining eligibility for CHIP.

Monthly Income Limits by Household Size for CHIP	
Household Size (Adults Plus Children)	Household Income
1*	\$2,523
2	\$3,424
3	\$4,325
4	\$5,226
For each additional person, add:	\$902

*A family of one might be a child who does not live with a parent or other relative.

Incarcerated Youth

Certain incarcerated people may be eligible for Medicaid coverage for inpatient medical services provided in a “free-world” medical facility not located on the premises of a jail or prison, including:

- Children age 18 and younger incarcerated by the Texas Department of Criminal Justice, the Texas Juvenile Justice Department or the Juvenile Probation Department.
- Pregnant women incarcerated by the Texas Department of Criminal Justice.

Children receiving Children’s Medicaid may have their coverage suspended upon admission into a juvenile facility and reinstated upon release.



Women

Medicaid for Pregnant Women

As part of House Bill 12 passed during the 88th legislative session, HHSC extended its postpartum Medicaid and CHIP coverage from two to 12 months for eligible women, effective March 1, 2024.

The 12 months of postpartum coverage begins the month after a pregnancy ends. Eligible women receive continuous coverage during the pregnancy and 12-month postpartum period unless they voluntarily withdraw, move out of state, or are determined to be ineligible due to client fraud or agency error.

See the table below for the Medicaid for Pregnant Women monthly income limits by household size, which includes unborn children.

Monthly Income Limits by Household Size for Medicaid for Pregnant Women	
Household Size	Monthly Household Income
1	\$2,485
2	\$3,373
3	\$4,261
4	\$5,148
For each additional person, add:	\$888

CHIP Perinatal

CHIP Perinatal (CHIP-P) services are for the unborn children of uninsured pregnant women who do not qualify for Medicaid for Pregnant Women. To qualify, pregnant women must have an income at or below 202 percent of the FPL.

For CHIP-P members at or below 198 percent of the FPL, the mother must apply for Emergency Medicaid to cover her labor and delivery. If the mother's labor and delivery are covered by Emergency Medicaid, her CHIP-P newborn may receive 12 months of continuous Medicaid coverage from date of birth.

CHIP-P newborns in families with incomes above 198 percent of the FPL receive CHIP benefits from date of birth through the remainder of the 12-month original eligibility period.

See the table below for the CHIP-P monthly income limits by household size.

Monthly Income Limits by Household Size for CHIP-P	
Household Size	Household Income
1	\$2,536
2	\$3,441
3	\$4,347
4	\$5,252
For each additional person, add:	\$906

Healthy Texas Women

Healthy Texas Women (HTW) provides free health and family planning services to low-income women age 15 through 44 who qualify. A woman is considered 44 years old through the last day of the month of her 45th birthday. Women age 15 through 17 must have a parent or legal guardian apply, renew and report changes on their behalf.

To qualify for HTW, a woman must:

- Not be eligible to receive full Medicaid benefits, CHIP, or Medicare Part A or B.
- Not be pregnant.
- Not have any other creditable health coverage — unless filing a claim would cause physical, emotional or other harm from a spouse, parent or another person.
- Have a household income at or below 204.2 percent of the FPL.

To provide continuity of care, women receiving Medicaid for Pregnant Women are automatically tested for HTW eligibility before their Medicaid for Pregnant Women coverage ends.

Healthy Texas Women Plus

HTW Plus provides enhanced and targeted postpartum care services to HTW clients whose pregnancies ended within the 12 months preceding their enrollment in HTW. Enhanced postpartum benefits include services that address:

- Postpartum depression and other mental health conditions.
- Cardiovascular and coronary conditions.
- Substance use disorders, including drug, alcohol and tobacco use.

There is no separate eligibility determination for HTW Plus. If a woman is determined eligible for HTW and her pregnancy ended within the 12 months preceding her HTW eligibility date, she is eligible for HTW Plus benefits during her 12-month HTW certification period.

Breast and Cervical Cancer Screening Services

Breast and Cervical Cancer Screening (BCCS) services are diagnostic services for women age 18 and older who either need to be tested for or already have a breast or cervical cancer diagnosis, including precancerous conditions. These services are offered at clinic sites across Texas.

BCCS clinics serve as the point of access for applying for Medicaid for Breast and Cervical Cancer (MBCC). Women cannot apply through an HHSC benefits office. To qualify for diagnostic services, women must:

- Not have health insurance.
- Have a net household income at or below 200 percent of the FPL.

Medicaid for Breast and Cervical Cancer Program

The MBCC program provides full Medicaid benefits, including services beyond the treatment of breast and cervical cancer, to eligible women applying through BCCS providers.

To qualify, a woman must:

- Be age 18 through 64.
- Be uninsured.
- Be diagnosed and in need of treatment for either a precancerous condition or a biopsy-confirmed, metastatic, or recurrent breast or cervical cancer.

Women continue to receive full Medicaid benefits if they meet the eligibility criteria at their coverage renewal period and provide proof from their doctor that they are receiving treatment for breast or cervical cancer.



Parents and Families

Medicaid for Parents and Caretaker Relatives

Adults who care for a child receiving Medicaid can also be eligible for Medicaid. To qualify, they must be a related caretaker to a child who has Medicaid and meet the program's income limits. The child must be living with them and be age 17 or younger or be age 18 and attending school full-time.

Related caretakers may be:

- A parent or stepparent.
- A sibling or stepsibling.
- A grandparent.
- An uncle or aunt.
- A nephew or niece.
- A first cousin or the child of a first cousin.

See the table below for the monthly income limits by household size.

Monthly Income Limits by Household Size for Medicaid for Parents and Caretaker Relatives		
Household Size	Monthly Income for One Parent Household	Monthly Income for Two Parent Household
1	\$103	-----
2	\$196	\$161
3	\$230	\$251
4	\$277	\$285
For each additional person, add:	\$52	\$52

Medically Needy with Spend Down Program

The Medically Needy with Spend Down program helps both families with children age 18 and younger and pregnant women who do not qualify for Medicaid to pay for unpaid medical expenses.

To qualify, a person or family subtracts their health care expenses from their income until they meet the program's income limits, also called "spending down." The income limit is \$275 per month for a family of three. The program also has an asset requirement. The asset limit is \$3,000 for households with a member who is elderly or has a disability and \$2,000 for all other households. Assets are not considered for pregnant women. Pregnant women who meet the spend-down amount receive continuous eligibility coverage throughout the pregnancy and postpartum coverage period without being required to meet the spend-down amount again.

The spend-down amount is different for each household and is any amount over the income limit. Clients who are determined eligible must submit their paid and unpaid medical bills. Medical bills greater than the spend-down amount are covered or reimbursed by Medicaid.

The Health Insurance Premium Payment Program

The Health Insurance Premium Payment (HIPP) program helps families where at least one person receives Medicaid pay for employer-sponsored health insurance premiums. When the cost of the premium is less than the cost of projected Medicaid expenditures, HIPP reimburses clients for their share of their employer-sponsored health insurance premium.

HIPP clients eligible for Medicaid do not pay out-of-pocket deductibles, copayments or coinsurance for Medicaid services delivered by a Medicaid provider. If a Medicaid service is not covered by their employer-sponsored health plan, clients can still get the service at no cost to them if a Medicaid provider administers the service.

HIPP clients not eligible for Medicaid must pay deductibles, copayments and coinsurance as required.



Children and Adults with Disabilities

Children and adults with disabilities who get Medicaid are usually in one or more of the following groups:

- They reside in a nursing facility or an intermediate care facility for individuals with an intellectual disability or related condition (ICF/IID).
- They get SSI.
- They get home and community-based services through a waiver program (see A Closer Look, page 23).

Children with disabilities can get coverage through the Children's Medicaid program or the Medicaid for the Elderly and People with Disabilities (MEPD) program. To qualify, children with disabilities must be age 20 and younger.

Medicaid for the Elderly and People with Disabilities

Adults with disabilities, some children with disabilities, and people age 65 and older may be able to get health coverage through the MEPD program.

To qualify, people with disabilities must meet the income and resource limits for Medicaid. Those with disabilities who do not receive SSI may qualify for MEPD through a facility, such as a nursing facility or ICF/IID, or through a waiver program.

Medicaid Buy-In for Children

The Medicaid Buy-In for Children (MBIC) program offers low-cost Medicaid services to children with disabilities in families that make too much money to get Medicaid. MBIC members may make monthly payments, which vary based on household income and health insurance status.

If a member has insurance through an employer, the payment could be up to \$230 per month. If they have insurance through an employer and get HIPP, the payment could be up to \$70 per month.

Those without health insurance through an employer and who cannot get HIPP do not have to make a monthly payment.

See the table below for the FY2024 MBIC monthly income limits by household size.

Monthly Income Limits by Household Size for Children and Adults with Disabilities	
Household Size (Adults Plus Children)	Monthly Household Income
1*	\$1,883
2	\$2,555
3	\$3,228
4	\$3,900
For each additional person, add:	\$672

*A family of one might be a child who does not live with a parent or other relative.

Medicaid Buy-In for Adults

The Medicaid Buy-In (MBI) for Adults program offers low-cost Medicaid services to adults with disabilities who work. Adults may qualify if they meet all the following criteria:

- They have a disability (the MBI program uses the Social Security disability guidelines to determine if applicants have a disability).
- They are working and meet the income limits for the program.
- They are not continuously living in a state institution or nursing facility.

The 2024 income limit for MBI is \$3,138 per month. Countable assets must total no more than \$2,000. Only earned income is counted toward this income limit. Unearned income is only considered in the calculation of the monthly premium, not eligibility.

How People Can Access Long-Term Services and Supports

Aging and Disability Resource Centers

Aging and Disability Resource Centers (ADRCs) provide person-centered services to people and caregivers regardless of age, income and disability. ADRCs partner with a network of local service agencies to coordinate information and referrals for people needing access to long-term services and supports (LTSS) programs and benefits, including Medicaid and Medicaid waiver programs.

LIDDAs and LMHAs

Local intellectual and development disability authorities (LIDDAs) serve as the point of entry for publicly funded intellectual and developmental disability (IDD) programs. LIDDAs help people enroll in Medicaid programs.

Local mental health authorities (LMHAs) provide information, recommendations and referrals to people seeking mental health services, including Medicaid.



People Age 65 and Older

Medicaid for the Elderly and People with Disabilities

The MEPD program covers people age 65 and older and those with disabilities who do not receive SSI. Applicants must meet income and resource limits for the program. People age 65 and older and those with disabilities may qualify for MEPD through a facility, such as a nursing facility or an ICF/IID, or through a waiver program. They may also qualify for MEPD if they do not have Medicare, or if they are dually eligible for Medicare and Medicaid.

People Eligible for Medicare and Medicaid

Medicare is a federally paid and administered health insurance program. Medicare covers inpatient hospital services (Part A), physician and related health services (Part B), Medicare managed care (Part C) and prescription drugs (Part D).

Some Medicare beneficiaries may be eligible for partial or full Medicaid benefits. Medicare beneficiaries who do not qualify for full Medicaid benefits may receive assistance through the Medicare Savings Programs (MSPs).

Medicare Savings Programs

Medicare beneficiaries with limited income and resources may qualify for the MSPs.

Medicare beneficiaries who qualify for the MSPs automatically qualify for a Low-Income Subsidy (LIS) program. The LIS “Extra Help” program pays all or a portion of a person’s Medicare Part D (prescription drug coverage) premiums, deductibles and copayments.

Resource limits for 2024 are \$9,430 for one person and \$14,130 for a couple for most dually eligible people. For the Qualified Disabled Working Individuals program, the resource limits for 2024 are \$4,000 for one person and \$6,000 for a couple.

Supplemental Security Income Recipients

Supplemental Security Income (SSI) recipients is a federal cash assistance program for low-income people with disabilities. Low-income people age 65 and older may also qualify for SSI. The Social Security Administration sets income eligibility limits, asset limits and benefit rates, and determines eligibility. In Texas, all people eligible for SSI are automatically eligible for Medicaid.



Waiver Program Interest List and Eligibility Process

HHSC maintains interest lists for the following Medicaid Home and Community-Based Services waiver programs:

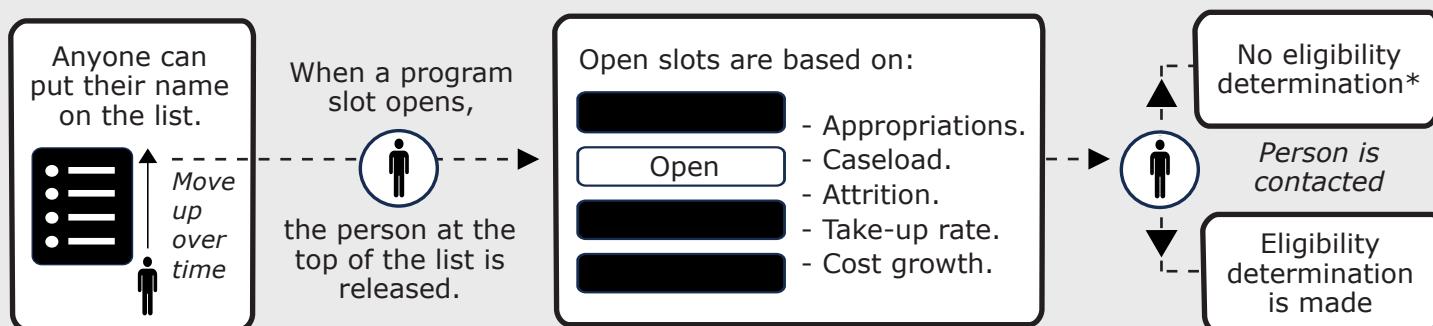
- Community Living Assistance and Support Services (CLASS)
- Deaf Blind with Multiple Disabilities (DBMD)
- Home and Community-based Services (HCS)
- Medically Dependent Children Program (MDCP)
- STAR+PLUS Home and Community Based Services (STAR+PLUS HCBS)
- Texas Home Living (TxHmL)

Annual contact is made by HHSC with everyone on the interest list to confirm they want to stay on the list and to get updates to their information. There are a limited number of slots per program. People in reserved capacity groups, such as the Promoting Independence Initiative or Crisis Diversion, are given priority when program slots become available.

Interest list counts and average wait times vary depending on demand and available slots. Generally, about half the people on interest lists are also receiving other Medicaid services through another waiver, state plan Medicaid benefits or general revenue-funded LTSS. For the most current interest list statistics visit:

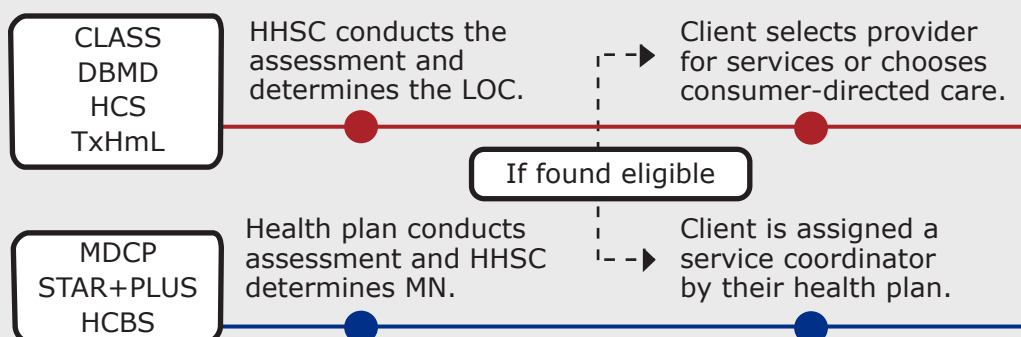
<https://www.hhs.texas.gov/about/records-statistics/interest-list-reduction>.

Interest List Process



Eligibility Determination

Eligibility determination is based on household income, resources and level of care (LOC) or medical necessity (MN). The criteria for LOC or MN a person must meet varies by program.



*Reasons: Deceased, declined, no response, unable to locate, withdrawn, other.

Chapter 2

What Medicaid and CHIP services are available for Texans?



At-a-Glance

What programs and services are available for Texans?

Two Service Delivery Models

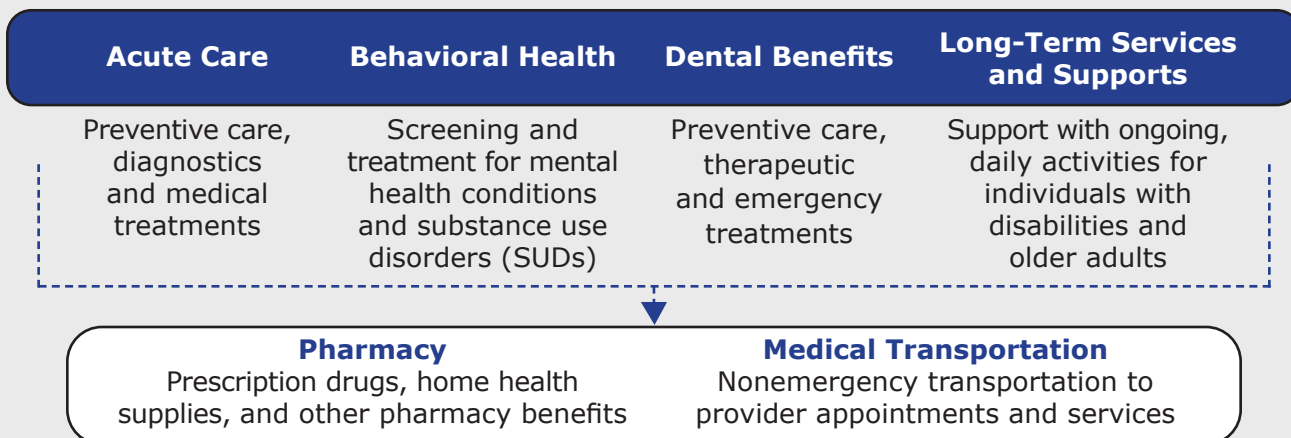
97% of Texans who receive services get them through **Managed Care**.

- HHSC contracts with managed care organizations (MCOs), also called health plans.
- MCOs must provide members with all covered, medically necessary services.
- In most cases, members stay within the health plan's contracted provider network.

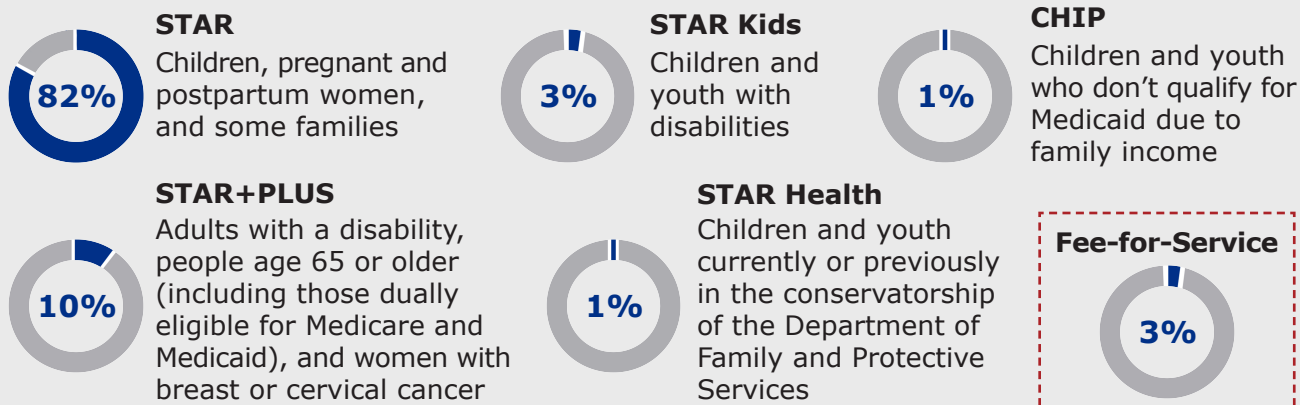
3% of Texans who receive services get them through **Fee-for-Service**.

- Under the fee-for-service (FFS) model, clients can go to any Medicaid provider.
- Clients can receive some services through managed care and others through FFS.
- Most FFS clients will transition to managed care once they have chosen an MCO.

Main Types of Services



Programs that Deliver Services



Percentages are rounded.

What Medicaid and CHIP Cover

Mandatory vs. Optional Services

Federal law requires state Medicaid programs to provide acute care and behavioral health services and long-term services and supports (LTSS). States may choose to provide some, all or no optional services.

States with a separate Children's Health Insurance Program (CHIP), like Texas, have flexibility in determining benefits. The Texas CHIP benefit package focuses on acute care services, but also includes some behavioral health services and dental benefits (see Appendix B, page 156, for a full list of Medicaid programs and services and CHIP benefits).

Acute Care Services

Medicaid and CHIP members can receive preventive care, diagnostics and medical treatments called acute care services. Acute care services may include:

- Doctor or clinic visits.
- Prescription drugs.
- Emergency services.
- Hospital inpatient and outpatient care.
- Vaccines.
- Vision and hearing care.
- X-rays and laboratory tests.
- Pregnancy, childbirth and postpartum care.

Both Medicaid and CHIP cover dental services for children and youth. Adults in certain Medicaid programs can access limited dental benefits.

Behavioral Health Services

Both Medicaid and CHIP cover screening and treatment services for mental health conditions and substance use disorders (SUDs), referred to as behavioral health services. These services include:

- Screening, brief intervention and referral to treatment.
- Residential and outpatient withdrawal management.
- Mental health rehabilitation.
- Peer specialist services.
- Individual, family and group psychotherapy.
- Inpatient psychiatric services.
- Psychological, neurobehavioral and neuropsychological testing.
- Mental health targeted case management.
- Psychiatric diagnostic evaluation.
- Medication management.
- Electroconvulsive therapy.
- SUD individual and group counseling.
- Collaborative care model services.

Two Medicaid programs provide specialized behavioral health services in Texas: Youth Empowerment Services (YES) and Home and Community-Based Services-Adult Mental Health (HCBS-AMH).

In-Lieu-of Services and Settings

In-lieu-of services and settings (ILOSs) are medically appropriate, cost-effective services offered by managed care organizations (MCOs) in place of Medicaid State Plan services or settings. These services must be voluntary for MCOs to provide and for Medicaid members to use. In 2024, MCOs began offering new ILOSs as a substitute for mental health or SUD services specified in the Medicaid State Plan. These services include partial hospitalization, intensive outpatient and coordinated specialty care services.

Long-Term Services and Supports

LTSS are provided to adults age 65 and older and people of all ages with physical, intellectual or developmental disabilities who need nursing care or help with tasks of daily living.

Medicaid covers LTSS through the Texas State Plan and through waiver programs. Services may be delivered through managed care, fee-for-service (FFS) or both.

LTSS may be provided in long-term care facilities, in community settings or within a person's home. The goal is to ensure people have seamless access to services and supports in the most appropriate, least restrictive settings.

Long-Term Care Facilities

Nursing Facilities

Nursing facilities provide services with the goal of maximizing resident autonomy, function, dignity and comfort. Required services provided in a nursing facility setting include:

- Room and board.
- Nursing.
- Medical supplies, equipment and accessories.
- Over-the-counter drugs.
- Personal-need items.
- Social services and activities.

Add-on services provided in a nursing facility setting may include:

- Ventilator care.
- Tracheostomy care for residents age 21 and younger.
- Physician-ordered rehabilitative therapies.
- Custom power wheelchairs.
- Augmentative communication devices.
- Emergency dental services.

Intermediate Care Facilities for Individuals with an Intellectual Disability

Intermediate care facilities for individuals with an intellectual disability or related condition (ICFs/IID) provide residential and habilitation services to people with intellectual disabilities or related conditions. ICFs/IID provide ongoing evaluation and individual program planning, 24-hour supervision, coordination, and integration of health or rehabilitative services. ICF/IID residential settings vary in size, from community settings serving four to 12 individuals (currently 95 percent of ICFs/IID) to large state-supported living centers serving several hundred.

Home and Community-Based Services

State Plan Home and Community-Based Services

Medicaid covers some home and community-based services like Personal Assistance Services (PAS), Community Attendant Services, Day Activity and Health Services (DAHS), Community First Choice (CFC) and Personal Care Services (PCS).

These services help individuals with:

- Activities of daily living like bathing, dressing and eating.
- Instrumental activities of daily living like cooking, grocery shopping and cleaning.
- Other essential home and personal care tasks.
- Some services, like DAHS, include nursing care, meals and community programs.

Medicaid Waiver Programs

Medicaid waiver programs provide people with care in less restrictive settings than institutions like long-term care or psychiatric facilities. Most of Texas' waiver programs are granted through Home and Community-Based Services 1915(c) waivers.

In Texas, services delivered through these programs may include:

- Respite.
- Medical supplies.
- Nursing.
- Professional therapies like physical, occupational and speech therapy.
- Adaptive aids and minor home modifications.
- Employment assistance and supported employment.

(See Appendix B, page 159, for a list of services provided by each waiver program.)

Texas Medicaid Waiver Programs

1. Community Living Assistance and Support Services (CLASS)
2. Deaf Blind with Multiple Disabilities (DBMD)
3. Home and Community-based Services (HCS)
4. Medically Dependent Children Program (MDCP)*
5. STAR+PLUS Home and Community Based Services (STAR+PLUS HCBS)**
6. Texas Home Living (TxHmL)
7. Youth Empowerment Services (YES)

*MDCP operates as a 1915(c) waiver concurrent with the 1115 Transformation Waiver authority.

**STAR+PLUS HCBS operates like a 1915(c) waiver but is provided through the 1115 Transformation Waiver (see Chapter 5, page 112).

Individual Cost Limit

Each waiver uses an individual cost limit or “cost cap” for services since it waives the requirement to serve individuals in an institution. Cost caps vary by waiver program but generally in Texas have been based on the cost of care in an institutional setting with some percentage adjustments.

The service planning team reviews assessments and other information about a person’s needs to develop a service plan that includes waiver and non-waiver services and supports. It is expected that the service plan will meet the person’s needs in the community setting without exceeding the cost cap. If a person exceeds the cost cap, the Health and Human Services Commission (HHSC) has a process to ensure their needs are met. This includes exploring non-waiver resources, transitioning the person to another waiver or, when appropriate, transitioning to institutional services. The services available under the waiver, the state plan and other sources must be enough to assure the health and welfare of the waiver’s participants.

How Home and Community-Based Services are Delivered

People who receive home and community-based services can choose how certain services, like attendant care, are delivered.

Agency Option

Services are delivered through a provider agency, which employs attendants or other service providers.

Consumer Directed Services Option

The person or their legally authorized representative (LAR) employs the attendants or other service providers. This gives the person greater choice and control over their services as the employer of record. People using the Consumer Directed Services (CDS) option must select a Financial Management Services Agency to provide orientation on the CDS option, issue payment to the person’s employees, and pay federal and state taxes on the employer’s behalf. People may choose the agency option for some services and the CDS option for others.

Service Responsibility Option

The MCO and a provider agency work with the person to provide them with increased control over the delivery of their services. The provider agency employs the attendants or other service providers. People work with their provider agency of choice to determine which attendants will deliver their services. The person or their LAR trains the staff to meet their needs and manages them on a day-to-day basis while the provider agency manages the business activities.

Prescription Drugs

People eligible for Medicaid or CHIP receive coverage for prescription drugs. The Vendor Drug Program (VDP) oversees the prescription drug benefit. This includes managing the Texas Formulary, which is a list of all covered drugs, limited home health supplies, select vitamin and mineral products, and other select non-drug products; and the Preferred Drug List (PDL). Drugs may be listed on the PDL as “preferred” or “non-preferred” based on their safety, efficacy and cost-effectiveness (see Chapter 3, page 63). “Non-preferred” drugs require prior authorization before they can be prescribed.

VDP also delivers outpatient prescription drugs for FFS clients, while MCOs cover the benefit in managed care. Texas currently requires Medicaid and CHIP MCOs to use the formulary, the PDL and prior authorization requirements.

Adults enrolled in FFS are limited to three prescriptions per month. There are no limits on how many prescription drugs can be authorized for the following people: children age 20 and younger enrolled in any Medicaid program or CHIP; adults enrolled in managed care; members in nursing facilities; and members enrolled in certain Medicaid waiver programs. For Medicare clients who are dually eligible for Medicaid, Medicare Part D covers their prescription drugs.

The FFS three-prescription limit does not apply to people in the waiver programs (see A Closer Look, page 44).

The number of new high-cost specialty drugs continues to increase as technology advances in drug research, development and manufacturing processes. Medicaid continues to focus on ensuring access to medications while exploring opportunities to develop cost-saving strategies to address rising drug costs.

Hospice Services

Hospice services provide care for terminally ill people for whom curative treatment is no longer desired and who have a physician’s prognosis of six months or less to live. Services are aimed at providing comfort and maintaining quality of life during a person’s end of life.

Services include physician services, nursing, PAS, therapies, prescription drugs, respite care and counseling, plus supportive care for their loved ones. These services are delivered through FFS and can be administered in the home or in community settings, long-term care facilities or in hospital settings.

Children age 20 and younger receiving hospice services may continue to receive curative care from non-hospice acute care providers.

Nonemergency Medical Transportation Services

Nonemergency medical transportation (NEMT) services provide cost-effective medical transportation to a doctor or specialist, a pharmacy, or other providers for covered health care services. NEMT services are only for people enrolled in Medicaid programs or the non-Medicaid programs Children with Special Health Care Needs and transportation for indigent cancer patients.

NEMT services include:

- Demand response transportation — curb-to-curb transportation using dispatched vehicles, including shared rides and transportation network company vehicles.
- Mass transit tickets — including bus, rail, ferry, publicly or privately-owned transit providing general or special service on a regular or continuing basis, and commercial airline transportation services.
- Mileage reimbursement through Individual Transportation Participant (ITP) requests. ITPs must register to participate in the mileage reimbursement program.
- Meals and lodging, when covered health care services require an overnight stay outside the county of residence for children and youth age 20 and younger.
- Advanced funds for transportation or travel-related services for people age 20 and younger.
- Out-of-state travel to Louisiana, Arkansas, Oklahoma, and New Mexico counties that share a border with Texas, and travel to states outside of the adjoining states for covered health care services not provided in Texas.

NEMT services for managed care members are coordinated by their MCO. NEMT services for FFS clients are coordinated by HHSC through the Medical Transportation Program.

Trips must be scheduled with at least two days' notice before the ride is needed. In certain circumstances, NEMT services can be requested with less than two days' notice. Examples of these trips include trips to the pharmacy, urgent care services or discharge from a health care facility. The MCO or HHSC may allow other exceptions to the two-day notice requirement.

Telehealth, Telemedicine and Home Telemonitoring Services

Texas Medicaid and CHIP cover telehealth, telemedicine and home telemonitoring (referred to as teleservices) for certain conditions when demonstrated to be clinically appropriate and cost effective. In response to the COVID-19 Public Health Emergency (PHE), HHSC allowed temporary flexibility for telehealth and telemedicine services to ensure access to medically necessary services.

The main types of teleservices provided include:

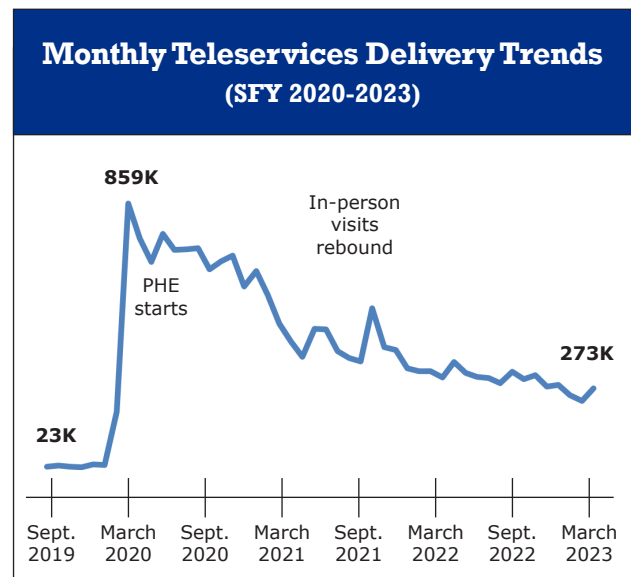
- Telemedicine — medical care delivered using audiovisual or audio-only telecommunications.
- Telehealth — non-medical, non-dental health services, like behavioral health services, delivered using audiovisual or audio-only telecommunications.
- Home Telemonitoring — remote monitoring and transmission of member health data to a telemonitoring provider, usually to manage chronic health conditions or high-risk pregnancies.

As the PHE ended, in-person visits rebounded, but teleservices delivery remains 10 times greater than prior to the PHE. In 2021, House Bill (H.B.) 4 (87th Legislature, Regular Session) required HHSC to permanently expand telemedicine and telehealth services to members if clinically appropriate and cost effective.

Telemedicine and telehealth are available for a broad range of services and conditions, including:

- Early Childhood Intervention (ECI).
- Emergency Triage, Treat and Transport.
- Obstetrics.
- Outpatient mental health.
- Physical therapy, occupational therapy and speech therapy.
- Physician evaluation and management.
- SUD.

Members of any age can get home telemonitoring for diabetes and/or hypertension. Members under age 21 may be home telemonitored for end-stage solid organ disease, organ transplantation or mechanical ventilation. H.B. 2727 (88th Legislature, Regular Session, 2023) authorized temporary home telemonitoring for high-risk pregnancies and required HHSC to evaluate other medical conditions for reimbursement (see Appendix A, page 151).



Managed Care in Texas

STAR

STAR is the first and largest managed care program in Texas. Most people who receive Medicaid in Texas are enrolled in STAR. The program primarily covers children, pregnant and postpartum women and some families. STAR MCOs also provide service coordination to members with special health care needs (see A Closer Look, page 50). There are 16 MCOs delivering the STAR program across 13 STAR service areas.

Other managed care programs in Texas include STAR Health, STAR Kids, STAR+PLUS, CHIP and dental.

Value-Added Services

Value-added services are extra services that MCOs can choose to provide to their members. Services offered vary across MCOs. Examples include diapers for newborns, smoking cessation programs, weight loss management and adult dental services for members in the STAR+PLUS program.



Children and Youth

Medicaid

Most Medicaid members are children. To qualify for Children's Medicaid, a child must be age 18 or younger. Newborns (under 12 months) born to mothers getting Medicaid at the time of delivery are automatically eligible for Children's Medicaid and remain eligible through the month of their first birthday. Children eligible for Medicaid can receive a wider range of health care services than adults.

Children and youth with Medicaid are eligible for behavioral health care, including Health and Behavior Assessment and Intervention (HBAI) services. HBAI services identify the psychological, behavioral, emotional, cognitive and social factors important to preventing, treating and managing physical health symptoms.

Children and youth also receive comprehensive dental services. Most dental benefits are provided through managed care and delivered by one of three dental maintenance organizations. Children enrolled in STAR Health or who reside in a long-term care facility receive dental care through the STAR Health MCO or their assigned facility, respectively.

Children's Medicaid

Most children receiving Children's Medicaid are enrolled in STAR. Children's Medicaid services — including acute care, pharmacy services, behavioral health and LTSS — are primarily delivered through managed care, although some LTSS may be covered through FFS.

Beginning Jan. 1, 2024, eligible children under the age of 19 receive 12 months of Medicaid or CHIP continuous eligibility coverage, regardless of changes in income.

Some children with disabilities who receive Children's Medicaid are enrolled in the STAR Kids managed care program (see STAR Kids, page 41).

Medicaid for Children in Foster Care

In partnership with the Texas Department of Family and Protective Services (DFPS), Medicaid provides STAR Health, a managed care program for children in state conservatorship. The program is administered by a single, statewide MCO.

Children in foster care and kinship care are considered to have special health care needs, which might include multiple and complex medical, physical, behavioral and developmental needs. Children and youth in foster care also have the typical health care needs of all children including routine well-child health care, immunizations and the treatment of acute childhood illnesses. To address these needs, STAR Health provides integrated physical and behavioral health care, including primary care, emergency services, inpatient and outpatient mental health care, and community-based LTSS, as well as dental, vision and pharmacy services. The program provides a medical home for children as soon as they enter state conservatorship and is available to the following eligible populations:

- Children and young adults in DFPS conservatorship.
- Young adults age 18 through the month of their 22nd birthday who voluntarily agree to continue in a foster care placement.
- Young adults from age 18 through the month of their 21st birthday who are participating in the Former Foster Care Children program or are participating in the Medicaid for Transitioning Foster Care Youth program.
- An infant born to a mother who is enrolled in STAR Health.
- Certain children or young adults, from birth through the month of their 21st birthday, who are enrolled in the Adoption Assistance program or the Permanency Care Assistance program.

STAR Health also offers services, including:

- Service coordination (see A Closer Look, page 50).
- A 24/7 nurse hotline for caregivers and caseworkers.
- A 24/7 behavioral health crisis services line.
- Health Passport — a web-based electronic health record system.

Children may be enrolled in the MDCP waiver and receive their MDCP services while enrolled in STAR Health (see Medically Dependent Children Program, page 42).

In addition, STAR Health promotes training and certification of behavioral health providers, caregivers and caseworkers in trauma-informed care, including evidence-based practices such as Trauma-Focused Cognitive Behavioral Therapy.

Texas Health Steps

Children enrolled in any Medicaid program receive Early and Periodic Screening, Diagnosis and Treatment services — also known as Texas Health Steps. This program provides comprehensive medical, dental and case management services for children with Medicaid from birth through age 20. This includes the Comprehensive Care Program (CCP) which expands coverage to any medically necessary services, even if the services are not covered by the Medicaid State Plan. Beginning in February 2022, Applied Behavior Analysis, a type of therapy for autism spectrum disorder (ASD), became available for children and youth with ASD through Texas Health Steps-CCP.

Texas Health Steps also provides case management for eligible children and pregnant women (see page 38). Case managers help people access services they may need, including assistance with school or special education needs, transition planning, and issues such as substance use, homelessness or domestic violence. Texas Health Steps is not a benefit option for children enrolled in CHIP.

Youth Empowerment Services

YES is a Medicaid waiver program that provides intensive community-based services for children and youth who have severe mental, emotional or behavioral disturbances and their families.

Services include but are not limited to community living supports, family supports and supportive family-based alternatives, non-medical transportation, paraprofessional services and transitional services (see Appendix B, page 162, for a full list of services).

Children's Health Insurance Program

CHIP provides acute care, behavioral health care and pharmacy services for children in families who have too much income to qualify for Medicaid but cannot afford to buy other health insurance.

CHIP offers no-cost mental health and SUD screening and treatment services, including SUD prevention and intervention services.

CHIP also provides dental benefits. CHIP members receive up to \$564 in dental benefits per 12-month enrollment period — not including emergency dental services — to cover preventive and therapeutic services like periodontics and prosthodontic services. CHIP members may receive certain preventive and medically necessary services beyond this cap through a prior authorization process.



Women

Medicaid and CHIP cover reproductive health care services, including screening and treatment for sexually transmitted infections, contraception, prenatal and postpartum care services, and labor and delivery. Medicaid also covers breast and cervical cancer screening and treatment. CHIP only covers contraception for medically necessary purposes.

Medicaid for Pregnant Women

Women who receive Medicaid for Pregnant Women are enrolled in STAR. While enrolled, their coverage includes full Medicaid benefits and perinatal services, including:

- Prenatal visits.
- Prescription prenatal vitamins.
- Labor and delivery.
- Postpartum care.

Women with high-risk pregnancies may receive service coordination through their STAR MCO or through Case Management for Children and Pregnant Women (CPW).

Case Management for Children and Pregnant Women

CPW services transitioned to managed care, in addition to FFS, in September 2022. CPW services are delivered by case managers who are registered nurses or licensed social workers. H.B. 1575 (88th Legislature, Regular Session, 2023) expanded the provider types eligible to provide these services to include doulas and community health workers. Case managers assist pregnant women with high-risk health conditions and children with a health condition or health risk to access medically necessary health, social, educational and other services related to their health condition.

Before the transition to managed care, these services were available through FFS providers or Department of State Health Services regional case managers. To request CPW services, clients could call Texas Health Steps or reach out to any CPW provider in their area. While there is no change in this process for FFS clients, members enrolled in managed care must now call their MCO to request CPW services or reach out to an in-network CPW provider.

CHIP Perinatal

CHIP Perinatal (CHIP-P) coverage provides care to unborn children of pregnant women who are not eligible for Medicaid and who have household income up to 202 percent of the federal poverty level. Once born, the child will receive Medicaid or CHIP benefits, depending on their income. CHIP-P provides a basic prenatal care package. Services include:

- Up to 20 prenatal visits.
- Prescription drugs, including prenatal vitamins.
- Some laboratory testing, assessments, planning services, education and counseling.
- Some diabetic supplies through pharmacies with a physician prescription.
- Labor and delivery.
- Two postpartum care visits within 60 days after delivery or end of pregnancy.

Members receiving CHIP-P benefits are exempt from the 90-day waiting period and all cost-sharing, including enrollment fees and copays, for the duration of their coverage period (see Chapter 4, page 92).

Healthy Texas Women

Healthy Texas Women (HTW) is a Medicaid 1115 Demonstration program that provides women's health, preventive health and family planning services at no cost to eligible women. HTW services include:

- Annual family planning examinations.
- Pregnancy testing and counseling.
- Pelvic examinations.
- Sexually transmitted infection services.
- HIV screenings.
- Breast and cervical cancer screenings and diagnostic services.
- Treatment of cervical dysplasia.
- Screening and treatment for cholesterol, diabetes and high blood pressure.
- Contraception, including condoms, birth control pills, long-acting reversible contraceptives, emergency contraception and permanent sterilization.
- Screening and treatment for postpartum depression (PPD).
- Vaccinations.

H.B. 12 (88th Legislature, Regular Session, 2023) extended postpartum care for women enrolled in Medicaid or CHIP from two months to 12 months. In spring 2024, HTW clients who were still within their 12-month postpartum period were transitioned back to full-coverage Medicaid or CHIP for the remainder of their 12-month postpartum period.

In January 2020, the Centers for Medicare & Medicaid Services (CMS) approved the Texas 1115 Demonstration application for HTW. HHSC began receiving federal matching funds for HTW services provided to adults on Feb. 18, 2020 (see Chapter 5, page 113).

Healthy Texas Women Plus

HTW Plus is an enhanced postpartum services package. HTW Plus clients are eligible to receive additional services to treat certain health conditions, including mental health conditions like PPD or SUDs and cardiovascular conditions. To receive HTW Plus, an HTW client must have been pregnant within the 12 months prior to their enrollment. HTW Plus services include:

- Individual, family and group psychotherapy services.
- Peer specialist services.
- Cardiovascular evaluation imaging and laboratory studies; blood pressure monitoring equipment; and anticoagulant, antiplatelet and antihypertensive medications.
- Screenings, brief interventions and referrals to treatment for SUDs.
- Outpatient substance-use counseling.
- Smoking cessation services.
- Medication-assisted treatment (MAT).
- Additional injectable insulin options, blood glucose testing and monitoring supplies.
- Asthma medications and supplies.

In December 2020, HHSC submitted an amendment to the HTW 1115 demonstration to receive federal funds for HTW Plus. When the amendment was submitted, HHSC requested an effective date of April 1, 2021. The amendment is pending with CMS as of December 2024.

As a result of the implementation of H.B. 12 (88th Legislature, Regular Session, 2023) HTW Plus coverage will be primarily available to women not enrolled in Medicaid or CHIP during their pregnancy.

Medicaid for Breast and Cervical Cancer

Women who receive Medicaid for Breast and Cervical Cancer (MBCC) are enrolled in STAR+PLUS. MBCC covers active cancer treatments, including:

- Chemotherapy and radiation.
- Surgery, including reconstruction.
- Disease surveillance for members with triple-negative receptor breast cancer.
- Medication, including hormone treatments.

Women eligible for MBCC receive full Medicaid benefits and service coordination and remain in the program for the duration of their cancer treatment if they continue to meet all other eligibility criteria.

Women can apply for MBCC through Breast and Cervical Cancer Services (BCCS) clinics. BCCS is not a Medicaid program. BCCS clinics provide breast and cervical cancer screening and diagnostic services to women while also serving as a point of access for MBCC.



Children and Adults with Disabilities

As previously described, people with disabilities may receive LTSS through Medicaid. LTSS may be provided in long-term care facilities or in home and community-based settings (see Long-Term Services and Supports, page 28).

Children

Children with disabilities can receive acute care, behavioral health services and LTSS through Children's Medicaid or Medicaid for the Elderly and People with Disabilities (MEPD). Some may receive home and community-based services through a Medicaid waiver program. Children who are both in foster care and enrolled in STAR Health receive all their acute care, behavioral health services and LTSS through their STAR Health MCO (see Medicaid for Children in Foster Care, page 35).

STAR Kids

STAR Kids is a managed care program that provides acute care services and LTSS benefits to children and youth age 20 and younger with disabilities. To enroll in STAR Kids, a person must also meet one of the following criteria:

- Receive Supplemental Security Income (SSI) or SSI-related Medicaid.
- Be eligible for MEPD or the Medicaid Buy-In for Children or Medicaid Buy-In programs.
- Be enrolled in a Medicaid waiver program.
- Reside in a long-term care facility.
- Be dually eligible for Medicare.

Medicaid for the Elderly and People with Disabilities

Children and adults with disabilities and people age 65 and older who do not receive SSI may qualify for Medicaid through MEPD.

Through MEPD, people may access care in Medicaid long-term care facilities and home and community-based services, which may be delivered through managed care, FFS or a combination of these models.

In addition to providing acute and long-term care services and supports to all STAR Kids members, MCOs also provide service coordination (see A Closer Look, page 50). If a person is enrolled in multiple programs, their STAR Kids service coordinator works with all other service coordinators or case managers to determine which programs will provide each of their services.

While STAR Kids can provide both acute care and LTSS to members, sometimes it may only provide acute care. For example, a person residing in a long-term care facility may be enrolled in STAR Kids for their acute care only while their LTSS are provided by their assigned facility and paid for through the FFS model.

In addition, FFS waiver programs can provide many of the same LTSS available through STAR Kids. If a person needs a service that is available in both STAR Kids and their waiver program, they must get the service through STAR Kids.

Medically Dependent Children Program

MDCP is a Medicaid waiver program delivered through managed care that provides home and community-based services to children and youth age 20 and younger as a cost-effective alternative to a nursing facility.

People enrolled in MDCP receive all services through their STAR Kids or STAR Health MCO (see Appendix B, page 161, for a full list of services available through MDCP).

School Health and Related Services Program

The School Health and Related Services (SHARS) program allows local education agencies, including public charter schools, to receive federal reimbursement for providing Medicaid services to participating Medicaid-eligible students age 20 and younger. Management of the SHARS program is a cooperative effort between the Texas Education Agency and HHSC. This program covers certain health-related services documented in a student's Individualized Education Program. Services include:

- Audiology services (pending CMS approval to also reimburse audiology services for students with a 504 plan).
- Physician and nursing services.
- Physical, speech and occupational therapies.
- PCS.
- Psychological services, including assessments and counseling.
- Transportation in a school setting.

Early Childhood Intervention

ECI is a statewide program that provides services to families with children from birth up to age 3 with developmental delays or disabilities. HHSC contracts with local entities, such as nonprofit organizations and some school districts, to provide services in all Texas counties. ECI provides targeted case management, specialized skills training, therapy services and other benefits for all children who qualify, whether they have Medicaid or not.



Waiver Programs for People with Intellectual and Developmental Disabilities

Medicaid 1915(c) waiver programs are designed to provide home and community-based services to people with intellectual and developmental disabilities (IDD) or related conditions as an alternative to placement in an ICF/IID.

Four waiver programs for people with IDD are described below. These programs are delivered through the FFS model and many provide the same services (see Appendix B, page 159).

Demand for some waiver programs exceeds capacity. These programs maintain interest lists that people can join at any time (see A Closer Look, page 23).

Community Living Assistance and Support Services

CLASS provides home and community-based services to people who have a related condition qualifying them for placement in an ICF/IID. Services include adaptive aids, case management, prevocational services, minor home modifications, residential habilitation transportation, specialized therapies, support family services and transition assistance services.

Deaf Blind with Multiple Disabilities

DBMD provides home and community-based services (as an alternative to residing in an ICF/IID) to people of all ages who have deaf blindness or have a condition that will result in deaf blindness and who have an additional disability. Services include case management, individualized skills and socialization, residential habilitation (including transportation), assisted living, audiology services, dietary services, behavioral support and intervener services.

Home and Community-based Services

HCS provides individualized services to people of all ages who qualify for ICF/IID level of care, yet live in their family's home, their own home, or other settings in the community as an alternative to living in an ICF/IID. Services include individualized skills and socialization, residential services, supported home living (including transportation) and transition assistance services.

Texas Home Living

TxHmL provides selected services and supports for people with IDD who live in their own home or their family's home. Services include behavioral support, community support (including transportation), respite and individualized skills and socialization.

Adults

Adults with disabilities can receive their acute care, behavioral health services and LTSS through Medicaid. Some may receive home and community-based services through a Medicaid waiver program. As with children with disabilities, adults may receive their services through managed care, FFS or a combination of these models.

Other programs available to eligible adults with disabilities include Programs of All-Inclusive Care for the Elderly (PACE), Medicaid-Medicare Plans (MMP) and Medicare Advantage Dual Eligible Special Needs Plans (D-SNPs) (see People Age 65 and Older, page 47).

STAR+PLUS

The STAR+PLUS managed care program provides acute care, behavioral health care and LTSS for adults who have a disability or who are age 65 and older. STAR+PLUS serves adults who:

- Receive SSI or SSI-related Medicaid.
- Are enrolled in STAR+PLUS HCBS.
- Reside in an ICF/IID or are enrolled in an IDD waiver program who are not dually eligible for Medicare.
- Are enrolled in the MBCC program.
- Are nursing facility residents.
- Are dually eligible for Medicare, except dually eligible individuals in an ICF/IID or IDD waiver program.

Members who are dually eligible for Medicare receive LTSS through STAR+PLUS and their acute care services through Medicare.

Members with complex medical conditions are assigned a service coordinator who develops a person-centered service plan with the member and manages the member's acute care and LTSS. All STAR+PLUS members can request to have a service coordinator.

People can choose whether they participate in STAR+PLUS if they meet one or both criteria:

- Are enrolled in PACE.
- Are a member of Federally Recognized Native American Tribes.

People are excluded from participation in STAR+PLUS if they:

- Are age 20 and younger, except for participants in MBCC who may be age 18 through 64.
- Are dually eligible and currently living in an ICF/IID or receiving IDD waiver services.
- Reside in a State Supported Living Center.
- Reside in a state veteran home or the Truman Smith Care Center.

STAR+PLUS Home and Community Based Services Program

STAR+PLUS Home and Community Based Services (STAR+PLUS HCBS) is a managed care program delivered through the 1115 Transformation Waiver that provides a cost-effective alternative to living in a nursing facility for adults age 21 and older who have disabilities or who are elderly.

People enrolled in the program receive all services through their STAR+PLUS MCO. Some of the services offered include adult foster care, assisted living services, emergency response services, home delivered meals and transition assistance services (see Appendix B, page 161).

Strengthening person-centered planning practices in HCBS continues to be a focus for this program in response to the CMS HCBS Settings Rule, which requires states to ensure HCBS settings meet minimum standards for integration and choice.

Home and Community-Based Services – Adult Mental Health

HCBS-AMH is a state plan program under Section 1915(i) that helps people with serious mental illness remain in their community. Adults with a diagnosis of serious mental illness have complex needs that can lead to extended psychiatric hospitalizations, repeated arrests and frequent emergency department visits. The HCBS-AMH program provides an array of intensive home and community-based services tailored to a person's assessed needs and considering the person's preferences and goals.

Services include but are not limited to host home/companion care, supervised living services, supported home living and psychosocial rehabilitative services. Services are provided in CMS-approved settings, which can be a person's home or apartment, an assisted living setting or small community-based residence.

Medicaid Substance Use Disorder Treatment Services

Medicaid SUD treatment services include assessment/evaluation, individual and group counseling, residential treatment, withdrawal management and MAT services. SUD treatment services must be provided by a licensed chemical dependency treatment facility (CDTF) or an opioid treatment provider (OTP), except for MAT services.

MAT, including methadone and buprenorphine, is primarily used for opioid-use disorder, but can also be used for alcohol-use disorder. Methadone for an opioid-use disorder may only be provided by an OTP. Buprenorphine treatment may be provided by OTPs, CDTFs, physicians and other qualified prescribers — including nurse practitioners, physician assistants, nurse midwives and nurse anesthetists.

Screening, Brief Intervention and Referral to Treatment

Screening, Brief Intervention and Referral to Treatment provides early intervention and treatment services for members age 10 and older who have a SUD or who are at risk of developing a SUD. The benefit is available in community-based settings and hospitals.



People Age 65 and Older

Adults age 65 and older can receive Medicaid through MEPD, delivered through the STAR+PLUS managed care program (as previously described on page 42). People who are aging and need LTSS can also get these services through MMPs, D-SNPs or PACE.

Medicare-Medicaid Dually Eligible Individuals

The Dual Demonstration (Medicare-Medicaid Plans)

The Dual Demonstration provides a fully integrated managed care model for people who are dually eligible for Medicare and Medicaid. Both Medicare and Medicaid services are provided under a single MMP.

The model involves a three-party contract between an MMP, HHSC and CMS. MMPs are available in five Texas counties: Bexar, Dallas, El Paso, Harris and Hidalgo. If eligible, people are automatically or passively enrolled into an MMP, but they may choose to opt out. Other dually eligible STAR+PLUS members who are not passively enrolled and who live in one of the five counties can choose to enroll.

The Dual Demonstration excludes dually eligible individuals receiving Medicaid services provided through an ICF/IID or through a Medicaid IDD waiver program, such as CLASS, DBMD, HCS or TxHmL.

The CMS Contract Year 2023 Medicare Advantage and Part D Final Rule requires states to end their Dual Demonstrations. In accordance with this federal regulation, HHSC is ending the Dual Demonstration Program on Dec. 31, 2025, and will transition the Dual Demonstration to an integrated D-SNP model on Jan. 1, 2026 (see D-SNP section below). This will advance Medicare-Medicaid integration between certain STAR+PLUS MCOs and D-SNPs for dually eligible STAR+PLUS members in the Dual Demonstration counties.

Rider 32 Implementation

H.B. 1, Article II, Rider 32 (88th Legislature, Regular Session, 2023) mandates HHSC to transition Medicaid-only services for dually eligible people enrolled in managed care from an FFS delivery model to a managed care service delivery model without imposing cost-sharing on dually eligible people. Beginning Sept. 1, 2025, HHSC will require MCOs to cover for dually eligible members:

- The Medicaid-only acute care services that MCOs cover for members who do not have Medicare.
- Drugs that are excluded from Medicare Part D and are paid by Medicaid after commercial insurance is billed or if there is no commercial insurance.

HHSC anticipates that these changes will streamline claims processing and improve MCOs' ability to coordinate services for their dually eligible members.

Medicare Advantage Dual Eligible Special Needs Plans

Medicare Advantage D-SNPs are Medicare Advantage managed care plans designed to provide Medicare services to people who are also fully or partially eligible for Medicaid (dual-eligible).

Full-benefit dually eligible persons are Medicare beneficiaries who can receive the full array of Medicaid benefits. Partial-benefit dually eligible persons are Medicare beneficiaries who are not eligible to receive the full array of Medicaid benefits but may receive Medicaid assistance for their Medicare Part A or B premiums and cost-sharing expenses.

HHSC contracts with D-SNPs to cover the Medicare cost-sharing obligations for partial-benefit dually eligible members in the Qualified Medicare Beneficiary Program and full-benefit dually eligible members. HHSC-contracted D-SNPs must also coordinate care between Medicare and Medicaid.

Programs of All-Inclusive Care for the Elderly

PACE provides acute care and LTSS for a capitated monthly fee below the cost of comparable care in a long-term care facility. PACE participants must be age 55 and older, live in a PACE service area, qualify for nursing facility level of care and be able to live safely in the community. PACE participants receive all medical and social services they need through their PACE provider.

PACE offers all health-related services for each participant, including inpatient and outpatient medical care, specialty services (e.g., dentistry, podiatry, and both physical and occupational therapy), social services, in-home care, meals, transportation and day activity services.

PACE is available in the cities of Amarillo, El Paso and Lubbock. People in these service areas who are also eligible for STAR+PLUS may choose to receive services either through STAR+PLUS or PACE but not both.



A Closer Look

Service Coordination in Managed Care

STAR Kids and STAR+PLUS

All STAR Kids members are connected with a service coordinator through their health plan. STAR+PLUS members with complex medical conditions can also get a service coordinator. All STAR+PLUS members can request to have a service coordinator.

Service coordination helps ensure members get the services they need. Using a person-centered approach, service coordinators help manage the member's health care and long-term care needs — including access to community resources and help with food and housing. Service coordinators work with the member, their primary care provider, and their specialty and non-medical providers to develop and carry out an Individualized Service Plan.

STAR and CHIP

STAR and CHIP members with special health care needs can get help from their health plans through service coordination. Special health care needs include serious, ongoing illnesses, chronic or complex conditions, disabilities, and conditions that require therapeutic intervention and evaluation by appropriately trained staff.

A service coordinator in STAR or CHIP helps ensure that members with special health care requirements get the services they need from their health plan. They can also help members get non-capitated services, which are services that might not be covered in STAR but are a Medicaid benefit. For example, for STAR members, these services might include programs like CFC or PCS.

STAR Health

All STAR Health members receive service coordination through their health plan. Children and youth in foster care are considered to have special health care needs, which might include multiple and complex medical, physical, behavioral and developmental needs.

Service coordination includes person-centered planning and assessment to help manage these complex health care needs and enhance a child's or youth's well-being. A STAR Health service coordinator works with the member, member's caregivers, primary care provider and specialty care providers to create and implement a service plan. Service coordinators help ensure access to and use of covered services. The health plan provides all STAR Health members with a named service coordinator to be the main point of contact for all members, medical consenters and caregivers.

Chapter 3

How does HHSC make sure clients get good care?



At-a-Glance

5.8 million
Texans served



16 Managed Care Organizations (MCOs)
3 Dental Maintenance Organizations (DMOs)



40+
Contracts

HHSC Contract Oversight Tools

Tools span a multitude of areas, administered by various expertise.



Access to services

Network adequacy monitoring, appointment availability studies, member satisfaction studies



Service delivery

Utilization reviews for acute care, managed care long-term service and drugs



Quality of care

Performance indicator dashboard for key quality measure indicators, value-based enrollment, improvement projects, pay-for-quality, alternative payment models, MCO report cards



Operations

Readiness reviews prior to serving members, biennial operational reviews, targeted reviews conducted on site



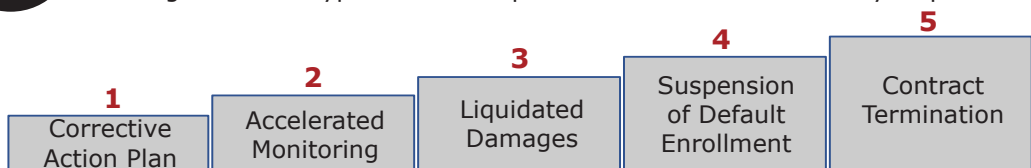
Financial

Validation of financial statistical reports, administrative expenses cap, profit limits, experience rebate, independent auditing



Contractual Non-Compliance:

Multiple stages of remedies are assessed for non-compliance discovered through oversight tools, each with an increased level of impact. Remedies are contingent on the type of non-compliance and are not necessarily sequential.



Numbers are subject to change. Number of MCOs and contracts current as of September 2024.

The Managed Care Delivery System

The Health and Human Services Commission (HHSC) has over the past two decades shifted its service delivery for Medicaid and the Children’s Health Insurance Program (CHIP) from the fee-for-service (FFS) model to managed care. HHSC contracts with 16 managed care organizations (MCOs) to coordinate services for most Medicaid members and all CHIP members. In addition, HHSC contracts with three dental maintenance organizations (DMOs) to manage dental care.

Managed care is an integrated service delivery system in which HHSC contracts with MCOs to provide all covered, medically necessary services to people receiving Medicaid or CHIP benefits, and DMOs to provide routine and preventive care and medically necessary dental services to people receiving Medicaid and CHIP benefits. HHSC pays MCOs and DMOs a monthly capitation rate for each member enrolled in their plans, and MCOs and DMOs reimburse providers for services provided to members.

One goal of transitioning from FFS to managed care is to provide value-based care.

Achieving Value-Based Care

MCOs and DMOs can achieve value-based care by improving or stabilizing member health and delivering services in a cost-effective manner. MCOs and DMOs receive a fixed dollar amount per member per month (PMPM) in advance based on historical costs. This is known as a capitation rate.

This model places MCOs and DMOs at financial risk if the cost of care exceeds this rate, which incentivizes them to improve care quality while keeping costs low. The “medical home” or “dental home” leverages the patient-provider relationship to improve health outcomes and ensure appropriate use of covered services.

The Medical Home

When members enroll with an MCO, they choose a primary care provider (PCP) — usually a family or general practitioner, pediatrician or OB/GYN — who serves as the member’s medical home. PCPs are central access points for their patients’ health care. They deliver comprehensive preventive and primary care, and provide referrals for specialty care and other covered services.

The medical home helps with cost management and service delivery. The referral system and emphasis on preventive and primary care can limit overuse of services, while still improving access to covered, medically necessary services. Members with chronic, complex health care needs also benefit from the medical home, as it promotes a patient-centered approach to care. The member's PCP, specialists and other caregivers directly engage with them to address any high-risk health conditions or other co-morbid conditions.

The Dental Home

When members enroll with a DMO, they choose a Main Dental Home (MDH) — usually a general dentist, pediatric dentist, rural health clinic or federally qualified health center. The MDH provides routine preventive, diagnostic, urgent, therapeutic, initial and primary care while maintaining the continuity of patient care and initiating referrals for specialized care.

HHSC's Managed Care Contract Oversight

HHSC's oversight of managed care seeks to ensure that Medicaid and CHIP improve health outcomes and MCOs and DMOs deliver high-quality, cost-effective services.

HHSC contracts with MCOs and DMOs to coordinate services for 97 percent of Medicaid members and 100 percent of CHIP members. Because of the size, scope and complexity of managed care contracts and the types of services MCOs and DMOs provide, there is no single oversight strategy.

MCO and DMO Selection and Contract Terms

Each MCO and DMO is selected through a competitive contract process that requires ongoing involvement from staff and leadership across the organization — starting with the solicitation phase and continuing throughout the evaluation phase, negotiations and contract formation.

The relationship between HHSC and any selected MCO and DMO is best described as a "partnership with accountability." This is established through robust contract terms at the onset which outline MCO and DMO responsibilities, including but not limited to:

- Reporting financial, member enrollment and encounter data.
- Providing member benefit packages that cover services required by the Texas Medicaid or CHIP programs.
- Delivering service coordination to eligible members.
- Establishing adequate and accessible provider networks.
- Timely processing of provider claims.
- Timely processing of encounters.
- Maintaining member and provider call centers.
- Resolving member and provider complaints and appeals.

All MCO and DMO contracts have the same Uniform Terms and Conditions but program-specific statements of work.

Readiness Reviews

After they have been selected through the competitive contracting process, MCOs and DMOs build out their operations. Before an MCO or DMO begins serving members, HHSC conducts readiness reviews to ensure that the MCO or DMO can provide all contracted services.

HHSC examines the following major areas to ensure MCOs and DMOs are ready to serve members:

Readiness Reviews Major Areas	MCO	DMO
Behavioral health referral process	✓	—
Claims processing	✓	✓
Complaint and appeal process	✓	✓
IT system readiness	✓	✓
Main Dental Home	—	✓
Member and provider hotlines	✓	✓
Member education materials	✓	✓
Pharmacy services	✓	✓
Provider network	✓	✓
Provider relations	✓	✓
Utilization management policies and procedures	✓	—
Website functionality and content for members and providers	✓	✓

The above list is not all-inclusive. Program-specific areas, such as service coordination for STAR, STAR Kids, STAR+PLUS and STAR Health, are also reviewed.

Contract Management and Oversight

HHSC employs several oversight tools to monitor MCOs and DMOs based on the requirements laid out in their contracts. The main areas of contract oversight include:

- Access to services.
- Service delivery.
- Quality of care.
- Operational reviews.
- Financial practices.

The oversight tools within these areas enable HHSC to verify MCO and DMO compliance with state and federal law and with the terms of their contract. If non-compliance issues are found, HHSC applies a graduated remedy process to address these issues.



Access to Services

MCOs and DMOs can reduce the use of emergency care by ensuring members have timely access to primary and preventive care. They are contractually required to build adequate networks of providers so their members can access care when needed, near where they live.

In Texas and across the United States, network adequacy for commercial health plans and Medicaid and CHIP MCOs and DMOs is influenced by many factors, including provider availability, administrative complexity and payment rates.

Network Adequacy

Building a strong network of providers is an ongoing effort, and HHSC continues to work with MCOs and DMOs to achieve this goal. HHSC uses a variety of tools to monitor MCO and DMO networks — including time and distance standards, provider directory quality checks, appointment availability studies and member satisfaction surveys. When deficiencies are discovered, HHSC addresses them through its established graduated remedy process (see Non-Compliance Remedies, page 79).

Time and Distance Standards

HHSC uses time or distance standards to monitor MCO and DMO provider networks and ensure members have timely access to services. These standards were developed based on those used for Medicare Advantage and by the Texas Department of Insurance (TDI) and are either consistent with or more stringent than state and federal requirements.

HHSC is continually evaluating and updating network adequacy standards as part of an ongoing effort to ensure members have access to a choice of quality health care providers and services.

- In September 2018, HHSC adopted new time and distance standards for long-term services and supports (LTSS) and pharmacy.
- In 2020, HHSC adopted new provider network standards for outpatient chemical dependency treatment facilities and outpatient opioid treatment programs.
- In 2021, HHSC implemented additional provider access standards for personal attendant services focused on timeliness and service coordinator turnover.

MCOs must allow members to choose their network provider to the extent possible. DMOs must ensure 95 percent of their members have access to a choice of at least two main dentists. MCOs and DMOs must also ensure that 90 percent of their members have access to at least two network providers within specific time or distance requirements.

HHSC uses geo-mapping to monitor the network access of individual MCOs and DMOs and to look for areas of the state where there are workforce provider shortage issues.

In response to House Bill (H.B.) 4, 87th Legislature, Regular Session, 2021, HHSC redesigned the network adequacy non-compliance process for telehealth and telemedicine. With the redesigned process, MCOs must provide a detailed description of their access to care plan, teleservices plan, and access to care interventions to ensure that there are no gaps in care for their members. HHSC allows MCOs the flexibility to outline how they use teleservices to address non-compliances. Responses demonstrating teleservices efforts will vary by MCO.

Time and Distance Standards							
		Distance in Miles			Travel Time in Minutes		
Provider Type		Metro County	Micro County	Rural County	Metro County	Micro County	Rural County
Behavioral Health-Outpatient		30	30	75	45	45	90
Hospital-Acute Care		30	30	30	45	45	45
Prenatal		10	20	30	15	30	40
Primary Care Provider*		10	20	30	15	30	40
Specialty Care Provider	Cardiovascular Disease	20	35	60	30	50	75
	ENT (Otolaryngology)	30	60	75	45	80	90
	General Surgeon	20	35	60	30	50	75
	OB/GYN	30	60	75	45	80	90
	Ophthalmologist	20	35	60	30	50	75
	Orthopedist	20	35	60	30	50	75
	Pediatric Sub-Specialists	20	35	60	30	50	75
	Psychiatrist	30	45	60	45	60	75
	Urologist	30	45	60	45	60	75
Occupational, Physical or Speech Therapy		30	60	60	45	80	75
Nursing Facility		75	75	75	N/A	N/A	N/A
Pharmacy		2	5	15	5	10	25
Pharmacy (24-hour)		75	75	75	90	90	90
Substance Use Disorder-Outpatient	Chemical Dependency Treatment Facilities	30	30	75	45	45	90
	Opioid Treatment Programs	30	30	75	45	45	90
Main Dentist (general or pediatric)		30	30	75	45	45	90
Dental Specialists	Pediatric Dental	30	30	75	45	45	90
	Endodontist	75	75	75	90	90	90
	Orthodontist	75	75	75	90	90	90
	Oral Surgeons	75	75	75	90	90	90
Long-Term Services and Support**		30	60	60	45	80	75

Metro = county with a population of 200,000 or greater; Micro = county with a population between 50,000-199,999; Rural = county with a population of 49,999 or less.

*Services for both adults and children include acute, chronic, preventive, routine or urgent care.

**Not applicable for Attendant Care, CFC Habilitation Services, Consumer Directed Services, In-Home Therapies – OT, PT, ST, In-Home Skilled Nursing and Private Duty Nursing.

The standard for LTSS provider services is measured by Provider Choice. There must be a choice of at least two providers in each county.

Appointment Availability

Appointment availability studies monitor the length of time a member must wait between scheduling an appointment with a provider and receiving treatment from the provider. The studies are for MCOs and do not include appointment availability for DMOs. HHSC contracts with an external quality review organization (EQRO) to conduct these studies annually. The EQRO performs four studies by provider type in the areas of prenatal, primary care, vision and behavioral health.

The EQRO uses a mystery shopper method to randomly sample providers to determine how soon an appointment can be scheduled. Provider office samples for the studies are pulled from member-facing provider directories submitted by the MCOs.

In 2022 and 2023, appointment availability studies were conducted for prenatal care, primary care, vision and behavioral health. Corrective Action Plans (CAPs) and Liquidated Damages (LDs) are assessed when MCOs fail to meet appointment availability standards. MCO appointment availability reports are published in the [EQRO Annual Technical Report](#) posted on the HHSC website. HHSC shares the raw data with MCOs so they can develop targeted interventions to improve future results.

HHSC uses results from appointment availability studies to target improvement efforts. For example, HHSC placed greater focus on prenatal care through the 2022 MCO Performance Improvement Projects (PIPs) and Pay-for-Quality (P4Q) measures. The 2022 PIP scores were posted to the HHSC website in 2024.

MCO Appointment Availability Standards	
Level/Type of Care	Time to Treatment
Urgent Care (child and adult)	Within 24 hours
Routine Primary Care (child and adult)	Within 14 calendar days
Preventive Health Services for New Child Members	No later than 90 calendar days after enrollment
Initial Outpatient Behavioral Health Visits (child and adult)	Within 14 calendar days
Preventive Health Services for Adults	Within 90 calendar days
Prenatal Care (not high risk)*	Within 14 calendar days
Prenatal Care (high risk)*	Within five calendar days
Prenatal Care (new member in third trimester)	Within five calendar days
Vision Care (ophthalmology, therapeutic optometry)	None indicated ("Access without primary care provider referral")

*Prenatal care appointment availability studies are only conducted for STAR.

Provider Directory Quality

MCOs and DMOs use the information submitted by providers during credentialing for their directories. However, inaccurate or out-of-date information can impact members' ability to access services. HHSC requires MCOs and DMOs to update online provider directories weekly and monitors both online and hard copy directories for accuracy.

HHSC makes monthly calls to a random sample of providers from MCO and DMO directories to confirm that:

- The provider's contact information is accurate.
- The provider is accepting patients.
- The provider is meeting appointment availability standards.
- The provider covers certain age limits.
- The provider is contracted with an MCO or DMO.
- The provider is accepting the correct MCO or DMO program.
- The provider type is correct.
- The provider's office hours are correct.

HHSC shares findings with MCOs (including any data collected from appointment availability studies) and DMOs, who then develop plans to address identified issues. Efforts to improve the quality of provider directories are ongoing.

Member Satisfaction

To understand the patient experience with health care, HHSC reviews member complaints, as well as results from the Consumer Assessment of Healthcare Providers and Systems (CAHPS) and the National Core Indicators-Aging and Disabilities (NCI-AD) surveys.

The surveys allow HHSC to monitor and evaluate quality of care, assist members in choosing their MCO, and gather information on the outcomes of quality improvement initiatives. Surveys alternate so that each member group is surveyed every other year, as shown in the table below.

Odd Years	Even Years
• Child members in STAR • CHIP • Medicaid and CHIP Dental	• Adults members in STAR • STAR+PLUS • STAR Health • STAR Kids

CAHPS surveys focus on consumer perceptions of quality, such as provider communication skills and ease of access to health care services. Member survey results can be viewed on the [Texas Healthcare Learning Collaborative \(THLC\) Portal](#). Most Texas Medicaid CAHPS survey composites and ratings were equal to or higher than national averages. HHSC uses these results in its annual MCO report cards, performance indicator dashboards and in P4Q (see Transparency with the Public, page 70).

National Core Indicator (NCI) surveys assess various aspects of the experiences and outcomes of Developmental Disability services provided to members with Intellectual and Developmental Disabilities (IDD) and their families and for older adults and members with physical disabilities.

Texas participates in two types of surveys for three different client populations on a biennial basis:

1. NCI-IDD: The survey cycle runs July 1 through June 30.
 - Adult In-Person Survey
 - Child Family Survey
2. NCI-AD: The survey cycle runs July 1 through May 31.



Service Delivery

MCOs are required to provide all covered, medically and functionally necessary services to their Medicaid and CHIP members in the appropriate amount, duration and scope. HHSC monitors MCO service delivery and contractual compliance through utilization reviews (URs).

Utilization Reviews

URs are overseen by the Office of the Medical Director and are conducted by a team of nurses, clinicians and contract experts. URs examine requests for medical treatments to ensure MCOs are authorizing, justifying and providing appropriate, medically necessary services without overutilization or underutilization. The reviews also ensure MCO adherence to federal and state laws and rules, their contracts with HHSC, and their own policies.

URs consist of two review units: Acute Care Utilization Review (ACUR) and Long-Term Services and Supports Utilization Review (LTSS UR).

Acute Care Utilization Review

The ACUR unit oversees the authorization of medical benefits — a key component of an MCO's management of Medicaid service utilization. This oversight reduces authorizations of unnecessary services while ensuring that members have access to the care they need.

The reviews are typically done as part of HHSC's comprehensive readiness and operational reviews. They can also be conducted as a targeted review if a need is identified (see Targeted Reviews, page 76). The team is comprised of registered nurses, behavioral health specialists, contract specialists, and occupational, speech and physical therapists.

Activities include analysis of prior and concurrent authorization data for a targeted sample of members to determine whether MCOs appropriately and accurately: deliver medically necessary services; process requests in a timely manner; and ensure members have appeal and fair hearing opportunities when a service is denied.

Long-Term Services and Supports Utilization Review

The LTSS UR team samples STAR+PLUS Home and Community Based Services (STAR+PLUS HCBS), STAR Kids and STAR Health members enrolled in the Medically Dependent Children Program to review how MCOs conduct assessments. They also review MCO procedures and related information used to determine appropriateness of member enrollment in these programs. The review includes ensuring MCOs are providing services according to their assessment of service needs. The team conducts desk reviews of MCO documentation and interviews each of the members in the sample. Member interviews are conducted using one of the following modalities: in-person (preferred), audio-only or remote audio-visual.

UR findings inform policy and contract updates and clarifications, MCO consultation and training, internal process improvements, and remediation actions with MCOs. UR teams also coordinate and communicate with MCOs to find solutions that meet HHSC's standards and expectations.

Prescription Drug Oversight

Under current regulations, Medicaid and CHIP must adhere to the Texas Formulary — a list of covered, Centers for Medicare & Medicaid Services (CMS)-approved drugs — when administering pharmacy benefits. The Vendor Drug Program (VDP) oversees the Formulary and protocols for drug-use management across Medicaid, CHIP and other HHSC programs.

The VDP also manages the Preferred Drug List (PDL) — a subset of many but not all drugs on the Formulary. Drugs listed as “preferred” are available without prior authorization (PA), while drugs listed as “non-preferred” require a PA before they may be dispensed. A drug’s status on the PDL is based on its safety, efficacy and cost-effectiveness. The PDL is required in FFS and managed care.

The Texas Drug Utilization Review Board

The Texas Drug Utilization Review (DUR) Board is an HHSC advisory board whose members are appointed by the HHSC executive commissioner. The board is comprised of physicians and pharmacists who provide services across the Medicaid population and represent a variety of specialties. Three representatives from the MCOs and one consumer advocate for Texas Medicaid members sit on the board.

The duties of the board include submitting recommendations to HHSC for the PDL, approving clinical prior authorizations, developing and reviewing educational interventions for Medicaid providers and reviewing drug utilization across the Medicaid program.

Drug Utilization Reviews

HHSC and MCOs perform DURs before and after dispensing medications to Medicaid members. These reviews evaluate the safety and appropriate use of drug therapies while seeking to contain costs.

H.B. 3286, 88th Legislature, 2023, allows VDP to add drugs to the formulary more quickly on a provisional basis, sets guidelines for creating a temporary non-preferred status for new-to-market drugs just added to the formulary and elaborates on PDL exceptions for when a non-preferred drug can be used. H.B. 3286 also changes the DUR Board by adding an additional MCO representative and making all MCO representatives voting members.

Prospective Drug Utilization Reviews

Prospective DURs are performed before each prescription is filled or delivered to the member, typically at the point-of-sale or point-of-distribution. This review evaluates the member’s medication history to support appropriate and medically necessary drug utilization. It also includes screening for therapeutic duplication, interactions with other health conditions or drugs, incorrect drug dosage or duration of treatment, and clinical abuse or misuse.

The point-of-sale claim adjudication process includes advisory messages concerning clinically significant conditions or situations. Upon identifying any clinically significant conditions or situations, the pharmacist should take appropriate steps to avoid or resolve the problem, including consultation with the prescribing provider.

Pharmacy Prior Authorizations

There are two types of prior authorizations (PAs) for prescription drugs: non-preferred and clinical.

Non-preferred PAs may occur when prescribers choose medications listed on the PDL as non-preferred for their Medicaid patients. To prescribe a non-preferred medication, the provider must obtain PA from HHSC or their patient's MCO before the drug is dispensed.

Clinical PAs may be required for an individual drug or an entire drug class included on the Formulary. Drugs may also have preferred or non-preferred status on the PDL. Clinical PAs are determined using evidence-based clinical criteria and nationally recognized peer-reviewed information. Participating MCOs must perform certain clinical PAs and may perform others at their discretion.

When a PA cannot be obtained and a member needs medication without delay, members may be able to access a 72-hour emergency supply of the prescribed drug.

Retrospective Drug Utilization Reviews

After a member has received medication, retrospective DURs are conducted to review the drug therapy. Reviews examine claims data to analyze prescribing practices, the member's medication use and pharmacy dispensing practices. This helps identify patterns of fraud and abuse, gross overuse, and inappropriate or medically unnecessary care. This also informs active, ongoing educational outreach for prescribing providers and pharmacists to improve prescribing and dispensing practices.

Pharmacy Benefits Manager Oversight

MCOs contract with pharmacy benefits managers (PBMs), who build the MCOs' pharmacy networks, and negotiate rates with and pay claims to pharmacists. By extension, PBMs are required to comply with state and federal regulations, use the PDL, and follow the PA criteria established by the VDP and the MCO.

Many of the same tools used to monitor MCOs are also used to ensure PBM compliance, including desk reviews, on-site reviews, readiness reviews and targeted reviews. These activities include review of MCOs' policies, procedures, contract deliverables, and self-reported data for encounters and claims.

HHSC contracts also prohibit “spread pricing” (when a PBM keeps a portion of the reimbursement rates intended for pharmacies instead of passing the full payments from the MCO to pharmacies). Drug prices and dispensing fees paid to the pharmacies from the PBM must be transparent to the state.

Electronic Visit Verification

Electronic Visit Verification (EVV) is a computer-based system that electronically documents and verifies service delivery information, such as the date, time, location and service type for certain Medicaid service visits. EVV helps prevent fraud, waste and abuse while ensuring Medicaid members receive authorized care. Texas requires EVV for certain Medicaid-funded home and community-based services provided through HHSC and MCOs.

The 21st Century Cures Act

The 21st Century Cures Act is a federal law passed in December 2016, requiring the use of EVV for all Medicaid personal care services and home health care services that need an in-home visit. States that do not implement EVV receive reduced Medicaid funding.

HHSC implemented EVV for Medicaid personal care services on Jan. 1, 2021, and for Medicaid home health care services on Jan. 1, 2024.



Quality of Care

HHSC has a strong focus on quality of care in Medicaid and CHIP and uses multiple approaches to measure and improve care quality. Quality-based payment programs and other initiatives, like recently implemented benchmarks and value-based enrollment, incentivize MCOs, DMOs, facilities and other providers to improve care quality and increase value.

HHSC collects data from several sources to support its quality initiatives. The broad range of data available includes measures such as disease prevention, chronic disease management, behavioral health, maternal health and preventable hospitalizations, and member and provider experience.

Under federal regulations, HHSC contracts with an EQRO. Texas’ EQRO follows CMS protocols to assess access, utilization and quality of care provided by all MCOs and DMOs participating in Medicaid and CHIP medical and dental managed care programs.

HHSC uses the EQRO to perform custom quality-of-care evaluations. The EQRO uses a variety of tools and measures, including: analyzing MCO and DMO documents, provider medical records, and administrative data (like enrollment and encounter data); interviewing MCO and DMO administrators; and surveying members, their caregivers and providers.

EQRO reports allow comparison of results across MCOs and DMOs in each program and are used to develop overarching goals and quality improvement activities for Medicaid and CHIP managed care programs. MCO and DMO results are compared to HHSC standards and national benchmarks, if applicable.

Accountability for MCOs' and DMOs' continuous improvement in quality of care for members is primarily accomplished through various improvement projects and quality-based payment reforms.



**A
Closer
Look**

Value-Based Enrollment

HHSC encourages new members to select an MCO using MCO report cards and other provided information once they are found eligible. If a Medicaid member does not choose a plan, HHSC assigns them to an MCO through a process called default enrollment.

In State Fiscal Year 2021, HHSC began using value-based enrollment (VBE) in which measures of quality and efficiency are incorporated into the default enrollment process. Under VBE, MCOs that perform better on key risk-adjusted cost and quality measures and earn higher member satisfaction receive a greater share of default enrollments.

HHSC scores MCOs to determine their respective monthly target default assignments. VBE measures align with the state's Managed Care Quality Strategy and are weighted to balance incentives for better outcomes, lower costs and higher member satisfaction.

How Value Score Measures are Weighted

40%

Cost and Efficiency

Risk-adjusted actual to expected spending ratio

20%

Cost and Quality

Risk-adjusted potentially preventable events (PPE) ratios

40%

Quality and Member Satisfaction

Composite MCO report card scores

Performance Improvement Projects

Federal regulations require all states with managed care programs to ensure that MCOs and DMOs conduct PIPs, an integral part of Texas' Managed Care Quality Strategy. HHSC, in consultation with the EQRO, determines topics for PIPs based on quality goals and objectives and health plan performance. MCOs and DMOs create a PIP plan, report on their progress yearly and submit a final report. The EQRO validates the PIPs in accordance with CMS protocols.

MCOs and DMOs design PIP interventions and measurements to achieve significant, sustained improvement over time with a favorable impact on member health and satisfaction with their care. Each PIP lasts two years. HHSC requires each MCO and DMO to implement two PIPs per program on a staggered schedule, such that one PIP is implemented each calendar year. One PIP must be a collaborative project with another MCO, DMO or a community organization.

For 2024, DMO PIPs were focused on increasing fluoride varnish applications by collaborating with MCOs. MCO topics focused on improving maternal and childhood health outcomes by:

- Reducing the rate of severe maternal morbidity among deliveries with preeclampsia.
- Reducing the rate of c-sections in low-risk pregnancies and deliveries.
- Increasing the rate of behavioral health follow-up care.

In 2023, PIPs for MCOs focused on potentially preventable admissions for behavioral health-related diagnoses. DMOs' 2023 PIP topics were designed to increase the percentage of children under 21 who received a comprehensive or periodic oral evaluation.

Quality Assessment and Performance Improvement

Federal and state regulations require MCOs and DMOs to develop, maintain and operate quality assessment and performance improvement (QAPI) programs. MCOs and DMOs report on their QAPI programs each year and these reports are evaluated by Texas' EQRO.

Long-Term Care Facility Monitoring

HHSC oversees the quality of care received by all Texans in long-term care (LTC) facilities and helps providers optimize the care they deliver. Staff use several tools for monitoring, including on-site visits, technical assistance and training.

HHSC nurses, dietitians, pharmacists and qualified intellectual disability professionals conduct on-site quality monitoring visits. These visits help staff understand the changing needs of people living in the LTC facility and identify otherwise unseen problems. Staff use an early warning system to identify medium to high-risk LTC facilities and dispatch a rapid response team to facilities identified as an immediate risk to the health and safety of residents.

HHSC program staff research and connect providers statewide with training, tools and best practices, including in-service events and annual conferences. Staff also help facilities comply with state and federal regulations, including Pre-Admission Screening and Resident Review.

QAPI programs are ongoing, comprehensive quality assessment and performance improvement programs for all the services an MCO or DMO provides, while PIPs are time-limited interventions targeting a specific aspect of care.

Quality-Based Payment Reform

Quality-based payment programs are an integral part of health care payment reform through which provider payments become linked to improved quality and efficiency rather than volume of services delivered. HHSC administers quality-based payment programs in managed care and for health care and long-term care facilities, including: the medical and dental P4Q programs; Alternative Payment Models (APMs); the Hospital Quality-based Payment Program (see Chapter 4, page 105); and directed payment programs (DPPs) targeting specific provider groups, (see Chapter 4, page 102), including the Quality Incentive Payment Program for nursing facilities (see Chapter 4, page 103).

More information about quality-based payment programs and reforms can be found on the HHSC website.

The Medical and Dental Pay-for-Quality Programs

The medical P4Q program evaluates MCOs on a set of quality measures — including disease prevention, chronic disease management, and maternal and infant health — and places a percentage of the MCOs' capitation at risk, depending on performance. The dental P4Q program places a portion of the DMOs' capitation at risk based on performance on a set of dental care quality measures.

These programs incentivize MCOs and providers to implement APMs — also called value-based payments — which help them meet or exceed their P4Q performance targets. MCOs and DMOs are required to meet APM contract requirements. MCOs should make at least 50 percent of their payments to providers through APMs connected to measures of quality and value, with 25 percent of total payments through models with downside financial risk for all contracted providers. For DMOs, the requirements are 50 percent overall with 10 percent risk-based (see Chapter 4, page 104). HHSC uses the Healthcare Payment-Learning and Action Network APM framework to help guide this effort. Visit [HCP-LAN.org](https://www.hcp-lan.org) for more information.

Beginning in 2025, HHSC is broadening its APM requirements to encourage more focus on specific agency priorities, such as improving maternal and birth outcomes, reducing preventable emergency room use, and integrating behavioral and primary health care.

Aligning Technology by Linking Interoperable Systems Program for Improved Quality and Client Outcomes

Starting fiscal year 2025, HHSC introduced the Medicaid Managed Care Aligning Technology by Linking Interoperable Systems (ATLIS) for Client Health Outcomes program. Designed under the authority of 42 C.F.R. § 438.6(b)(2), HHSC developed the ATLIS program to improve the exchange of electronic data between MCOs and their network providers. The program is intended to advance quality strategy goals, accelerate the development of APMs and improve client outcomes. The program is initially focused on implementing improved data exchange between MCOs and certain hospital providers. ATLIS is an incentive program for MCOs. The MCOs may earn incentives from 0.05 to 5 percent of their capitation, depending on milestone achievement related to certain hospital classes within their networks.

Transparency with the Public

HHSC is committed to increasing transparency with the public regarding managed care performance and quality-related measures, including MCO report cards and the [THLC portal](#).

Managed Care Report Cards

Report cards apply to MCOs only and do not apply to DMOs.

HHSC uses a one- through five-star rating system to evaluate MCOs on their overall performance and on specific measures, such as the quality of care provided for chronic conditions like asthma or diabetes. Ratings are developed by surveying current members and analyzing encounter data and are updated annually.

These report cards allow members to compare the MCOs in their service area and make informed selections when enrolling or changing plans. MCO report cards are posted on the HHSC website and are included in the Medicaid and CHIP enrollment packets.

Texas Healthcare Learning Collaborative Portal

This website serves as a public reporting platform, contract oversight tool and a tool for MCO and DMO quality improvement efforts. HHSC, MCOs, DMOs, providers and the public can use the site to get up-to-date data on key quality-of-care measures — including PPEs, Healthcare Effectiveness Data and Information Set (HEDIS), hospital performance, and other health care quality information.

Providers can also see performance data, by MCO and DMO, within a service area over time. This data may serve as an important tool for providers to find unmet needs they can address. A provider who excels in an area of care identified for quality improvement may find common ground to engage with MCOs and DMOs on value-based contracting. The THLC portal can be accessed at [THLCPortal.com](https://thlcportal.com).



Quality Monitoring Sources and Measures

HHSC tracks and monitors program performance using a performance indicator dashboard — a combination of national and state-developed measures by program. MCOs are subject to contract remedies for failing to meet minimum standards for one-third or more quality measures, which includes member survey measures.

Sources	Measures
National Committee for Quality Assurance	Nationally recognized and validated set of measures used to gauge quality of care provided to members, including HEDIS.
Healthcare Effectiveness Data and Information Set (HEDIS)	Domains include effectiveness of care, access and availability of care, experience with care, and health care utilization.
Agency for Healthcare Research and Quality Pediatric Quality Indicators (PDIs)/ Prevention Quality Indicators (PQIs)	- PDIs and PQIs use hospital discharge data to measure quality of care for ambulatory care sensitive conditions — which are conditions where good outpatient care or early intervention can prevent hospitalization, complications or more severe disease. - PDIs specifically screen for problems experienced by children and youth.
3M Software for Potentially Preventable Events (PPEs)	The EQRO calculates rates across all MCOs and programs for PPEs, including potentially preventable: - Admissions. - Readmissions. - Emergency department visits. - Complications. - Ancillary services.
Consumer Assessment of Healthcare Providers & Systems (CAHPS) Surveys	- CAHPS health plan survey is a nationally recognized and validated tool for collecting standardized information on members' experiences with MCOs and services. - The EQRO alternates CAHPS surveys so that members or caregivers from each program are surveyed every other year.
Dental Quality Alliance (DQA)	- The DQA is an organization convened by the American Dental Association at the request of CMS. - DQA has developed national evidence-based oral health care performance measures that have been tested for feasibility, validity, reliability and usability.

Member Complaints and Appeals

When members are dissatisfied with their Medicaid services, they can formally let their MCO or DMO and HHSC know. The process they follow depends on the issue.

- **Submit a complaint:** A member can submit a complaint to the MCO, DMO and to HHSC, if they have an issue with the care they receive. Examples include concerns with customer service, delays in getting an appointment, PA issues, finding an in-network doctor/dentist, and problems contacting their MCO or DMO.
- **File an appeal:** A member can file an appeal directly with their MCO or DMO, if they have an issue with the MCO's denial to cover a service, medical supply, durable medical equipment or prescription, or the DMO's denial or limited authorization of a requested service.

HHSC monitors member complaints and appeals as an oversight tool to look for early warnings of potential systemic problems, larger MCO or DMO operational issues or the need for policy clarifications.

Member Complaints

Managed care complaint intake is centralized with the Office of the Ombudsman. This reduces confusion for members about whom they should contact to submit a complaint and where to find resources that explain how to submit a complaint and what to expect.

Centralization with the Ombudsman also allows for more comprehensive tracking. Standard definitions for categories improve data quality on trending and systemic issues. For example, now MCOs, DMOs and HHSC track access to care, customer service and quality of care complaints the same way, resulting in more reliable data. Since centralization, MCOs and DMOs submit complaint data more frequently to allow for timelier analysis and action.

Submission and Resolution

Members are encouraged to first submit their complaints directly to their MCO or DMO. If they still need assistance, they can also submit complaints to the Ombudsman.

MCOs and DMOs must resolve complaints within 30 days, regardless of point of entry. Complaints about urgent access to care are escalated for a faster resolution. CHIP MCO complaints are submitted to TDI and the MCOs. FFS complaints are handled through the Medicaid helpline.

Provider Complaints

Providers can also submit complaints. The most common provider complaints relate to MCO or DMO administrative functions, including claims or billing disputes and service authorizations.

Providers are encouraged to exhaust the relevant MCO or DMO complaints process before submitting a complaint to HHSC. Managed care provider complaints are addressed by Medicaid and CHIP Services. Provider complaints not related to managed care are submitted to the Texas Medicaid and Healthcare Partnership.

Member Appeals

When a member gets a letter from their MCO letting them know a service, medical supply, durable medical equipment or prescription is not covered or denied, they can ask the MCO to reconsider by asking for a plan-level appeal. Members who receive a letter from their DMO of a denied or limited authorization of a requested service may ask the DMO for a plan-level appeal.

If the member still disagrees with their MCO's or DMO's decision after their appeal, they can ask for a State Fair Hearing with HHSC. Members can ask to keep getting services during the health plan appeal and State Fair Hearings process that follows.

External Medical Reviews

In 2022, HHSC implemented an external medical review (EMR) option during the State Fair Hearings process for managed care members in the STAR, STAR Kids, STAR+PLUS, STAR Health programs and dental programs.

If a managed care member requests to get an EMR along with their State Fair Hearing, the review occurs before their State Fair Hearing. HHSC sends the member's case to an independent review organization (IRO) where medical experts privately review the MCO's or DMO's decision. The IRO can uphold or change the MCO's or DMO's decision. Once the member gets the results of the EMR, they can decide if they want to cancel or move forward with their State Fair Hearing.

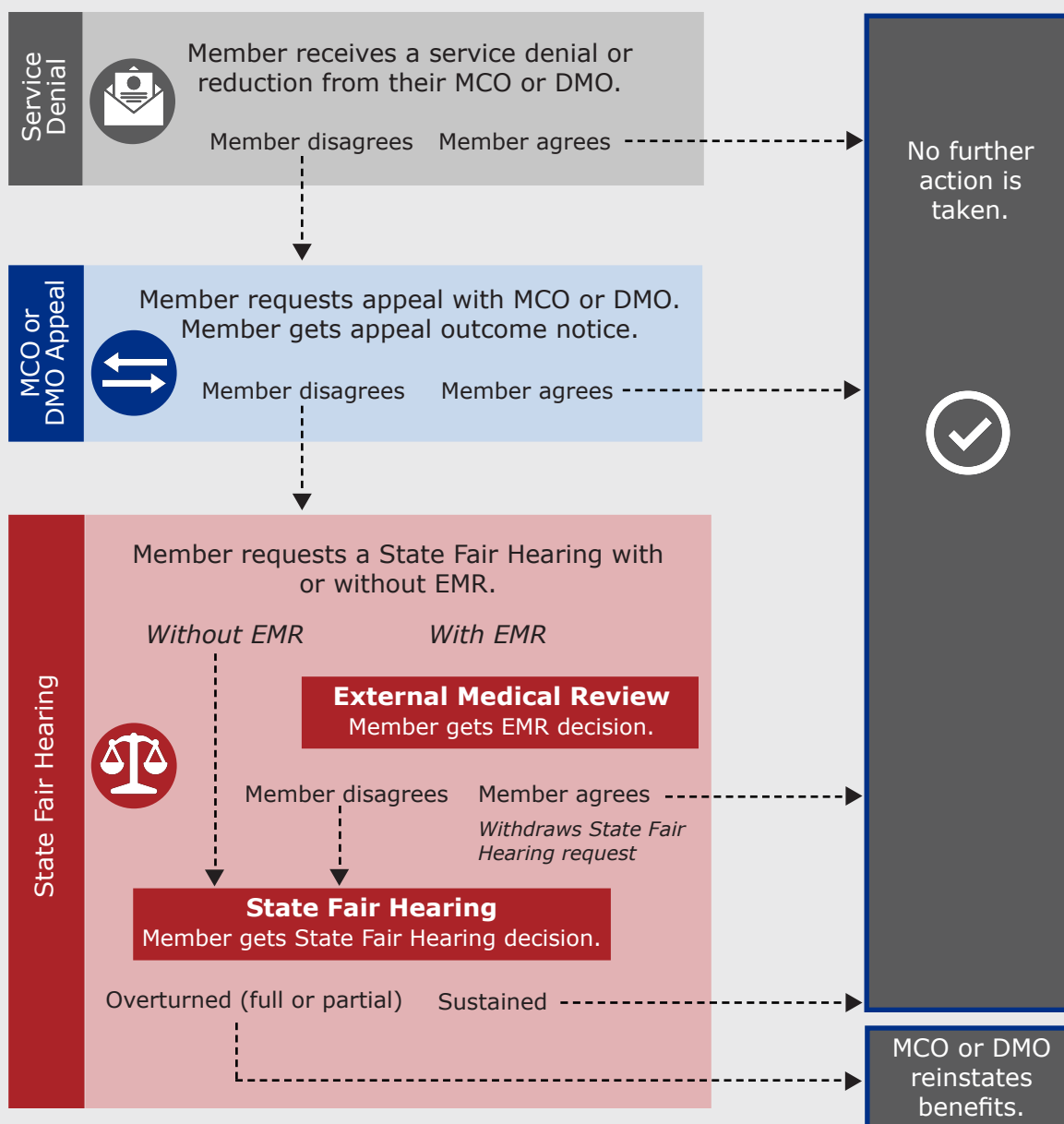
(See A Closer Look, page 74, for the member's journey through the MCO and DMO appeal process.)

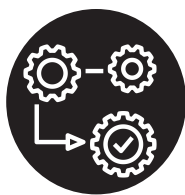


A Closer Look

MCO and DMO Appeal Process

Below is a high-level visual illustration of the member's journey through the health plan appeal process. A member can ask for a State Fair Hearing with or without an EMR after they receive their MCO's health plan appeal decision.





Operations

The way MCOs and DMOs conduct their business operations can have a direct impact on the care members receive.

Operational Reviews

HHSC conducts biennial operational reviews of MCOs and DMOs. These operational reviews generally comprise an in-depth review of MCO and DMO operational compliance and performance across several areas to ensure policies and practices align with performance standards. A multi-disciplinary team reviews key functions and requirements as stipulated in the MCO's or DMO's contract. Each subject area has its own tools for data collection. The HHSC Office of Inspector General (OIG) is also invited to participate in procurement readiness and operational reviews. MCO and DMO staff interviews are part of the review process.

Operational reviews can be conducted on site, virtually or a hybrid of both methods.

Examples of operations reviewed on site or through virtual meeting and review of materials include:

- Claims processing.
- Member and provider training.
- Complaints and appeals.
- Encounter data.
- PA processes.

If any problems are discovered during the operational reviews, HHSC takes appropriate steps to address performance. Operational reviews can also inform planned third-party performance audits.

Third-Party Performance Audits

Independent auditors conduct performance audits and self-reported data audits as determined by risk. Medicaid and CHIP Services also works with the OIG to collaborate on audit topics. The target of the audits varies, but typically focuses on two areas:

1. MCO or DMO data, such as complaints and appeals
2. Operational processes, such as claims processing or subcontractor monitoring, including PBMs

Targeted Reviews

In addition to the established biennial operational review process for MCOs and DMOs, HHSC also conducts targeted reviews when a significant or recurring problem with an MCO or DMO is identified — for example, claims timeliness. This can occur in response to review of other compliance deliverables or to complaints from members, providers or other stakeholder groups. The scope, entity and focus of targeted reviews vary based on the topics raised by complaints received and past instances of noncompliance.

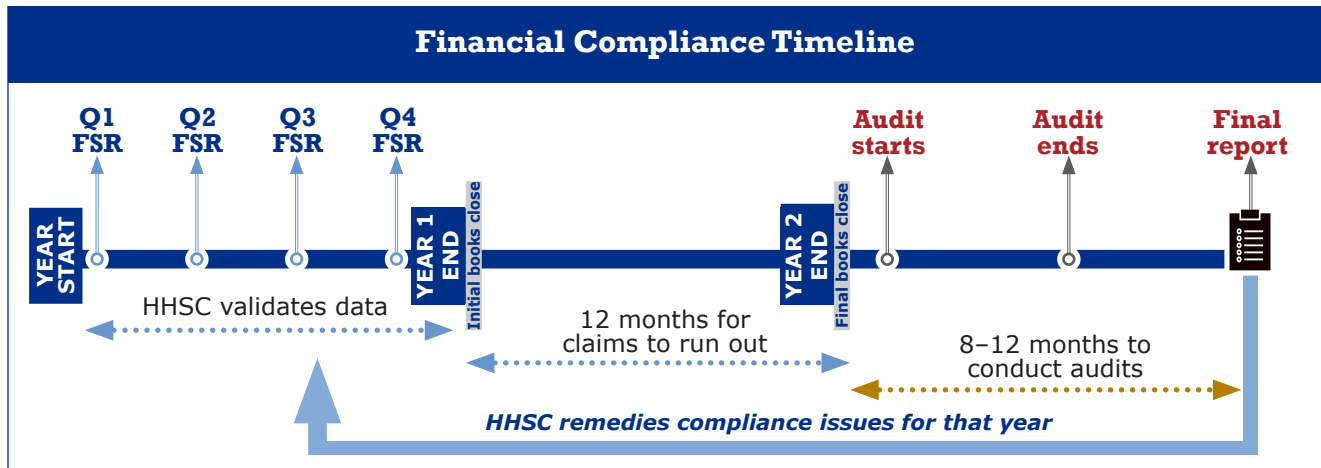


Financial

The contract defines financial requirements of the MCOs and DMOs, including the standards for financial data they must report to HHSC. MCOs submit financial statistical reports (FSRs) that include information on medical and administrative expenses. MCO contracts include limits on administrative expenses recognized by the Medicaid program and establish profit-sharing provisions to limit profits. Like MCOs, DMOs are subject to financial oversight based on their contract terms and must submit FSRs. There are some differences in DMOs' contract terms, such as no limits on the allowable administrative expenses, and differences in the profit limits including the experience rebate tiers and recovery rates. The experience rebates process determines the amount of profit-sharing with the State (see A Closer Look, page 78).

FSRs are one source for establishing capitation rates in future years, making the validation of them an important component of contract oversight. HHSC financial analysts validate MCO-reported medical expenses to encounter data on a quarterly basis. Independent auditors review the administrative expenses reported by the MCOs and provide additional data validation by comparing medical expenses to paid claims.

The timeline to complete oversight for MCO financial activity for a given year is 20–24 months after the end of that year. Full audits by independent auditors can only occur after the final books close and all claims have run out for that given year. The timeline may be impacted by workforce changes and the timeliness of audit processes.



While audits occur annually, HHSC financial analysts can also determine the need for any supplemental audits or reviews based on other identified issues.

MCOs are paid PMPM capitation rates for the delivery of services. Capitation rates are established each year based on actual MCO expenditures on medical services from previous time periods. Reductions in spending on medical services will have the effect of reducing future capitation rates. If spending to provide contracted services exceeds their capitation rate payments, MCO profit margins are at risk.



**A
Closer
Look**

Administrative Expense and Profit Limits

MCO Administrative Expense Limits

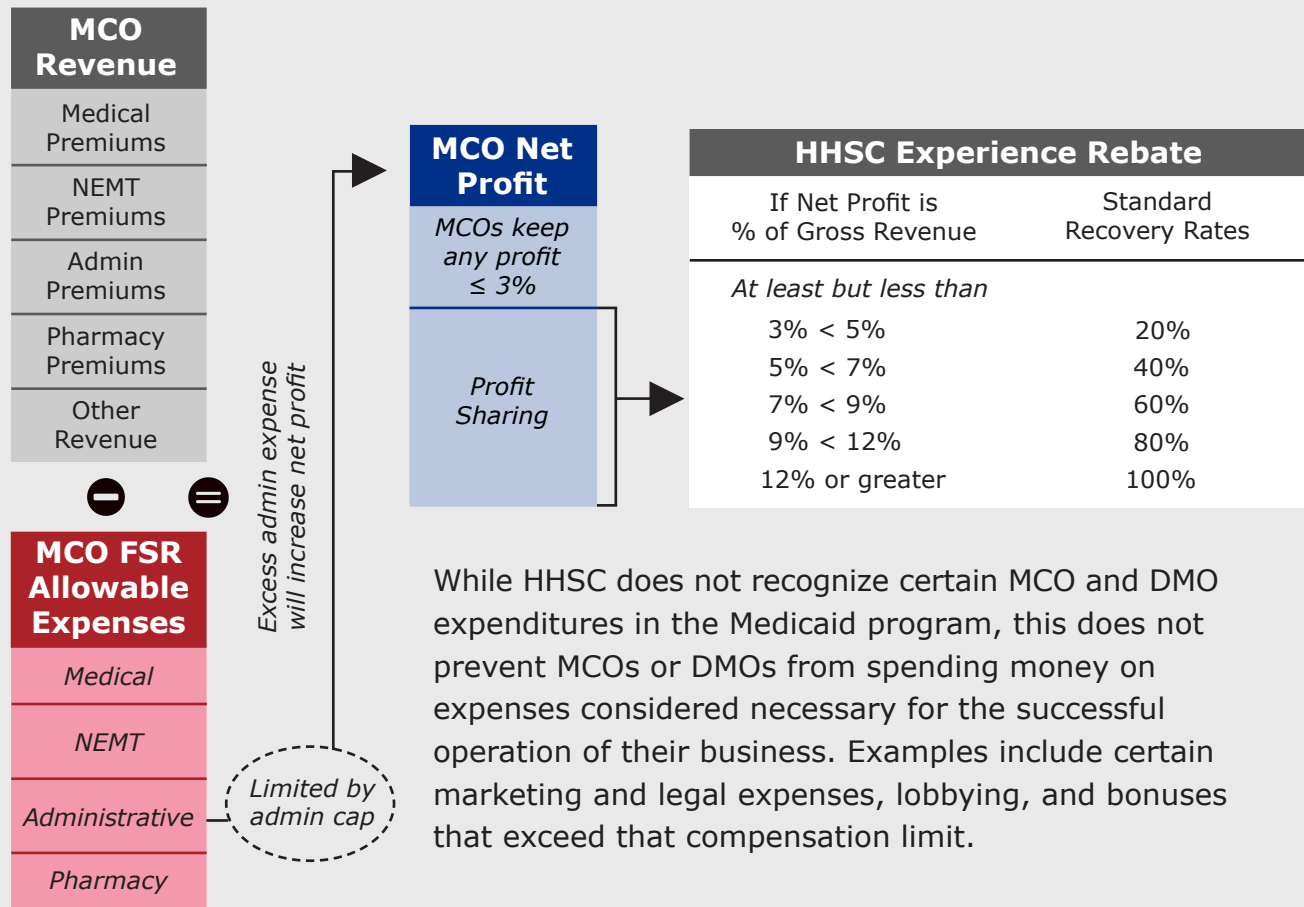
Contract terms define an allowed and disallowed administrative expense. Additionally, HHSC limits administrative expenses that Medicaid will pay in the contract, and this is referred to as the admin cap.

- The cap is compared to MCO's reported administrative expenses.
- Any amounts over the admin cap also become disallowed expenses, and the MCO's net income is increased by that amount.

MCO Profit Limits

- MCOs will retain all their net income before taxes that is equal to or less than 3 percent of the total revenues received.
- Net income greater than 3 percent of total revenues will be shared based on a graduated experience rebate method.
- The experience rebate structure is tier-based.

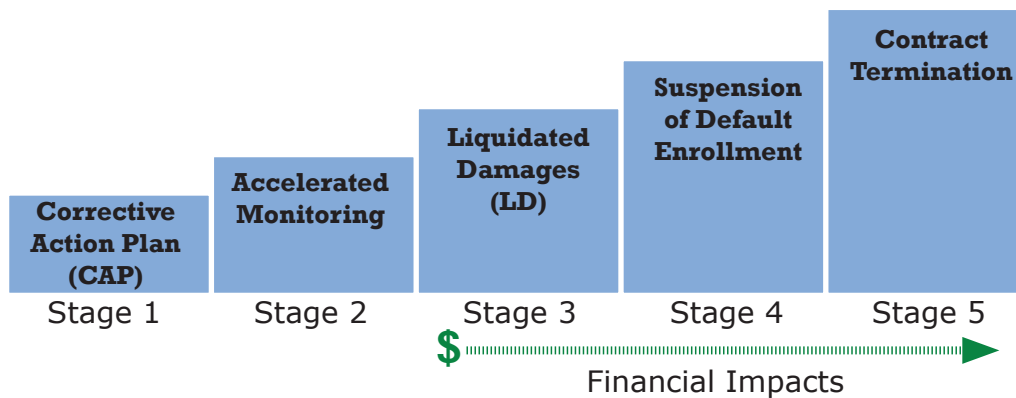
Net Profit and Experience Rebate





Non-Compliance Remedies

HHSC may use multiple types of enforcement actions, including monetary damages and CAPs, to hold MCOs and DMOs accountable for not meeting contract terms. As specified in the managed care contracts, HHSC may impose one or more of the following remedies at its discretion:



The type of remedy issued is contingent upon the type and severity of the non-compliance. Remedies may not be issued sequentially.

Assessment of Contract Remedies

CAPs and LDs are the primary remedies HHSC uses to address performance issues or contractual non-compliance. The goal of any contract action is to encourage compliance with contract standards.

Corrective Action Plans

HHSC assesses the need for CAPs monthly. Through the CAP process, MCOs and DMOs are required to provide HHSC with:

- A detailed explanation of the reasons for the deficiency.
- An assessment or diagnosis of the cause.
- Actions taken to resolve the deficiencies.
- Actions taken to prevent future occurrences.

HHSC closes the CAP once the MCO or DMO demonstrates it has taken appropriate action to address contractual non-compliance. Examples of CAPs include non-compliance with network adequacy standards and issues identified through URs.

To increase transparency, the issuance of CAPs was added to the HHSC website. This allows the public and other states that are contemplating contracting with an MCO or DMO to see its performance history.

Accelerated Monitoring

HHSC can implement accelerated monitoring practices, which are more frequent or extensive than standard monitoring practices for an MCO or DMO.

Examples of accelerated monitoring practices include additional reporting requirements, escalated CAPs and targeted on-site reviews.

Liquidated Damages

LDs are not intended to be a penalty but are meant to assess and recover HHSC's projected financial loss and damage resulting from MCO or DMO non-compliance, including losses due to project delays.

HHSC assesses LDs quarterly. Examples of LDs include non-compliance when submitting encounter data, provider payment timeliness and delivery of appropriate services. All LD decisions and reconsideration determinations require written approval.

Suspension of Default Enrollment

Members who do not proactively choose an MCO or DMO are automatically assigned one through a process known as default enrollment. Every MCO or DMO receives a percentage of their members through default enrollment. HHSC may suspend default enrollment for an MCO or DMO with persistent contractual non-compliance.

HHSC's executive commissioner must approve the suspension of default enrollment. The suspension is effective either for a minimum of 90 days or until HHSC determines the issue is resolved or identifies another appropriate timeline.



The Office of Inspector General

The mission of the OIG is to ensure the health and safety of Texans through the prevention and detection of fraud, waste and abuse in publicly funded, state-run health and human services programs. OIG has five main tools for detecting, deterring and preventing fraud, waste and abuse to ensure taxpayer funds for health and human services are properly spent:

1. Performance audits focus on risk-based topics related to providers, contractors and programs across HHS and the Department of Family and Protective Services.
2. Inspections focus on systemic issues and risk assessment.
3. Investigations focus on alleged fraudulent, wasteful or abusive practices.
4. Reviews focus on fraud, waste and abuse in documentation, billing and payments.
5. Data analytics drive OIG work by identifying trends and outliers for further review.

Performance Audits

Audits may identify non-compliance with requirements, overpayments, and program policy gaps. OIG auditors may propose recommendations to prevent fraud, waste and abuse. OIG staff conduct risk-based performance audits, including audits of HHSC agencies, contractors and providers. Those audits cover a range of topics, such as the accuracy of medical provider payments; the performance of HHSC contractors; information technology; and the functions, processes and systems within HHSC programs. Examples of previous audits include provider delivery of telehealth services, the effective utilization of state funding by HHSC contractors, and the controls in place for certain activities at MCOs and DMOs, such as the security of confidential information.

Inspections

Inspections are targeted examinations of providers and HHSC programs, systems or functions that may identify systemic trends of fraud, waste and abuse. Findings from an inspection may result in the OIG's use of another tool to further review the topic or actions to address the OIG's observations. Topics from past inspections include ambulance claims oversight, mental health private psychiatric bed funds and managed care claims submitted by durable medical equipment providers.

Investigations

Provider Investigations

This unit investigates allegations of fraud, waste and abuse by health care providers. The results of an investigation may lead to recoupment of overpayments, imposition of sanctions or administrative actions, and referrals to licensing boards and the Office of the Attorney General's Medicaid Fraud Control Unit.

Special Investigations

This unit comprises several functions, including the Major Case Unit, which investigates complex cases across the HHS system. Other Special Investigations functions include retailer monitoring and HHS oversight.

Benefits Program Integrity

This unit examines members suspected of abusing HHSC programs — including Medicaid, CHIP, the Supplemental Nutrition Assistance Program, the Temporary Assistance for Needy Families program, and the Special Supplemental Nutrition Program for Women, Infants and Children. Findings can include the provision of false information to apply for services or using someone else's benefits for services.

Reviews

This unit reviews utilization of acute care services (e.g., provider office visits, laboratory and x-ray services), hospital services (e.g., surgical specialists), and nursing facility services (e.g., rehabilitation, long-term care).

Medicaid Lock-In Program

The Medicaid Lock-In program operates by “locking in” a member to one provider and pharmacy to prescribe and dispense certain drugs, like controlled substances (e.g., opioids), to prevent their abuse or overuse. This prevents members from visiting more than one Medicaid provider to attain prescriptions for controlled substances. The OIG analyzes Medicaid claims data to identify members who reach a pre-defined threshold of prescriptions or provider visits. Providers and MCOs also make referrals to the program.

Data Analytics

The OIG develops complex algorithms to find trends, patterns and outliers in Medicaid billing that could indicate fraud, waste or abuse. Advanced data analytics allow the OIG to better focus its audits, inspections, investigations and reviews. In addition, data analysts conduct data-informed policy research, develop data intelligence solutions and provide data system support for other OIG functions.

Chapter 4

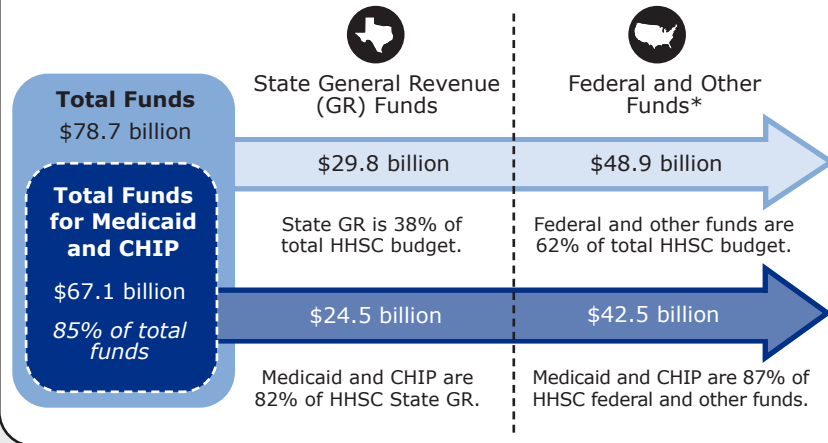
What are the financial features of Medicaid and CHIP?



At-a-Glance

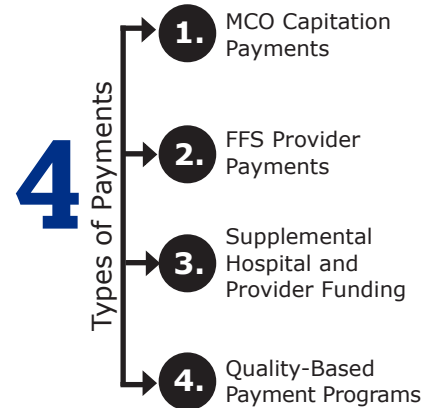
Funding

2022–2023 HHSC Biennial Appropriations

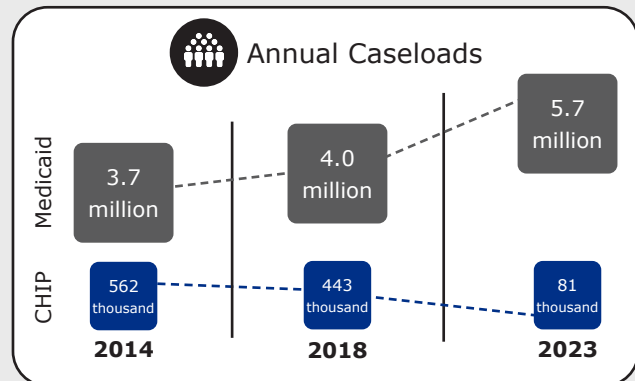
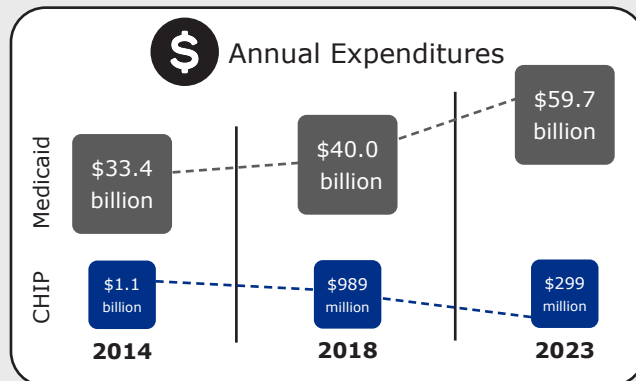


*Other funds include, but are not limited to, Appropriated Receipts, Interagency Contracts, Medicaid Subrogation Receipts (State Share), and WIC Rates.

Payments



Growth Trends



Includes DSH, OC, OC/PHP/CCP, FFS and Supplemental Payment Program expenditures.

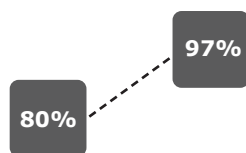
Full details for annual expenditures and caseloads can be found on page 168.

Caseloads impacted due to federal PHE.

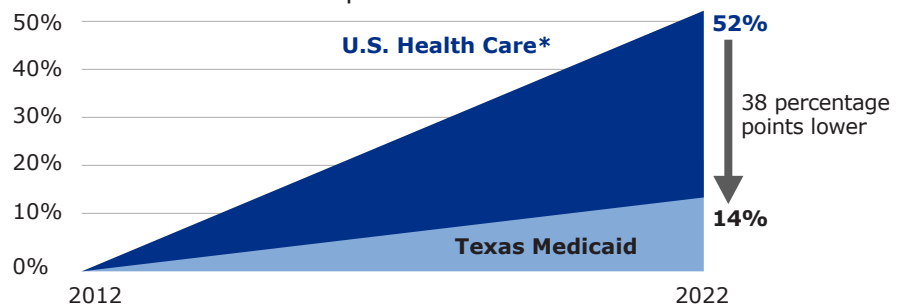
10-Year Cost Growth Comparison

The number of Texans enrolled in managed care has grown by approximately 2 million in 10 years. The value of managed care is demonstrated by Texas Medicaid's cost per person growth, trending lower than U.S. national average.

Managed Care Percent of Caseload



Cost per Person



Texas Medicaid cost per person is based on full benefit clients.

*Source: CMS, Office of the Actuary 2012-2022 – data is for CY12-CY22.



Budget Development

There are several factors that impact the state Medicaid budget, including what types of services Texas chooses to cover and the amount of federal matching funds that certain programs will receive.

Health and Human Services Commission (HHSC) staff develop the estimates of future Medicaid caseloads and spending that, in turn, form the basis for state appropriations requests. Estimates are based on:

- Projections of the number of people eligible for and applying for the program.
- Estimations of cost trends.
- Analyses of any new federal mandates or state changes affecting eligibility, services or program policy.

Ultimately, decisions about funding are determined by the Texas Legislature.

The budget takes effect at the beginning of the biennium in September of odd-numbered years. A significant amount of time elapses between the development of the initial agency budget request and the passage of a finalized appropriations bill.

Legislative Appropriations Timeline	
August	State agencies submit legislative appropriations requests for the next biennium
January	Legislature convenes
April	Legislature works on appropriations bills; last chance to provide up-to-date Medicaid projections for bills
May	Legislature finishes appropriations for the next biennium
September	New biennium begins (in odd-numbered years only)

Medicaid Matching Funds

Federal funds are a critical component of health care financing for the State of Texas. The amount of federal Medicaid funds Texas receives is based primarily on the federal medical assistance percentage (FMAP), or Medicaid matching rate. With some exceptions, such as waivers or the Disproportionate Share Hospital (DSH) program, there is no cap on federal funds for Medicaid expenditures.

Derived from each state's average per capita income, the Centers for Medicare & Medicaid Services (CMS) updates the rate annually. Consequently, the percentage of total Medicaid spending that is paid with federal funds also changes annually. As the state's per capita income increases in relation to the national per capita income, the FMAP rate decreases. For Federal Fiscal Year (FFY) 2025, Texas' Medicaid FMAP is 60.00 percent.

Texas uses a one-month differential FMAP figure that considers differences between the FFY, which runs October through September, and the State Fiscal Year (SFY), which runs September through August. The one-month differential FMAP for Texas in SFY 2025 is 60.01 percent — which includes one month of the FFY 2024 rate of 60.53 percent and 11 months of the FFY 2025 rate of 60.00 percent.

CHIP Matching Funds

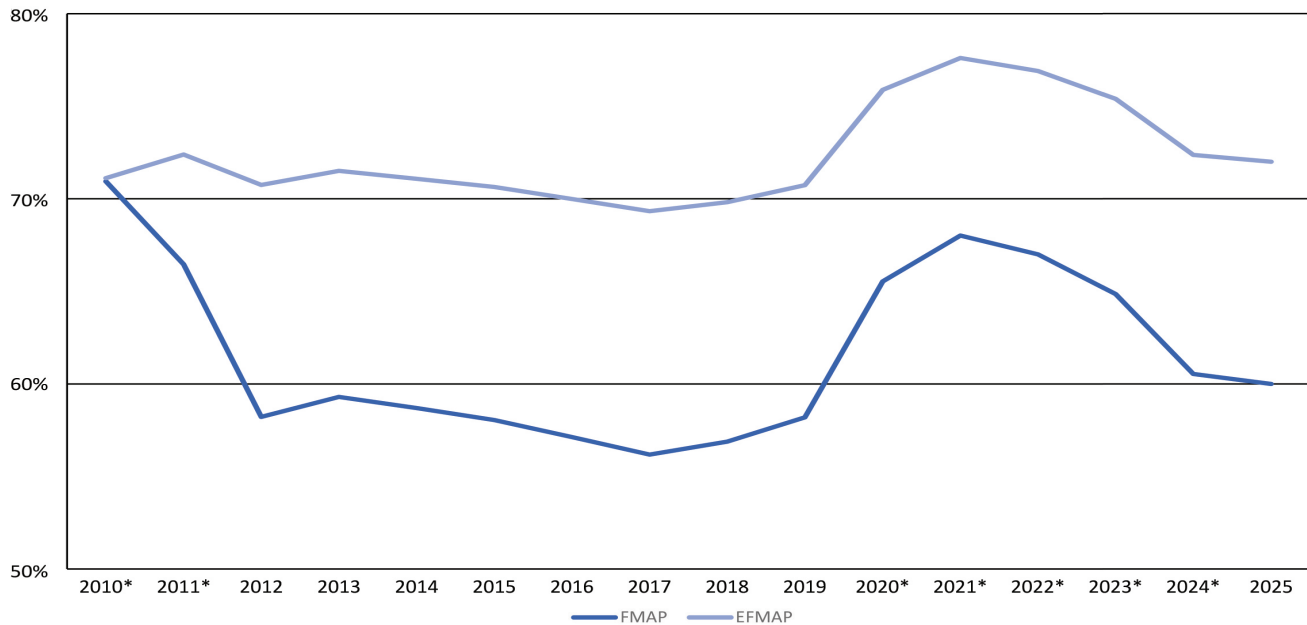
Unlike Medicaid, the Children's Health Insurance Program (CHIP) is a federal block grant program rather than an entitlement program. Total federal funds allotted to CHIP each year are capped, as are the funds allotted to each state. Each state is allotted a portion of the total federal funds based on a formula set in federal statute, and each state receives federal matching payments up to the allotment. The FFY 2023 allocation was fully expended, and the 2024 allocation is estimated to be fully expended in 2025. The federal allocation for Texas in FFY 2023 was \$1.4 billion.

In addition, CHIP offers a more favorable federal matching rate than Medicaid. The federal CHIP funds that states receive are based on the enhanced federal medical assistance percentage (EFMAP). Derived from each state's average per capita income, CMS updates this rate annually. Consequently, the percentage of total CHIP spending paid with federal funds also changes annually.

Health Services Initiatives

States have the option to use unexpended federal CHIP funds within the 10 percent administrative cap to develop Health Services Initiatives (HSI). An HSI must directly improve the health of low-income children less than 19 years of age who are eligible for CHIP or Medicaid but may serve children regardless of income.

Texas Federal Medical Assistance Percentages, FFYs 2010–2025



*Indicates stimulus FMAPs in effect.

The Affordable Care Act (ACA) temporarily increased the EFMAP for FFYs 2016 through 2019. Following the end of this increase, the EFMAP rate was decreased by 23 percentage points. Due to the federal Public Health Emergency (PHE), the 2020-2024 FMAPs include an additional federal match of:

- 6.2% for the period January 2020 - March 2023
- 5.0% for the period of April 2023 - June 2023
- 2.5% for the period of July 2023 - September 2023
- 1.5% for the period of October 2023 - December 2023

The EFMAP for Texas in FFY 2025 is 72.00 percent and in SFY 2025 is 72.01 percent.

Deferrals and Disallowances

Deferrals and disallowances impact the availability of federal financial participation:

- **Deferral:** If CMS determines that a state may be out of compliance with federal regulations or its Medicaid State Plan, then CMS may withhold funds until compliance is proven or until the state provides additional information to support the validity of the claim.
- **Disallowance:** If CMS alleges a claim is not allowable, then it can recoup federal funds.

CMS can impose deferrals or disallowances following a federal audit or a change to the Medicaid State Plan.

A deferral or disallowance may be imposed for the federal fiscal quarters for which CMS asserts the state is out of compliance with CMS regulations or its Medicaid State Plan. In the case of a disallowance, CMS may retroactively encompass several years of claims.

States have the option to appeal the CMS determination with the U.S. Department of Health and Human Services' Departmental Appeals Board. The Departmental Appeals Board will make a ruling based on the written records provided by both parties or can hold a hearing prior to making a ruling.

Mandatory and Optional Spending

Texas Medicaid is federally required to provide certain acute care services and long-term services and supports (LTSS) — including inpatient and outpatient care, physician services, family planning services and supplies, extended services for pregnant women, and nursing facility services for clients age 21 and older (see Appendix B, page 156).

Texas also chooses to cover some of the optional services allowed, but not required, by the federal government. These services do not necessarily increase costs.

In fact, eliminating some optional services and eligibility categories could increase Medicaid costs. For example, dropping the option of covering prescription drugs could ultimately cost Medicaid more. People who do not receive needed drugs may require more physician services, increased hospitalizations or even LTSS. Similarly, Texas potentially saves money by covering pregnant women up to 198 percent of the federal poverty level (FPL), because some women may not otherwise receive adequate prenatal care. This coverage helps prevent adverse and costly pregnancy outcomes.

In addition, some of the optional services covered by Texas Medicaid were originally paid for with 100 percent state or local funds. By adding coverage for those services through Medicaid, part of the cost is now covered with federal matching dollars.

For example, services for individuals with intellectual or developmental disabilities (IDD) that are provided by state supported living centers and community-based residential settings now receive federal Medicaid matching dollars in addition to state funds.

The 1115 Transformation Waiver and Budget Neutrality

The 1115 Transformation Waiver, also known as the Texas Healthcare Transformation and Quality Improvement Program (THTQIP), provides the federal authority for operations of most of the state's managed care programs — including STAR, STAR+PLUS, STAR Kids and the Children's Dental Program — while preserving supplemental funding for providers, including hospitals (see Chapter 5, page 112). On Jan. 15, 2021, CMS approved an extension of the waiver through September 2030.

Medicaid spending under a Section 1115 demonstration waiver is required to be budget neutral, meaning that federal spending under the demonstration cannot exceed projected costs in the absence of the demonstration (referred to as the without-waiver baseline). If state spending is below the without-waiver baseline, the difference is considered savings.

The savings accumulated by a state may be used to fund supplemental payments to providers not otherwise permitted under federal regulations. Texas' 1115 waiver originally contained two such funding pools: the Uncompensated Care (UC) and the Delivery System Reform Incentive Payment (DSRIP) pools. Under the current waiver, these savings support the UC and Public Health Provider-Charity Care pools (see Chapter 4, page 101).

DSRIP ended in September 2021 with funding expiring in January 2023. Under the current waiver, HHSC has implemented a transition plan that preserves budget neutrality and funding for providers while continuing health care transformation in Texas. The Quality Incentive Payment Program (QIPP), and our new directed payment programs (DPPs), now operate in conjunction with the waiver (see Chapter 4, page 103).



Rate Setting

Managed Care Organization Rates

Since most Medicaid clients are enrolled in managed care, managed care organization (MCO) capitation rates are the primary way that the state pays for services. These rates act as the state's payments to MCOs for contractually required services. MCOs then negotiate rates for services with providers and pay them to administer services to members.

Payment rates to MCOs are developed using actuarially sound practices and principles. Payments are based on a "per member per month" (PMPM) rate for each risk group within each service delivery area and the MCOs' membership enrollment. PMPM rates differ across risk groups and service delivery areas and differ by MCO based on the health status, or acuity, of the MCOs' membership.

For example, STAR MCO capitation rates are derived from MCO historical claims experience, also called encounter data, from a base period of time. Encounter data includes records of the health care services for which MCOs pay and the amounts MCOs pay to providers of those services. Rates are established each year based on actual MCO expenditures. Reductions in spending for Medicaid services will have the effect of reducing future capitation rates.

From this, the base cost data are totaled, and trends are calculated for the prospective time period during which the rates will apply. The cost data are also adjusted for MCO expenses, such as reinsurance, capitated contract payments and changes in plan benefits. A reasonable provision for administrative expenses, taxes and risk margin is also added to the claims component to project the total cost for the rating period. A risk margin is included to recognize financial risk.

For STAR MCOs, newborn delivery expenses are removed from the PMPM rates, resulting in an adjusted capitation rate for each service area. A separate lump-sum payment, called the delivery supplemental payment, is computed for each service area for expenses related to each newborn delivery.

A final adjustment is made to reflect the acuity of the population enrolled in each MCO. The purpose of the acuity risk adjustment is to recognize the anticipated cost differential among multiple MCOs in a service area, due to the variable health status of their respective memberships. The final capitated premiums paid to the MCOs are based on this risk-adjusted PMPM for each combination of service area and risk group.

Pharmacy rates are similarly calculated. Certain high-cost drugs are paid for outside of the capitation rate.

Rates for the other managed care programs — STAR+PLUS, STAR Kids and STAR Health — are also determined using the same methods, with certain exceptions:

- STAR+PLUS, STAR Kids and STAR Health MCOs do not receive a delivery supplemental payment for newborn deliveries. Provisions for these costs are included in the capitation rates.
- Due to low caseload among risk groups for STAR Kids clients younger than age 1 and receiving services through the Youth Empowerment Services waiver, capitation rates for these risk groups are calculated on a statewide basis.
- There is only one STAR Health MCO. This MCO is reimbursed using a single capitation rate that does not vary by age, gender or area.
- For the STAR Health MCO, there is a special allowance for the additional administrative services in the program, including the Health Passport.

HHSC also includes provision for DPPs in the STAR, STAR+PLUS, STAR Kids and Dual Demonstration programs' capitation rates, which are used to increase funding for providers and facilities, including hospitals and nursing facilities (see Chapter 4, page 99).

Children's Medicaid Dental Services Rates

Children's Medicaid Dental Services capitation rates are based on claims from the covered population in the base period. The base cost is totaled, and it trends to the time period for which the applicable rates are calculated. A reasonable provision for administrative expenses, taxes and risk margin is added to the claims component to project the total cost for the rating period. These projected total costs are used to set statewide rates that vary by age group.

CHIP Rates

The rate-setting process for CHIP is like the process used for the STAR MCOs. CHIP MCO rates, including pharmacy costs, are derived from MCO historical claims experience, also called encounter data, for a base period of time.

From this, the base cost data are totaled, and trends are calculated for the prospective time period during which the rates will apply. The cost data are also adjusted for MCO expenses, such as reinsurance, capitated contract payments and changes in plan benefits. A reasonable provision for administrative expenses, taxes and risk margin is also added to the claims component to project the total cost for the rating period. A risk margin is included to recognize financial risk.

The removal of newborn delivery expenses from the total cost rate results in an adjusted capitation rate for each service area. A separate lump-sum payment, called the delivery supplemental payment, is computed for expenses related to each newborn delivery. While the delivery supplemental payment can vary by service area for the STAR MCOs, all CHIP MCOs receive the same lump-sum payment of \$3,100 for each birth.

The resulting underlying base rates vary by service area and age group. A final adjustment is made to reflect the acuity of the population enrolled in each MCO. The purpose of the acuity risk adjustment is to recognize the anticipated cost differential among multiple MCOs in a service area due to the variable health status of their respective memberships. The final capitated premium paid to the MCOs is based on this acuity risk-adjusted premium and covers all non-maternity medical services.

CHIP dental benefits are reimbursed through a separate set of capitation rates, and dental maintenance organizations (DMOs) manage this benefit. The rate-setting process for these services is similar to the Children's Medicaid Dental Services program.

CHIP Perinatal Rates

Capitation rates for CHIP Perinatal (CHIP-P) are derived using a methodology similar to that described for CHIP. CHIP-P covers the unborn children of pregnant women who are uninsured and do not qualify for Medicaid. The more focused scope of benefits and eligibility for CHIP-P clients and the absence of an acuity adjustment produce some differences in the methodology.

MCO historical claims are totaled, and trends are calculated to project forward to the time period for which rates are to apply. The cost data is adjusted for MCO expenses, changes in plan benefits and other miscellaneous costs. Final rates vary by risk group and service area. However, due to low caseload among risk groups with income over 198 percent and up to and including 202 percent of the FPL, capitation rates for these risk groups are calculated on a statewide basis.

CHIP Cost-Sharing

Most families in CHIP pay an annual enrollment fee to cover all children in the family. All CHIP families pay copayments for doctor visits, prescription drugs, inpatient hospital care and non-emergent care provided in an emergency room setting. CHIP annual enrollment fees and copayments vary based on family income. The total amount a family can be required to contribute out-of-pocket toward the cost of health care services will not exceed five percent of family income.

CHIP cost-sharing rates are published online annually (hhs.texas.gov/services/health/medicaid-chip/programs-services/children-families/childrens-medicaid-chip).



How HHSC Pays for Prescription Drugs

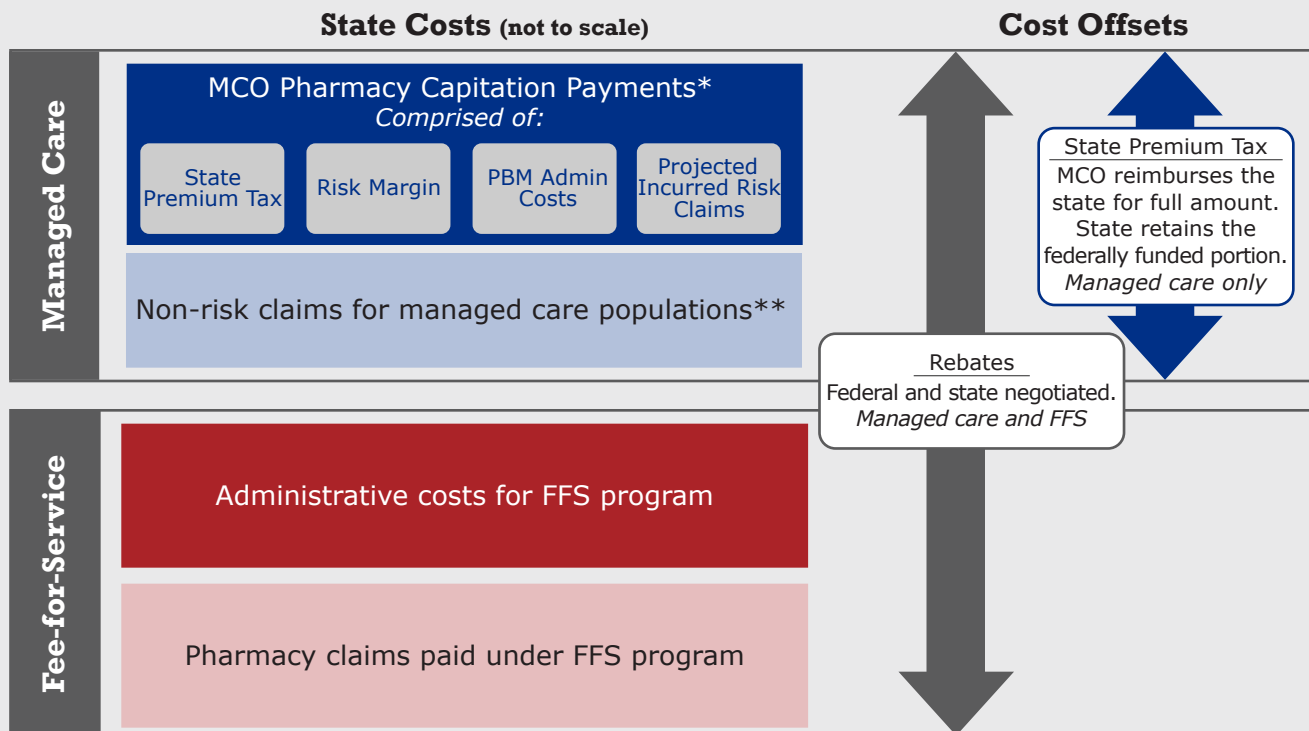
In recent years, prescription drugs have become increasingly associated with rising costs. For Medicaid and CHIP, MCOs administer most of the prescription drug benefit. A small portion of the benefit is administered by HHSC on a fee-for-service (FFS) basis.

In managed care, pharmacies are paid through pharmacy benefit managers (PBMs) contracted with MCOs (see Chapter 3, page 64). PBMs pay ingredient costs and dispensing fees to pharmacies. MCO pharmacy capitation rates are developed to cover pharmacy claims and administrative costs and to include a risk margin.

As part of the capitation rate, MCOs must pay the state premium tax through which HHSC receives federal matching dollars. MCO capitation rates do not include the costs of certain drugs for which HHSC makes non-risk payments to the MCO. Non-risk payments are limited to the actual amounts MCOs pay to providers for the ingredient cost of these drugs up to the FFS reimbursement amount.

Pharmacies are paid directly in FFS. Rates for prescription drugs include fees to cover the cost of ingredients — which are determined using the National Average Drug Acquisition Cost (NADAC) or the wholesale acquisition cost, and a dispensing fee.

The primary drivers of cost are rising drug prices and client utilization. HHSC offsets some of these costs through the state premium tax in managed care and through federal and state rebate programs. HHSC also regulates how MCOs administer the prescription drug benefit to manage costs and limit overutilization (see Chapter 3, page 63).



*The ACA Health Insurance Providers Fee was repealed as of Jan. 1, 2021.

**Certain drugs are not included in the capitation rate and are paid on a cost-settlement basis with the MCO.

Fee-for-Service Rates

Even though most Medicaid clients are under managed care, three percent of clients continue to get services paid through FFS. HHSC is responsible for establishing FFS reimbursement methodologies. Changes may be authorized by rule or approval from CMS. FFS rates are paid directly to providers, physicians, other medical practitioners, pharmacists and hospitals.

HHSC consults with stakeholders and advisory committees when considering changes to FFS reimbursement rates. All proposed rates are subject to a public hearing, and all proposed reimbursement methodology rule changes are subject to a 30-day public comment period, as part of the approval process. Changes in FFS reimbursement rates will often impact MCO rates. Many contracts between MCOs and providers incorporate payment rates based on a percentage of the FFS rate for the same service. HHSC does not require MCOs to use the FFS rates, and some MCOs use alternate reimbursement models. Rates for services delivered by physicians and other practitioners are uniform statewide and are either access-based fees or resource-based fees.

HHSC also establishes reimbursement rates for services such as laboratory and x-ray services, radiation therapy, physical and occupational therapy, dental services, and maternity clinic services. Reimbursement rates for most services are evaluated biennially.

For physician-administered drugs and biologicals, physicians are reimbursed at the lesser of their billed charges and the reimbursement rate, which is an estimate of the provider's acquisition cost for the specific drug or biological. Rates for physician-administered drugs and biologicals are reviewed semi-annually.

Fee-for-Service Rates for Inpatient and Outpatient Hospital Care

General acute care hospital reimbursement rates for FFS clients are set using a prospective payment system (PPS) based on the All Patient Refined Diagnosis Related Groups patient classification system. Under this system, each patient is classified into a diagnosis related group (DRG) based on clinical information.

Hospitals are paid a pre-determined rate for each DRG admission. The rate is calculated using a standardized average cost of treating a Medicaid inpatient admission and a relative weight for each DRG. Outlier payments are made in addition to the base DRG payment for clients age 20 and younger whose treatments are exceptionally costly or who have long-length stays.

Outpatient hospital services provided to FFS clients are reimbursed at a portion of the hospital's reasonable cost, and reimbursements are dependent on the type of hospital and the patient volume.

HHSC has developed several supplemental payment programs to address the difference between rate and facility costs (see Supplemental Hospital and Provider Funding, page 99).

Reimbursement Rates by Hospital Type	
Rural Hospital	Rates are based on the standard dollar amount required to treat a Medicaid inpatient admission and are derived from base year costs.
Children's Hospital	Rates are established using a statewide standard dollar amount which is derived from base year costs. Add-ons are used with this base standard dollar amount to make payments for additional services like medical education, geographic location and safety-net designation.
Psychiatric Hospital	Rates are reimbursed on a PPS per diem — based on the federal base per diem — with facility-specific adjustments for wages, rural location and length of stay.
State-Owned Teaching Hospital	Rates paid are for the reasonable cost of providing care to Medicaid clients using the cost principles from the Tax Equity and Fiscal Responsibility Act of 1982.
Urban Hospital	Rates are established using a statewide standard dollar amount, which is derived from base year costs. Add-ons are used with this base standard dollar amount to make payments for additional services — such as medical education, geographic location and safety-net designation.

Fee-for-Service Pharmacy Rates

The Vendor Drug Program administers prescription drugs under the FFS model. Rates for prescription drugs include fees to cover the cost of ingredients, which are determined using the NADAC or the wholesale acquisition cost. After the ingredient cost is calculated, the dispensing fee is calculated and added to the total reimbursement amount. Costs will differ based on the type of pharmacy.

Pharmacies that provide free delivery services to FFS clients may be eligible for a delivery incentive per prescription. Another incentive may be added if the pharmacy dispenses a premium preferred generic. Reimbursements are reduced to a pharmacy's reported "Usual and Customary" or "Gross Amount Due" price if either is less than the total reimbursement.

Fee-for-Service Reimbursement Rates

Reimbursement rates for acute services — which encompass most covered services in Medicaid — are published in fee schedules available at <https://public.tmhp.com/FeeSchedules/Default.aspx>. Reimbursement methodologies that outline how HHSC sets those rates can be found in Chapter 355 of the Texas Administrative Code (TAC) and Attachment 4.19-B of the Texas State Plan.

Reimbursement rates for hospital services encompass both outpatient and inpatient facility services in Texas. Reimbursement methodologies that outline how HHSC sets those rates can be found in Chapter 355 of the TAC and Attachment 4.19-B of the Texas State Plan.

Reimbursement rates for LTSS programs are published in fee schedules available at <https://pfd.hhs.texas.gov/long-term-services-supports>. Reimbursement methodologies that outline how HHSC sets LTSS rates can be found in Chapter 355 of the TAC.

Rates for Other Care Facilities

Nursing Facilities

Nursing facilities are reimbursed for services provided to Medicaid residents through daily payment rates, which are uniform statewide by level of service. Enhanced rates are available for enhanced direct care compensation and staffing. The total daily payment rate for each level of service may be retroactively adjusted based on failure to meet specific staffing or spending requirements.

Rates are calculated based on costs submitted by providers on applicable Medicaid cost reports collected by HHSC. The rates are limited by appropriation level. Costs are categorized into five rate components:

1. Direct care staff.
2. Other resident care.
3. Dietary and nutritional services.
4. General and administrative.
5. A fixed capital asset use fee.

Each rate component is calculated separately based on HHSC formulas. Direct care and other resident care components vary according to residents' acuity. The total rate for each level of service is calculated by adding together the appropriate rate components. Nursing facility cost reports are subjected to either a desk review or on-site audit to determine whether reported costs are allowable. MCOs are currently required to reimburse nursing facilities, at a minimum, the same daily payment rate — including any enhancements — as would have been paid under FFS.

Intermediate Care Facilities for Individuals with an Intellectual Disability or Related Condition

Intermediate care facilities for individuals with an intellectual disability or related condition (ICFs/IID) are reimbursed for services delivered to Medicaid residents through daily payment rates, which are prospective and uniform statewide by facility size and level of need. The total daily payment rate may be retroactively adjusted if a provider fails to meet specific direct care spending requirements.

The modeled rates are updated, when funds are available, using the service providers' most recent audited cost reports. Enhanced rates are available for enhanced attendant compensation.

ICF/IID cost reports are subjected to a desk review or on-site audit to determine whether reported costs are allowable. ICF/IID rates are recalculated biennially.

Federally Qualified Health Centers and Rural Health Clinics

Federally qualified health centers (FQHCs) include all organizations receiving grants under Section 330 of the Public Health Service Act. FQHCs qualify for enhanced reimbursement from Medicare and Medicaid, as well as other benefits. FQHCs serve underserved areas or populations, offer a sliding-scale fee, provide comprehensive services, have an ongoing quality assurance program, and have a governing board of directors.

A rural health clinic (RHC) is a clinic located in a rural area, designated by the U.S. Health Resources and Services Administration as a shortage area. Medicare has several requirements a clinic must meet to qualify as an RHC, including that it must be located in a non-urbanized area that is medically underserved, as defined by the U.S. Census Bureau.

To participate in the Texas Medicaid program, FQHCs and RHCs must:

- Comply with all federal, state and local laws and regulations applicable to the services provided.
- Sign a written provider agreement with HHSC, and then comply with the terms of the agreement and all requirements of the Texas Medicaid program.
- Bill for covered services in the manner and format prescribed by HHSC.

Covered services are limited to either services as described in the Social Security Act or other ambulatory services covered by the Texas Medicaid program when provided by other enrolled providers.

FQHCs and RHCs are reimbursed 100 percent of the average reasonable and allowable costs for the clinic in the base year of 2000.

Texas Medicaid reimburses FQHCs through a PPS or an alternative prospective payment system (APPS). PPS rates are inflated annually using the Medicare Economic Index (MEI) for primary care, while APPS rates are inflated annually using 100.5 percent of the MEI. However, if increases in an FQHC's costs are greater than the inflation amount under PPS and APPS, the provider can request an adjustment to their rate. If an FQHC chooses the APPS method, rates may be prospectively reduced to better reflect the reasonable costs or the PPS rates.

RHCs are reimbursed through a PPS methodology. The intent of the state is to ensure that each RHC is reimbursed either at 100 percent of its reasonable costs or the Medicare maximum payment per visit — also called the federal ceiling — as applicable. PPS rates are adjusted annually using the MEI for primary care. If the increases in an RHC's costs are greater than the inflation amount in either system, the provider can request an adjustment to their rate.

Renal Dialysis Facilities

Renal dialysis facilities provide services to patients with end-stage renal dialysis or acute kidney injury. These facilities must be certified by Medicare to enroll in Texas Medicaid. Renal dialysis facilities are reimbursed according to facility-specific composite rates. Composite rates follow the CMS-specified calculations (prior to CMS adoption of a different reimbursement method in January 2011). Composite rates are based on available funds and are subject to legislative appropriations.



Supplemental Hospital and Provider Funding

Historically, rates paid to hospitals for services have been below the average costs facilities incur to provide Medicaid-covered services. In addition, the expansion of managed care has greatly impacted the way hospitals are funded in Texas.

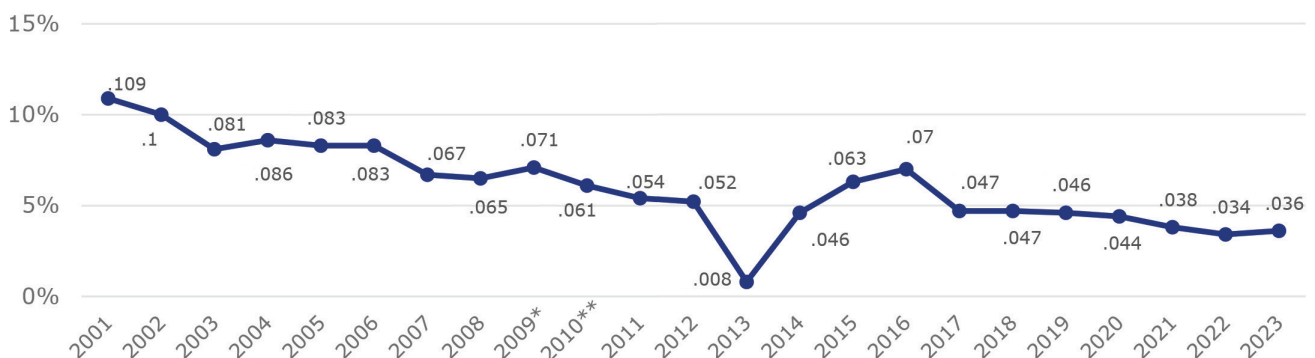
To comply with federal regulations and to preserve federal hospital funding while expanding managed care, HHSC submitted a five-year Section 1115 Transformation Waiver in 2011. On Jan. 15, 2021, CMS approved the most recent extension of the waiver through September 2030 (see Chapter 5, page 112).

Through the 1115 Transformation Waiver, also known as THTQIP, HHSC administers supplemental funding for hospitals and other facilities to help cover the cost of uncompensated care and incentivize improvements to service delivery. These programs also include quality-based payment programs (see Quality-Based Payment Programs, page 104).

Disproportionate Share Hospital Funding

DSH funding is special funding for hospitals that serve a disproportionately large number of Medicaid and low-income patients. DSH funds are not tied to specific services for Medicaid-eligible patients.

DSH Funds as a Percentage of the Total Medicaid Budget, FFYs 2001–2023



*2009 includes \$23.5 million in American Recovery and Reinvestment Act (ARRA) federal stimulus funds.

**2010 includes \$47.6 million in ARRA federal stimulus funds.

There are no federal or state restrictions on how disproportionate share hospitals can use their funds. Hospitals may use DSH payments to cover the uncompensated costs of care for low-income patients, including Medicaid patients. DSH payments have proven an important source of revenue by helping hospitals expand health care services to the uninsured, defraying the cost of treating low-income patients, and recruiting physicians and other health care professionals.

To qualify for DSH funds, hospitals must meet all program eligibility criteria, conditions of participation, and at least one of the following qualification criteria:

- A disproportionate total number of inpatient days are attributed to Medicaid patients.
- A disproportionate percentage of all inpatient days are attributed to Medicaid patients.
- A disproportionate percentage of all inpatient days are attributed to low-income patients.

All state-owned hospitals in Texas are deemed disproportionate share hospitals, provided they meet federal and state eligibility criteria and conditions of participation.

As in other matching Medicaid programs, the federal government and non-federal sources each pay a share of total DSH program costs. Under the ACA, federal DSH allocations were set to decrease in size in anticipation of the reduction of the uninsured population.

The Coronavirus Aid, Relief and Economic Security Act delayed the FFY 2020 \$4 billion DSH allotment reduction until Dec. 1, 2020. The reduction was further delayed through Dec. 31, 2024, by the Consolidated Appropriations Act 2024. Currently, the statutory reductions in the federal share of DSH payments for all states from FFY 2020 through FFY 2025 are:

FFY	2020	2021	2022	2023	2024	2025
Reduction for all states	\$4.0 billion	\$8.0 billion	\$8.0 billion	\$8.0 billion	\$8.0 billion	\$8.0 billion

DSH payment information reports are published annually online at:
<https://pfd.hhs.texas.gov/hospitals-clinic/hospital-services/disproportionate-share-hospitals>.

Uncompensated Care Funding

Created under the 1115 Transformation Waiver, the Uncompensated Care (UC) Pool includes \$38.5 billion in allocated funds over the period of the waiver. To receive UC payments, providers must meet certain eligibility requirements. Payments are cost-based and help offset the costs of uncompensated care provided by hospitals and other providers. Though previously defined as unreimbursed costs for Medicaid and uninsured patients incurred by hospitals, uncompensated care costs are currently defined as unreimbursed charity care costs. UC payments will be based on each provider's UC costs, as reported on a UC application.

The Public Health Provider-Charity Care Program

Authorized under the current 1115 Transformation Waiver, the Public Health Provider-Charity Care Program (PHP-CCP) is designed to allow qualified providers to receive reimbursement for the cost of delivering uncompensated health care services, including behavioral health services, immunizations and other preventative services.

Eligible providers include Community Mental Health Clinics, Community Centers, Local Behavioral Health Authorities, Local Mental Health Authorities, Local Health Departments and Public Health Districts.

The PHP-CCP began operating on Oct. 1, 2021. The first year of the program consisted of uncompensated care. In its second year, the program has transferred to charity care only. Total funding is set not to exceed \$500 million over these first two years of the program. This pool was resized in 2024 to approximately \$498 million per year based on charity care costs incurred by eligible providers.

Graduate Medical Education

Hospitals that operate medical residency training programs incur higher expenses than hospitals without training programs. Graduate medical education (GME) supplemental payments cover the costs of program administrative staff, allocated facility overhead, and salaries and fringe benefits for residents and teaching physicians.

Historically, the Medicaid share of these additional costs have been covered by GME payments to state-owned teaching hospitals. These payments are contingent upon receipt of intergovernmental transfers (IGT) of funds from state-owned teaching hospitals for the non-federal share of Medicaid GME payments.

Effective Oct. 1, 2018, GME supplemental payments have been made to non-state public hospitals. These payments are contingent upon receipt of IGT of funds from the non-state public teaching hospitals for the non-federal share of Medicaid GME payments.

Effective April 1, 2019, GME supplemental payments have been made to private hospitals. These payments are contingent upon receipt of IGT of funds from a local government entity for the non-federal share of Medicaid GME payments. The private GME State Plan Amendment (SPA) was approved by CMS for private hospitals on Aug. 15, 2023, retroactively to April 1, 2019.

Hospital Augmented Reimbursement Program

The Hospital Augmented Reimbursement Program (HARP) is a statewide supplemental program providing Medicaid payments to hospitals for inpatient and outpatient services that serve Texas Medicaid FFS patients. The program serves as a financial transition for providers historically participating in the DSRIP program. HARP provides additional funding to hospitals to assist in offsetting the costs hospitals incur while providing Medicaid services.

Eligible participants include non-state government-owned and -operated public hospitals and private hospitals. The public HARP SPA was approved for non-state government-owned and -operated hospitals on Aug. 31, 2022. The private HARP SPA was approved for private hospitals on Aug. 15, 2023. Total annual funding for HARP in FFY 2024 was approximately \$1.4 billion.

Network Access Improvement Program

The Network Access Improvement Program (NAIP) is a “pass-through” payment program to help fund public hospitals and health-related institutions and increase access to primary and specialty care for managed care members. Under federal law, NAIP will be discontinued. Pass-through payments to physicians have been phased out as of July 1, 2022. Pass-through payments must be phased out by July 1, 2027.

Directed Payment Programs

DPPs are supplemental payment programs for providers and are designed to advance at least one of the objectives of Texas’ Managed Care Quality Strategy. Enrolled providers submit qualitative and quantitative data on quality measures that are used to evaluate the programs. Previously, the following DPPs were developed in coordination with the 1115 Transformation Waiver.

The Uniform Hospital Reform Increase Program

The Uniform Hospital Reform Increase Program (UHRIP), a DPP designed to reduce hospitals’ uncompensated care costs through enhanced payments to hospitals for medically necessary, covered services provided to Medicaid managed care members. The Comprehensive Hospital Increase Reimbursement Program (CHIRP) replaced UHRIP as of Sept. 1, 2021.

The Quality Incentive Payment Program

QIPP, a DPP focused on improving quality of care in nursing facilities seeks to improve quality and innovation in nursing facility services for residents enrolled in STAR+PLUS. Both public and private nursing facilities can participate in the program, and more than 1,000 of the state's almost 1,200 nursing facilities are enrolled in QIPP for SFY 2025 with a proposed budget of \$1.75 billion. QIPP is now in its eighth year of operation. HHSC revised the program for FY 2025 to include only pay-for-performance quality measures developed and monitored by CMS across four program components: Components 1, 3 and 4 include ten clinical Minimum Data Set (MDS) quality measures; Component 2 incentivizes improvement in staff-to-patient ratios. On Aug. 14, 2024, CMS approved QIPP as a value-based DPP for a three-year period.

The DSRIP program ended in September 2021 and was replaced by the following four DPPs to continue health care transformation initiatives. All currently operating DPPs require annual approval from CMS.

The Comprehensive Hospital Increase Reimbursement Program

CHIRP is a DPP for hospitals providing health care services to STAR and STAR+PLUS members. Eligible hospitals include children's hospitals, rural hospitals, mental health hospitals, state-owned hospitals and urban hospitals. In FY 2025, CHIRP has an estimated program size of \$6.5 billion. CHIRP funds are paid through three components: 1) the UHRIP component which enhances payments to hospitals, 2) the Average Commercial Incentive Award component — an optional incentive award component, and 3) the Alternate Participating Hospital Reimbursement for Improving Quality Award component — an optional pay-for-performance component beginning in FY 2025. Quality measures address hospital safety, maternal care, pediatric care, rural health care and care transitions. CMS approved CHIRP on Sept. 13, 2024 for SFY 2025.

The Directed Payment Program for Behavioral Health Services

The Directed Payment Program for Behavioral Health Services (DPP BHS) is a DPP for community mental health centers and local behavioral health authorities. It seeks to promote and improve access to behavioral health services, care coordination, and successful care transitions. It also incentivizes the Certified Community Behavioral Health Clinic model of care. In FY 2025, DPP BHS has an estimated program size of \$157 million. CMS approved DPP BHS on July 29, 2024 for SFY 2025.

The Rural Access to Primary and Preventive Services Program

The Rural Access to Primary and Preventive Services (RAPPS) program is a DPP for RHCs that provides primary care and long-term care services to STAR, STAR+PLUS and STAR Kids members. Eligible providers include hospital-based RHCs, which include non-state government-owned and private RHCs, and free-standing RHCs. In FY 2025, RAPPS has an estimated program size of \$22 million. CMS approved RAPPS on Aug. 15, 2024 for SFY 2025.

Texas Incentives for Physicians and Professional Services

Texas Incentives for Physicians and Professional Services (TIPPS) is a DPP for certain physician groups providing health care services to children and adults enrolled in STAR, STAR+PLUS and STAR Kids. Eligible physician groups include health-related institutions, indirect medical education physician groups affiliated with hospitals and other physician groups. In FY 2025, TIPPS has an estimated program size of \$787 million. CMS approved TIPPS on July 31, 2024 for SFY 2025.



Quality-Based Payment Programs

Pay-for-Quality and Managed Care Payment Reform

To reward the use of evidence-based practices and promote health care coordination and efficacy among MCOs, HHSC implements medical pay-for-quality (P4Q) programs for STAR, STAR+PLUS, STAR Kids, CHIP and a dental P4Q program. The medical and dental P4Q programs were suspended for 2020 and 2021 due to the COVID-19 Public Health Emergency. MCOs were not subject to any recoupments or distributions based on calendar year 2020 and 2021 performance. The medical and dental P4Q programs resumed in 2022.

The medical P4Q program evaluates MCOs on a set of quality measures — with a focus on prevention and chronic disease management, including behavioral, maternal and infant health. MCOs can earn or lose money based on their level of improvement or decline from the prior year and their performance relative to benchmarks set by HHSC. For the medical P4Q program, three percent of MCOs' capitation is at-risk. In the dental P4Q program, 1.5 percent of DMOs' capitation is at-risk.

A strong medical P4Q program incentivizes MCOs to pursue quality-based alternative payment models (APMs) with providers to help them achieve higher performance on P4Q measures.

To further accelerate this effort, HHSC also has implemented contractual requirements to advance the use of APMs between MCOs and providers.

APMs are health care payment models that link a percentage of the provider's overall payment to a measure of either quality or quality and cost. MCOs and DMOs must have a certain percentage of their overall provider payments associated with an APM. For a certain percentage of these payments, the provider must have some degree of risk.

In 2023, HHSC designed the next phase (2024–2027) of its APM strategy, by launching its APM Performance Framework (APM-PF). In the APM-PF, MCOs' performance in progressing the adoption of APMs in their provider contracts will be measured through a point system across five domains: (1) achievement levels, (2) quality performance, (3) priorities, (4) pilots & initiatives and (5) provider support. MCOs submit an annual report to HHSC documenting their performance in the five domains.

Hospital Quality-Based Payment Program

HHSC operates a quality monitoring program that adjusts payments to hospitals according to their performance with FFS and managed care patients — with the goal of improving quality and lowering costs.

Hospitals and MCOs are financially accountable for certain potentially preventable events, including potentially preventable complications (PPCs) and potentially preventable readmissions (PPRs).

- A PPC is a harmful event or negative outcome, such as an infection or surgical complication, which occurs after the patient's admission and may have resulted from the treatment or lack of treatment provided, rather than from a natural progression of their condition.
- A PPR is a return hospitalization that may have resulted from deficiencies in care or treatment provided during a previous hospital stay or from inadequate post-hospital discharge follow-up.

If hospitals fail to meet certain PPC or PPR thresholds, then adjustments in the form of reductions are made to FFS hospital inpatient claim payments. Similar adjustments are made to MCO capitation rates based on an assumed similar reduction in MCO payments for hospital inpatient claims.



Fund Recovery

The Office of Inspector General (OIG) performs the functions listed below (see Chapter 3, page 81).

Third-Party Liability

Under federal law, Medicaid and CHIP are the payer of last resort. This means other sources of health insurance that a client may have — such as commercial health insurance or medical coverage under car insurance — may be responsible to pay for Medicaid and CHIP clients. This requirement, called third-party liability (TPL), ensures the responsible party other than Medicaid or CHIP pays for care.

To implement TPL requirements, federal and state rules require states to take reasonable measures to identify potentially liable third parties and process claims accordingly. As a condition of eligibility, Medicaid and CHIP clients also must cooperate with state efforts to pursue other sources of coverage.

States rely on two main sources of information to determine whether a liable third party exists for a claim: clients, and data matches with other insurers or data clearinghouses. HHSC works to reduce health care expenditures by shifting claims expense to third-party payers, utilizing either cost avoidance or cost recovery:

- Cost avoidance occurs when the state is aware that a client has potential third-party coverage when a claim is filed. The state rejects the claim and instructs the provider to submit it to the potential primary payer. After the potential primary payer has processed the claim, the provider may resubmit a claim for any portion of the claim not covered by the primary payer.
- Cost recovery, also known as “pay and chase,” occurs when a third party is identified after a claim is filed or paid. HHSC must promptly seek reimbursement from the liable third party.

MCOs and DMOs are subject to the state and federal requirements related to cost avoidance and cost recovery. Each MCO and DMO has the obligation to cost avoid claims and cost recover when there is a liable third party.

Medicaid Estate Recovery Program

Texas implements the Medicaid Estate Recovery Program (MERP) in compliance with federal Medicaid laws. Under MERP, the state may recover the cost of Medicaid services provided by filing claims against the estate of certain deceased Medicaid clients who were age 55 and older and had received LTSS benefits on or after March 1, 2005.

Claims may include certain Medicaid long-term care services and related costs of hospital and prescription drug services for clients in nursing facilities, ICFs/IID, Community Attendant Services or waiver programs.

There are certain exemptions from recovery, as required by federal and state law. When no exemptions apply, the heirs may request a hardship waiver if certain conditions are met. When no exemptions or hardship conditions exist, the state files a claim against the descendant's assets that are subject to probate. The estate representative is responsible for paying the lesser of the MERP claim amount or the estate value — after all higher priority estate debts have been paid. This is paid through the estate, not the resources of any heirs or family members.

The claims filing component of the MERP program has been contracted to a private company through a competitive procurement process.

Chapter 5

What is the governing framework for Medicaid and CHIP?



At-a-Glance

Medicaid and the Children's Health Insurance Program (CHIP) change in response to federal and state requirements.

Key Federal Concepts

Fundamental Requirements:

The basic principles for Medicaid programs established by the Social Security Act (SSA).

Centers for Medicare & Medicaid Services (CMS):

The agency within the U.S. Department of Health and Human Services that oversees the Medicaid program.

Single State Agency:

Federal regulations require each state to designate a single state agency responsible for that state's Medicaid program.

Medicaid State Plan:

Submitted by states individually, this is a dynamic document that serves as the contract between each state and CMS.

Waivers:

States apply to CMS for waivers to test new ways to deliver and pay for services.



Requirements Directed by the Texas Legislature

Requirements Established by the SSA

Medicaid operates according to the following fundamental requirements:

1

Statewide Availability

All Medicaid services must be available to all eligible clients statewide, without local restrictions.

2

Sufficient Coverage

States must cover each service in amount, duration and scope that is "reasonably sufficient."

3

Service Comparability

All clients must be able to access the same amount, duration and scope of services, except where federal law specifically requires a broader range or allows a reduced package of services.

4

Freedom of Choice

Clients must be allowed to go to any Medicaid health care provider who meets program standards.

Key Federal Mandate Categories

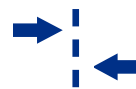
Populations



Services



Limits



See Appendix A, page 149 for a list of highlights from the Texas Legislature and relevant federal changes.

How Texas Administers Medicaid and CHIP

Texas maintains a state plan for both Medicaid and the Children’s Health Insurance Program (CHIP) that serves as the contract between the State and Centers for Medicare & Medicaid Services (CMS). These state plans describe the nature and scope of the Medicaid program and CHIP — including administration, client eligibility, benefits and provider reimbursement.

The state plans give the Health and Human Services Commission (HHSC) sole authority to administer Medicaid and CHIP in Texas. HHSC’s responsibilities include:

- Serving as the primary point of contact with the federal government.
- Coordinating initiatives to maximize federal funding and administering the Medical Care Advisory Committee (mandated by federal Medicaid law to advise the Medicaid agency about health and medical care services).
- Establishing policy direction for Medicaid and CHIP, administering the state plans, and coordinating with other HHSC departments and state agencies to carry out operations.
- Determining program eligibility for Medicaid and CHIP.
- Establishing Medicaid policies (including medical and dental benefits policies), rules, reimbursement rates and oversight of Medicaid program operations, including managed care organization (MCO) contract compliance.

CHIP State Plan

States can design their CHIP plan using one of three frameworks established by federal law. States may include CHIP within their Medicaid program, create a separate CHIP plan or create a combination of these two options. Texas operates CHIP separately.

Medicaid Waivers

Federal law allows states to apply to CMS for permission to depart from certain Medicaid requirements. Federal law allows three main types of waivers: Research and Demonstration 1115 waivers, Freedom of Choice 1915(b) waivers and Home and Community-Based Services (HCBS) 1915(c) waivers. These waivers allow states to develop creative alternatives to the traditional Medicaid program.

States may seek waivers to:

- Provide services above and beyond state plan services to selected populations.
- Expand services in certain geographical areas.
- Limit free choice of providers.
- Implement innovative new service delivery and management models.

States must provide regular reports and evaluations showing cost-effectiveness, cost neutrality or budget neutrality (based on the type of waiver), and that requirements for the waiver are being met.

Research and Demonstration 1115 Waivers

Section 1115 waivers give states the flexibility to test new ideas for operating their Medicaid programs — including implementing statewide health system reforms, providing services not typically covered by Medicaid, or allowing innovative service delivery systems to improve care, increase efficiencies and reduce costs.

The Texas Healthcare Transformation and Quality Improvement Program

The Texas Healthcare Transformation and Quality Improvement Program (THTQIP), also known as the 1115 Transformation Waiver, allowed Texas to expand managed care — including pharmacy and dental services — while preserving federal hospital funding. STAR, STAR+PLUS, STAR Kids and dental managed care services are covered under this waiver.

CMS approved an extension of the waiver through September 2030.

The waiver, first approved in 2011, originally included two funding pools: the Uncompensated Care (UC) and Delivery System Reform Incentive Payment (DSRIP) pools. The UC pool is available to help cover the actual uncompensated care costs incurred by hospitals and other eligible providers for serving uninsured people in the state. The DSRIP pool provided incentive payments to providers to support their efforts to enhance access to care, improve quality and reform the health care delivery system. DSRIP ended in September 2021 and funding expired in January 2023.

Managed care directed payment programs (DPP) operate in conjunction with the 1115 waiver to fund targeted rate increases for certain types of providers through MCOs, and to advance at least one goal and objective of the Texas Managed Care Quality Strategy.

The DPPs include the:

- Comprehensive Hospital Increase Reimbursement Program.
- Directed Payment Program for Behavioral Health Services.
- Quality Incentive Payment Program.
- Rural Access to Primary and Preventive Services.
- Texas Incentives for Physicians and Professional Services.

For more information on DPPs, see Chapter 4, page 102.

Healthy Texas Women

In January 2020, CMS approved a 1115 Demonstration Waiver for the Healthy Texas Women (HTW) program. HHSC began receiving federal funds for HTW in February 2020, and the demonstration was approved through December 2024. The demonstration allows HTW to receive federal matching funds and requires HTW to comply with certain Medicaid requirements — including eligibility application, verification and demonstration regulations.

In December 2020, HHSC submitted an amendment to the HTW demonstration to receive federal funds for HTW Plus. At the time the amendment was submitted, HHSC requested an effective date of April 1, 2021. As of December 2024, the amendment is still pending with CMS.

In March 2024, HHSC submitted a request to CMS to extend the HTW demonstration for a period of five years from Jan. 1, 2025 through Dec. 31, 2029. As of December 2024, this request is still pending.

The goals and objectives of the HTW demonstration are to:

- Increase access to women’s health and family planning services, avert unintended pregnancies, and positively impact the outcome of future pregnancies and the health and well-being of women and their families.
- Increase access to preventive health care — including screening and treatment for hypertension, diabetes and high cholesterol — to improve maternal health and reduce maternal mortality.
- Increase access to women’s breast and cervical cancer services to promote early cancer detection.
- Implement the State policy to favor childbirth and family planning services that do not include elective abortions or the promotion of elective abortions within the continuum of care or services, and to avoid the direct or indirect use of State funds to promote or support elective abortions.
- Reduce the overall cost of publicly funded health care by providing low-income Texans with access to safe, effective services consistent with the goals above.

Freedom of Choice 1915(b) Waivers

Section 1915(b) waivers provide states the flexibility to modify their service delivery systems and authorize them to implement a managed care model and limit provider networks. How states use 1915(b) authority depends on the end goals of the program.

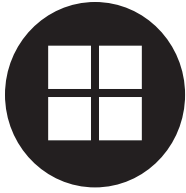
For example, in Texas, the selective contracting authority granted under 1915(b)(4) waivers is used for programs and services such as Community First Choice (CFC) Medicaid State Plan services. This allows the State to limit the provider base for CFC clients in an Intellectual or Developmental Disabilities waiver to their waiver providers. Texas also uses a 1915(b)(4) waiver for aspects of the Medical Transportation Program (MTP). Most nonemergency medical transportation services are provided through managed care. The fee-for-service (FFS) delivery model for MTP continues to operate under the authority of the State Plan and a 1915(b)(4) waiver, allowing HHSC to contract directly with the transportation providers to provide demand response transportation services for the FFS population.

Home and Community-based Services 1915(c) Waivers

Section 1915(c) waivers allow states to provide home and community-based services as an alternative for people who are eligible for care in an institution (nursing facility, intermediate care facility for individuals with an intellectual disability or related condition, or hospital).

States may use these waivers to serve people age 65 and older and those with physical disabilities, an intellectual or other developmental disability, mental health conditions, or more specialized populations, such as people with traumatic brain injuries or sensory impairment.

Texas 1915(c) waivers include the Medically Dependent Children Program, Home and Community-based Services, Texas Home Living, Community Living Assistance and Support Services, Deaf Blind with Multiple Disabilities, and Youth Empowerment Services. Texas also has the THTQIP, a section 1115 waiver, that includes the STAR+PLUS Home and Community Based Services (STAR+PLUS HCBS) program that operates similar to a 1915(c) waiver (see Chapter 5, page 112).



Fundamental Requirements

Statewide Availability

Also referred to as “statewideness,” this federal law requires state Medicaid programs to offer the same benefits to everyone throughout the state. Exceptions to this requirement are possible through Medicaid waiver programs and special contracting options. Since the move toward managed care in Texas, statewide availability is primarily assured through federal and state regulations that establish minimum provider network standards, and by state oversight of MCO compliance with these regulations. Texas has implemented access standards, including time and distance standards, and appointment availability standards (see Chapter 3, page 56).

Sufficient Coverage

Federal law specifies a set of mandatory Medicaid program benefits and a set of optional benefits that states may choose to provide. Federal law allows states to determine what constitutes reasonably sufficient coverage in terms of the amount, duration and scope of services. Because each state defines these parameters, state Medicaid plans vary in what and how much they cover. Below are two examples of limits set in Texas for Medicaid services:

- A 30-day limit on inpatient hospital stays per spell of illness for adults served in FFS or in the STAR+PLUS managed care program. Exceptions include:
 - More than one 30-day hospital visit can be paid for if the stays are separated by 60 or more consecutive days.
 - Clients receiving a pre-approved, medically necessary transplant are allowed an additional 30 days of inpatient care, beginning on the date of the transplant.
 - The limit does not apply to STAR+PLUS members admitted to an inpatient hospital due to a primary diagnosis of a severe and persistent mental illness.
 - The limit does not apply to children age 20 or younger.
- Three prescriptions per month for outpatient drugs for adults in FFS. Exceptions to this limit include:
 - Family planning drugs are not limited.
 - The limit does not apply to children age 20 or younger, adults enrolled in managed care or clients in nursing facilities or enrolled in certain 1915(c) waiver programs.

Service Comparability

In general, service comparability requires states to provide the same level of services to all clients, except where federal law specifically requires a broader range of services or allows a reduced package of services. Additional steps have been taken to increase access to care for certain demographic groups, such as children and youth, as the result of the *Frew v. Young* litigation.

Coverage for Children

The federally required Early and Periodic Screening, Diagnosis and Treatment (EPSDT) program — known in Texas as Texas Health Steps (THSteps) — provides preventive health and comprehensive care services for Medicaid-enrolled children and youth age 20 and younger (see Chapter 2, page 36). Federal changes expanded the benefits of this program so that children and youth age 20 or younger are eligible for any medically necessary and appropriate health care service covered by Medicaid, regardless of the limitations of the state's Medicaid program.

Children's Health Care Case Law

In addition to the federal requirements specific to children and youth, court cases have played a significant role in the delivery of children's health care through the Texas Medicaid program.

- *Alberto N. v. Young*, a federal lawsuit settled in May 2005, requires HHSC to comply with Title XIX of the Social Security Act (SSA) (42 U.S.C. §1396 et seq.) by providing all medically necessary in-home Medicaid services to children age 20 and younger who are eligible for the EPSDT program.
- *Frew v. Young*, a class action lawsuit filed against Texas in 1993, alleged that the State neither adequately informed parents and guardians about nor adequately provided EPSDT services. In 1995, a consent decree was negotiated that imposed certain requirements on the State. In 2007, the State negotiated a set of corrective action orders with the plaintiffs to implement the consent decree and increase access to EPSDT services. Since 2007, Texas HHSC and Department of State Health Services have actively worked to meet the requirements of the corrective action orders. As a result, some portions of the consent decree and corrective action orders have since been dismissed.

Freedom of Choice

In general, a state must ensure that Medicaid clients are free to obtain services from any qualified provider. Exceptions are possible through Medicaid waivers and special contract options. THSteps clients have freedom of choice regarding a medical checkup provider, even if they are not the child's primary care provider.

To maintain freedom of choice under managed care, each service delivery area in the state has at least two health plans or MCOs from which eligible members can choose. The exception is STAR Health, which is administered statewide by a single MCO. Once members select their health plan, they also choose a primary care provider from the MCO's provider network.

Appendix A

Key Medicaid and CHIP legislation

Texas Medicaid and the Children’s Health Insurance Program (CHIP) change in response to legislative requirements at the state and federal levels. This appendix provides a brief legislative history of Medicaid and CHIP, including highlights of key federal and state legislation by topic.

Medicaid

Founding Legislation

In 1965, the Social Security Act (SSA) of 1935 was amended to add Title XIX, which created Medicaid as a state-administered health care program, jointly funded by the federal government. The SSA was further amended in 1967 to mandate inclusion of the Early and Periodic Screening, Diagnosis and Treatment program for children in Medicaid, and again in 1972 to allow states to cover inpatient psychiatric care for children and youth. In 1977, the Centers for Medicare & Medicaid Services (CMS) was created to oversee and manage Medicare, Medicaid and CHIP.

The SSA dictates the principles that state Medicaid programs must fulfill, including:

- **Section 1902(a)(1)** requires that state Medicaid programs be in effect “in all political subdivisions of the state.”
- **Section 1902(a)(10)** requires that state Medicaid programs provide services to people that are comparable in amount, duration and scope.
- **Section 1902(a)(23)** requires that state Medicaid programs ensure that clients have the freedom to choose any qualified provider to deliver a covered service.

Also included within the SSA are sections that allow states to waive various Medicaid requirements and implement different service delivery models, eligibility criteria and benefits:

- **Section 1115(a)** allows states to waive provisions of Medicaid law to test new concepts consistent with the goals of the Medicaid program. Systemwide changes are possible under this provision. CMS must approve waivers.
- **Section 1903(m)** allows state Medicaid programs to develop risk contracts with managed care organizations (MCOs) or comparable entities.
- **Section 1915(b)** allows states to waive freedom of choice when choosing providers. States may require that members enroll in MCOs or other programs. CMS must approve managed care waivers.

- **Section 1915(c)** allows states to waive various Medicaid requirements to establish alternative, home and community-based services for people who qualify to receive services in an intermediate care facility for individuals with an intellectual disability or related condition (ICF/IID), nursing facility, institution for mental disease or inpatient hospital. CMS must approve waivers.
- **Section 1915(i)** allows states to offer a variety of services under a state plan Home and Community-Based Services (HCBS) benefit. People must meet state-defined criteria based on need and typically get a combination of acute care medical services and long-term services in home and community-based settings.
- **Section 1929** allows states to provide a broad range of home and community-based care to people with functional disabilities as an optional state plan benefit. In all states but Texas, the option is only available to people age 65 and older. In Texas, people of any age may qualify to receive personal care services through Section 1929 if they meet the state's functional disability test and financial eligibility criteria.

Program Reforms

Federal and state laws also determine who is eligible for Medicaid and what Medicaid covers. Federal and state legislation have reformed and expanded the program over time — adjusting program eligibility criteria, as well as the type, amount, duration and scope of Medicaid services.

Federal Legislation

Historically, all Medicaid clients either had Supplemental Security Income (SSI) or welfare. Beginning in the 1980s, several federal laws expanded Medicaid coverage to populations ineligible for SSI or Temporary Assistance for Needy Families (TANF) and, eventually, delinked financial assistance from Medicaid eligibility. A few of these federal laws are highlighted below:

The Omnibus Budget Reconciliation Act of 1986 (OBRA of 1986)

- Required states to cover emergency care services, including labor and delivery, for undocumented immigrants and people who are homeless.
- Allowed states to cover infants and pregnant women under 100 percent of the federal poverty level (FPL), to create a phase-in for children age 4 and younger under 100 percent of the FPL, and to cover prenatal care while a Medicaid application is pending, along with guaranteed coverage for the full term of pregnancy and postpartum care. States may waive asset tests for this group.

The Omnibus Budget Reconciliation Act of 1987 (OBRA of 1987)

- Required states to extend coverage to children age 6 and younger born after Sept. 30, 1983, whose families meet TANF financial standards, even if the family does not qualify for TANF. States may choose whether to extend coverage through age 7.
- Allowed states to cover infants age 1 and younger and pregnant women under 185 percent of the FPL, with immediate coverage (no phase-in) for children age 4 and younger under 100 percent of the FPL.
- Mandated enhanced nursing facility standards, including the creation of the Preadmission Screening and Resident Review process — a requirement that all current and prospective nursing facility clients be screened for mental illness, intellectual disability or related conditions.

The Medicare Catastrophic Coverage Act of 1988

- Expanded Medicaid coverage for infants, young children and pregnant women.
- Provided phased-in coverage of infants through their first birthday and pregnant women under 100 percent of the FPL.
- Required more comprehensive coverage of hospital services for infants and expanded payments for hospital services for infants in all hospitals and for children age 5 and younger in disproportionate share hospitals.
- Amended eligibility criteria and services provided to dually eligible Medicare clients — including providing phased-in coverage of out-of-pocket costs (premiums, deductibles and coinsurance) for Qualified Medicare Beneficiaries under 100 percent of the FPL and establishing minimum standards for income and asset protection for spouses of Medicaid clients in nursing homes.
- Expanded Medicaid coverage for students with disabilities who meet eligibility requirements for special education described in the Individuals with Disabilities Education Act (IDEA).

The Omnibus Budget Reconciliation Act of 1989 (OBRA of 1989)

- Prohibited states from limiting the amount, duration, scope or availability of state plan services to children in Medicaid.

The Personal Responsibility and Work Opportunity Reconciliation Act (PRWORA) of 1996

- De-linked cash assistance and Medicaid eligibility. While Medicaid financial eligibility is determined using the FPL, the TANF monthly cap is based on a set dollar amount, and each state sets its own income eligibility guidelines for TANF cash assistance. Prior to PRWORA, children age 18 and younger and their related caretakers who qualified for TANF cash assistance were also automatically qualified for Medicaid. Under PRWORA, if households need both TANF cash assistance and Medicaid, they must apply separately for each program.
- Required TANF clients to participate in work activities within two years of entering the program and prohibited them from receiving federally funded TANF benefits for more than 60 months over a lifetime. This requirement is thought to be partly responsible for the drop in the Medicaid caseload in Texas in the mid-to-late 1990s.
- Allowed states to decide whether to continue providing Medicaid to most legal immigrants. Most immigrants entering the United States (U.S.) after Aug. 22, 1996, are subject to a five-year waiting period during which no federal Medicaid funds can be accessed for their care. The Balanced Budget Act (BBA) of 1997 restored SSI benefits for legal immigrants who arrived in the U.S. prior to Aug. 22, 1996, but limited the benefit until after the first seven years of a person's residence in the U.S.

Various laws have been passed to extend PRWORA beyond its initial expiration date in September 2002. **The Deficit Reduction Act (DRA) of 2005** reauthorized TANF through 2010, and continuing resolutions have extended the program since then.

The DRA reduced federal Medicaid and Medicare spending by \$39 billion from 2006–2010 through changes to prescription drug regulations and long-term services and supports (LTSS) eligibility rules, among others.

The Balanced Budget Act (BBA) of 1997:

A landmark piece of federal legislation that:

- Created CHIP and changed Medicaid and Medicare rules and regulations.
- Added several new eligibility options including:
 - Guaranteed Eligibility — allowed states to choose to guarantee Medicaid coverage for up to 12 months for all children, even if they no longer meet Medicaid income eligibility tests.
 - Medicaid Buy-in — allowed states to offer people with disabilities, and income below 250 percent of the FPL, an opportunity to buy in to the Medicaid program.

- Medicaid Buy-in for Children — allowed states to offer children age 18 and younger with disabilities an opportunity to buy in to the Medicaid program.
- Repealed the Boren Amendment from the Omnibus Budget Reconciliation Act of 1980 (OBRA of 1980), delinking Medicaid nursing home rates from federal and state quality-of-care standards and replacing them with rates that are “reasonable and adequate.”
- Allowed states to require most Medicaid-eligible people to enroll in managed care plans without a waiver, such as an 1115 or 1915(b) — but mandated all states with Medicaid managed care programs to ensure MCOs conduct Performance Improvement Projects (PIPs) (see Chapter 3, page 67).

The BBA was later amended by the **Balanced Budget Refinement Act (BBRA) of 1999**, which provided approximately \$17 billion in “BBRA relief” over five years. The BBRA also extended the phaseout of cost-based reimbursement for Federally Qualified Health Centers (FQHCs) and rural health clinics and changed Medicaid Disproportionate Share Hospital (DSH) payments and rules. It also prohibited states from using CHIP federal funds for DSH.

The Affordable Care Act (ACA) of 2010

Made several significant changes to the health care system in the U.S., including:

- Prohibited coverage denials based on a preexisting condition.
- Allowed children to remain on their parent’s health plan until age 26.
- Required people to have health insurance or pay a penalty. The **Tax Cut and Jobs Act of 2017** eliminated this penalty starting in 2019.
- Required states to establish health insurance marketplaces or refer people to the federally facilitated marketplace to assist people and small employers in accessing health insurance. Texas currently utilizes the federally facilitated marketplace.
- Mandated that states modify their Medicaid program and CHIP plan, including their state plans and financial eligibility criteria.
- Gave states the option to expand Medicaid eligibility up to 133 percent of the FPL for people age 64 and younger, starting with 100 percent federal matching funds for the first three calendar years. The federal matching rate decreased gradually each year, reaching 90 percent in 2020. Texas has not expanded Medicaid eligibility to this optional adult group.

In anticipation of the uninsured population decreasing following Medicaid expansions and the implementation of the state health insurance marketplaces, the ACA intended to decrease DSH allotments. Since 2010, various pieces of legislation have delayed the implementation of DSH funding cuts under the ACA and changed the amounts of the reductions.

The ACA also changed the federal drug rebate program created under OBRA of 1990 — increasing the minimum federal rebate percentages that drug manufacturers are required to pay to participate in the Medicaid program, while expanding the rebate program to cover claims paid by MCOs (see page 128).

Medicaid and CHIP Managed Care Final Rule, 2016

On April 25, 2016, CMS published the Medicaid and CHIP Managed Care Final Rule, which aligned key rules with those of other health insurance coverage programs, modernized how states purchase managed care for members, and strengthened the consumer experience and key consumer protections. Specific changes included:

- Provided flexibility for states to have value-based purchasing models, delivery system reform initiatives, or provider reimbursement requirements in the managed care contract.
- Strengthened existing quality improvement approaches with respect to managed care plans.
- Required states to implement a quality rating system for Medicaid and CHIP managed care plans and to report plan performance.
- Required states to provide notices to explain implications of enrollees' choices as well as all disenrollment opportunities.
- Required managed care plans to implement and maintain administrative and managerial procedures to prevent fraud, waste and abuse.
- Conditioned payment of federal financial participation on timely, accurate, and complete reporting of encounter data.

This legislation also applies to CHIP.

Consolidated Appropriations Act (CAA), 2023

Congress passed an omnibus appropriations bill and signed it into law in December 2022. The CAA made several changes related to the COVID-19 Public Health Emergency (PHE) and the following longstanding provisions to Medicaid and CHIP services:

- Required states to provide 12 months of continuous eligibility for children age 18 and younger in Medicaid and CHIP.
- Extended federal funding for CHIP.
- Established a permanent state option to provide 12 months postpartum coverage in Medicaid and CHIP.
- Instituted several Medicaid and CHIP improvements related to mental health and juvenile justice.

See also the Consolidated Appropriations Act (CAA) on page 131 for more details on changes related to the PHE.

State Legislation

Senate Bill (S.B.) 11, 85th Legislature, Regular Session, 2017

- Required the Department of Family and Protective Services (DFPS) to ensure that children taken into state conservatorship receive an initial medical exam within three business days if they are removed from their home as the result of:
 - Sexual abuse, physical abuse or an obvious physical injury.
 - A chronic medical condition, a medically complex condition or a diagnosed mental illness.
- Prohibited a physician or other health care provider from administering a vaccination as part of the required exam — except for an emergency tetanus vaccination — without parental consent and until DFPS has been named managing conservator.
- Required DFPS to ensure that any child who enters state conservatorship receives any necessary emergency medical care as soon as possible.

S.B. 1 (Article II, HHSC, Rider 42), 87th Legislature, Regular Session, 2021

- Required the Health and Human Services Commission (HHSC) to implement the Emergency Triage, Treat, and Transport Model to reimburse Medicaid-enrolled emergency ambulance services provided for:
 - Transporting Medicaid clients to alternative destinations, other than an emergency department, as approved by HHSC.
 - Facilitating appropriate treatment in place at the scene.
 - Facilitating appropriate treatment in place via telehealth.

Women's Health

House Bill (H.B.) 2466, 85th Legislature, Regular Session, 2017

- Required Medicaid and CHIP to cover a maternal depression screening for mothers of children who are eligible for Medicaid. This screening occurs during the covered well-child visit, which takes place before the child's first birthday.

S.B. 750, 86th Legislature, Regular Session, 2019

- Required HHSC to:
 - Apply for federal funds to implement a model of care to improve the quality and accessibility of care for pregnant women with opioid use disorder. CMS awarded HHSC funding under the Maternal Opioid Misuse Model.
 - Develop a postpartum depression treatment network.
 - Collaborate with MCOs to develop and implement cost-effective, evidence-based, and enhanced prenatal services for high-risk pregnant women covered under Medicaid.
 - Assess the feasibility of providing services in the Healthy Texas Women (HTW) program through managed care and implement policies to improve the quality of maternal health in managed care.
 - Develop an enhanced, cost-effective and limited postpartum care services package in HTW called HTW Plus.
 - Implement strategies to ensure continuity of care for women transitioning from Medicaid to the HTW program.
 - Develop or enhance statewide initiatives to improve the quality of maternal health care services and outcomes for women in Texas.

S.B. 748, 86th Legislature, Regular Session, 2019

- Established two separate pilot programs for pregnant women. One provides services to pregnant women through a pregnancy medical home, while the other provides prenatal and postpartum services through telemedicine and telehealth.
- Required a variety of program evaluations related to maternal health.

H.B. 133, 87th Legislature, Regular Session, 2021

- Required HHSC to:
 - Transition Case Management for Children and Pregnant Women (CPW) services to managed care.
 - Transition HTW to a managed care delivery model (if necessary federal authority is obtained).
 - Extend postpartum coverage for Medicaid for pregnant women from 60 days to six months.

Prescription Drugs

Federal Legislation

The Omnibus Budget Reconciliation Act of 1990 (OBRA of 1990)

- Established the Federal Rebate Program, requiring drug manufacturers to pay rebates for state Medicare formulary drugs dispensed under state outpatient drug programs.
- Required states to cover all drugs for which a manufacturer provides rebates under the terms of the law and to maintain an open formulary for all drugs from manufacturers that have signed a federal rebate agreement. CMS determines rebate amounts per unit.

OBRA of 1990 was later amended under the DRA of 2005, which extended the rebate program to outpatient drugs administered in a physician's office or another outpatient facility.

The ACA also made changes by increasing the minimum federal rebate percentages that drug manufacturers are required to pay to participate in the Medicaid program, and by expanding the rebate program to cover claims paid by MCOs. The federal government keeps 100 percent of the increased rebate amount.

The Vendor Drug Program (VDP) within the Texas Medicaid program manages the federal manufacturer drug rebate program. VDP collects rebates for medications that are dispensed by pharmacies and administered by physicians to people enrolled in fee-for-service (FFS) and managed care. Texas negotiates additional state rebates for preferred drugs. HHSC also collects rebates for drugs provided to people enrolled in CHIP and three state health programs, including HTW.

The Medicare Prescription Drug Improvement and Modernization Act (MMA) (2003)

- Created a new Medicare prescription drug benefit (Part D) and made other program and payment changes. Medicare Part D is a voluntary Medicare prescription drug benefit where people in traditional Medicare may choose a private, drug-only plan. Those who choose to enroll in an MCO may choose a plan offering a drug benefit. Medicare Part D is available to dual eligibles receiving Medicaid.

- Amended provider payments, some of which had been reduced or constrained by previous legislation. Major provisions affecting the Medicaid program include:
 - A “clawback” provision that recoups part of the federal cost of the drug benefit by requiring states to refund a portion of their savings that result from Medicare providing drug coverage to dual eligibles.
 - New preventive benefits to Medicare and the elimination of copays for home health services, some of which were previously covered by Medicaid.
 - Increased premiums for Medicare Part B, which covers physician services, lab services, etc. Medicaid pays these premiums on behalf of certain clients who are dually eligible for Medicare and Medicaid.

State Legislation

H.B. 2292, 78th Legislature, Regular Session, 2003

- Directed HHSC on how to implement the Medicaid Preferred Drug List (PDL) — a tool used by many states to control growing Medicaid drug costs (see Chapter 3, page 63).
- Required HHSC to collect supplemental rebates from both FFS and MCO prescription drug claims. Supplemental rebate revenue is shared with CMS at the same federal medical assistance percentage (FMAP) used to pay claims.

H.B. 1917, 85th Legislature, Regular Session, 2017

- Provided that the PDL will carve into managed care, allowing each MCO to determine the drugs that are on their preferred drug list on Sept. 1, 2023.
- H.B. 1283, 88th Legislature, Regular Session, 2021 extended this date to Aug. 31, 2033.

S.B. 1780, 86th Legislature, Regular Session, 2019

- Allowed HHSC to negotiate a value-based arrangement with a prescription drug manufacturer.

H.B. 2822, 87th Legislature, Regular Session, 2021

- Required HHSC to automate prior authorization (PA) for drugs in the antipsychotic class and to issue specific messaging for PA denials for drugs in the antipsychotic class.
- Allows adult Medicaid members to bypass preferred drug PAs in certain circumstances.

S.B. 1 (Article II, HHSC, Rider 22/ Exceptional Item 3), 87th Legislature, Regular Session, 2021

- Appropriated funds for HHSC to expand access to direct-acting antiviral medications for Medicaid clients with chronic Hepatitis C by entering into a cost-effective rebate agreement with a drug manufacturer.

Economic Crises and Public Health Emergencies

Since Medicaid primarily serves low-income people, a rise in unemployment can result in an increase in the number of people eligible for Medicaid. During the 2008 economic recession, for example, the rise in unemployment — and subsequent federal legislation passed to meet the crisis — caused sudden growth in Medicaid enrollment.

Similarly, the recent public health crisis caused by COVID-19 spurred federal legislation to address an economic recession and assist state and local governments, which included changes to Medicaid and CHIP.

The American Recovery and Reinvestment Act (ARRA) of 2009

- Temporarily increased the FMAP from October 2008 through December 2010.
- Temporarily prohibited states from making changes to any Medicaid eligibility standards, methodologies or procedures that were more restrictive than those in effect on July 1, 2008.
- Extended the TANF Supplemental Funds, created a new TANF Emergency Fund and increased the DSH allotment.

ARRA also included reforms and financial incentives for health care technology under the Health Information Technology for Economic and Clinical Health (HITECH) Act (see page 136).

The Families First Coronavirus Response Act (FFCRA) of 2020

- Required states to maintain continuous Medicaid coverage for most people active or certified for Medicaid starting on March 18, 2020, until the end of the month in which the federal PHE ended. The continuous coverage requirement was later delinked from the PHE (see Chapter 1, page 12).
- Required private and public health insurers — including Medicare, Medicaid and CHIP — to provide coverage for COVID-19 testing.

- Temporarily increased the Medicaid FMAP by 6.2 percentage points from Jan. 1, 2020, throughout the duration of the emergency period. As a condition of receiving the increased FMAP, states were required to maintain eligibility and services for any Medicaid recipients eligible as of March 18, 2020. This was later changed by the Consolidated Appropriations Act, 2023.
- Provided paid emergency sick leave and family medical leave for people and families impacted by the PHE.

The Coronavirus Aid, Relief and Economic Security (CARES) Act of 2020

- Delayed the reductions in DSH allotments for federal fiscal year (FFY) 2020 until Dec. 1, 2020. This was later changed by the Consolidated Appropriations Act, 2024 to Jan. 1, 2025 [see Section 1923(f)(7)(A) of the Social Security Act (42 U.S.C. 1396r-4(f)(7)(A))].

The American Rescue Plan Act (ARPA) of 2021

- Required mandatory coverage of COVID-19 vaccines and administration and treatment under Medicaid.
- Provided states the option to modify certain coverage under Medicaid for pregnant and postpartum women.
- Provided states the option to provide qualifying community-based mobile crisis intervention services.

Consolidated Appropriations Act (CAA), 2023

- Delinked requirements for Medicaid continuous coverage from the COVID-19 PHE.
- Set an end date of March 31, 2023, for the continuous Medicaid eligibility requirement established under the FFCRA.

Rights and Entitlements for People with Disabilities

Federal Legislation and Policy

The Americans with Disabilities Act (ADA) of 1990

- Prohibited discrimination based on disability in the areas of: employment; public services provided by state and local governments; and public services operated by private entities, transportation and telecommunications.

The Ticket to Work and Work Incentives Improvement Act (TWWIIA) of 1999

- Expanded the eligibility options and funding for Medicaid Buy-in programs for people age 16 through 64.
- Extended Medicare coverage for people with disabilities returning to work.
- Created the Ticket to Work Program, which enables SSI and Social Security Disability Insurance recipients to obtain employment services from both private and public providers.

Olmstead v. L. C., 527 U.S. 581 (1999)

- The 1999 Supreme Court decision, *Olmstead v. L.C.*, determined that people with intellectual disabilities or serious mental illnesses have the right to live in community-based settings rather than in institutions. The ruling mandated that publicly run programs provide the option for community-based services to people with disabilities when such services are appropriate and can be reasonably accommodated. In response to the *Olmstead* decision and other federal policy priorities, Texas Medicaid has expanded coverage for a broad range of LTSS.

Promoting Independence Initiative, 2001

- Established in response to the U.S. Supreme Court ruling *Olmstead v. Zimring*. The Promoting Independence Initiative is also called “Money Follows the Person.” General Appropriations Act, S.B. 1 (Article II, HHSC, Rider 37), 77th Legislature Regular Session, 2001 allowed funding for people moving from nursing facilities to community-based services to be transferred from the nursing facility budget to the community-based services budget.
- Received a demonstration award through the DRA of 2005. Federal authorization of the demonstration ended in September 2016, but states were given supplemental grants through Sept. 30, 2020. The 2020 through 2023 Consolidated Appropriations Acts extended authorization of the demonstration for FFYs 2021 through 2027. Even though the authorization extends to 2027, there may not be funds for each year. The available funds will depend on re-obligation and congressional appropriations.
- Later codified by H.B. 1867, 79th Legislature, Regular Session, 2005.

Home and Community-Based Services Settings Final Rule, 2014

In March 2014, new federal rules provided direction on the settings where home and community-based services are received. The final home and community-based services regulations set forth new requirements for several Medicaid authorities under which states may provide home and community-based LTSS. The regulations enhance the quality of home and community-based services and provide additional protections to people who receive services under these Medicaid authorities.

State Legislation and Policy

S.B. 7, 83rd Legislature, Regular Session, 2013

- Directed HHSC to implement an acute care and LTSS system for people with intellectual and developmental disabilities (IDD), a managed care delivery system, and the federal Community First Choice (CFC) option.
- Authorized HHSC to operate a pilot program by Sept. 1, 2018, to test one or more managed care service delivery models to deliver Medicaid LTSS to people with IDD.

H.B. 3295, 85th Legislature, Regular Session, 2017

- Extended completion of the managed care pilot program for people with IDD (under Chapter 534) from Sept. 1, 2018, to Sept. 1, 2019.
- Changed the transition of the Texas Home Living (TxHmL) waiver program to managed care from Sept. 1, 2018, to Sept. 1, 2020, which was then extended to Sept. 1, 2027.

S.B. 547, 85th Legislature, Regular Session, 2017

- Allowed state supported living centers (SSLCs) to provide certain nonresidential Medicaid services to support people with IDD and directed HHSC to establish reimbursement rates for SSLCs for these services.

H.B. 4533, 86th Legislature, Regular Session, 2019

- Required HHSC to develop and implement a 24-month pilot program by Sept. 1, 2023, for people with IDD, traumatic brain injury and similar functional needs in STAR+PLUS.
- Required HHSC to conduct a pilot to test the feasibility and cost efficiency of transitioning Home and Community-based Services (HCS), Deaf Blind with Multiple Disabilities (DBMD) residential services and community-based ICF/IID services to managed care.
- Updated the plan to transition waiver program services to managed care:
 - TxHmL will transition by Sept. 1, 2027.
 - Community Living Assistance and Support Services (CLASS) will transition by Sept. 1, 2029.
 - HCS and DBMD services will transition by Sept. 1, 2031.

H.B. 706, 86th Legislature, Regular Session, 2019

- Expands audiology services as a benefit under the School Health and Related Services Program to students with a Section 504 Plan.

S.B. 1, (Article II, HHSC), 87th Legislature, Regular Session, 2021

- Rider 23/Exceptional Item 9 required HHSC to comply with the CMS-issued regulations governing the settings in which home and community-based services are provided. CMS requires that a setting must support a person's full access to the community, including opportunities to engage in community life, work in competitive integrated settings and control personal resources. Replace day habilitation with a new, more integrated service called Individualized Skills and Socialization. Day habilitation services provided in some home and community-based services programs were not compliant with federal regulations.
- Rider 28 required HHSC to create a new children's Medicaid Applied Behavior Analysis medical benefit as soon as possible but not later than Feb. 1, 2022.
- In Exceptional Item 4, HHSC received approval from CMS to fund additional waiver program slots through Section 9817 of the American Rescue Act.

S.B. 1648, 87th Legislature, Regular Session, 2021

- Required HHSC to:
 - Conduct a pilot to enhance health homes and care coordination for children with medically complex conditions to improve health outcomes (the Comprehensive Health Homes for Integrated Care Kids pilot).
 - Allow Medicaid managed care members with complex medical needs to continue receiving care from a specialty provider who they have an established relationship with, regardless of whether the member also has primary health benefit plan coverage.

H.B. 3720, 87th Legislature, Regular Session, 2021

- Required HHSC to develop a questionnaire for people who are on an interest list for any of the home and community-based services Medicaid waiver programs.

Behavioral Health

Federal Legislation

The Paul Wellstone and Pete Domenici Mental Health Parity and Addition Equity Act (MHPAEA) of 2008

- Required group health plans that offer behavioral health benefits to provide those services at parity with medical and surgical benefits. This does not impact FFS but does apply to Medicaid and CHIP managed care programs.

The Substance Use Disorder Prevention that Promotes Opioid Recovery and Treatment for Patients and Communities (SUPPORT) Act of 2018

- Directed funding to federal agencies and states to increase access to addiction treatment and to take mitigating steps, such as preventing overprescribing opioids.
- Required Medicaid to report on adult behavioral health outcome measures, expand drug utilization reviews, share data between Medicaid and the state's prescription drug monitoring databases, expand the "qualified practitioners" who can prescribe buprenorphine for opioid treatment, and ensure health insurance coverage continuity for former foster youth up to age 26 in any state they reside.

State Legislation

S.B. 1, 81st Legislature, Regular Session, 2009 (Article IX, Health-Related Provisions, Section 17.15)

- Authorized HHSC to add comprehensive substance use disorder (SUD) benefits for adults in Medicaid to reduce medical expenditures related to substance abuse.

S.B. 58, 83rd Legislature, Regular Session, 2013

- Required HHSC to integrate behavioral health and physical health services into Medicaid managed care programs by adding both mental health targeted case management and mental health rehabilitative services to the array of services provided by MCOs by Sept. 1, 2014.

H.B. 10, 85th Legislature, Regular Session, 2017

- Created a behavioral health ombudsman in the HHSC Office of the Ombudsman and established a mental health condition and SUD parity work group.
- Required HHSC and the Texas Department of Insurance to study and report on benefits provided by health benefit plans, both for medical or surgical expenses and for mental health conditions and SUDs.

S.B. 1, 85th Legislature, Regular Session, 2017 (Article II, HHSC, Rider 45)

- Required HHSC to develop performance metrics to increase accountability of MCOs for members with severe mental illness (SMI). These metrics must include integrated care, jail and emergency department diversion, post-release linkage to care, homelessness reduction, supportive housing and medication adherence.
- Required HHSC to improve outcomes, care integration and cost control against an established baseline for members with SMI. If shown to be cost-effective, HHSC must develop and procure a managed care program in at least one service area to serve members with SMI. After conducting extensive research, HHSC did not procure a managed care program for members with SMI. As required, HHSC submitted a report outlining the results to the Legislative Budget Board.

S.B. 1177, 86th Legislature, Regular Session, 2019

- Directed HHSC to work with the State Medicaid Managed Care Advisory Committee to determine services that could potentially be provided in lieu of state plan services, and to implement in lieu of services if medically appropriate and cost-effective.

S.B. 1, 87th Legislature, Regular Session, 2021 (Article II, HHSC, Rider 33)

- Authorizes Medicaid to continue reimbursement for select behavioral health services to be delivered via audio-only modalities, as well as any other benefit HHSC determines to be clinically and cost-effective.

S.B. 672, 87th Legislature, Regular Session, 2021

- Required HHSC to reimburse Medicaid providers for collaborative care management services for children and adults.

Technology, Patient Privacy and Fraud Prevention

Federal Legislation

The Health Insurance Portability and Accountability Act (HIPAA) of 1996

- Mandated that all health plans, health care clearinghouses and health care providers must protect all individually identifiable health information that is held or transmitted by a covered entity or business associate. Protected health information includes digital, paper and oral information.
- Required standardized electronic exchange of administrative and financial health services information for all health plans, including Medicaid. All health care organizations must implement secure electronic access to health data and remain in compliance with privacy regulations set by the U.S. Department of Health and Human Services.
- Implemented the National Provider Identifier (NPI) system in which each health care entity — including individuals, employers, health plans and health care providers — must have a unique 10-digit NPI number.

The Health Information Technology for Economic and Clinical Health (HITECH) Act of 2009

- Incentivized providers to adopt and meaningfully use electronic health records (EHRs) and advance health information exchange systems. The EHR incentive program ended Dec. 31, 2021.
- Established health information exchange and data breach notification rules that build upon HIPAA privacy and security regulations.

The 21st Century Cures Act of 2016

- Expanded the scope of programs and services required to use Electronic Visit Verification (EVV) to include all FFS and managed care personal care services, home health nursing and therapy services provided in the member's home, including a Consumer Directed Services (CDS) option or service responsibility option. The expansion includes the CLASS, DBMD, HCS, Medically Dependent Children Program (MDCP), TxHmL and the Youth Empowerment Services (YES) 1915(c) waivers; the 1915(i) Home and Community-Based Services-Adult Mental Health (HCBS-AMH) program; and 1915(k) CFC.
- Required the use of EVV for all Medicaid personal care services and home health care services by Jan. 1, 2020, a deadline that was later extended by CMS to Jan. 1, 2021. HHSC implemented EVV for personal care services on Jan. 1, 2021.
- Required states to implement EVV for Medicaid home health care services by Jan. 1, 2023, a deadline that was later extended by CMS to Jan. 1, 2024.
- HHSC implemented EVV for Medicaid home health care services on Jan. 1, 2024.

State Legislation

S.B. 7, 82nd Legislature, First Called Session, 2011

- Added Section 531.024172 to the Texas Government Code, which required HHSC to pilot EVV in home health skilled nursing services, home health aide services and private duty nursing services.
- If HHSC determined that EVV would be cost-effective and feasible, HHSC must implement the program.

H.B. 4533, 86th Legislature, Regular Session, 2019

- Required HHSC to phase out use of the Texas Provider Identifier and switch to only using the NPI to identify.

S.B. 1991, 86th Legislature, Regular Session, 2019

- Amended Section 531.024172 of the Texas Government Code to add language to subsection (g) and to add subsections (g-1) and (g-2), which allowed service providers to utilize their own proprietary EVV system and, if feasible, to be reimbursed by HHSC.
- Allowed service providers to utilize their own proprietary EVV system and, if feasible, to be reimbursed by HHSC.
- Required HHSC to adopt rules that amend the MCO payment recovery and recoupment efforts process.

S.B. 1, 87th Legislature, Regular Session, 2021 (Article II, HHSC, Rider 27)

- Rider 27 provided direction to HHSC on Medicaid operations as follows:
 - Deployed the Provider Enrollment Management System, allowing for online NPI-based enrollment and revalidation.
 - Streamlined managed care enrollment and disenrollment via implementation of automatic enrollment into managed care.
 - Reduced paper waste by promoting the use of electronic provider directories for Medicaid and CHIP.
 - Authorized Medicaid and CHIP MCOs to communicate with members, with their consent, through text messaging and email.

S.B. 8, 87th Legislative Session, Third Called Session, 2021

- Appropriated \$5 million to HHSC for Medicaid eligibility system upgrades.
- Required HHSC to implement a streamlined portal for provider enrollment and credentialing information.

Telehealth and Telemedicine

State Legislation

S.B. 1107, 85th Legislature, Regular Session, 2017

- Standardized requirements for telemedicine and telehealth services by specifying acceptable telemedicine and telehealth service delivery requirements.
- Directed the Texas Medical Board, Texas Board of Nursing, Texas Physician Assistant Board, and Texas State Board of Pharmacy to jointly develop administrative rules for valid prescriptions generated during a telemedicine visit.

H.B. 1063, 86th Legislature, Regular Session, 2019

- Expanded home telemonitoring to pediatric patients who have been diagnosed with end-stage solid organ disease, have received an organ transplant or require mechanical ventilation.
- Directed HHSC to assess whether telemedicine, telehealth and home telemonitoring services are cost-effective for the Medicaid program in its biennial report on telemedicine, telehealth and home telemonitoring services.

S.B. 670, 86th Legislature, Regular Session, 2019

- Ensures that MCOs do not deny reimbursement for a covered health care service or procedure solely because the covered service was delivered through telemedicine or telehealth.
- Ensured that MCOs' utilization of telemedicine or telehealth services promotes and supports patient-centered medical homes by allowing a Medicaid recipient to receive telemedicine or telehealth services from a provider other than the recipient's primary care physician or provider in certain circumstances.
- Authorized HHSC to reimburse FQHCs for the patient-site facility fee and distant-site practitioner fee for covered telemedicine medical services and telehealth services delivered by a health care provider to a Medicaid recipient.
- Directed HHSC to develop, document and implement a monitoring process to ensure that MCOs' use of telemedicine medical services or telehealth services promotes and supports patient-centered medical homes and care coordination.

H.B. 4, 87th Legislature, Regular Session, 2021

- Required HHSC to expand the use of telemedicine, telehealth and audio-only platforms to deliver a range of services, including but not limited to office visits, behavioral health audio-only services, assessments and case management.
- Required HHSC to make permanent the flexibilities that were authorized during the COVID-19 PHE where it is clinically appropriate and cost-effective.

H.B. 2056, 87th Legislature, Regular Session, 2021

- Required Medicaid to cover teledentistry services. Synchronous teledentistry allows a dentist or other health care professional, such as a dental hygienist, to provide a dental service to a patient at a different physical location using synchronous audiovisual technology.

Children's Health Insurance Program

Founding Legislation

Federal Legislation

The BBA of 1997 created the State Children's Health Insurance Program (SCHIP) under Title XXI of the SSA and appropriated nearly \$40 billion for the program for FFYs 1998 through 2007. SCHIP offered states three options when designing a program:

- Use SCHIP funds to expand Medicaid eligibility to children who were previously ineligible for the program.
- Design a separate state children's health insurance program approved by the U.S. Secretary of Health and Human Services.
- Combine both the Medicaid and separate state program options.

States that chose to expand their Medicaid programs were required to provide all mandatory and optional services covered under their Medicaid State Plan and follow the Medicaid cost-sharing rules. States that chose to implement a separate program had more flexibility. Within federal guidelines, they determined their own SCHIP benefit packages.

Texas originally opted to expand Medicaid eligibility using SCHIP funds. In July 1998, Texas implemented Phase I of SCHIP, providing Medicaid to children age 15 through 17 whose family income was under 100 percent of the FPL. Phase I of SCHIP operated from July 1998 through September 2002. The program was phased out as Medicaid expanded to cover those children.

State Legislation

S.B. 445, 76th Legislature, Regular Session, 1999 enacted Phase II of SCHIP, which created Texas CHIP. S.B. 445 specified that coverage under CHIP be available to children in families with incomes up to 200 percent of the FPL. Coverage under Phase II of the program began on May 1, 2000.

Texas CHIP was designed and operates as a separate child health program. Texas elected secretary-approved coverage for CHIP.

Program Reforms

Federal Legislation

The Medicare, Medicaid and SCHIP Benefits Improvement and Protection Act (BIPA) of 2000

- Allowed states to retain unexpended FFYs 1998 through 1999 federal allocations through FFY 2004.
- Granted states additional time to spend 50 percent of unused FFY 2000 and FFY 2001 federal allocations through FFY 2004 and FFY 2005, respectively.

The Children’s Health Insurance Program Reauthorization Act (CHIPRA) of 2009

- Appropriated nearly \$69 billion for FFY 2009 through 2013 and redesigned the allocation formula, so that it more closely reflected state spending.
- Required states to verify a CHIP applicant’s citizenship.
- Mandated that states apply mental health parity standards established under the MHPAEA of 2008.
- Allowed states to cover pregnant women above 185 percent of the FPL and up to the income eligibility level for children in CHIP.
- Allowed states to provide Medicaid and CHIP coverage to qualified immigrant children and pregnant women without the previously required five-year delay.

Texas has implemented additional changes in accordance with CHIPRA guidance — including applying certain managed care safeguards to CHIP, expanding dental services, and implementing mental health parity in CHIP.

Following CHIPRA, the ACA of 2010 extended federal funding for CHIP through FFY 2015 and increased the federal match rate for FFYs 2016 through 2019. Maintenance of effort (MOE) requirements prohibited states from restricting CHIP eligibility standards, methodologies or procedures through Sept. 30, 2019. Medicaid payments are contingent upon meeting this CHIP MOE requirement. The ACA eliminated asset tests and most income disregards in CHIP financial eligibility determinations.

The Medicare Access and CHIP Reauthorization Act (MACRA) of 2015

- Extended federal funding for CHIP through FFY 2017 and fully funded state CHIP allotments through Sept. 30, 2017, including funding the 23 percentage-point increase in federal matching funds authorized by the ACA.
- Extended CHIPRA outreach and enrollment grants and CHIPRA quality provisions, plus maintained MOE for children's coverage in Medicaid and CHIP through 2019.
- Made permanent the authorization for Transitional Medical Assistance, which provides time-limited Medicaid to low-income parents transitioning to employment at higher wages that otherwise would make them ineligible for Medicaid.

Bipartisan Budget Act of 2018

- Reauthorized CHIP funding through FFY 2027. The reauthorization legislation also extended the MOE provision but phased out the associated super-enhanced federal match rate. In FFY 2020, the 23 percent bump reduced to 11.5 percentage points, and in FFY 2021, the match will return to the standard CHIP enhanced rate.
- Federal funding for CHIP lapsed from October 2017 through February 2018. While some states received additional grant funding from CMS, Texas continued to fund CHIP using FFY 2017 carry-over funds.

The Substance Use Disorder Prevention that Promotes Opioid Recovery and Treatment for Patients and Communities (SUPPORT) Act of 2018

- Required mental health and SUD services to be included as a category for basic services in CHIP (Section 5022). These services include inpatient and outpatient services, psychosocial rehabilitation and crisis stabilization.

State Legislation

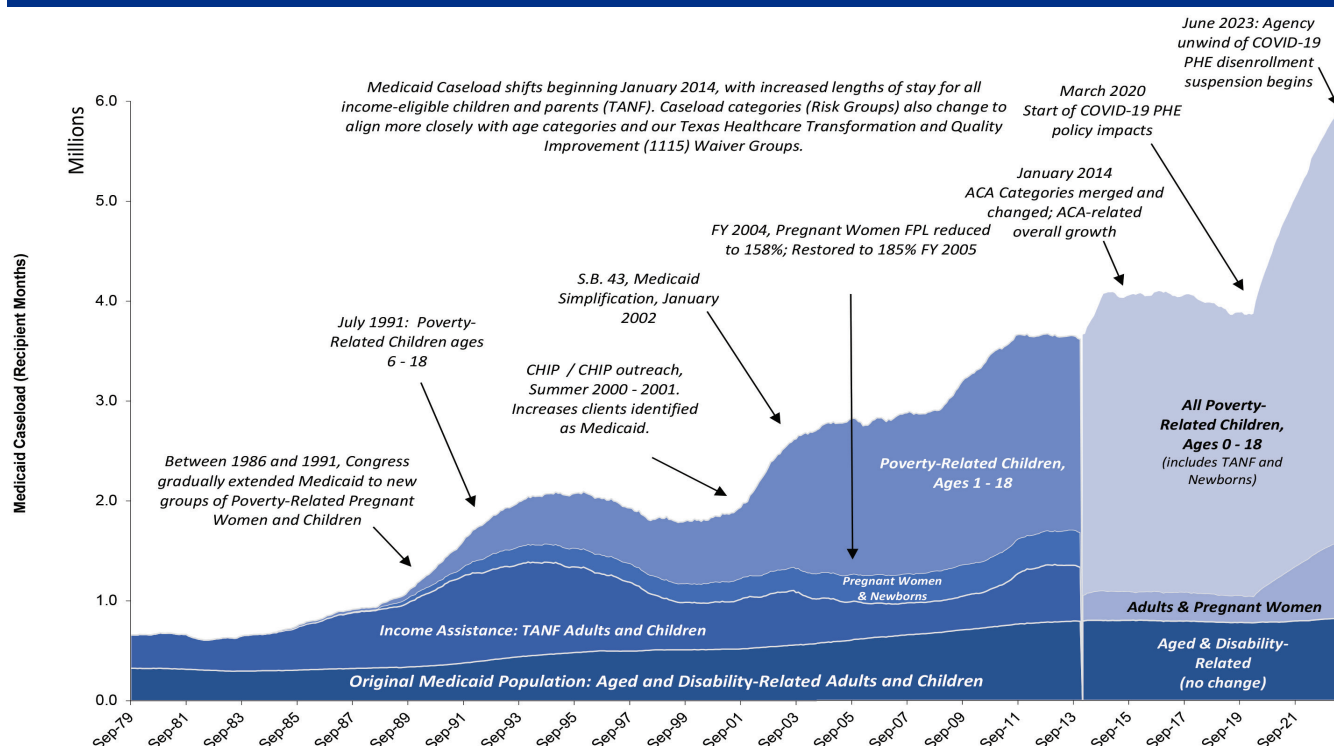
S.B. 1, 79th Legislature, Regular Session, 2005 (Article II, HHSC, Rider 70)

- Authorized HHSC to expend funds to provide unborn children with health benefit coverage under CHIP. The result was CHIP Perinatal, which began in January 2007.
- Determined that CHIP Perinatal services are for the unborn children of pregnant women who are uninsured and do not qualify for Medicaid due to income or immigration status.

Legislative Impacts on Caseload

Enrollment in Texas Medicaid and CHIP can change in response to federal and state legislative requirements (see figure below).

A.1: Texas Medicaid Caseload by Group, SFYs 1980–2023



Medicaid eligibility was historically linked to receiving financial assistance. In the 1980s, several federal laws extended coverage to populations ineligible for financial assistance. This expansion — to include a greater number of people with disabilities, children, pregnant women and older people — fueled the growth of the Medicaid program.

Caseloads declined in the 1990s after Medicaid eligibility was de-linked from financial assistance. In 2002, the number of children enrolled in Medicaid grew sharply when the application process was simplified. The ACA of 2010 also contributed to caseload growth because of required eligibility changes.

To receive increased federal funding, the FFCRA required states to maintain continuous Medicaid coverage for most people active or certified for Medicaid starting on March 18, 2020. This contributed to increases in caseloads. Although the PHE officially ended on May 11, 2023, federal legislation ended continuous coverage on March 31, 2023. (see Chapter 1, page 12). The first disenrollments in Texas began in June 2023. With the end of continuous coverage, caseloads will reflect return to normal business operations.

Managed Care in Texas

State Legislation

S.B. 10, 74th Legislature, Regular Session, 1995

- Directed HHSC to enact a comprehensive statewide restructuring of Medicaid, incorporating a managed care delivery system. HHSC began expanding, and continues to expand, its managed care program through 1915(b) waivers under the authority of S.B. 10.

S.B. 6, 79th Legislature, Regular Session, 2005

- Directed HHSC and DFPS to develop a statewide health care delivery model for all Medicaid children in foster care. STAR Health was implemented on April 1, 2008.

H.B. 1, 82nd Legislature, Regular Session, 2011, and S.B. 7, 82nd Legislature, First Called Session, 2011

- Instructed HHSC to expand its use of managed care. The Legislature directed HHSC to preserve federal hospital funding, which was historically received as supplemental payments under the upper payment level program.

S.B. 7, 83rd Legislature, Regular Session, 2013

- Directed HHSC to: implement cost-effective options for delivering basic attendant care and a voluntary LTSS managed care pilot program; expand STAR+PLUS to the Medicaid Rural Service Area; and integrate acute care and LTSS for people age 65 and older and those with disabilities. Most people being served through one of the 1915(c) waivers for people with IDD — and living in community-based settings or in an ICF/IID — are now receiving acute care services through STAR+PLUS or STAR Kids.
- Directed HHSC to develop the STAR Kids managed care program, tailored for children with disabilities — including children receiving MDCP waiver benefits. STAR Kids was implemented statewide on Nov. 1, 2016.

- Directed HHSC to develop quality-based outcome and process measures in quality-based payment systems by measuring potentially preventable events, rewarding use of evidence-based practices, and promoting health care coordination, collaboration and efficacy. HHSC initially implemented the medical and dental pay-for-quality (P4Q) programs in 2014. The programs were subsequently redesigned in 2016 and 2017. New medical P4Q programs were implemented in 2018 for STAR, STAR+PLUS and CHIP, as well as a new dental P4Q program. STAR Kids was added for 2020. Due to the federal PHE, HHSC suspended medical and dental P4Q for 2020 and 2021. P4Q resumed in 2022. Results for previous years are available on the Texas Healthcare Learning Collaborative Portal ([THLCPortal.com](https://thlcpportal.com)).

S.B. 1, 83rd Legislature, Regular Session, 2013 (Article II, HHSC, Rider 51b[15])

- Directed HHSC to transition remaining FFS populations into managed care as a cost containment measure. The rider stated that this reduction should be achieved by improving care coordination through a capitated managed care program.

H.B. 72, 86th Legislature, Regular Session, 2019

- Directed HHSC to change managed care enrollment rules for children and young adults in the Adoption Assistance (AA) and Permanency Care Assistance (PCA) programs.
- As of Sept. 1, 2020, children who transition from foster care to AA or PCA remain in STAR Health for an additional two months to improve continuity of care. Certain AA and PCA children with disabilities have a choice between enrolling in STAR Kids or staying in STAR Health.

H.B. 1, 86th Legislature, Regular Session, 2019 (Article II, HHSC, Rider 43)

- Required HHSC to create an incentive program that allows MCOs to increase the percentage of default enrollment clients they receive based on quality, efficiency and effectiveness, and performance metrics.

H.B. 4533, 86th Legislature, Regular Session, 2019

- Required HHSC to standardize and implement consistent definitions for MCO grievance processes, as well as reporting and data collection for Medicaid.
- Required HHSC to implement a pilot program for people with IDD, traumatic brain injury, and similar functional needs in STAR+PLUS. Established new timelines and processes for transitioning IDD LTSS into managed care.
- Required HHSC to expand the availability of the CDS option in the MDCP waiver program.

H.B. 1576, 86th Legislature, Regular Session, 2019

- Required HHSC to add non-emergency transportation services to managed care for coordination by the MCO.
- Required HHSC to provide nonmedical transportation services, which are a subset of demand response transportation services, for certain trips requested with less than 48-hour notice.

H.B. 3041, 86th Legislature, Regular Session, 2019

- Required MCOs to publish information about submitting a PA and the process for communicating with the MCO about a PA request on their website.
- Required HHSC to allow, if practicable, renewal of a PA within 60 days of expiration for managed care and FFS.

S.B. 1207, 86th Legislature, Regular Session, 2019

- Required HHSC and MCOs to issue notices to providers and members with clear and detailed information regarding either a denial, reduction or termination of coverage or a denial of a PA request.
- Required that HHSC contract with an independent external medical reviewer (see Chapter 3, page 63).
- Directed, for MDCP only, that the MCOs implement measures to ensure that STAR Kids service coordinators provide results of the STAR Kids Screening Assessment Instrument to the child's parent or legally authorized representative. It also required MCOs to offer an opportunity for a peer-to-peer with a physician of the member's choice.
- Required HHSC to establish time-frame guidelines for MCOs to issue decisions on PA requests, which must be published on MCO websites.
- Required HHSC to establish a uniform process and timeline for MCOs that receive an insufficient or inadequate PA request.

S.B. 1, 87th Legislature, Regular Session, 2021 (Article II, HHSC, Rider 39)

- Required that, if Medicaid provider rates are increased due to a legislative appropriation, MCOs reimburse the full amount of the appropriated funds to providers, to the extent allowed by federal laws and regulations.

H.B. 2658, 87th Legislature, Regular Session, 2021

- Among other operational changes, directed HHSC to adopt rules establishing minimum performance standards for nursing facility providers that participate in the STAR+PLUS managed care program.

A.2: Development of Medicaid Managed Care in Texas, SFYs 1994-2023

State Fiscal Year	Managed Care Carve-in	Total Medicaid Managed Care Enrollment	% of Medicaid Population in Managed Care
1994	STAR implemented in Travis County and in the Tri-County Area.	58,243	2.86%
1995	No change.	65,388	3.16%
1996	The Tri-County area was expanded to three additional counties and was renamed from Lone STAR Health Initiative to STAR.	71,435	3.46%
1997	STAR expanded to the Bexar, Lubbock, and Tarrant service areas (SAs), and the Travis County area was expanded to include surrounding counties.	274,694	13.82%
1998	STAR expanded to the Harris SA, and STAR+PLUS implemented in the Harris SA.	364,336	19.56%
1999	STAR expanded to the Dallas SA.	425,069	23.45%
2000	SSTAR expanded to the El Paso SA.	523,832	28.98%
2001	No change.	623,883	33.35%
2002	No change.	755,698	35.92%
2003	No change.	988,389	39.71%
2004	No change.	1,112,002	41.43%
2005	No change.	1,191,139	42.85%
2006	Primary Care Case Management (PCCM) implemented in 197 counties.	1,835,390	65.72%
2007	STAR expanded to the Nueces SA, and STAR+PLUS expanded to the Bexar, Travis, Nueces, and Harris Contiguous SAs. STAR replaced PCCM in all urban areas.	1,921,651	67.83%
2008	Integrated Care Management (ICM) implemented in the Dallas and Tarrant SAs, and STAR Health implemented statewide.	2,039,340	70.86%
2009	The ICM program was discontinued.	2,127,382	70.78%
2010	No change.	2,362,091	71.62%
2011	STAR+PLUS expanded to the Dallas and Tarrant SAs.	2,676,149	75.53%

A.2: Development of Medicaid Managed Care in Texas, SFYs 1994-2023

State Fiscal Year	Managed Care Carve-in	Total Medicaid Managed Care Enrollment	% of Medicaid Population in Managed Care
2012	STAR expanded to Medicaid Rural Service Areas (MRSAs), replacing PCCM in all rural areas and discontinuing that program. Pharmacy benefits were carved into all managed care programs, and inpatient hospital benefits were carved into STAR+PLUS. The Children's Medicaid Dental Services program implemented statewide.	2,893,965	79.16%
2013	No change.	2,982,923	81.53%
2014	No change.	3,012,265	80.41%
2015	STAR+PLUS expanded to all areas of the state, non-dual eligible clients in IDD waivers and nursing facility benefits were carved into STAR+PLUS, mental health targeted case management and mental health rehabilitative services were carved into all managed care programs, and the Dual Demonstration program implemented in Bexar, Dallas, El Paso, Harris, Hidalgo, and Tarrant counties.	3,524,581	86.88%
2016	No change.	3,570,411	87.93%
2017	STAR Kids implemented statewide. NorthSTAR was discontinued.	3,721,646	91.50%
2018	PCA programs were carved into STAR, and the Medicaid for Breast and Cervical Cancer program was carved into STAR+PLUS.	3,776,096	93.89%
2019	No change.	3,676,441	93.91%
2020	No change.	3,760,023	94.36%
2021	No change.	4,517,225	96.46%
2022	No change.	5,121,247	96.42%
2023*	No change.	5,564,399	97.15%

*Caseloads impacted due to federal PHE.

Recent Medicaid and CHIP State Legislation

The table below highlights key pieces of state legislation passed during the 88th Texas Legislature.

88th Legislature, 2023	
<i>Benefit Changes</i>	
S.B. 989	Required HHSC to: • Provide coverage of biomarker testing for the purpose of diagnosis, treatment, appropriate management, or ongoing monitoring of an enrollee's disease or condition when supported by medical and scientific evidence. • Manage coverage decisions for biomarker tests demonstrating clinical utility and administering benefits in a way that limits disruptions in care.
<i>Home and Community-Based Services</i>	
H.B. 4169	Required HHSC to seek federal approval to include prevocational services under certain Medicaid waiver programs providing LTSS. The goal is to help the recipient achieve permanent, integrated employment in the community, making at least minimum wage.
H.B. 3550	Required HHSC to: • Establish minimum standards for transportation services for prescribed pediatric extended care centers (PPECC). • Identify the documentation a PPECC must obtain and maintain to be eligible for reimbursement. • Authorize PPECCs to provide nursing services in a group setting, consistent with appropriate staffing ratios. • Allow a PPECC to obtain all required guardian signatures for admission on a single consent document.
H.B. 1, Article II, Rider 25	Required HHSC to transition the nursing facility reimbursement methodology and individual cost-limits methodology for the MDCP and STAR+PLUS Home and Community Based Services waiver programs from the resource utilization group to the Patient Driven Payment Model Long Term Care reimbursement methodology. The change simplifies the reimbursement structure, enabling a significant number of benefits.

88th Legislature, 2023	
<i>Home and Community-Based Services</i>	
H.B. 1, Article II, Rider 29 (Exception Item #6b)	Required HHSC to increase the case management rate in the DBMD Medicaid waiver program from an hourly rate to a monthly rate. The case management rate change improves efficiency for providers and more accurately reflects service delivery.
H.B. 1, Article II, Rider 30(a)	Required HHSC to increase the base wage for attendants in various programs from \$8.11 to \$10.60 per hour. The increase in attendant rates supports services that are non-technical, medically related, or personal care services in home or community living settings.
<i>Member Communication</i>	
H.B. 2802	Allowed MCOs to communicate with participants through any electronic means, including telephone, text message and email. They may not contact participants for marketing purposes.
<i>Prescription Drugs</i>	
H.B. 3286	Required HHSC to: • Create a provisional formulary coverage status, which allows Medicaid and CHIP to cover the drug while the Certificate of Information is under review. • Create a temporary non-preferred status for new-to-market drugs on the formulary that have not been reviewed for PDL status. The bill also: • Elaborated on existing PDL exceptions for when a non-preferred drug can be used. • Changed the Drug Utilization Review Board by adding an additional MCO representative and making all MCO representatives voting members.
H.B. 916	Required HHSC to: • Provide up to a three-month supply of covered prescription contraceptive drugs the first time an enrollee obtains the drug. • Provide a 12-month supply each subsequent time the enrollee obtains the same drug, regardless of whether the enrollee was enrolled in the health benefit plan the first time they obtained the drug.

88th Legislature, 2023

Telemedicine, Telehealth and Teleservices

H.B. 2727	Required HHSC to: • Add FQHCs and Rural Health Clinics as home telemonitoring providers. • Clarify that “remote patient monitoring” is synonymous with home telemonitoring. • Allow home telemonitoring services for other conditions if they are clinically effective and cost-effective. • Change eligibility for home telemonitoring services so that a participant can qualify with only one risk factor, rather than requiring two or more. • Require providers to establish a plan of care with outcome measures for home telemonitoring clients. • Share the plan of care with the client’s physician.
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Women’s Health

H.B. 12	Required HHSC to extend continuous Medicaid coverage to eligible women for 12 months after pregnancy ends, regardless of eligibility status changes. The 12-month postpartum coverage also applies to CHIP and begins after the last month of pregnancy.
H.B. 1575	Required HHSC to: • Develop standardized nonmedical screening questions for pregnant women in Medicaid and the Thriving Texas Families program. • Require MCOs and the Thriving Texas Families program to administer a nonmedical health-related needs screening of pregnant women and obtain informed consent prior to administering the screening. • Add community health workers and doulas as providers that would be eligible to provide CPW case management services. • Specify the form and manner for data collection, collect data from MCOs and Thriving Texas Families Program providers, and develop an ongoing summation report of the data collected and one-time report describing the implementation of the provider case management services to pregnant women.

Summary of New Federal Access and Managed Care Rules

In May 2024, CMS released final rules on “Ensuring Access to Medicaid Services” (Access) and “Managed Care Access, Finance, and Quality” (Managed Care). Implementation deadlines have a range based on when the rules became effective from 60 days to four years, with some home and community-based services quality measures phased in over seven years.

Summary of Major Access Rule Changes*

Key Topics	Access Rules
Advisory Committees	• Expands the scope of existing advisory committees and creates a Beneficiary Advisory Council. • Requires an advisory group to consult on FFS rates paid for personal care, home health aides and homemaker services (direct care workers).
Home and Community-Based Services	• Revises quality requirements and monitoring of person-centered planning. • Expands Critical Incident Management System. • Adds grievance systems requirements for FFS HCBS programs. • Requires reporting on percent of payment for direct care worker compensation (vs. other costs) and public posting of hourly rates for FFS direct care workers. • Adds public reporting on interest lists, quality, performance and compliance.
Fee-for-Service (FFS)	• New public reporting requirements for FFS Medicaid fee schedule rates and benchmarking of Medicare service rates. • New requirement to show sufficient access to care when submitting state plan amendments with rate reductions.
Website Transparency	• Codifies requirements on accessibility, architecture (how many clicks to get to information), quarterly verification that links are working and prominent placement of information on how to obtain language assistance.

Summary of Major Managed Care Rule Changes*

Key Topics	Managed Care Rules
Timely Access to Care	• New maximum appointment wait times and compliance standards. • New requirements for secret shopper surveys, member experience surveys, provider directory accuracy, and a single, easy-to-use public webpage. • Requires annual comparisons of managed care provider rates to Medicare, or Medicaid FFS for direct care services.
State Directed Payments (SDP)	• Changes to contract requirements, evaluations, performance measures, reporting, submission timelines, and payment and payment reconciliation.
In-Lieu-of Services and Settings (ILOSs)	• Codifies specific ILOSs requirements and requires ongoing monitoring and an evaluation after five years if ILOSs costs exceed 1.5 percent.
Medical Loss Ratios	• New reporting and contract requirements for provider incentive payments. • Managed care plans must report overpayments to states within 30 days.
Quality Strategy and External Review	• Adds new External Quality Review Organization (EQRO) evaluations and changes due date for the EQRO technical report. • Adds requirements for public comment, website posting and frequency of update to the state's quality strategy.
Quality Rating System (QRS)	• Defines QRS measures and methodology and offers CMS-approved options. • Requires a new interactive QRS website with specific comparative measures.
CHIP Alignment with Medicaid	• Requires CHIP alignment on nearly all access-related provisions except for those related to State Directed Payments and dually eligible members.

*This is a high-level summary only. New rules have extensive details not captured in the above chart.

Appendix B

Texas Medicaid and CHIP services

Medicaid State Plan

Mandatory and Optional Services

The Social Security Act specifies a set of benefits that state Medicaid programs must provide and a set of optional benefits that states may choose to provide. A state may choose to provide some, all or none of the optional benefits. Some optional services that Texas chooses to provide are available only to members (a) age 20 and younger, or (b) age 20 and younger or age 65 and older who are in an institution for mental disease. If the member is age 20 and younger, all federally allowable and medically necessary services must be provided.

Mandatory and optional state plan services provided by Texas Medicaid include:

B.1: Acute Care Services	
Mandatory	Optional*
• Inpatient hospital services • Outpatient hospital services • Laboratory and x-ray services • Physician services • Medical and surgical services provided by a dentist • Early and Periodic Screening, Diagnosis and Treatment services for people age 20 and younger • Family planning services and supplies • Federally Qualified Health Center services • Rural Health Clinic services • Nurse-midwife services • Certified pediatric and family nurse practitioner services • Home health services • Freestanding birth center services (when licensed or otherwise recognized by the state) • Transportation to medically necessary services • Tobacco cessation counseling for pregnant women • Extended services for pregnant women	• Prescription drugs • Medical or remedial care by other licensed practitioners: ▫ Nurse practitioners/certified nurse specialists ▫ Physician assistants ▫ Licensed midwives ▫ Certified registered nurse anesthetists ▫ Anesthesiologist assistants ▫ Psychologists ▫ Licensed clinical social workers** ▫ Licensed professional counselors ▫ Licensed marriage and family therapists ▫ Licensed behavior analysts • Podiatry*** • Limited chiropractic services • Optometry (including eyeglasses and contacts) • Telemedicine • Home telemonitoring • Hearing instruments and related audiology • Home health supplies provided by a pharmacy • Clinic services: ▫ Maternity clinic services ▫ Renal dialysis facility services ▫ Ambulatory surgical center services • Tuberculosis clinic services • Peer specialist services • Rehabilitation and other therapies: ▫ Mental health rehabilitative services ▫ Rehabilitation and other therapy services

B.1: Acute Care Services	
Mandatory	Optional*
	▫ Substance use disorder treatment ▫ Physical, occupational and speech therapy ▫ Autism services, to include Applied Behavior Analysis • Case management services for pregnant women with high-risk conditions • Pregnancy-related and postpartum services for 12 months after the pregnancy ends • Services for any other medical conditions that may complicate pregnancy • Respiratory care services • Ambulance services • Emergency hospital services • Private duty nursing • Prescribed Pediatric Extended Care Centers

B.2: Long-Term Services and Supports	
Mandatory	Optional*
• Nursing facility services for clients age 21 and older	• Intermediate care facility services for individuals with an intellectual disability or related condition (ICF/IID) • Inpatient services for members who are in an institution for mental disease and who are either age 21 and younger or age 65 and older • Services furnished under a Program of All-Inclusive Care for the Elderly • Day Activity and Health Services • 1915(i) Home and Community-Based Services-Adult Mental Health • 1915(k) Community First Choice services: ▫ Attendant care (including habilitation) ▫ Emergency response services • Attendant services: ▫ Personal care/assistance services ▫ Community attendant services • Targeted case management for: ▫ Infants and toddlers with intellectual or developmental disabilities ▫ Adults with intellectual or developmental disabilities ▫ People with chronic mental illness • Nursing facility services for people age 20 and younger • Services provided in religious non-medical health care institutions

*Includes optional Medicaid services provided in Texas. Does not include all optional services allowed under federal policy.

**Except when delivered in a Federally Qualified Health Center setting.

***Except when delivered by a medical doctor or doctor of osteopathic medicine.

CHIP State Plan

States can design their Children’s Health Insurance Program (CHIP) using one of three frameworks established by federal law. States may include their CHIP plan within their Medicaid program, create a separate CHIP plan, or create a hybrid of these two options. Texas CHIP operates as a separate program, but many CHIP services are the same as Texas Medicaid services.

Services provided by Texas CHIP include:

B.3: Services Covered by Texas CHIP

- Surgical services
- Transplants
- Skilled nursing facilities (including rehabilitation hospitals)
- Outpatient hospital, comprehensive outpatient rehabilitation hospital, clinic (including health center) and ambulatory health care center services
- Physician/physician extender professional services (including well-child exams and preventive health services, such as immunizations)
- Laboratory and radiological services
- Durable medical equipment, prosthetic devices and disposable medical supplies
- Home and community-based health services
- Federally Qualified Health Center services
- Rural Health Clinic services
- Nursing care services
- Inpatient mental health services
- Outpatient mental health services
- Inpatient and residential substance abuse treatment services
- Outpatient substance abuse treatment services
- Rehabilitation and habilitation services (including developmental assessments and physical, occupational and speech therapy)
- Hospice care services
- Emergency services (including emergency hospitals, physicians and ambulance services)
- Emergency medical transportation (ground, air or water)
- Care coordination
- Case management
- Prescription drugs
- Dental services
- Vision
- Chiropractic services
- Tobacco cessation

Home and Community-Based Waiver Programs

Federal law allows states to apply to the Centers for Medicare & Medicaid Services for permission to depart from certain Medicaid requirements. These waivers allow states to develop creative alternatives to the traditional Medicaid program. The table below provides a list of Texas Medicaid waiver programs.

B.4: Texas Medicaid Waivers		
Waiver	Description	Services Covered
Community Living Assistance and Support Services (CLASS)	CLASS provides home and community-based services to people who have related conditions as an alternative for placement in an ICF/IID. A related condition is a disability other than an intellectual or developmental disability which originates before age 22 and substantially limits life activity.	• Adaptive aids and minor home modifications • Medical supplies • Dental services • Nursing • Respite • Professional therapies* • Auditory integration/enhancement training • Specialized therapies*** • Employment assistance and supported employment • Case management • Prevocational services • Residential habilitation (transportation) • Support family services • Transition assistance services • Continued family services • Financial management services and support consultation** • Prescribed drugs (extended state plan services)****

B.4: Texas Medicaid Waivers

Waiver	Description	Services Covered
Deaf Blind with Multiple Disabilities (DBMD)	DBMD provides home and community-based services as an alternative to residing in an ICF/IID to people of all ages who are deaf, blind, or have a condition that will result in deafblindness, and who also have an additional disability. The program focuses on increasing opportunities for people to communicate and interact with their environment.	• Adaptive aids and minor home modifications • Medical supplies • Dental services • Nursing • Respite • Professional therapies* • Employment assistance and supported employment • Case management • Residential habilitation (transportation) • Assisted living (1-3 bed and 4-6 bed homes) • Transition assistance services • Intervener • Orientation and mobility • Audiology services • Individualized skills and socialization • Chore services • Prescription drugs (extended state plan services)**** • Financial management services and support consultation**
Home and Community-based Services (HCS)	HCS provides home and community-based services as an alternative to residing in an ICF/IID to people of all ages who qualify for ICF/IID level of care, yet live in their family's home, their own home, a small HCS residential home or other setting in the community.	• Adaptive aids and minor home modifications • Dental services • Nursing • Respite • Professional therapies* • Employment assistance and supported employment • Individualized skills and socialization • Prescription drugs (extended state plan services)**** • Individualized skills and socialization • Residential assistance services • Supported Home Living (transportation) • Transition assistance services • Financial management services and support consultation**

B.4: Texas Medicaid Waivers

Waiver	Description	Services Covered
Medically Dependent Children Program (MDCP)	MDCP provides community-based services to children and youth age 20 and younger as an alternative to residing in a nursing facility.	• Adaptive aids and minor home modifications • Respite • Employment assistance and supported employment • Flexible family support services • Transition assistance services • Financial management services
STAR+PLUS Home and Community Based Services (STAR+PLUS HCBS)	STAR+PLUS HCBS provides community-based services to adults age 21 and older with disabilities and people age 65 and older as an alternative to residing in a long-term care facility. These services are delivered through the STAR+PLUS managed care program.	• Adaptive aids and medical supplies • Minor home modifications • Respite • Employment assistance and supported employment • Dental services • Adult foster care • Assisted living services • Emergency response services • Home-delivered meals • Transition assistance services • Personal assistance services • Nursing services • Professional therapies* • Cognitive rehabilitation therapy • Financial management services and support consultation**

B.4: Texas Medicaid Waivers		
Waiver	Description	Services Covered
Texas Home Living (TxHmL)	TxHmL provides selected services and supports as an alternative to residing in an ICF/IID to people of all ages who qualify for ICF/IID level of care who live in their own home or their family's home.	• Adaptive aids and minor home modifications • Dental services • Nursing • Respite • Professional therapies* • Employment assistance and supported employment • Prescription medication (extended state plan service)**** • Community support (transportation) • Individualized skills and socialization • Financial management services and support consultation**
Youth Empowerment Services (YES)	YES is a home and community-based waiver that allows for more flexibility in the funding of intensive community-based services for children and adolescents with severe emotional disturbances and their families.	• Adaptive aids and supports • Minor home modifications • Respite • Specialized therapies*** • Employment assistance and supported employment • Community living support • Family supports and supportive family-based alternatives • Non-medical transportation • Paraprofessional services • Transitional services

*Professional therapies may include physical therapy, occupational therapy, speech and language pathology, audiology, behavioral support, dietary services and cognitive rehabilitation therapy. Each waiver program may differ in what therapies are available.

**These services are only available to people using the Consumer Directed Services delivery option.

***Specialized therapies may include: massage therapy, hippotherapy, therapeutic horseback riding, music therapy, aquatic therapy, recreational therapy, art therapy, animal-assisted therapy and nutritional counseling. Each waiver program may differ in what therapies are available.

****Allows access to unlimited prescriptions for people in the waiver program who do not already have access to unlimited prescriptions. People in managed care or under age 21 have access to unlimited prescriptions.

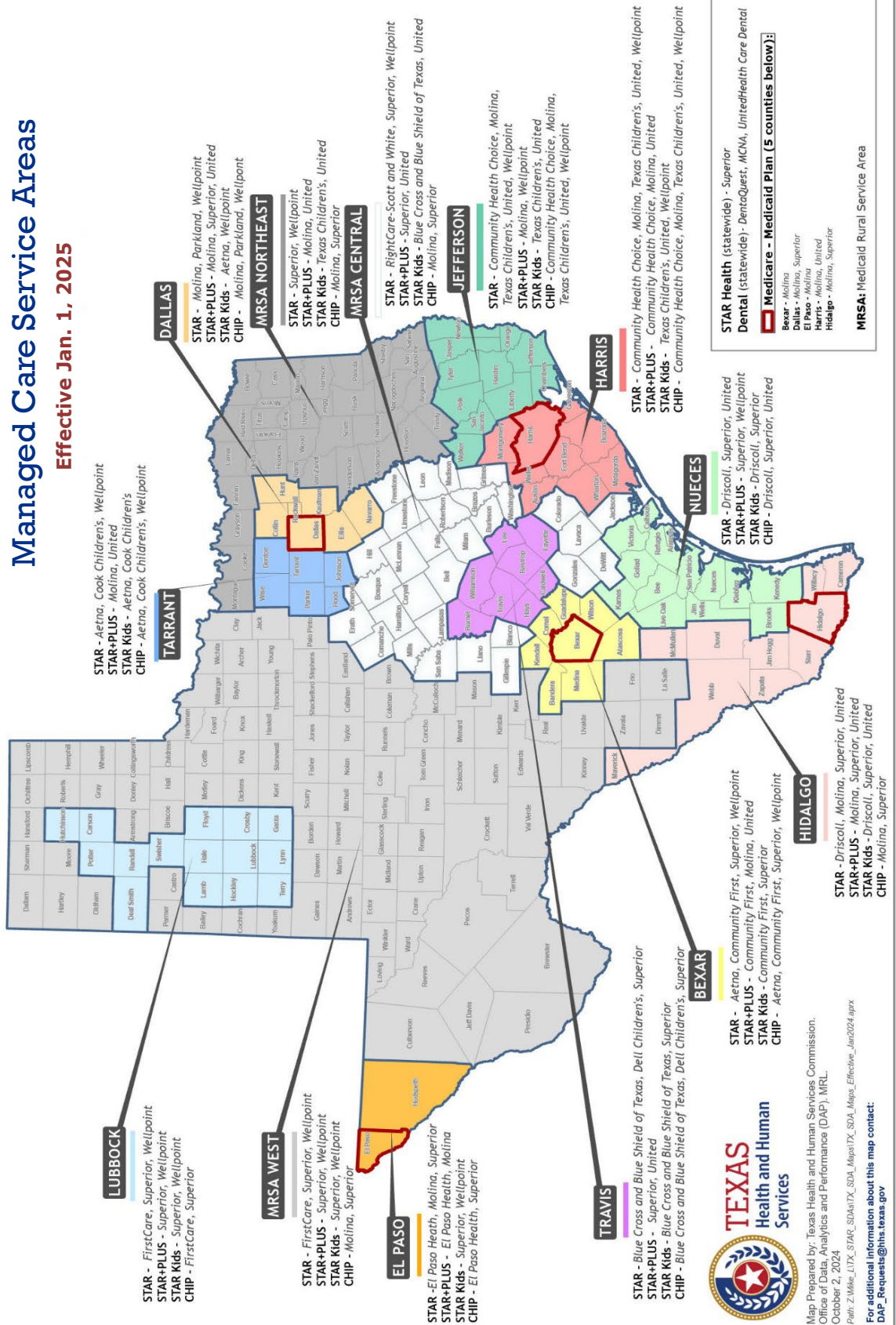
Appendix C

Texas Medicaid and CHIP map and figures

C.1: Texas Managed Care Services Delivery Map

Managed Care Service Areas

Effective Jan. 1, 2025

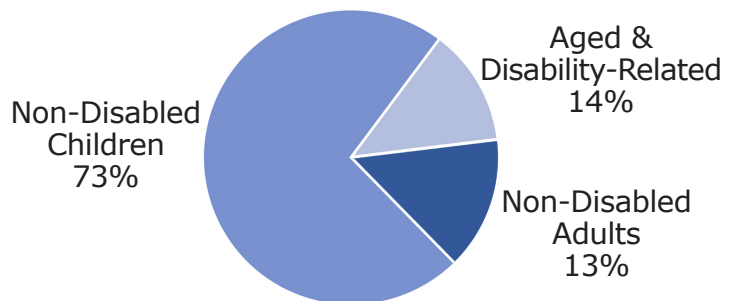


Demographics

The number of Texas Medicaid and Children’s Health Insurance Program (CHIP) clients can be expressed as a monthly average count or an annual unduplicated count. The monthly average count is the average number of clients on Medicaid or CHIP per month. The unduplicated count is the total number of individual Texans who received Medicaid or CHIP services over a period of time. In 2023, the monthly average count for Medicaid was approximately 5.7 million clients, and the monthly average count for CHIP was approximately 80,600 clients. This low number of CHIP clients is a result of the Public Health Emergency’s federal requirements to continue Medicaid coverage for all recipients, even if their eligibility changed. This prevented states from reducing or ending Medicaid coverage regardless of changes in a recipient’s circumstances. As a result, the number of individuals receiving Medicaid assistance significantly increased and those receiving CHIP assistance decreased.

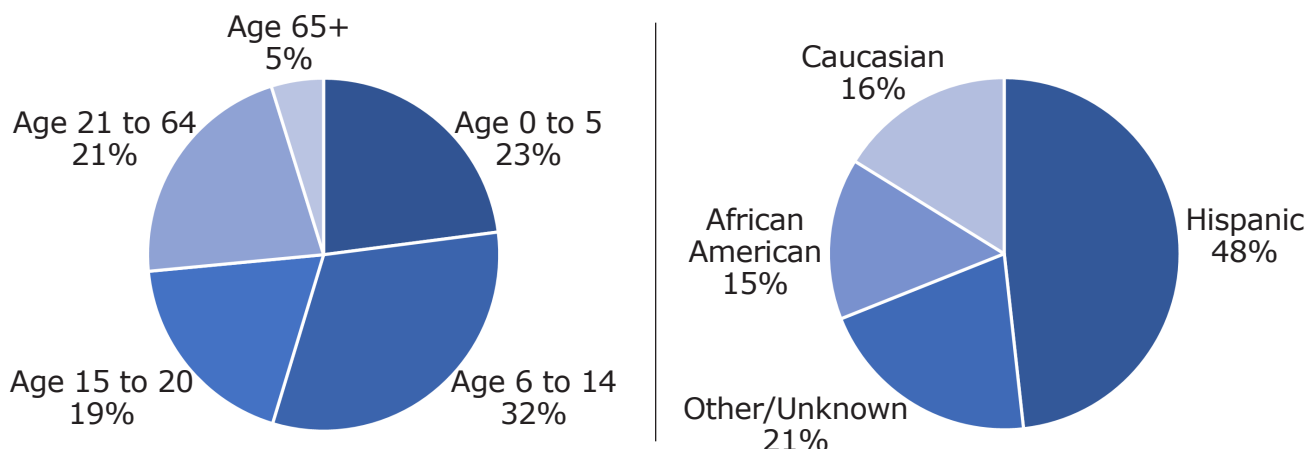
In 2023, about 14 percent of Texas Medicaid clients were recipients due to age and disability-related reasons, 13 percent were non-disabled adults and 73 percent were non-disabled children. CHIP covers children, birth through age 18, who are not eligible for Medicaid due to income but cannot afford private health insurance.

C.2: Medicaid Recipients by Age and Disability Reason, 2023



The following figures break down the total statistics for unduplicated clients who received Medicaid in 2023 by age group and ethnicity. As previously indicated, most Medicaid clients in any given year are children. In addition, Hispanics accounted for the largest portion of Medicaid clients, comprising 48 percent of the population.

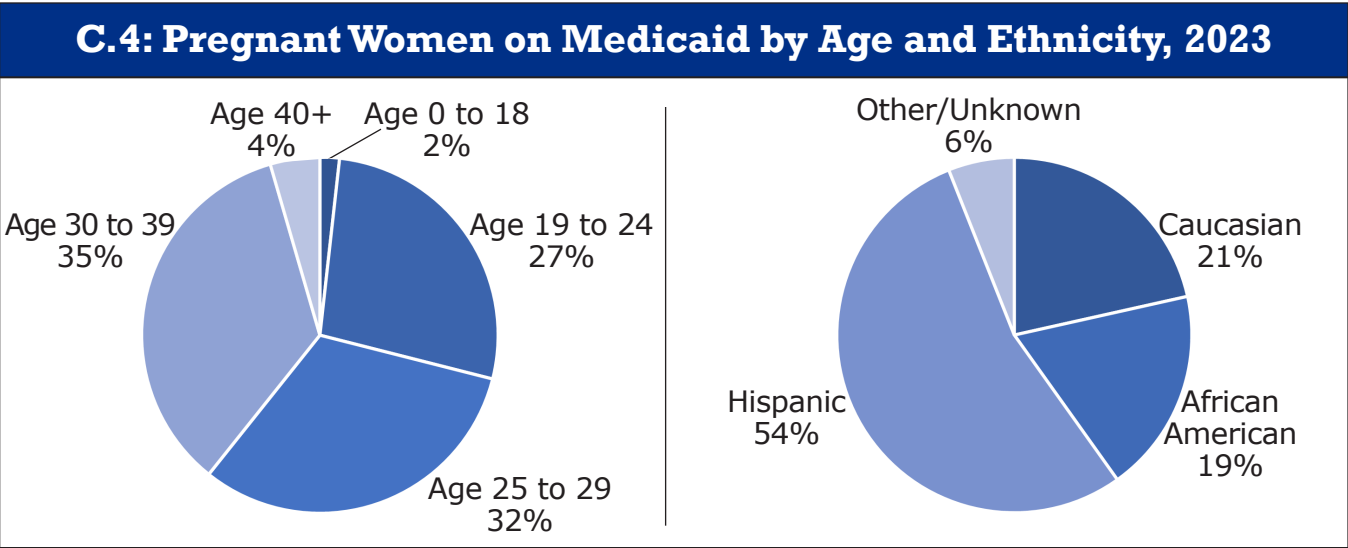
C.3: Medicaid Recipient by Age and Ethnicity, 2023



Meanwhile, for CHIP: 11 percent of CHIP clients were age 5 and younger, 63 percent were age 6 through age 14, and 26 percent were age 15 through age 18.

Medicaid covers low-income women for pregnancy-related services and newborns in low-income families. Pregnant women make up 9 percent of the Texas Medicaid and CHIP caseload. The following figures illustrate the distribution of pregnant women enrolled in Medicaid by age group and ethnicity. Fifty-nine percent of the pregnant women in the Texas Medicaid program are between age 19 and age 29, while only 2 percent are age 18 and younger. Reflective of the Medicaid population as a whole, just over half, or 54 percent, of pregnant women receiving Medicaid are Hispanic.

Of the pregnant women enrolled in Texas Medicaid or CHIP in 2023, 95 percent receive services from Medicaid. The remaining 5 percent receive services from CHIP Perinatal.



CHIP Perinatal covers the unborn children of pregnant women who do not qualify for Medicaid due to income but cannot afford private health insurance. In 2023, the monthly average caseload for CHIP Perinatal was 26,997.

Beginning in 2010, newborns under 185 percent of the federal poverty level (FPL) began moving out of CHIP Perinatal into Medicaid due to changes in eligibility. The Affordable Care Act (ACA) changed this income limit to 198 percent of the FPL effective January 2014 (see Appendix A, page 124).

The following table shows the total average monthly caseload for the CHIP Perinatal population from 2011 through 2023 — including the number of perinates and newborns below 185 percent of the FPL prior to the 2014 change and the number of perinates and newborns below 198 percent of the FPL after the 2014 change.

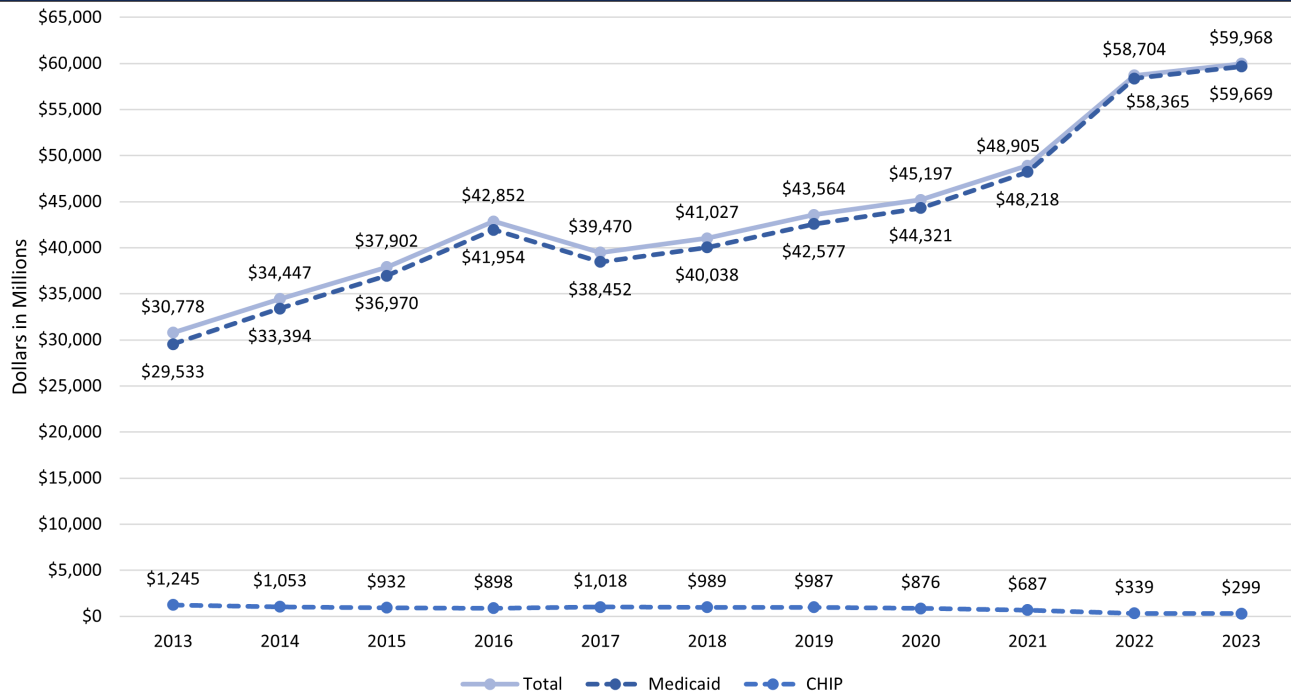
C.5: CHIP Perinatal Caseload Summary, 2011–2023

State Fiscal Year	Total Caseload	Perinates 0%–184% FPL	Perinates 185%–200% FPL	Newborns 0%–184% FPL	Newborns 185%–200% FPL
2011	44,214	36,775	546	6,582	310
2012	37,190	36,238	652	0	300
2013	37,027	36,068	640	0	319
State Fiscal Year	Total Caseload	Perinates 0%–197% FPL	Perinates 198%–202% FPL	Newborns 0%–197% FPL	Newborns 198%–202% FPL
2014*	36,800	36,203	509	0	88
2015	36,535	35,703	742	0	90
2016	35,071	34,638	329	0	104
2017	34,458	33,969	389	0	100
2018	32,696	32,158	417	0	121
2019	30,856	30,272	445	0	139
2020	28,670	28,134	358	0	178
2021	25,916	25,524	263	0	129
2022	27,716	27,397	264	0	55
2023	26,997	26,713	219	0	65

*ACA income limit changes effective Jan. 1, 2014.

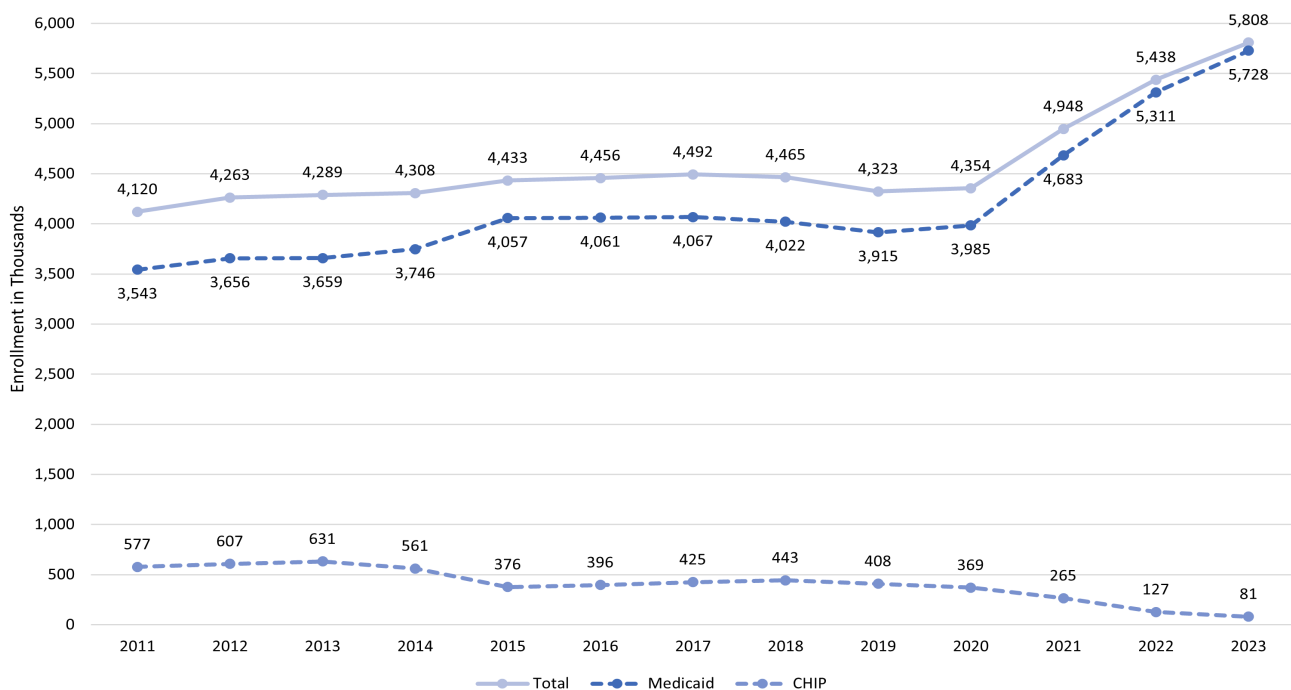
Budget and Enrollment

C.6: Annual Expenditures for Medicaid and CHIP, 2013-2023



Data is for FFYs 2013-2023.

C.7: Average Monthly Medicaid and CHIP Enrollment, SFYs 2011-2023



Caseloads impacted due to federal PHE.

C.8: Medicaid Expenditure History, FFYs 2018–2023

Federal Fiscal Year	Payer	Grant Benefits	Dispropor- tionate Share Hospital	Uncompen- sated Care	Upper Payment Level	DSRIP	Clawback	MCHIP	Admin- istration	Survey and Certification	Total Medicaid
2023	FED	29,698,214,302	1,297,458,302	2,996,380,169	--	57,164,858	--	531,818,960	1,164,373,760	29,271,449	35,774,681,800
2023	FED-ARRA	--	--	--	--	--	--	--	4,160,752	--	4,160,752
2023	FED-COVID	2,482,165,141	125,165,112	244,868,659	--	5,191,392	--	--	--	--	2,857,390,305
2023	NONFED	17,072,287,749	741,529,513	1,762,370,151	--	31,315,079	502,005,845	173,581,102	740,217,385	9,757,150	21,033,063,974
2023	TOTAL	49,252,667,193	2,164,152,926	5,003,618,979	--	93,671,330	502,005,845	705,400,062	1,908,751,897	39,028,599	59,669,296,831
2022	FED	29,813,895,518	1,208,645,550	1,874,142,541	--	1,326,903,660	--	1,021,700,248	977,752,634	24,537,673	36,247,577,824
2022	FED-ARRA	--	--	--	--	--	--	--	4,145,700	--	4,145,700
2022	FED-COVID	2,824,259,985	123,243,055	191,030,657	--	135,309,255	--	--	--	--	3,273,842,952
2022	NONFED	15,052,625,465	655,902,608	1,015,966,436	--	720,194,421	435,545,121	306,151,861	645,259,465	8,179,224	18,839,824,599
2022	TOTAL	47,690,780,968	1,987,791,213	3,081,139,634	--	2,182,407,335	435,545,121	1,327,852,109	1,627,157,799	32,716,897	58,365,391,075
2021	FED	24,081,645,447	1,131,887,983	969,320,168	--	1,844,928,510	--	710,768,051	957,506,308	29,030,662	29,725,087,128
2021	FED-ARRA	--	--	--	--	--	--	--	8,663,541	--	8,663,541
2021	FED-COVID	2,375,881,093	113,505,723	98,244,081	--	185,109,398	--	--	--	--	2,772,740,295
2021	NONFED	12,422,968,086	585,343,759	517,017,707	--	954,826,982	395,729,425	205,411,647	620,498,480	9,676,887	15,711,472,974
2021	TOTAL	38,880,494,626	1,830,737,465	1,584,581,956	--	2,984,864,890	395,729,425	916,179,698	1,586,668,330	38,707,549	48,217,963,939
2020	FED	20,285,815,677	1,173,783,843	2,308,350,002	--	1,756,478,472	--	517,172,436	902,797,147	31,732,309	26,976,129,886
2020	FED-ARRA	--	--	--	--	--	--	--	8,215,900	--	8,215,900
2020	FED-COVID	1,496,152,433	87,268,951	242,682,786	--	182,814,252	--	--	--	--	2,008,918,422
2020	NONFED	11,151,989,333	704,874,796	1,406,065,047	--	1,002,620,058	388,870,566	74,741,357	587,743,987	10,587,588	15,327,492,732
2020	TOTAL	32,933,957,443	1,965,927,590	3,957,097,835	--	2,941,912,782	388,870,566	591,913,793	1,498,757,034	42,319,897	44,320,756,940
2019	FED	18,648,454,476	1,135,454,021	2,087,928,703	--	1,489,493,368	--	527,505,019	880,280,799	34,755,055	24,803,871,441
2019	FED-ARRA	--	--	--	--	--	--	--	12,697,190	--	12,697,190
2019	NONFED	13,261,592,251	815,280,033	1,517,288,370	--	1,070,166,468	481,830,280	35,391,653	567,268,631	11,589,907	17,760,407,593
2019	TOTAL	31,910,046,727	1,950,734,054	3,605,217,073	--	2,559,659,836	481,830,280	562,896,672	1,460,246,620	46,344,962	42,576,976,224
2018	FED	17,106,627,997	1,081,579,027	1,540,814,930	--	1,754,634,974	--	507,257,806	822,708,662	29,036,137	22,842,659,533
2018	FED-ARRA	--	--	--	--	--	--	--	22,003,400	--	22,003,400
2018	NONFED	12,777,021,171	819,932,090	1,174,000,086	--	1,330,166,318	489,795,321	40,162,197	532,639,351	9,976,953	17,173,693,487
2018	TOTAL	29,883,649,168	1,901,511,117	2,714,815,016	--	3,084,801,292	489,795,321	547,420,003	1,377,351,413	39,013,090	40,038,356,420

C.9: Medicaid Expenditure History, FFYs 2012–2017

Federal Fiscal Year	Payer	Grant Benefits	Dispropor- tionate Share Hospital	Uncompen- sated Care	Upper Payment Level	DSRIP	Clawback	MCHIP	Admin- istration	Survey and Certification	Total Medicaid
2017	FED	15,940,244,087	1,017,299,170	1,540,241,906	--	1,604,753,947	--	733,099,118	924,173,410	31,011,499	21,790,823,137
2017	FED-ARRA	9,009,355	--	--	--	--	--	--	42,830,813	--	51,840,168
2017	FED-ACA	(41,561)	--	--	--	--	--	--	--	--	(41,561)
2017	NONFED	12,290,313,332	793,261,567	1,198,100,572	--	1,251,691,974	462,902,271	60,944,177	540,679,435	11,292,410	16,609,185,738
2017	TOTAL	28,239,525,213	1,810,560,737	2,738,342,478	--	2,856,445,921	462,902,271	794,043,295	1,507,683,658	42,303,909	38,451,807,482
2016	FED	16,020,648,741	1,680,300,179	3,513,327,443	--	1,510,079,934	--	406,135,213	920,274,804	31,487,209	24,082,253,523
2016	FED-ARRA	9,730,750	--	--	--	--	--	--	47,920,889	--	57,651,639
2016	FED-ACA	(5,829,324)	--	--	--	--	--	--	--	--	(5,829,324)
2016	NONFED	11,879,923,210	1,239,498,723	2,582,312,821	--	1,133,154,677	406,131,303	31,185,078	536,844,295	11,224,325	17,820,274,432
2016	TOTAL	27,904,473,377	2,919,798,902	6,095,640,264	--	2,643,234,611	406,131,303	437,320,291	1,505,039,988	42,711,534	41,954,350,270
2015	FED	15,442,364,936	1,367,834,218	2,080,629,579	--	1,432,632,157	--	297,843,117	886,094,144	22,605,040	21,530,003,191
2015	FED-ARRA	--	--	--	--	--	--	--	86,157,065	--	86,157,065
2015	FED-ACA	106,855,855	--	--	--	--	--	--	--	--	106,855,855
2015	NONFED	10,789,116,883	970,120,562	1,466,393,531	--	1,035,305,295	370,092,170	123,939,980	484,172,478	7,535,012	15,246,675,911
2015	TOTAL	26,338,337,674	2,337,954,780	3,547,023,110	--	2,467,937,452	370,092,170	421,783,097	1,456,423,687	30,140,052	36,969,692,022
2014	FED	14,982,927,389	905,058,004	1,451,339,911	--	1,348,925,140	--	117,437,709	737,229,930	19,236,614	19,562,154,776
2014	FED-ARRA	--	--	--	--	--	--	--	146,532,831	--	146,532,831
2014	FED-ACA	101,838,600	--	--	--	--	--	--	--	--	101,838,600
2014	NONFED	10,027,061,826	621,178,089	997,553,985	--	949,464,943	372,286,605	47,794,335	562,215,287	6,412,204	13,583,967,274
2014	TOTAL	25,111,827,814	1,526,236,093	2,448,893,976	--	2,298,390,083	372,286,605	165,232,044	1,445,978,048	25,648,818	33,394,493,481
2013	FED	14,209,243,885	133,812,750	1,885,406,398	--	287,667,948	--	28,657,109	646,050,231	22,731,210	17,213,569,531
2013	FED-ARRA	(2,722,794)	--	--	--	--	--	--	185,694,886	--	182,972,092
2013	FED-ACA	82,774,190	--	--	--	--	--	--	--	--	82,774,190
2013	NONFED	9,540,324,425	92,935,191	1,325,138,105	--	197,438,203	376,596,140	11,728,774	502,399,429	7,577,070	12,054,137,337
2013	TOTAL	23,829,619,706	226,747,941	3,210,544,503	--	485,161,151	376,596,140	40,385,883	1,334,144,546	30,308,280	29,533,453,150
2012	FED	13,773,419,669	882,595,210	--	1,400,669,919	--	--	27,808,056	651,723,257	22,796,293	16,759,012,404
2012	FED-ARRA	6,301,635	--	--	12,501,171	--	--	--	209,039,020	--	227,841,826
2012	NONFED	9,833,604,615	633,370,455	--	981,318,763	--	344,689,503	11,494,337	549,535,172	7,598,764	12,361,311,609
2012	TOTAL	23,613,025,919	1,515,965,665	--	2,394,489,853	--	344,689,503	39,302,393	1,410,297,449	30,395,057	29,348,165,839

C.10: Medicaid Expenditure History, FFYs 2010–2011

Federal Fiscal Year	Payer	Grant Benefits	Dispropor- tionate Share Hospital	Uncompen- sated Care	Upper Payment Level	DSRIP	Clawback	MCHIP	Admin- istration	Survey and Certification	Total Medicaid
2011	FED	14,093,145,087	956,328,092	--	1,829,711,562	--	--	20,859,639	589,699,290	23,918,822	17,513,662,492
2011	FED-ARRA	1,395,373,748	--	--	231,747,606	--	--	--	167,790,509	--	1,794,911,863
2011	NONFED	7,741,106,553	622,813,407	--	977,218,223	--	277,468,044	7,982,215	490,315,493	7,912,940	10,124,816,875
2011	TOTAL	23,229,625,388	1,579,141,499	--	3,038,677,391	--	277,468,044	28,841,854	1,247,805,292	31,831,762	29,433,391,230
2010	FED	12,670,015,218	991,515,974	--	1,849,499,135	--	--	--	585,452,634	23,347,881	16,119,830,842
2010	FED-ARRA	2,599,216,338	--	--	366,322,520	--	--	--	1,368,805	--	2,966,907,663
2010	NONFED	6,231,480,571	696,673,993	--	925,963,561	--	188,351,774	--	513,545,911	7,782,627	8,563,798,437
2010	TOTAL	21,500,712,127	1,688,189,967	--	3,141,785,216	--	188,351,774	--	1,100,367,350	31,130,508	27,650,536,942

Appendix D

Texas Medicaid and CHIP Roadmaps

About these Roadmaps



Over four million Texans receive services through Medicaid and CHIP, representing 15% of the state population. The mission of Texas Medicaid is to deliver quality, cost-effective services to improve the health and well-being of people across the state.

The following core goals were identified to support this mission:

- ✓ Ensure members have timely access to the services they need.
- ✓ Encourage providers to participate in the Medicaid program.
- ✓ Incentivize value using innovation in the service delivery model.
- ✓ Strengthen partnerships through transparency and accountability.

The roadmaps do not encompass all the work that Texas Medicaid does but provide a big-picture perspective on agency priorities shaped by direction from state leadership and collaboration with a diverse portfolio of stakeholders. They highlight steps to meet the mission in the following key categories: Pediatric Health, Older Adults & Adults with Disabilities, Women's Health, Behavioral Health, Value-based Care and Business Operations.

These are living documents that are subject to change as the needs of members shift or state and federal requirements evolve. Through inclusion of recent accomplishments, targeted future milestones and measures that demonstrate progress in reaching our destinations, the roadmaps provide perspective on where Texas Medicaid has been, where it is now and the direction of the path forward.

Pediatric Health



MCS Goal

Ensure members have timely access to the services they need.

Destination

Infants, children and youth getting the care and services they need to reach their fullest potential.

Enhanced children's dental preventive services policy launches

This policy adds caries arresting medicament, a minimally invasive therapy for treatment of dental cavities to prevent them from growing and spreading.

Sept.
2023

HHSC implements payment rate increase

This is the first provider rate increase for pediatric evaluation and management services in approximately 20 years.

Sept.
2023

Youth Empowerment Services (YES) waiver renews for five years

YES is a Medicaid waiver program that provides intensive community-based services for children and youth who have severe mental, emotional or behavioral disturbances, and their families.

April
2023

Texas Health Steps Online Provider Education Platform is enhanced

Texas Health Steps' award-winning Online Provider Education program collaborated with experts to develop over 50 targeted trainings and free continuing education courses for pediatric providers.

2023



START

Children and adolescents make up 76% of the Medicaid caseload

- 51% of Texas births are covered by Medicaid.
- 47% of Texas children on Medicaid or CHIP.
- In 2017, STAR Kids becomes a managed care program serving 164,723 children and youth with disabilities.

2021

HHSC implements enhancements to developmental screenings

Items added to screenings during Texas Health Steps visits include "Survey of Well Being of Young Children" and "Rapid Assessment for Adolescent Preventive Services".

Progress Indicators

Quality Measures



- Asthma medication ratio
- Upper respiratory infection treatment
- Weight assessment & counseling
- Caregiver Experience of Care survey

Access Measures



- Well-child visits
- Immunization status
- Oral evaluation – dental services
- SK SAI: Services & Coordination

HHSC implements federally required continuous eligibility for children

Eligible children under 19 years of age now receive 12 months of continuous eligibility, regardless of changes in income.

Jan.
2024

Pediatric dental coverage expands to support special health care needs

Enhanced facility services for dental procedures for a patient who requires monitored anesthesia is now a benefit. This increases access to care for young children and special health care needs members who require dental anesthesia to receive dental care.

Sept.
2024

HHSC is selected to participate in CMS Infant Well-Child Affinity Group

HHSC supported MCOs with quality improvement projects that increased attendance rates of well-child visits for children ages 0-15 months.

2022

Comprehensive Health Homes for Integrated Care (CHIC) Kids Pilot Program launches

HHSC partnered with MCOs and specialty clinics to implement a pilot program in STAR Kids to provide coordinated care for children with complex medical conditions through a health home.

Dec.
2022

HHSC implements Medicaid Autism Services policy

The policy includes services, such as applied behavior analysis, that promote a holistic approach to supporting children and families impacted by autism spectrum disorder.

Feb.
2022

2022

Streamlined STAR Kids Screening and Assessment Instrument (SK-SAI) begins

The revised SK-SAI is more efficient and better captures the needs of children and youth enrolled in the STAR Kids program.

2022

Medical Pay-for-Quality (P4Q) program expands

The P4Q program now includes STAR Kids.

Older Adults & Adults with Disabilities



MCS Goal

Ensure members have timely access to the services they need.

Destination

Increased choice and control for greater independence and a better quality of life.

New STAR+PLUS contracts are operational

The STAR+PLUS contracts awarded by HHSC to seven MCOs, including four current MCOs and three new MCOs, take effect. The new contracts enhance service coordination, service planning and the transition process for youth aging into STAR+PLUS.

Prevocational services are added to certain Medicaid waiver programs

Employment readiness is added to the Home and Community-based Services (HCS), Texas Home Living (TxHmL) and Deaf Blind with Multiple Disabilities (DBMD) waiver programs to support people's progression toward their employment goals.

Managed care person-centered planning is enhanced

To enhance managed care person-centered planning practices and comply with federal requirements, HHSC implements measures to assess whether MCOs providing or coordinating LTSS are delivering high-quality, person-centered care.

High-impact initiatives enhance member autonomy and access

- Since 2007, because of Promoting Independence initiatives, 14,670 people have transitioned from institutions to home and community-based services (HCBS).
- 2014: Texas begins delivering certain long-term services and supports (LTSS) programs statewide through managed care.
- 2017: Quality Incentive Payment Program (QIPP) for nursing facilities and utilization reviews launch.

March
2023

New Individualized Skills and Socialization service launches

This new service replaces day habilitation services in the waiver programs. The service provides person-centered activities, assists with personal care needs and medications, and helps people learn new skills or practice existing skills while exploring their community.

START

Progress Indicators

Quality Measures



- Money Follows the Person tracking
- QIPP performance measures
- Aging & Disability survey metrics related to autonomy and fulfillment

Service Measures



- Access: Network adequacy
- Delivery: Utilization review results

Medicaid wraparound benefits for dually eligible people in managed care are streamlined

Fee-for-service wraparound services are transitioned into managed care for dually eligible people enrolled in managed care. This improves claims processing for providers and care coordination for members.

2025

Integrated Dual Eligible Special Needs Plans (D-SNPs) transition starts

The Dual Demonstration managed care program ends, and certain Medicare-Medicaid Plans will transition to an integrated D-SNP model.

2026

HHSC implements the final electronic visit verification (EVV) requirements of the federal 21st Century Cures Act

Following the Jan. 1, 2021, implementation of all personal care services, HHSC implements the use of EVV for home health care services.

Jan.
2024

Centers for Medicare & Medicaid Services (CMS) approves implementation of a Medically Fragile Program

The program ensures that people who are medically fragile and are receiving STAR+PLUS HCBS can receive existing HCBS services that exceed the program cost limit. Implementation is scheduled through March 2025.

Nov.
2023

March
2023

Sept.
2023

The questionnaire for people on waiver program interest lists is improved

The revised questionnaire captures new information to help HHSC better determine the types of services an individual needs and when the services are needed.

The base wage for attendants increases

Increases in attendant rates support services that are non-technical, medically related, or personal care services in home or community living settings.

Women's Health



MCS Goal

Ensure members have timely access to the services they need.

Destination

Seamless care throughout each phase of life.

Expand network of case management providers for CPW Services

Community Health Workers and doulas will be new providers of Medicaid case management for CPW services.

Improvement projects target on maternal and childhood health outcomes

MCOs must create Performance Improvement Projects (PIPs) and report on their progress annually. Each PIP lasts two years. 2024 PIPs focus on:

- Reducing the rate of severe maternal morbidity among deliveries with preeclampsia.
- Reducing the rate of c-sections in low-risk pregnancies and deliveries.
- Increasing the rate of behavioral health follow-up care.

2024

2024

Implement a new nonmedical health-related screening for pregnant women

MCOs will screen all pregnant women who opt-in for nonmedical health-related needs, then coordinate services and referrals.

2024

Dec.
2019

START

Opportunity for impact started with the STAR Program for Pregnant Women

Services are provided through managed care.

- Pregnant women make up about 4.3% of Texas Medicaid and CHIP caseload (~185,000 of average monthly).
- 53% of all Texas births are covered by Medicaid.

HHSC awarded federal funding to implement the Maternal Opioid Misuse (MOM) Model

The MOM Model aims to improve quality of care for Medicaid-eligible pregnant and postpartum women with opioid use disorder. Participating clinics ensure access to treatment during and after pregnancy, and coordinate with postpartum treatment programs, peer coaches and physicians to ensure continuity of care.

Progress Indicators

Quality Measures



- Prenatal and postpartum care
- Preventative screenings
- Birth outcomes

Access Measures



- Network adequacy
- Appointment availability

Value-based payment programs strengthen focus on maternal health

Pay-for-Quality (P4Q) program and newly established Alternative Payment Models (APM) Performance Framework have incentives for improved maternal health outcomes:

- 2024 and 2025 STAR P4Q includes at-risk measures on prenatal and postpartum care.
- STAR MCOs that implement or sustain Maternal Medical Homes or other similar APMs receive additional credit toward meeting the overall APM contract requirement.

2024

2025

A postpartum depression treatment network

HHSC is collaborating with MCOs and HTW providers to develop a PPD treatment network for women enrolled in these programs. This network will help Medicaid providers refer women with PPD symptoms to mental health specialists, improving access to care and care continuity.

Postpartum Medicaid and CHIP coverage is now 12 months

Postpartum coverage expanded to add an additional 10 months, going from two months to 12 months for eligible women.

March
2024

Case management for children and pregnant women (CPW) managed care carve-in

CPW is carved-in to managed care, improving care coordination for children and pregnant women.

Sept.
2022

Jan.
2020

Sept.
2020

Healthy Texas Women (HTW) becomes a Medicaid program

Centers for Medicare & Medicaid Services (CMS) approves the HTW 1115 Demonstration Waiver. HHSC uses the waiver to enhance women's health care services by increasing access to and participation in the HTW program.

HTW Plus launches

HTW Plus is a postpartum benefits package covering outpatient services for major physical and mental health conditions that contribute to maternal morbidity and mortality in the postpartum period. It also covers chronic conditions, such as asthma and diabetes.

Behavioral Health



MCS Goal

Ensure members have timely access to the services they need.

Destination

Integrated behavioral health care when and where it's needed.

Increase access with in-lieu-of services and settings

MCOs will begin offering new medically appropriate, cost-effective services in-lieu-of mental health or SUD services specified in the Medicaid State Plan. These services include partial hospitalization, intensive outpatient and coordinated specialty care services.

HHSC encourages primary and behavioral health integration through Alternative Payment Models (APMs)

A newly updated MCO APM Framework encourages MCOs to establish APMs that integrate primary and behavioral health care, using collaborative care codes.

2024

2024

Certain behavioral health teleservices are permanently available

HHSC permanently extends behavioral health telehealth flexibilities that were initially allowed during public health emergency, further enhancing timely access to services.

Sept.
2022

New Collaborative Care Model implements

This new Medicaid State Plan benefit integrates behavioral health services with primary care providers, allowing them to manage behavioral health conditions (mental health or substance use) as chronic diseases rather than treating acute symptoms alone.

June
2022

Mental Health Parity and Addiction Equity Act of 2008 increases focus on integrated care

Additionally, HHSC:

- Implements Substance Use Disorder (SUD) benefit.
- Integrates Behavioral Health (BH) into managed care, including mental health benefits (mental health targeted case management and mental health rehabilitative services).

Sept.
2020

Healthy Texas Women (HTW) Plus launches

HTW is a postpartum benefits package covering outpatient services for major physical and mental health conditions that contribute to severe maternal morbidity and mortality in the postpartum period, including chronic conditions and perinatal mood and anxiety disorders like postpartum depression and substance use disorders.

START

Progress Indicators

Quality Measures



- Follow-up visits for critical BH incidents (ER, hospital)
- Effective treatment or medication initiation

Access Measures



- Network adequacy
- Appointment availability

Behavioral health quality measures now part of the Quality Incentive Payment Program (QIPP)

QIPP expands to include a behavioral health quality measure with a focus on depressive symptoms for all participating nursing facilities.

2024

HHSC addresses workforce shortage issues

HHSC will update the minimum requirements for qualified mental health professional-community services (QMHP-CS). QMHP-CSs deliver mental health targeted case management and most mental health rehabilitation state plan services.

2025

Directed Payment Program for Behavioral Health Services (DPP BHS) is established to support community mental health services

HHSC established the DPP BHS for community mental health centers and local behavioral health authorities to promote and improve access to behavioral health services, care coordination and successful care transitions for individuals enrolled in STAR, STAR+PLUS and STAR Kids. DPP BHS also promotes the Texas Certified Community Behavioral Health Clinic model of care.

Sept.
2021

Aug.
2021

HHSC implements uniform opioid policy

This policy aligns managed care and fee-for-service processes when prescribing opioids, encourages appropriate use of opioids and seeks to reduce over-prescribing. It includes claim review limitations, required retrospective drug utilization review activities, required monitoring of antipsychotic medications for children, and fraud and abuse identification.

June
2021

Early intervention and treatment services expand

Screening, Brief Intervention, and Referral to Treatment (SBIRT) services for adolescents and adults who have, or are at risk of developing, a substance abuse disorder are offered in community-based settings or hospitals. This expansion enhances care integration by allowing SBIRT services to be delivered in skilled nursing facilities, extended care facilities and intermediate care facilities.

Value-Based Care



MCS Goal

Incentivize value using innovation in the service delivery model.

Destination

Promoting high-value care for a healthier Texas.

HHSC adds more data to MCO report cards to increase transparency

In addition to survey data and relevant quality measures, MCO report cards now include complaints data. This allows members to make an even more informed decision when using this key tool for choosing a plan.

MCO Alternative Payment Model (APM) performance requirements are updated to a more comprehensive framework

HHSC develops contractual requirements with MCOs and DMOs that move away from a specific focus on APM targets and give MCOs credit for a broader range of work promoting value-based care.

HHSC provides a report on the development of benchmarks for MCOs

Pursuant to the 2022-23 General Appropriations Act, HHSC Rider 20, HHSC is required to develop and report on the development of quality of care and cost-efficiency benchmarks for Medicaid and CHIP MCOs.

The medical Pay-for-Quality (P4Q) program expands

In addition to STAR, STAR+PLUS, CHIP and a dental P4Q program, the P4Q program now includes STAR Kids.

Initiatives with a deliberate and measured focus on quality outcomes

- Centers for Medicare & Medicaid Services (CMS) approves the Delivery System Reform Incentive Payment (DSRIP) program under the 1115 Healthcare Transformation Waiver.
- Medicaid establishes quality-based payment initiatives, MCO performance measurement and reporting and the hospital quality-based payment program.

START

2023

2023

2022

2022

2019

Quality data on health outcomes is made available to the public

The Performance Indicator Dashboard is added to the quality measure results on the Texas Healthcare Learning Collaborative website, allowing users to compare MCO performance on key health outcome measures by program.

Progress Indicators

Partner Measures



- MCO: Pay-for-Quality, Alternative Payment Model (APM) benchmarks
- Direct Payment Program (DPP) metrics

Outcome Measures



- Overall reduction in potentially preventable events (PPE)

Non-Medical Drivers of Health (NMDOH) Action Plan launches

The NMDOH Action Plan outlines goals and actions to coordinate activities and support the work of MCOs and Medicaid providers addressing health-related social needs. The plan focuses on addressing food insecurity, housing instability and lack of transportation – all factors that can negatively impact a person's health.

Feb.
2023

An incentive program to expand the exchange and use of electronic health information implements

Aligning Technology by Linking Interoperable Systems (ATLIS) for Client Health Outcomes is designed to improve client outcomes and advance alternative payment models by requiring MCOs to formally assess in-network providers' Health Information Exchange Connectivity and incentivize exchange of actionable data.

2024

New directed payment programs start

As part of DSRIP transition, HHSC introduces four new directed payment programs in late 2021 and early 2022 to continue advancing quality and DSRIP best practices. The programs are for hospitals, physician groups, community mental health centers and rural health clinics.

March
2021

2020

HHSC receives CMS approval to request value-based drug manufacturer proposals

HHSC begins reviewing proposals from drug manufacturers that focus on tying payment to outcomes for drugs.

Value-Based Enrollment (VBE) Incentive program launches

HHSC uses a default methodology to enroll clients who do not select an MCO. Under the VBE program, MCOs that perform better on quality measures, member satisfaction and efficiency receive a greater share of default enrollments.

Dec.
2020

Business Operations



MCS Goal

Strengthen partnerships through transparency and accountability.

Destination

Reliable and efficient for the people we work with and in the work that we do.

Electronic Visit Verification (EVV) expands to cover additional services

To ensure compliance with federal law and reduce fraud, waste, and abuse, providers are now required to use EVV for Medicaid home health in addition to personal care services. Providers can use the state's EVV vendor or their own proprietary system.

HHSC modernizes technology for the Vendor Drug Program (VDP)

VDP transitions to a comprehensive vendor to manage pharmacy claims, rebate administration, formulary and preferred drug list management, and retro and prospective URs. This transition supports the goals of supporting better health, better care and lower costs.

HHSC implements Interest List Self-Service Portal

Individuals on a waiver interest list can check their status, update their contact information, complete a questionnaire or add themselves to an interest list through [YourTexasBenefits.com](https://www.texasbenefits.com).

Program providers move from the CARE system to Texas Medicaid & Healthcare Partnership (TMHP)

This move provides Home and Community-based Services (HCS) and Texas Home Living (TxHmL) program providers increased efficiencies, system-driven processes, and improved data capabilities. This transition allows for providers to become electronic data interchange submitters, resulting in less data entry and the ability to batch claims.



START

Texas Medicaid, a large complex organization committed to ongoing improvement

- 5.8 million clients served in Medicaid and CHIP*
- 6,000+ total contracts
- 140K enrolled providers
- 27 advisory committees and subcommittees

*Caseloads impacted due to federal PHE.

Feb.
2020

HHSC improves timely identification of issues from member complaints

Increased frequency of review and review of member-level complaints provides additional enhancements to oversight.

Jan.
2024

Aug.
2023

May
2022

March
2024

Progress Indicators

Partner Measures

- Timeliness of provider enrollment
- Complaint trends
- Stakeholder feedback

Accountability Measures

- Overall contract compliance
- Timeframe for closure of resolution for missed Key Performance Indicators (KPIs)

HHSC digitizes the waiver provider contract process

The Contract Application and Management for Providers site will allow Intellectual and Developmental Disability waiver providers to apply for and manage their contracts using a modernized digital interface rather than a paper process.

2025

HHSC streamlines provider enrollment and credentialing processes

Providers and facilities will be able to apply simultaneously for Medicaid and CHIP enrollment and MCO/DMO credentialing via PEMS+.

2025

External medical reviews (EMRs) now offered to managed care members as part of the state fair hearing process

An EMR is an optional step a member can take before a state fair hearing. Independent healthcare experts review and can uphold or overturn an MCO's decision to deny a benefit or reduce services.

May
2022

HHSC begins enhanced service coordination MCO oversight

MCOs start using the new service coordination managed care report, which includes data tracking on new members, newly identified members with special health care needs, service plan status and other data.

June
2022

Sept.
2020

HHSC removes the requirement for paper provider directories

STAR+PLUS and STAR Kids MCOs are no longer required to provide hard copies of provider directories to all members at enrollment except upon request. Online directories allow for searchability and can be kept more current.

Dec.
2021

Streamlined Provider Enrollment and Management System (PEMS) launches

PEMS is a National Provider Identifier-based system that is designed to provide efficiencies with provider enrollment and information management, including eliminating the need for a Texas Provider Identifier number.

Glossary



Abuse

Provider or member practices that result in unnecessary costs to Medicaid — including reimbursements for services that are not medically necessary or that fail to meet professionally recognized standards for health care, and actions that are inconsistent with sound fiscal, business or medical practices.

Accelerated Monitoring

A contractual non-compliance remedy that uses more frequent or extensive monitoring than standard monitoring practices. Examples of accelerated monitoring practices include additional reporting requirements, escalated corrective action plans (CAPs) and on-site reviews.

Access-Based Fee (ABF)

Under the fee-for-service (FFS) delivery model, access-based fees (ABFs) are a type of Medicaid rate used to pay physicians and other practitioners for services. Rates for physicians and other practitioners are uniform statewide and are categorized as either resource-based fees (RBFs) or ABFs. ABFs are calculated based on the following: historical charges, the current Medicare FFS rates, reviews of Medicaid fees paid by other states, surveys of providers' costs to deliver a service, and Medicaid fees for similar services. ABFs account for deficiencies in RBF methodology to ensure access to health care services for Medicaid members. Reimbursement rates for physician and other practitioner services are evaluated at least once every two years as part of a biennial fee review process. See Fee-for-Service (FFS) Rates and Resource-Based Fee (RBF).

Activities of Daily Living (ADLs)

Activities of daily living (ADLs) are activities essential to daily personal care — including bathing or showering, dressing, grooming, transferring, toileting and eating. Assistance with ADLs is part of attendant services, including personal care services (PCS) and personal assistance services (PAS). These services are available in several state plan and Medicaid waiver long-term services and supports (LTSS) programs, including Community Attendant Services (CAS), Primary Home Care (PHC), Day Activity and Health Services (DAHS), Community First Choice (CFC), and home and community-based services programs, including STAR+PLUS HCBS. See Long-Term Services and Supports (LTSS).

Acuity Risk Adjustment

Before capitation rates for managed care organizations (MCOs) are finalized, an acuity risk adjustment is made to reflect the health status, or acuity, of the population enrolled in each MCO. This adjustment anticipates the cost differential among multiple MCOs in a service area due to the variable health status of their respective memberships. See Capitation Rates.

Acute Care Services

Acute care services focus on preventive care, diagnostics and treatments. Acute care services covered by Medicaid include but are not limited to inpatient and outpatient hospital services, laboratory and x-ray services, and physician services. Medicaid covers acute care services for all clients. See Appendix B, page 156, for a full list of acute care services offered in Texas.

Acute Care Utilization Review (ACUR)

An Acute Care Utilization Review (ACUR) is designed to monitor managed care organizations (MCOs) and ensure they are authorizing, justifying and providing appropriate, medically necessary services to Medicaid members. For this review, a team of nurses conducts a desk review of a targeted sample set of medical records. This sample set is selected based on complaints, the severity and frequency of non-compliance instances, and the volume or cost of specific services. Nurses from the ACUR team conduct an in-depth review of the sample cases — including their authorization process, medical necessity determination, timeliness and accuracy of the resolution.

Administrative Expense Limits (Admin Cap)

The amount of allowable Medicaid administrative expenses by managed care organizations (MCOs) is confined by administrative expense limits (Admin Cap). The Admin Cap is compared to reported administrative expenses by the MCO. Any amounts over the Admin Cap become disallowed expenses and are added to the MCO's net income. MCOs are not prevented from incurring expenses that they consider necessary to the successful operation of their business. The Admin Cap only pertains to their ability to record those expenses on their financial statistical reports (FSRs). See Financial Statistical Report (FSR).

Agency Option

The agency option refers to one way Medicaid clients can receive certain long-term services and supports (LTSS). With the agency option, services are delivered through

a provider agency, which employs or contracts with attendants or other direct service workers and manages all employment and business operations. Either the MCO service coordinator, Local Intellectual and Developmental Disability Authority (LIDDA) service coordinator (for Home and Community-based Services [HCS] and Texas Home Living [TxHmL]), case management agency (for the Community Living Assistance and Support Services [CLASS] waiver program) or provider agency (for Deaf Blind with Multiple Disabilities [DBMD]) coordinates with the person or authorized representative to monitor and ensure clients are satisfied with their services. See Provider Agency.

Aging and Disability Resource Center (ADRC)

Aging and Disability Resource Centers (ADRCs) provide older adults, people with disabilities, and their family members with educational information about long-term services and supports (LTSS) and serve as points of access for LTSS programs. Each ADRC partners with a network of local service agencies to coordinate information and referrals for people needing access to both private and public LTSS programs and benefits, including Medicaid. There are 28 ADRCs operating throughout Texas.

Alberto N. v. Young

A federal lawsuit settled in May 2005 that requires the Health and Human Services Commission (HHSC) to comply with Title XIX of the Social Security Act (42 U.S.C. §1396 et seq.) by providing all medically necessary in-home Medicaid services to children age 20 and younger who are eligible for the Medicaid Early and Periodic Screening, Diagnosis and Treatment (EPSDT) program.

All Patient Refined Diagnosis Related Group (APR-DRG)

All Patient Refined Diagnosis Related Group (APR-DRG) comes from the Diagnosis Related Group (DRG) classification system, which provides a means of relating the type of patients a hospital treats to the costs incurred by the hospital. APR-DRGs expand on the basic DRGs to be more representative of non-Medicare patients and to incorporate severity of illness subclasses. See Diagnosis Related Group (DRG).

Alternative Payment Model (APM)

An alternative payment model (APM) is a payment approach that rewards providers for delivering high-quality, cost-efficient care. APMs link a percentage of the provider's overall payment to a measure of either quality or quality and cost. As part of building a strong quality (P4Q) program that incentivizes managed care organizations (MCOs) to achieve better performance, the Health and Human Services Commission (HHSC) developed contractual requirements minimum thresholds of MCOs' overall payments to health care providers be in the form of an APM. For a certain percentage of these payments, the provider must have some degree of risk, called the risk threshold. See Pay-for-Quality (P4Q).

Alternative Prospective Payment System (APPS)

Like prospective payment systems (PPSs), alternative prospective payment systems (APPSs) are methods of reimbursement in which payment is based on a predetermined, fixed amount based on the classification system of that service. However, where PPS rates are inflated annually for primary care using the Medicare Economic Index (MEI), APPS rates are inflated annually using 100.5 percent of the MEI. APPSs may be used by Federally Qualified Health Centers (FQHCs). If an FQHC chooses the APPS method, rates may be prospectively reduced to better reflect the reasonable costs or the PPS rates.

Amount, Duration and Scope

Amount, duration and scope are criteria used to define Medicaid benefits and limitations in a State's Medicaid plan. Each state defines these parameters, so state Medicaid plans vary in what they cover.

Applied Behavior Analysis

Applied behavior analysis (ABA) is a type of therapy commonly used for children diagnosed with autism spectrum disorder (ASD). ABA therapy helps people with autism improve their social skills and reduce negative behaviors. Beginning February 2022, ABA services became available to children and youth with ASD and receiving Medicaid through the Health Steps Comprehensive Care Program. ABA services are delivered by a licensed behavior analyst (LBA). See Texas Health Steps.

Appointment Availability

Appointment availability is a measurement used to assess member access to care within the managed care delivery system. It is measured by the time between when a member contacts a provider and the date of the first available appointment. Managed care organizations (MCOs) can reduce the use of emergent care by ensuring members have timely access to regular and preventive care. The Health and Human Services Commission (HHSC) contractually requires MCOs to follow appointment availability standards — for STAR, Children's Health Insurance Program (CHIP), STAR Kids, STAR Health and STAR+PLUS — based on the type of medical appointment requested. Texas' external quality review organization (EQRO) conducts secret shopper studies to evaluate MCO compliance with these availability standards. See Mystery Shopper Method.

Audits

An audit is an official inspection of performance or finances that is conducted by independent contractors or outside agencies, such as the Office of Inspector General (OIG). See also Office of Inspector General (OIG).

B

Behavioral Health Services

Behavioral health services refer to the treatment of mental health conditions and substance use disorders (SUDs). These services may be provided by the following: therapists in private practice, physicians, private and public psychiatric hospitals, community mental health centers, comprehensive provider agencies and chemical dependency treatment facilities. Treatment services for mental health conditions include: inpatient psychiatric services; psychiatric diagnostic evaluation with and without medical services; psychotherapy (individual, family and group); psychological, neurobehavioral and neuropsychological testing services; mental health targeted case management and mental health rehabilitative services; electroconvulsive therapy; medication management; and peer specialist services. Treatment services for SUD include assessment and evaluation, individual and group counseling, medication-assisted therapy, and residential treatment and withdrawal management services. Screening services include Health and Behavior Assessment Intervention (HBAI) and Referral to Treatment. See Health and Behavior Assessment and Intervention (HBAI) and Substance Use Disorder (SUD).

Breast and Cervical Cancer Services (BCCS)

Breast and Cervical Cancer Services (BCCS) clinic sites provide quality, low-cost and accessible breast and cervical cancer screening and diagnostic services to women. These clinic sites are the points of access for women applying to the Medicaid Breast and Cervical Cancer (MBCC) program. See the Medicaid Breast and Cervical Cancer (MBCC) Program.



Capitation Rate

Through actuarially sound methods, Texas develops a per member per month (PMPM) rate, or capitation rate, for each risk group within all state service areas for its Medicaid and Children's Health Insurance Program (CHIP) managed care programs. Capitation rates differ across risk groups and service areas, but they are the same for each managed care organization (MCO) within a service area. The managed care rate-setting process involves a series of mathematical adjustments to arrive at the final rates paid to the MCOs. Capitation rates are primarily based on MCO historical claims experience, also called encounter data. A risk margin is typically added in case of possible changes in claims cost. How capitation rates are determined varies by program, though the calculation method is similar. Capitation rates, paid monthly to MCOs, are the primary method the state uses to pay for services. See Encounter Data and Per Member Per Month (PMPM).

Case Management for Children and Pregnant Women

Case Management for Children and Pregnant Women (CPW) services are for children and youth age 20 and younger and pregnant women who are eligible for Medicaid. Case managers assist eligible clients in gaining access to medically necessary medical, social and educational services — as well as other services related to their health condition, health risk or high-risk condition. For pregnant women, services can continue up to 12 months postpartum. Services include assessing member needs, developing a service plan with clients and families, making referrals, problem-solving, advocacy and follow-up.

Centers for Medicare & Medicaid Services (CMS)

The Centers for Medicare & Medicaid Services (CMS) is the federal agency within the U.S. Department of Health & Human Services responsible for administering Medicare and overseeing state administration of Medicaid. States submit a Medicaid State Plan that serves as the contract between the state and CMS. CMS must approve the plan and any amendments to the plan. CMS also approves any waivers for which states can apply. See State Plan.

Chemical Dependency Treatment Facility (CDTF)

Chemical dependency treatment facilities (CDTFs) are licensed facilities that offer outpatient and residential treatment for persons with a substance use disorder (SUD). Medicaid SUD services must be provided in a CDTF, except for Medication-Assisted Treatment services.

Children's Health Insurance Program (CHIP)

The Children's Health Insurance Program (CHIP) provides acute care, behavioral health care, dental services and pharmacy services for children in families with too much income to qualify for Medicaid but who cannot afford to buy private health insurance. Children covered through CHIP receive similar services as children covered through Medicaid. See Children's Health Insurance Program (CHIP) Perinatal.

Children's Health Insurance Program (CHIP) Perinatal

Children's Health Insurance Program (CHIP) Perinatal services are for the unborn children of pregnant women who are uninsured and do not qualify for Medicaid. Services include prenatal visits, prescription prenatal vitamins, labor and delivery, and postpartum care.

Children's Hospital

According to the Centers of Medicare and Medicaid Services (CMS), a certified children's hospital is a freestanding or hospital-within-hospital that treats people mostly age 20 and younger.

Children's Medicaid

Children's Medicaid serves children age 18 and younger who meet the program's household income limits. Children's Medicaid provides acute care, behavioral health services, dental services, prescription drug benefits and long-term services and supports (LTSS). Children in the program are typically enrolled in the STAR managed care program, although some children with disabilities may be enrolled in STAR Kids. Children in Medicaid also receive Early and Periodic Screening, Diagnosis and Treatment (EPSDT) program services, known in Texas as Texas Health Steps. See Early and Periodic Screening, Diagnosis and Treatment (EPSDT) Program; STAR; STAR Kids; and Texas Health Steps.

Children's Medicaid Dental Services (CMDs) Program

The Children's Medicaid Dental Services (CMDs) program is a managed care program that provides dental benefits for children and youth on Medicaid from birth through age 20. There are three dental maintenance organizations (DMOs) operating statewide. See Dental Maintenance Organization (DMO).

Chronic Care Management

Managed care organizations (MCOs) must provide chronic care management programs and services using a person-centered approach that addresses the needs of high-risk members with complex chronic or co-morbid conditions. See Service Coordination.

Client

The word client is used in financial and encounter reporting to refer to 100 percent of eligible enrollees, including those using managed care and fee-for-service (FFS) delivery systems. The word member refers to an eligible person who is enrolled in Texas Medicaid or Children's Health Insurance Program (CHIP) managed care programs, which represents 97 percent of the total Medicaid/CHIP population.

Clinical Prior Authorization

Clinical prior authorizations are evidence-based reviews designed to ensure the clinical appropriateness of a drug or drug class based on factors such as age, availability of alternative medications, or possible drug interactions. They may apply to an individual drug or a drug class that is included on the federal formulary and may have preferred or non-preferred status on the Preferred Drug List (PDL). With the assistance of the Texas Drug Utilization Review Board, the Vendor Drug Program (VDP) develops, manages and reviews clinical prior authorizations across both fee-for-service (FFS) and the managed care programs. Participating managed care organizations (MCOs) must perform certain clinical prior authorizations and may perform others at their discretion. See Non-Preferred Prior Authorization, Preferred Drug List (PDL), Prior Authorization, Texas Formulary and Vendor Drug Program (VDP).

Community Attendant Services (CAS)

Community Attendant Services (CAS) are state plan, community-based, long-term services and supports (LTSS) that include assistance with activities of daily living (ADLs) and instrumental activities of daily living (IADLs). These services are available to eligible adults and children.

Collaborative Care Model (CoCM)

The collaborative care model (CoCM) is a systematic approach to the treatment of behavioral health conditions (mental health or substance use) in primary care settings. The model integrates the services of behavioral health care managers and psychiatric consultants into the primary care setting to proactively manage behavioral health conditions as chronic diseases rather than treating acute care symptoms.

Community First Choice (CFC)

Community First Choice (CFC) provides Medicaid State Plan home and community-based services to Medicaid recipients who meet an institutional level of care (LOC) for a nursing home, intermediate care facility, or institution for mental diseases. CFC services include personal assistance services (PAS), habilitation (HAB), emergency response services and support management. CFC services must be provided in community-based settings. See Personal Assistance Services (PAS).

Community Living Assistance and Support Services (CLASS)

Community Living Assistance and Support Services (CLASS) is a 1915(c) waiver program that provides home and community-based services and supports to persons who have a related condition diagnosis as an alternative to placement in an intermediate care facility for individuals with an intellectual disability or related condition (ICF/IID). A related condition is a disability other than an intellectual or developmental disability, which originates before age 22 and substantially limits life activity. See Appendix B, page 159, for a full list of services offered through CLASS. Eligibility determinations for CLASS are based on level of care (LOC) criteria outlined in the Texas Administrative Code. CLASS has an interest list. See Intermediate Care Facility for Individuals with an Intellectual Disability or Related Condition (ICF/IID) and Interest List.

Complaints

The Health and Human Services Commission (HHSC) monitors managed care organization (MCO) complaints, grievances and appeals processes for both members and providers. Data acts as an early warning to flag for potential systemic problems that warrant investigation, point to the need for policy clarifications or signal larger operational issues.

Comprehensive Care Program (CCP)

Federal law expanded the Early and Periodic Screening, Diagnosis and Treatment (EPSDT) program, known in Texas as Texas Health Steps, to cover any medically necessary and appropriate health care services used to treat all physical and mental diagnoses found in a screening. These benefits are included in Texas Health Steps but are referred to in Texas as the Comprehensive Care Program (CCP). See Early and Periodic Screening, Diagnosis and Treatment (EPSDT) Program and Texas Health Steps.

Comprehensive Hospital Increase Reimbursement Program (CHIRP)

The Comprehensive Hospital Increase Reimbursement Program (CHIRP) is a directed payment program for hospitals providing health care services to STAR and STAR+PLUS members. Eligible hospitals include children's hospitals, rural hospitals, mental health hospitals, state-owned hospitals and urban hospitals. Quality measures address hospital safety, maternal care, pediatric care, rural health care and care transitions. The program includes two components, including portions from the Uniform Hospital Rate Increase Program (UHRIP), currently in its fourth year of operation. See Directed Payment Program (DPP) and Uniform Hospital Rate Increase Program (UHRIP).

Consumer Assessment of Healthcare Providers and Systems (CAHPS) Survey

The Consumer Assessment of Healthcare Providers and Systems (CAHPS) survey is a nationally recognized and validated tool for collecting standardized information on member experiences with health plans and services. This survey focuses on consumer perceptions of quality, such as the communication skills of providers and ease of access to health care services. See National Core Indicators-Aging and Disabilities (NCI-AD) Survey.

Consumer Directed Services (CDS) Option

Consumer Directed Services (CDS) is a service delivery option for people receiving home and community-based long-term services and supports (LTSS) that allows the person or the person's legally authorized representative to be the employer of record for the direct care workers providing services — giving the person greater choice and control over the delivery of services. The person or legally authorized representative can hire, train, supervise and dismiss the employee when necessary. People may appoint a designated representative to assist with some employer responsibilities, like approving time sheets.

Continuous Medicaid Coverage

Also referred to as continuous eligibility, continuous Medicaid coverage refers to maintaining Medicaid coverage for people active or certified on or after March 18, 2020. Instituted as a part of the COVID-19 Public Health Emergency (PHE), this was a federal requirement for Texas Medicaid to qualify for an increase in federal funding for the program. Continuous Medicaid coverage increased the number of Texans on Medicaid from 3.9 million to about 6 million. The federal government ended continuous Medicaid coverage on March 31, 2023, and by March 31, 2024, HHSC initiated eligibility reviews for 100 percent of active Medicaid members. See Continuous Medicaid Coverage During the COVID-19 Public Health Emergency, page 12.

Contractor

A person or organization with which the state has successfully negotiated an agreement for the provision of required tasks.

Corrective Action Plan (CAP)

In cases of contractual non-compliance, the Health and Human Services Commission (HHSC) may use corrective action plans (CAPs) to hold managed care organizations (MCOs) accountable. The CAP process requires MCOs to provide HHSC with a detailed explanation of the reasons for the deficiency, an assessment or diagnosis of the cause, actions taken to cure or resolve the deficiencies — including short- and long-term solutions — and actions taken to prevent future occurrences. HHSC administers CAPs monthly and reviews, approves and closes CAPs once appropriate actions have been taken to address the non-compliance.

Cost Cap

A cost cap is the individual cost limit placed upon a service plan for a person enrolled in a waiver program. Each waiver program is designed to use reasonable methods to control overall spending, including the use of an individual cost limit, or cost cap. All waiver service plans must be within the cost limit. As a safeguard to members, the service plan must factor in the cost cap; available Medicaid and non-Medicaid services; and any service limits, and still ensure that the plan is sufficient to protect the health and welfare of the person receiving services.

Cost-Sharing

Cost-sharing refers to a copaying arrangement in which the state shares the cost of care with another party. Most families participating in Children's Health Insurance Program (CHIP), for example, pay an annual enrollment fee and make co-pays for doctor visits, prescription drugs, inpatient hospital care and non-emergent care provided in an emergency room setting. See Children's Health Insurance Program (CHIP).

COVID-19

Coronavirus disease (COVID-19) is an infectious disease caused by the novel SARS-CoV-2 virus, first identified in 2019. The rapid spread of COVID-19 led to a global pandemic beginning in 2020. In response to the pandemic, the United States federal government declared a Public Health Emergency (PHE). Texas Medicaid and CHIP have experienced many programmatic impacts because of the PHE. See Public Health Emergency (PHE).

Crisis Diversion

Crisis Diversion is a reserved capacity group for the Home and Community-based Services (HCS) program. People qualify as members of this reserved capacity group if they have an intellectual or developmental disability and are at imminent risk of being admitted or re-admitted to an institution. See Home and Community-based Services (HCS) and Reserved Capacity Group.

D

Day Activity and Health Services (DAHS)

Day Activity and Health Services (DAHS) are state plan, community-based, long-term services and supports (LTSS). DAHS is offered during the day, Monday through Friday, to eligible people living in the community. Services provided at licensed day activity and health services centers include nursing and personal care, meals, transportation, and social and recreational activities.

Deaf Blind with Multiple Disabilities (DBMD)

Deaf Blind with Multiple Disabilities (DBMD) is a 1915(c) waiver program that provides home and community-based services to people who are deaf and blind, and have a third disability (e.g., an intellectual disability), as an alternative to institutional care in an intermediate care facility for individuals with an intellectual disability or related condition (ICF/IID). See Appendix B, page 160, for a full list of services offered through DBMD. Eligibility determinations for DBMD are based on level of care (LOC) criteria outlined in the Texas Administrative Code. DBMD has an interest list. See Intermediate Care Facility for Individuals with an Intellectual Disability or Related Condition (ICF/IID).

Default Enrollment

Default enrollment occurs when members do not exercise their right to choose a managed care organization (MCO) and are automatically assigned to a health plan.

Deferral

A deferral refers to the withholding of federal funding, if the Texas Medicaid or Children's Health Insurance Program (CHIP) is determined to be out of compliance with federal regulations by the Centers of Medicare & Medicaid Services (CMS).

Delivery Supplemental Payment

Delivery supplemental payments are separate lump sums paid to managed care organizations (MCOs) to cover newborn delivery expenses. This payment is computed as part of the capitation payment for each service area.

Delivery System Reform Incentive Payment (DSRIP) Pool

The Delivery System Reform Incentive Payment (DSRIP) pool was one of two hospital funding pools under the Texas' section 1115 Texas Healthcare Transformation and Quality Improvement Program waiver, 1115 Transformation Waiver along with the Uncompensated Care (UC) pool. DSRIP funding provided financial incentives to focus hospitals and other providers on achieving quality health outcomes. Participating providers would develop and implement programs, strategies and investments to enhance the following: access to health care services, quality of health care and health systems, cost-effectiveness of services and health systems, and the health of the patients and families served. To earn DSRIP funds, providers undertook projects from a menu agreed upon by Centers for Medicare & Medicaid Services (CMS) and Health and Human Services Commission (HHSC) within the Regional Healthcare Partnership (RHP) planning protocol. The DSRIP program ended in September 2021. HHSC implemented a transition plan that preserved funding for providers and will continue the health care transformation efforts begun under the 1115 Transformation Waiver. See Directed Payment Program (DPP) and Uncompensated Care (UC) Pool.

Dental Maintenance Organization (DMO)

Dental maintenance organizations (DMOs) deliver and manage comprehensive dental services to eligible Medicaid and Children's Health Insurance Program (CHIP) members. For Medicaid and the Children's Health Insurance Program (CHIP), three DMOs operate throughout Texas.

Dental Quality Alliance (DQA)

The Dental Quality Alliance (DQA) is an organization convened by the American Dental Association at the request of Centers for Medicare & Medicaid Services (CMS). The DQA develops national evidence-based oral health care performance measures that are tested for feasibility, validity, reliability and usability.

Diagnosis Related Group (DRG)

The Diagnosis Related Groups (DRGs) are a patient classification system that relates the type of patients a hospital treats to hospital costs. The system was originally used by the Centers for Medicare & Medicaid Services (CMS) for Medicare hospital payments, and it has been expanded to be more inclusive of non-Medicare payments. Under this system, each patient is classified into a DRG based on clinical information. Hospitals receive a pre-determined rate for each DRG admission. For people receiving fee-for-service (FFS) inpatient or outpatient care, the rate is calculated using a standardized average cost of treating a Medicaid inpatient admission and a relative weight for each DRG.

Directed Payment Programs (DPPs)

Directed payment programs (DPPs) are designed to help managed care programs achieve delivery system and payment reform and performance improvement. Following the end of the Delivery System Reform Incentive Payment (DSRIP) program, HHSC implemented four new DPPs to preserve provider funding and further transform the Medicaid and CHIP health care system in Texas. HHSC currently administers five DPPs in Medicaid and CHIP.

Each DPP must be approved annually by the Centers for Medicare & Medicaid Services (CMS). See Comprehensive Hospital Increase Reimbursement Program (CHIRP), Directed Payment Program for Behavioral Health Services (DPP BHS), Quality Improvement Payment Program (QIPP).

Directed Payment Program for Behavioral Health Services (DPP BHS)

The Directed Payment Program for Behavioral Health Services (DPP BHS) is a DPP for community mental health centers and local behavioral health authorities. It seeks to promote and improve access to behavioral health services, service coordination, and successful care transitions for STAR, STAR+PLUS, and STAR Kids members. It also incentivizes using the Certified Community Behavioral Health Clinic model of care. See Directed Payment Programs (DPPs).

Disallowance

A disallowance refers to the recoupment of federal funds by the Centers for Medicare & Medicaid Services (CMS), should CMS allege that certain claims are not allowable.

Disproportionate Share Hospital (DSH)

Disproportionate Share Hospital (DSH) is a designation for hospitals that serve a higher-than-average number of Medicaid and other low-income patients. See also Disproportionate Share Hospital (DSH) Funding.

Disproportionate Share Hospital (DSH) Funding

Disproportionate Share Hospital (DSH) funding is special funding for hospitals that serve a disproportionately large number of Medicaid and low-income patients. DSH funds are not tied to specific services for Medicaid-eligible patients. There are no federal or state restrictions on how disproportionate share hospitals may use their funds. They may use the payments to cover the uncompensated costs of care for low-income patients, including Medicaid patients. DSH payments have been an important source of revenue by helping hospitals expand health care services to the uninsured, defray the cost of treating low-income patients, and recruit physicians and other health care professionals to treat patients.

Drug Utilization Review (DUR)

Drug utilization reviews (DURs), prospective and retrospective, are used by the Health and Human Services Commission (HHSC) and managed care organizations (MCOs) to evaluate member use of prescription drugs. See Prospective Drug Utilization Review (DUR), Retrospective Drug Utilization Review (DUR) and Texas Drug Utilization Review (DUR) Board.

Dually Eligible

People who are dually eligible qualify for both Medicare and Medicaid. Dually eligible people may receive Medicare Part A, Part B, or both, as well as full or partial Medicaid benefits. Typically, Medicare covers acute-care services and Medicaid covers long-term services and supports (LTSS).

Full-benefit dually eligible people are eligible to receive the full array of Medicaid services not covered by Medicare. Full-benefit and partial-benefit dually eligible people are eligible to receive assistance with Medicare premiums or Medicare cost-sharing, depending on the Medicare Savings Program they are in. See Medicaid-Medicare Plan (MMP), Medicare Savings Program and STAR+PLUS.

***Early and Periodic Screening, Diagnosis and Treatment (EPSDT) Program***

The Early and Periodic Screening, Diagnosis and Treatment (EPSDT) program is a required Medicaid State Plan benefit for eligible children and youth from birth through age 20. In Texas, this program is referred to as Texas Health Steps. Federal law expanded Texas Health Steps to cover any medically necessary and appropriate health care services used to treat all physical and mental diagnoses found in a screening. See Texas Health Steps and Comprehensive Care Program (CCP).

Early Childhood Intervention (ECI)

Early Childhood Intervention (ECI) is a statewide program that provides services to families with children age 3 and younger who have developmental delays or disabilities.

Electronic Visit Verification (EVV)

Electronic Visit Verification (EVV) is a computer-based tracking system that electronically verifies the occurrence of in-home personal attendant, therapy and nursing service visits by documenting the precise time a service delivery begins and ends. EVV helps prevent fraud, waste and abuse with the goal of ensuring Medicaid recipients receive care that is authorized.

Emergency Medicaid

The Emergency Medicaid program provides medical coverage designed to meet a sudden, critical health condition. Most covered people apply for this benefit only after an emergency occurs. If determined eligible, the person is covered by Medicaid only from the start of a qualifying emergency medical condition to when the event is stabilized, as verified by a medical provider.

Encounter Data

Used in the calculation of managed care organization (MCO) capitation rates, encounter data refers to the historical claims experience of MCOs for a base period. Encounter data includes the records of reimbursable health care services and the amounts MCOs pay to providers of those services. See Capitation Rates.

Enhanced Federal Medical Assistance Percentage (EFMAP)

The enhanced federal medical assistance percentage (EFMAP) is used to determine federal matching funds for the Children's Health Insurance Program (CHIP). Derived from each state's average per capita income, the Centers for Medicare & Medicaid Services (CMS) updates this rate annually. Consequently, the percentage of total CHIP spending that is paid for with federal funds also changes annually. The blended EFMAP rate for Texas in federal fiscal year 2023 was 75.39 percent and in state fiscal year 2023 was 75.66 percent. See Federal Medical Assistance Percentage (FMAP) and Matching Funds.

Experience Rebate

Experience rebate is the amount of profit earned through the managed care program that a managed care organization (MCO) or dental maintenance organization (DMO) must share with the state of Texas. The Health and Human Services Commission (HHSC) requires MCOs and DMOs to share any profits in excess of three percent of gross revenue earned through the managed care program.

External Medical Review (EMR)

Medicaid members who are denied services can appeal directly to the Health and Human Services Commission (HHSC) by requesting a “fair hearing.” As part of that request, they have the option to ask for an external medical review (EMR) before their fair hearing. During an EMR, an independent review organization (IRO) examines the member’s case and can uphold or change the service denial or eligibility determination. Unlike a fair hearing, EMRs are privately conducted, and the member, the MCO and HHSC cannot submit additional evidence. EMRs must occur after a health plan appeal if a managed care organization (MCO) made the original decision to deny services. See Complaints, Fair Hearing, Health Plan Appeal and Independent Review Organization.

External Quality Review Organization (EQRO)

An external quality review organization (EQRO) assesses managed care organization (MCO) performance on several metrics — access to care, utilization of care and quality of care — for all MCOs participating in Medicaid and Children’s Health Insurance Program (CHIP) medical and dental managed care programs. The Institute for Child Health Policy at the University of Florida has been the EQRO for Texas since 2002.

***Fair Hearing***

Medicaid clients who are denied services or program eligibility can appeal directly to the Health and Human Services Commission (HHSC) by requesting a fair hearing. During a fair hearing, an HHSC fair hearings officer publicly reviews evidence submitted by the client, the managed care organization (MCO) or HHSC program staff — and can uphold or change the original eligibility decision. While additional evidence may be submitted by either party, evidence submitted by the MCO or HHSC must be presented to the client before the hearing. Fair hearings must occur after a health plan appeal if an MCO made the original decision to deny services or eligibility. See Complaints, External Medical Review (EMR) and Health Plan Appeal.

Federal Medical Assistance Percentage (FMAP)

The Medicaid program is jointly funded by the federal government and states. The federal medical assistance percentage (FMAP) determines the amount of federal matching funds Texas receives for Medicaid. Derived from each state’s average per capita income, the Centers for Medicare & Medicaid Services (CMS) updates the rate annually.

Consequently, the percentage of total Medicaid spending that is paid with federal funds also changes annually. As the state's per capita income increases in relation to the national per capita income, the FMAP rate decreases. For federal fiscal year 2023, Texas' Medicaid blended FMAP rate was 64.85 percent. See Enhanced Federal Medical Assistance Percentage (EFMAP) and Matching Funds.

Federal Poverty Level (FPL)

The Federal Poverty Level (FPL) is a measure of income set by the U.S. Department of Health and Human Services used to determine eligibility for government programs and services, like Medicaid and Children's Health Insurance Program (CHIP). The FPL is updated yearly.

Federally Qualified Health Center (FQHC)

Federally Qualified Health Centers (FQHCs) include all organizations receiving grants under Section 330 of the Public Health Service Act. FQHCs qualify for enhanced reimbursement from Medicare and Medicaid, as well as other benefits. FQHCs serve underserved areas or populations, offer a sliding-scale fee, provide comprehensive services, and have an ongoing quality assurance program and a governing board of directors.

Fee-for-Service (FFS)

Fee-for-service (FFS) is a health care payment system under which providers receive a payment for each unit of service they provide. Under FFS, people can go to any Medicaid provider, and the provider will submit claims directly for Medicaid covered services.

Currently, only 3 percent of people on Medicaid in Texas still receive acute care services through FFS. The remaining 97 percent are enrolled into one of the 5 managed care programs. See Managed Care.

Fee-for-Service (FFS) Rates

Fee-for-service (FFS) is a healthcare payment system under which providers receive a payment for each unit of service they provide.

Financial Criteria

Texans who apply for Medicaid or the Children's Health Insurance Program (CHIP) must meet certain financial criteria to be eligible for services. Financial eligibility is generally measured by comparing an applicant's income to the U.S. Department of Health and Human Services definition of the federal poverty level (FPL) for annual household incomes. Federal law currently requires that Modified Adjusted Gross Income (MAGI) be the primary measurement tool used to calculate financial eligibility for most Medicaid applicants. See Federal Poverty Level (FPL) and Modified Adjusted Gross Income (MAGI).

Financial Statistical Report (FSR)

Managed care organizations (MCOs) are required to submit quarterly financial statistical reports (FSRs). FSRs include information on medical and administrative expenses and are one source for establishing future capitation rates. Validation of these reports is an important component of contract oversight, and FSRs are audited yearly.

Fraud

Fraud refers to an intentional deception or misrepresentation made by a person or entity with the knowledge that the deception could result in some unauthorized benefit to himself or some other person. This term does not include unintentional technical, clerical or administrative errors.

Freedom of Choice

In general, a state must ensure that Medicaid members are free to obtain services from any qualified provider. Exceptions are possible through Medicaid waivers and special contract options.

Freedom of Choice 1915(b) Waivers

Section 1915(b) waivers allow states to use a central broker (e.g., enrollment broker) to assist people with choosing a managed care organization (MCO), to use cost savings to provide additional services or to limit choice of Medicaid providers by requiring people to join MCOs.

Frew v. Young

A class action lawsuit filed against Texas in 1993 alleging that the state did not adequately provide Medicaid Early and Periodic Screening, Diagnosis and Treatment (EPSDT) services. In 1995, the State negotiated a consent decree that imposed certain requirements on the state. In 2007, the State negotiated corrective action orders with the plaintiffs to implement the consent decree and increase access to EPSDT services.

Since 2007, the Health and Human Services Commission (HHSC) and the Department of State Health Services (DSHS) have actively worked to meet the requirements of the corrective action orders. Since 2007, the Health and Human Services Commission (HHSC) and the Department of State Health Services (DSHS) have actively worked to meet the requirements of the corrective action orders.



Graduate Medical Education (GME) Payments

Graduate medical education (GME) payments to state-owned teaching hospitals cover the costs of program administrative staff, allocated facility overhead, and salaries and fringe benefits for residents and teaching physicians. See State-Owned Teaching Hospitals.



Health and Behavior Assessment and Intervention (HBAI)

Health and Behavior Assessment and Intervention (HBAI) services are designed to identify the psychological, behavioral, emotional, cognitive and social factors that impact the treatment or management of physical health symptoms for children and youth age 20 and younger. HBAI services help promote physical and behavioral health integration.

Health and Human Services Commission (HHSC)

The Health and Human Services Commission (HHSC) is the single state agency implementing and overseeing Medicaid and the Children's Health Insurance Program (CHIP) for Texas. See Single State Agency.

Health Insurance Premium Payment (HIPP) Program

The Health Insurance Premium Payment (HIPP) program reimburses eligible people for their share of an employer-sponsored health insurance premium if it is determined more cost-effective than reimbursing the person's Medicaid expenditures. To be eligible for HIPP, a person must be enrolled in Medicaid or have a family member enrolled in Medicaid and have employer sponsored insurance that covers the Medicaid-enrolled family member

Health Plan

Managed care organizations (MCOs) are often referred to as health plans — a term which describes their function in providing medical coverage and coordinated care to Medicaid and Children's Health Insurance Program (CHIP) members. See Managed Care Organization (MCO).

Health Plan Appeal

Medicaid members who are denied services or eligibility by their managed care organization (MCO) can ask for a health plan appeal. During a health plan appeal, the MCO reconsiders their original decision and can change or uphold it. If the member disagrees with the outcome of their health plan appeal, they have the right to appeal directly to the Health and Human Services Commission (HHSC) through a fair hearing. See Complaints, External Medical Review (EMR) and Fair Hearing.

Healthcare Effectiveness Data and Information Set (HEDIS)

The Healthcare Effectiveness Data and Information Set (HEDIS) is a widely used set of performance measures for managed care organizations (MCOs). HEDIS data for Texas Medicaid and Children's Health Insurance Program (CHIP) managed care programs is posted and regularly updated on the Texas Healthcare Learning Collaborative (THLC) public portal. See Texas Healthcare Learning Collaborative (THLC).

Healthcare Payment Learning and Action Network (HCP-LAN)

The Health and Human Services Commission (HHSC) is using the Healthcare Payment Learning and Action Network (HCP-LAN) to accelerate the transition to alternative payment models (APMs) within managed care organizations (MCOs) in Texas Medicaid and the Children's Health Insurance Program (CHIP). See Alternative Payment Model and Pay-for-Quality (P4Q).

Healthy Texas Women (HTW)

Healthy Texas Women (HTW) is a women's health and family planning Medicaid program for low-income women age 15 through 44. This program is authorized through a 1115 Demonstration Waiver. To ensure continuity of care, HTW auto-tests women into the program when their Medicaid for Pregnant Women coverage ends. Women can also apply for HTW like they would for other programs. See Healthy Texas Women Plus (HTW Plus).

Healthy Texas Women Plus (HTW Plus)

Healthy Texas Women Plus (HTW Plus) is an enhanced postpartum service for eligible women enrolled in Healthy Texas Women (HTW). HTW Plus services include treatment for mental health conditions, such as postpartum depression, substance use disorders (SUDs) and cardiovascular conditions. To receive HTW Plus, a woman enrolled in HTW must have been pregnant within the 12 months prior to her enrollment. In December 2020, HHSC submitted an amendment to the HTW 1115 demonstration to receive federal funds for HTW Plus. As of December 2024, the amendment is pending with CMS.

Heightened Scrutiny

Heightened scrutiny is a review process for ensuring home and community-based services settings comply with federal regulations. Under the 2014 Home and Community-Based Services Settings Rule, the Centers for Medicare & Medicaid Services (CMS) requires states to ensure home and community-based services settings meet minimum standards for integration and choice.

Home and Community-Based Services (HCS)

Home and Community-based Services (HCS) is a 1915(c) waiver program that provides community-based services to people with intellectual disabilities as an alternative to institutional care in an intermediate care facility for individuals with an intellectual disability or related condition (ICF/IID). See Appendix B, page 160, for a full list of services offered through HCS. Eligibility determinations for HCS are based on level of care (LOC) criteria outlined in the Texas Administrative Code. HCS has an interest list. See Interest List and Intermediate Care Facility for Individuals with an Intellectual Disability or Related Condition (ICF/IID).

Home and Community-Based Services–Adult Mental Health (HCBS-AMH)

The Home and Community-based Services–Adult Mental Health (HCBS-AMH) program helps people with serious mental illness remain in the community. The HCBS-AMH program provides an array of intensive home and community-based services tailored to a person’s assessed needs, preferences and goals.

Home and Community-Based Services (HCBS) 1915(c) Waivers

Home and Community-Based Services (HCBS) 1915(c) waivers allow states to provide home and community-based services as an alternative for people who meet eligibility criteria for care in an institution. See Community Living Assistance and Support Services (CLASS), Deaf Blind with Multiple Disabilities (DBMD), Home and Community-Based Services (HCS), Medically Dependent Children Program (MDCP), Texas Home Living (TxHmL) and Youth Empowerment Services (YES).

Home Telemonitoring

Home telemonitoring is a health service that requires scheduled remote monitoring of data related to a client’s health, and transmission of the data from the client’s home to a licensed home and community support services agency, a federally qualified health center, a rural health clinic, or a hospital. The data transmission must comply with standards set by HIPAA.

Hospital Quality-Based Payment Program

The Hospital Quality-Based Payment Program is a quality monitoring program that pays or recoups funds from hospitals according to their performance with fee-for-service (FFS) and managed care patients — with the goal of improving quality and lowering costs. Through the program, hospitals and managed care organizations (MCOs) are financially accountable for certain potentially preventable events (PPEs) and can receive bonus payments for achieving low PPE rates. See Potentially Preventable Event (PPE).



Income Disregards

Income disregards are income sources or amounts that are deducted or disregarded in a financial eligibility determination. For applicants subject to the Modified Adjusted Gross Income (MAGI) methodology, a standard income disregard of 5 percent of the federal poverty level (FPL) is granted. For applicants not subject to MAGI, there are program-specific income disregards. See Federal Poverty Level (FPL), Financial Criteria and Modified Adjusted Gross Income (MAGI).

Income Limits

Income limits are used in financial eligibility determinations. They are calculated based on household size and a certain percentage of the federal poverty level (FPL), which varies by program. Medicaid and Children's Health Insurance Program (CHIP) applicants must meet income limits and other financial criteria to be eligible for services. See Federal Poverty Level (FPL), Financial Criteria and Modified Adjusted Gross Income (MAGI).

Independent Review Organization (IRO)

An independent review organization (IRO) is a group of medical experts who are unaffiliated with a managed care organization (MCO) or the Health and Human Services Commission (HHSC) and are contracted to conduct external medical reviews (EMRs). During an EMR, an IRO can uphold or change an MCO's or HHSC's decision to deny services or program eligibility. See External Medical Review (EMR).

Individualized Skills and Socialization

Individualized Skills and Socialization is a community-integrated service for people with intellectual and developmental disabilities. It is designed to replace day habilitation services in the waiver programs. The service provides person-centered activities, assists with personal care needs and medications, and helps people learn new skills or practice existing skills while exploring their community.

Inspection

Inspections, such as those conducted by the Office of Inspector General (OIG), are targeted examinations into specific areas of the Health and Human Services Commission (HHSC) programs, systems or functions that may identify systemic trends of fraud, waste and abuse.

Instrumental Activities of Daily Living (IADLs)

Instrumental activities of daily living (IADLs) are activities essential to independent daily living — including preparing meals, shopping for groceries or personal items, performing light housework, and using a telephone. Assistance with IADLs is part of attendant services, including personal care services (PCS) and personal assistance services (PAS). These services are available in several state plan and Medicaid waiver long-term services and supports (LTSS) programs, including Community Attendant Services (CAS), Primary Home Care (PHC), Day Activity and Health Services (DAHS), Community First Choice (CFC), and home and community-based services (HCBS) programs, including STAR+PLUS HCBS. See Long-Term Services and Supports (LTSS).

Interest List

Demand for 1915(c) waiver programs, like Home and Community-based Services (HCS) and Community Living Assistance and Support Services (CLASS), often exceed capacity. Many Medicaid waiver programs maintain interest lists, which individuals can join at any time. When a program slot becomes available, the next person on the interest list may undergo an eligibility determination for the program. If they are found eligible, they may take the open slot. An individual can be on multiple interest lists at the same time and may stay on a program's interest list while enrolled in a different Medicaid program.

Intermediate Care Facility for Individuals with an Intellectual Disability or Related Condition (ICF/IID)

Intermediate care facilities for individuals with an intellectual disability or related condition (ICFs/IID) provide ongoing evaluation and individual program planning — as well as 24-hour supervision, coordination, and integration of health or rehabilitative services in a residential setting — to help people with an intellectual disability or related condition function to their greatest ability.

Interstate Compact on the Placement of Children (ICPC)

The Interstate Compact on the Placement of Children (ICPC) is an agreement between all 50 states, the District of Columbia and the U.S. Virgin Islands, authorizing them to work together to ensure children placed across state lines for foster care or adoption receive adequate protection and support services. It establishes procedures for the placement of children across state lines and responsibilities for the agencies and people involved.



Lawful Permanent Resident (LPR)

Legal permanent residents (LPRs) are non-citizens who are lawfully authorized to live permanently within the U.S. They are often called “green card holders.”

Level of Care (LOC)

Level of care (LOC) refers to the type and amount of care required by a resident of either a nursing facility or an intermediate care facility for individuals with an intellectual disability or related condition (ICF/IID). It is determined by evaluating the resident’s medical, nursing care and personal-care needs. See Intermediate Care Facility for Individuals with an Intellectual Disability or Related Condition (ICF/IID) and Nursing Facility.

Liquidated Damages (LDs)

Liquidated damages (LDs) are compensation for managed care contractual non-compliance. The Health and Human Services Commission (HHSC) assesses LDs quarterly to address any harm incurred due to managed care organization (MCO) contractual non-compliance.

Local Intellectual and Developmental Disability Authority (LIDDA)

Local intellectual and developmental disability authorities (LIDDAs) serve as the point of entry for publicly funded intellectual and developmental disability (IDD) programs, whether the program is provided by a public or private entity. LIDDAs provide or contract to provide an array of services and supports for people with IDD and enroll those who are eligible into Medicaid programs, such as intermediate care facilities for individuals with an intellectual disability or related condition (ICFs/IID), Home and Community-based Services (HCS) and Texas Home Living (TxHmL). They are also responsible for Permanency Planning for eligible people seeking to move from an institutional setting to a community-based setting.

Local Mental Health Authority (LMHA)

Local mental health authorities (LMHAs), along with local behavioral health authorities (LBHAs), evaluate the mental health needs of the communities in their designated areas and plan, develop policy, and coordinate services and resources to address those needs. LMHAs/LBHAs provide information, treatment, recommendations and referrals to people seeking mental health services.

Long-Acting Reversible Contraception (LARC)

Long-acting Reversible Contraception (LARC) devices are highly effective for preventing pregnancy, are easy to use, and last for several years. The Health and Human Services Commission (HHSC) established an add-on reimbursement to incentivize the use of immediate postpartum (IP) LARC for women enrolled in Medicaid for Pregnant Women.

Long-Term Services and Supports (LTSS)

Long-term services and supports (LTSS) provide a person with medical and personal services to assist with ongoing, day-to-day activities. People typically eligible for LTSS include adults age 65 and older and those with physical or intellectual disabilities. LTSS may be delivered through managed care or fee-for-service (FFS) and in conjunction with a waiver program.



Managed Care

Managed care is a service delivery model where the Health and Human Services Commission (HHSC) contracts with managed care organizations (MCOs) to provide Medicaid and Children’s Health Insurance Program (CHIP) services to members. MCOs coordinate overall care of members under managed care to improve quality and control costs. In Texas, 97 percent of Medicaid members are enrolled in managed care — where they select a health plan available in their service area and a primary care provider that coordinates their care. Default enrollment occurs when members do not exercise their right to choose an MCO and are automatically assigned to a health plan. See Default Enrollment, Health Plan, Managed Care Organization (MCO), Service Area and Primary Care Provider (PCP).

Managed Care Long-term Services and Supports Utilization Review (MLTSS UR)

The Managed Care Long-Term Services and Supports Utilization Review (MLTSS UR) team conducts sampled reviews of STAR+PLUS Home and Community Based Services (STAR+PLUS HCBS) to determine how they, as managed care organizations (MCOs), assess members and use procedures and related information to determine appropriate member enrollment. The review ensures MCOs are providing services according to their assessment of service needs.

Managed Care Organization (MCO)

A managed care organization (MCO) delivers and manages health services under a risk-based arrangement. The Health and Human Services Commission (HHSC) contracts with MCOs and pays them a per member per month (PMPM) rate, or capitation payment that incentivizes MCOs to control costs and invest in preventive health care. MCOs must provide all covered, medically necessary services to their members with the capitated payment. Members generally have a choice between at least two MCOs operating in their service area. See Capitation Rate, Managed Care and Per Member Per Month (PMPM).

Managed Care Report Card

A managed care report card evaluates the performance of a managed care organization (MCO) using a five-star rating system. The External Quality Review Organization (EQRO) develops annual ratings by surveying current members and analyzing encounter data. Report cards are available on the Health and Human Services Commission (HHSC) website and are provided in Medicaid and Children's Health Insurance Program (CHIP) enrollment packets. See External Quality Review Organization (EQRO).

Mandatory Benefits

The Social Security Act specifies a mandatory set of benefits state Medicaid programs must provide and an optional menu of services from which states can choose. States are not required to provide optional services. See Appendix B, page 156, for a complete list of mandatory and optional benefits. See Optional Benefits.

Matching Funds

Federal funds are a critical component of health care financing for the state of Texas. The amount of federal Medicaid funds that Texas receives is based primarily on the federal medical assistance percentage (FMAP), or Medicaid matching rate. The federal Children's Health Insurance Program (CHIP) funds that Texas receives are based on the enhanced federal medical assistance percentage (EFMAP). See Enhanced Federal Medical Assistance Percentage (EFMAP) and Federal Medical Assistance Percentage (FMAP).

Maternal Opioid Misuse (MOM) Model

The Maternal Opioid Misuse (MOM) model is a project to facilitate better integration of prenatal care, addiction medicine and psychiatric care for pregnant women with an opioid use disorder (OUD) who are enrolled in Medicaid.

Medicaid

A joint federal-state entitlement program that pays for medical care on behalf of certain groups of low-income persons. The program was enacted federally in 1965 under Title XIX of the Social Security Act. Texas participation in Medicaid began Sept. 1, 1967.

Medicaid Breast and Cervical Cancer (MBCC) Program

The Medicaid Breast and Cervical Cancer (MBCC) program provides Medicaid to eligible women diagnosed with breast or cervical cancer, including pre-cancerous conditions. To get MBCC, a woman must receive a screening at a Breast and Cervical Cancer Services (BCCS) clinic and apply for the program through the clinic. Women who have MBCC are enrolled into STAR+PLUS. Women remain eligible for MBCC if they are receiving active treatment, such as chemotherapy or radiation for breast or cervical cancer. See Breast and Cervical Cancer Services (BCCS).

Medicaid Buy-In for Children (MBIC)

The Medicaid Buy-In for Children (MBIC) program offers low-cost Medicaid services to children with disabilities in families that make too much money to get Medicaid. Children with family countable income at or below 300 percent of the federal poverty level (FPL) may qualify for the program, and households at or below 150 percent of the FPL will not pay a premium. MBIC families make monthly payments according to a sliding scale based on family income.

Medicaid Buy-In (MBI) for Adults

The Medicaid Buy-In (MBI) for Adults program offers low-cost Medicaid services to working adults with disabilities. Adults may qualify if they have a disability, are working and meet the income limits for the program and are not continuously living in an institution or nursing home. The income limit for MBI is \$3,138 per month and countable assets must total no more than \$2,000.

Medicaid Estate Recovery Program (MERP)

The Medicaid Estate Recovery Program (MERP) is required by federal and state law to recover — after time of death — certain long-term care and associated Medicaid service costs provided to members age 55 and older.

Medicaid for Former Foster Care Children (FFCC)

Medicaid for Former Foster Care Children (FFCC) covers Medicaid members who aged out of the foster care system in Texas at age 18 and received federally funded Medicaid when they aged out. Members of FFCC are automatically enrolled in STAR Health through the month of their 21st birthday. Members may opt out of STAR Health for STAR, which allows for a choice of health plans. After a young person reaches age 21, coverage will transfer to STAR. FFCC members may continue to be eligible up to the month of their 26th birthday. See Medicaid for Transitioning Foster Care Youth (MTFCY).

Medicaid for Parents and Caretaker Relatives

Medicaid for Parents and Caretaker Relatives is a program for adults who are a related caretaker for a child who already has Medicaid and meet the program's income limits.

Medicaid for Pregnant Women

Medicaid for Pregnant Women provides health coverage to low-income pregnant women — including acute care services, like prenatal care, labor and delivery, and postpartum care. Texas extends Medicaid eligibility to pregnant women with a household income at or below 198 percent of the federal poverty level (FPL) — well above the federal requirement of 133 percent of the FPL. Pregnant women who qualify receive services through the STAR program and receive coverage through 12 months postpartum. Newborns whose mothers qualified for Medicaid for Pregnant Women or Emergency Medicaid for their labor and delivery are enrolled into Children's Medicaid and receive 12 months of continuous coverage from their date of birth.

Medicaid for the Elderly and People with Disabilities (MEPD)

Medicaid for the Elderly and People with Disabilities (MEPD) serves people age 65 and older and individuals with disabilities who do not receive Supplemental Security Income (SSI). Individuals may qualify for this program through a facility, such as a nursing facility or an intermediate care facility for individuals with an intellectual disability or related condition (ICF/IID). MEPD covers long-term services and supports (LTSS), including long-term care facilities and state plan home and community-based services. See also Long-Term Services and Supports (LTSS).

Medicaid for Transitioning Foster Care Youth (MTFCY)

Medicaid for Transitioning Foster Care Youth (MTFCY) covers former foster care youth who were not receiving Medicaid when they aged out of foster care at age 18. Such people are eligible through the month of their 21st birthday to receive services through the fee-for-service (FFS) or managed care models. See Medicaid for Former Foster Care Children (FFCC).

Medicaid Hospice Services

Medicaid hospice services provide palliative care consisting of medical, social and support services to people with terminal illness and their loved ones. Hospice becomes available when curative treatment is no longer desired or possible and a physician indicates the person has 6 months or fewer to live.

Medicaid Lock-in Program

The Medicaid Lock-in Program is an Office of Inspector General (OIG) prescription drug oversight program that limits certain members to receiving or purchasing their prescriptions to one provider or pharmacy to prevent controlled substance abuse or overuse. People identified for and enrolled in the oversight program may only purchase prescriptions from the pharmacy to which they are “locked in.”

Medicaid-Medicare Plan (MMP)

Medicare-Medicaid Plans (MMPs) are designed to provide a fully integrated managed care model for people age 21 and older who are dually eligible for Medicare and full Medicaid benefits. The model involves a 3-party contract — between an MMP, the Health and Human Services Commission (HHSC), and the Centers for Medicare and Medicaid Services (CMS) — for the provision of the full array of Medicare & Medicaid services. All covered services, including acute care and long-term services and supports (LTSS), are provided by a single health plan. Participating members must be enrolled in STAR+PLUS. If eligible, people are passively enrolled into the program but may choose to opt-out. MMPs operate in Bexar, Dallas, El Paso, Harris, Hidalgo and Tarrant counties. See Dually Eligible.

Medical Home

A medical home consists of a patient’s primary care provider (PCP) who delivers comprehensive preventive and primary care and provides referrals for specialty care and other covered services. See Primary Care Provider (PCP).

Medical Necessity

Medical necessity is a measure used to determine a person’s eligibility for a program or service. Authorized health care providers assess whether a program or service is reasonable, appropriate and consistent with health care practice guidelines. The Health and Human Services Commission (HHSC) uses medical necessity to determine eligibility for programs and services delivered through fee-for-service (FFS). Managed care organizations (MCOs) also use medical necessity to make eligibility determinations for the programs and services they deliver. HHSC monitors the medical necessity determination for all MCOs through utilization reviews (URs). See Acute Care Utilization Reviews (ACURs) and Managed Care Long-Term Services and Supports Utilization Review (MLTSS UR).

Medical Transportation Program (MTP)

The Medical Transportation Program (MTP) is responsible for ensuring consistent, appropriate, reasonably prompt and cost-effective nonemergency medical transportation services to fee-for-service (FFS) clients, Children with Special Health Care Needs, and qualified low-income cancer patients who need transportation to covered health care services. See Nonemergency Medical Transportation (NEMT).

Medically Dependent Children Program (MDCP)

The Medically Dependent Children Program (MDCP) is a 1915(c) waiver program delivered through the STAR Kids managed care program. MDCP provides a cost-effective alternative to living in a nursing facility to children with disabilities. Eligibility determinations for MDCP are based on medical necessity as outlined in the Texas Administrative Code. Managed care organizations (MCOs) provide medical necessity assessments, which are used by the Health and Human Services Commission (HHSC) to determine whether a person qualifies for MDCP. MDCP has an interest list. See Appendix B, page 161, for a full list of MDCP services. See STAR Kids.

Medically Needy with Spend Down Program

Through the Medically Needy with Spend Down Program, Medicaid pays for unpaid medical expenses for medical services provided to children age 18 and younger and to pregnant women who meet the required Spend Down limits. Spend Down is the difference between an applicant's household income and the Medically Needy income limit. Applicants must have unpaid medical bills that exceed the Spend Down amount to receive benefits under the program. See Medicaid for Pregnant Women.

Medicare Advantage Dual Eligible Special Needs Plan (D-SNP)

A Medicare Advantage Dual Eligible Special Needs Plan (D-SNP) is a managed care delivery model specifically designed to coordinate care between Medicare and Medicaid covered services for people who are dually eligible for both programs. D-SNPs can serve both full and partial benefit dually eligible people. D-SNPs may be affiliated with managed care organizations (MCOs). See Dually Eligible.

Medicare Savings Program

Through the Medicare Savings Program (MSP), Texas Medicaid provides limited assistance to certain dually eligible Medicare beneficiaries. People eligible for MSP receive assistance, through Medicaid, for out-of-pocket Medicare expenses, such as Medicare premiums, deductibles and coinsurance.

Medication Assisted Therapy (MAT)

Medication Assisted Therapy (MAT) is the use of medications in combination with counseling and behavioral therapies for the treatment of substance use disorders (SUDs), which can help some people sustain recovery. An example is the use of methadone for opioid use disorder (OUD).

Member

The word member refers to an eligible person who is enrolled in Texas Medicaid or CHIP managed care programs, which represent 97 percent of the total Medicaid/CHIP population. The word client is used in financial and encounter reporting to refer to 100 percent of eligible enrollees, including those using managed care and fee-for-service (FFS) delivery systems. See Client.

Members with Special Health Care Needs (MSHCN)

Members with special health care needs (MSHCN) refers to managed care members who (a) have a serious, ongoing illness, a chronic or complex condition, or a disability that has lasted or is anticipated to last for a significant period of time and (b) requires regular, ongoing therapeutic intervention and evaluation by appropriately trained health care personnel. MSHCN may include women with high-risk pregnancies, members with high-cost catastrophic cases, and people with mental illness and co-occurring substance abuse. All members in STAR+PLUS, STAR Kids, STAR Health and MMP are considered MSHCN. See Service Coordination, STAR+PLUS, STAR Kids, STAR Health and Medicaid-Medicare Plan (MMP).

Modified Adjusted Gross Income (MAGI)

Modified Adjusted Gross Income (MAGI) is a tool used for financial eligibility determinations. MAGI uses federal income tax rules for determining income and household composition. Federal law requires financial eligibility for programs like Medicaid be determined using MAGI, except for people age 65 and older, people with a disability and those receiving Supplemental Security Income (SSI).

Mystery Shopper Method

The state's external quality review organization (EQRO) uses the mystery shopper method to evaluate whether providers meet appointment availability standards and to quantify the extent of provider directory issues. Using contact information provided by managed care organizations (MCOs), mystery shoppers (also termed "secret shoppers") contact network providers to see how quickly they can get appointments across all Medicaid programs. See External Quality Review Organization (EQRO).



National Core Indicators-Aging and Disabilities (NCI-AD) Survey

On a biennial basis, the Health and Human Services Commission (HHSC) participates in the National Core Indicators-Aging and Disabilities (NCI-AD) survey, which is similar to the Consumer Assessment of Healthcare Providers and Systems (CAHPS) survey. The core indicators address key areas of concern including service planning, rights, community inclusion, choice, health and service coordination, and safety and relationships. This allows the State to assess the outcomes of services provided to members who participate in STAR+PLUS Home and Community Based Services (STAR+PLUS HCBS) and the Programs of All-Inclusive Care for the Elderly (PACE).

Network Adequacy

Network adequacy refers to access to network providers that managed care organization (MCO) provider networks must meet to ensure members can access all medically necessary covered services. MCO provider networks must meet minimum provider access standards — including minimum distance, travel time and appointment availability for member access to providers; expedited credentialing for certain provider types specified by the Health and Human Services Commission (HHSC); and weekly updates of online provider directories.

Nonemergency Medical Transportation (NEMT)

Nonemergency medical transportation (NEMT) services provide cost-effective medical transportation to a doctor or specialist, a pharmacy, or other providers for covered health care services. NEMT services are only for people enrolled in managed care or fee-for-service (FFS), Children with Special Health Care Needs and qualified low-income cancer patients. Managed care NEMT services are coordinated by the MCO. FFS NEMT services are coordinated by the Medical Transportation Program (MTP). See Medical Transportation Program (MTP).

Non-Financial Criteria

In addition to meeting financial eligibility criteria, Medicaid applicants must meet non-financial criteria. They must meet program-specific age limits, reside and intend to remain in Texas, provide a Social Security number (SSN) or apply for one, and meet citizenship or alien status program requirements. See Qualified Aliens.

Non-Medical Drivers of Health (NMDOH)

Non-medical drivers of health (NMDOH) are the conditions in places where people live, learn, work, and play that affect a wide range of health risks and outcomes. This definition is adapted from the Centers for Disease Control and Prevention's (CDC) definition for "social determinants of health". The NMDOH Action Plan is a set of guiding priorities and strategic goals for Medicaid & CHIP Services (MCS) to coordinate NMDOH activities and support the work of managed care organizations (MCOs) and Medicaid providers. A detailed version of the action plan is available on the HHS NMDOH webpage.

Non-Preferred Prior Authorization

Non-preferred prior authorization may occur when a Medicaid member is prescribed a medication listed as non-preferred on the Preferred Drug List (PDL). Providers must obtain prior authorization for non-preferred medications from the Health and Human Services Commission (HHSC) or the patient's managed care organization (MCO) before the drug can be dispensed. See Clinical Prior Authorization, Preferred Drug List (PDL) and Prior Authorization.

Nursing Facility

Nursing facilities are long-term care facilities that provide services to meet the medical, nursing and psychological needs of individuals whose medical condition regularly requires the skills of licensed nurses.



Office of Inspector General (OIG)

The Office of Inspector General (OIG) is charged with safeguarding state health and human services by detecting and preventing fraud, waste and abuse and by ensuring the health and safety of Texans. The OIG engages in variety of activities related to Medicaid program integrity, including investigations, audits, inspections and reviews.

Operational Review

An operational review allows the Health and Human Services Commission (HHSC) to conduct an in-depth review of operational compliance and performance of a managed care organization (MCO) across several areas to ensure policies and practices align with performance standards. HHSC conducts on-site biennial operational reviews

of MCOs—including review modules on claims processing, member and provider training, complaints/appeals, encounter data, utilization management, and website critical elements.

Optional Benefits

The Social Security Act specifies a set of benefits state Medicaid programs must provide and a set of optional benefits states may choose to provide. The state may choose to provide some, all, or no optional services specified under federal law. See also Mandatory Benefits.



Pay-for-Quality (P4Q)

Pay-for-quality (P4Q) refers to programs that reward the use of evidence-based practices and promote health care coordination and efficacy among managed care organizations (MCOs). The Health and Human Services Commission (HHSC) implements medical P4Q programs for STAR, STAR+PLUS, Children's Health Insurance Program (CHIP) and a dental P4Q program. Strong P4Q programs incentivize MCOs to pursue quality-based alternative payment models (APMs) with providers to help them achieve higher performance on P4Q measures. P4Q programs were suspended for 2020 and 2021 in response to the federal Public Health Emergency. See Alternative Payment Model (APM).

Pediatric Quality Indicator (PDI)

Pediatric quality indicators (PDIs), like prevention quality indicators (PQIs), use hospital discharge data to measure quality of care. They target ambulatory care sensitive conditions of children and youth for which effective outpatient care or early intervention may prevent hospitalization, complications or more severe disease. See Prevention Quality Indicators (PQIs).

Performance Improvement Project (PIP)

Performance improvement projects (PIPs) are an integral part of Texas' Managed Care Quality Improvement Strategy. Under federal law, all states with managed care programs must ensure that managed care organizations (MCOs) conduct PIPs. PIPs must use

ongoing measurements and interventions to achieve significant improvement over time in health outcomes and enrollee satisfaction. MCOs conduct PIPs to improve areas of care identified as needing improvement by the Health and Human Services Commission (HHSC) in consultation with Texas' external quality review organization (EQRO). See External Quality Review Organization (EQRO).

Per Member Per Month (PMPM)

Per member per month (PMPM) rates are calculated for each risk group within each of the service areas in Texas. PMPM rates are based on encounter data and form the basis for the capitation rates paid to managed care organizations (MCOs) for the delivery of contractually required services under the Medicaid and Children's Health Insurance Program (CHIP) programs. See Capitation Rate and Encounter Data.

Personal Assistance Services (PAS)

Personal Assistance Services (PAS) are state plan or Medicaid waiver, community-based, long-term services and supports (LTSS) benefits for children and adults. PAS includes assistance with activities of daily living (ADLs) or instrumental activities of daily living (IADLs).

In managed care, PAS can be delivered as an entitlement service, through the STAR+PLUS home and community-based services component, STAR Health, or through Community First Choice (CFC). See Activities of Daily Living (ADLs), Community First Choice (CFC), Instrumental Activities of Daily Living (IADLs) and Long-Term Services and Supports (LTSS).

Personal Care Services (PCS)

Personal Care Services (PCS) are state plan, community-based, long-term services and supports (LTSS) benefits for children that include assistance with activities of daily living (ADLs) or instrumental activities of daily living (IADLs). See Activities of Daily Living (ADLs), Instrumental Activities of Daily Living (IADLs) and Long-Term Services and Supports (LTSS).

Pharmacy Benefits Manager

Each Medicaid and Children's Health Insurance Program (CHIP) managed care organization (MCO) contracts with a pharmacy benefits manager (PBM) to process prescription claims. The PBMs contract with pharmacies that dispense medications to Medicaid and CHIP managed care members. MCOs must allow any pharmacy provider that is willing to accept the financial terms and conditions of the contract to enroll in the MCO's network.

Postpartum Depression

Postpartum depression (PPD) is a mental health disorder that women may experience after giving birth. Symptoms must meet the diagnostic criteria set forth in the latest edition of the American Psychiatric Association's Diagnostic and Statistical Manual of Mental Disorders (DSM). Given the far-reaching impacts of PPD on the health of women and their children, the Health and Human Services Commission (HHSC) launched several initiatives and published a five-year PPD strategic plan to increase awareness, education and continuity of care for women with PPD.

Potentially Preventable Admission (PPA)

A potentially preventable admission (PPA) is a hospital admission or long-term care facility stay that might have been reasonably prevented with adequate access to ambulatory care or health care coordination. See Potentially Preventable Event (PPE).

Potentially Preventable Complication (PPC)

A potentially preventable complication (PPC) is a harmful event or negative outcome, such as an infection or surgical complication, which occurs after a hospital admission or a long-term care facility stay and that might have resulted from care, lack of care or treatment during the admission or stay. See Potentially Preventable Event (PPE).

Potentially Preventable Emergency Department Visit (PPV)

A potentially preventable emergency department visit (PPV) is emergency treatment for a condition that could have been treated or prevented by a physician or other health care provider in a non-emergency setting. See Potentially Preventable Event (PPE).

Potentially Preventable Event (PPE)

Potentially preventable events (PPEs) are encounters that could have been prevented that lead to unnecessary services or contribute to poor quality of care. PPEs are used to measure care quality. See Potentially Preventable Hospital Admission (PPA), Potentially Preventable Complication (PPC), Potentially Preventable Readmission (PPR) and Potentially Preventable Emergency Department Visit (PPV).

Potentially Preventable Readmission (PPR)

A potentially preventable readmission (PPR) is a return hospitalization within a set time that might have resulted from problems in care during a previous hospital stay or from deficiencies in a post-hospital discharge follow up. See Potentially Preventable Event (PPE).

Preadmission Screening and Resident Review (PASRR)

A Preadmission Screening and Resident Review (PASRR) supports people with a severe mental illness, intellectual disability or developmental disability (who are moving into a nursing facility) to ensure they are thoroughly evaluated and are able to live in the most appropriate and least restrictive setting of their choice. The PASRR process screens and identifies the goals, wishes, and needed services and supports for people to live safely and integrated within their community to maximize their potential. If a person with a severe mental illness, intellectual disability or developmental disability is moving into a nursing facility and needs higher-level services and supports, then they may be PASRR positive and eligible for enhanced levels of care and specialized services.

Preferred Drug List

The Preferred Drug List (PDL) is a tool used to control growing Medicaid drug costs while also ensuring program recipients can obtain medically necessary medicines. The PDL in Texas classifies drugs as preferred or non-preferred based on safety, efficacy and cost-effectiveness. Prescribers who choose non-preferred medications for their patients must obtain prior authorization. The Vendor Drug Program (VDP) and its vendors perform supplemental rebate negotiation with manufacturers and manage the PDL centrally across all Medicaid programs. Participating managed care organizations (MCOs) must use the PDL in administering pharmacy benefits to their members. See Non-Preferred Prior Authorization, Texas Drug Utilization Review (DUR) Board and Vendor Drug Program (VDP).

Prescription Drugs

Prescription drugs are pharmaceuticals that require a medical prescription from a provider to be dispensed. Outpatient prescription drugs are a benefit of the Children's Health Insurance Program (CHIP) and all managed care programs. For those enrolled in fee-for-service (FFS), Texas pays for all outpatient drug coverage through the Vendor Drug Program (VDP). See Vendor Drug Program (VDP).

Prevention Quality Indicator (PQI)

Prevention quality indicators (PQIs) use hospital discharge data to measure quality of care for ambulatory care sensitive conditions for which effective outpatient care or early intervention could prevent hospitalization, complications or more severe disease.

Primary Care Provider (PCP)

In managed care, primary care providers (PCPs) coordinate the care of Medicaid and CHIP members and are responsible for providing initial and primary care to patients, maintaining continuity of care and making referrals to specialists. PCPs tend to be general practitioners, family practice doctors, pediatricians, OB/GYNs, specialty trained nurses

or health clinics. Once enrolled in one of the managed care programs, members pick a managed care organization (MCO), which includes a directory of PCPs contracting with that plan. See Service Coordination.

Primary Home Care (PHC)

Primary Home Care (PHC) services are state plan, community-based, long-term services and supports (LTSS) benefits for children and adults that include assistance with activities of daily living (ADLs) or instrumental activities of daily living (IADLs).

Prior Authorization

A member's physician issues a prior authorization to document the medical necessity of a medication or medical service in certain situations before a managed care organization (MCO) can approve that medication or service. See Clinical Prior Authorization.

Profit Limit

Managed care organizations (MCOs) are paid on a per member per month (PMPM) basis, also called a capitation rate. This rate includes a risk margin to account for fluctuations in predicted claims cost, which may result in profit for the plan. The Health and Human Services Commission (HHSC) recovers any earned profits greater than three percent through a tiered experience rebate system. See Capitation Rate, Experience Rebate, Per Member Per Month (PMPM) and Risk Margin.

Programs of All-Inclusive Care for the Elderly (PACE)

Programs of All-inclusive Care for the Elderly (PACE) is a comprehensive care approach providing an array of services for a capitated monthly fee below the cost of comparable institutional care. PACE participants must be age 55 or older, live in a PACE service area, qualify for nursing facility level of care, and be able to live safely in the community at the time of enrollment. PACE participants receive all medical and social services needed through their PACE provider. PACE is available in Amarillo, El Paso and Lubbock.

Promoting Independence (PI) Initiative

The Promoting Independence (PI) Initiative, also called Money Follows the Person (MFP) policy, helps people in need of long-term services and supports (LTSS) move from facilities to community-based services. This better allows people to choose how and where they receive their LTSS. Other support services have since been developed to help identify people who want to leave an institutional setting and to assist them in relocating back to the community. PI is considered a reserve capacity group. See Reserve Capacity Group.

Prospective Drug Utilization Review (DUR)

Prospective Drug Utilization Reviews (DURs) evaluate each member's drug history before medication is dispensed to ensure appropriate and medically necessary utilization. Therapeutic criteria for prospective DURs are determined by the Texas DUR Board. See Texas Drug Utilization Review (DUR) Board.

Prospective Payment System (PPS)

A prospective payment system (PPS) is a method of reimbursement in which payment is made based on a predetermined, fixed amount. The payment amount for a particular service is based on the classification system of that service. PPSs are used for inpatient and outpatient hospital reimbursement and rural health clinic (RHC) reimbursement. Federally Qualified Health Centers (FQHCs) may be reimbursed using a PPS system or an alternative prospective payment system (APPS). PPS rates are adjusted for inflation annually using the Medicare Economic Index (MEI) for primary care. See Alternative Prospective Payment System (APPS).

Provider Agency

Provider agencies are the employers of attendants or other direct service workers, who provide long-term services and supports (LTSS), and are responsible for all employment and business operations-related activities. Provider agencies are licensed or certified by the Health and Human Services Commission (HHSC) and must comply with HHSC licensure and program rules. See Agency Option.

Provider Credentialing

The process through which managed care organizations (MCOs) ensure that each health care provider meets all professional standards, including licensure.

Provider Enrollment Screening

The Office of Inspector General (OIG), in close collaboration with the Health and Human Services Commission (HHSC), completes the required state and federal disclosure and screening activities for high-risk providers seeking to enroll, re-enroll or revalidate their enrollment in Medicaid and other HHSC programs. See Office of Inspector General (OIG).

Psychiatric Hospital

A psychiatric hospital is an institution primarily engaged in providing psychiatric services for the diagnosis and treatment of mentally ill people. The provisions for certification apply only where the entire institution is primarily for the treatment of mental illness. A psychiatric wing of a general hospital may not be certified.

Public Health Emergency

The President of the United States or the United States Secretary of Health and Human Services may declare a public health emergency (PHE) when a disease or disorder presents a significant risk to public health, including outbreaks of infectious diseases or bioterrorist attacks. PHEs authorize the Secretary of Health and Human Services to take certain discretionary actions. If the president authorizes the PHE, these actions can include waiving or modifying certain requirements for Medicare, Medicaid and State Children's Health Insurance Programs (SCHIPs). Many elements of Texas Medicaid and CHIP, including enrollment, funding, services and program oversight, were impacted by the most recent federal PHE, declared in March 2020 in response to the COVID-19 pandemic. The federal government officially ended the COVID-19 PHE on May 11, 2023. See Continuous Medicaid Coverage and COVID-19.

Public Health Provider-Charity Care Program

Authorized under the current Texas Healthcare Transformation and Quality Improvement Program (THTQIP) also referred to as the 1115 Transformation Waiver, the Public Health Provider-Charity Care Program (PHP-CCP) began operating in October 2021 and is designed to allow qualified providers to receive reimbursement for the cost of delivering uncompensated health care services, including behavioral health services, immunizations and other preventative services. Eligible providers include Community Mental Health Clinics (CMHCs), Community Centers, Local Behavioral Health Authorities (LBHAs), Local Mental Health Authorities (LMHAs), Local Health Departments (LHDs) and Public Health Districts (PHDs).

***Qualified Aliens***

Qualified aliens are non-citizen Texas residents who are potentially eligible for certain programs like Medicaid. Qualified aliens may be subject to different requirements, such as waiting periods and time-based eligibility limitations. There are several categories, including but not limited to: Legal Permanent Residents (LPRs), asylees and refugees, and victims of human trafficking. See Legal Permanent Resident (LPR).

Qualified Disabled Working Individuals

Qualified Disabled Working Individuals are individuals who are no longer entitled to free Medicare Part A benefits because they are working. Their income must be less than or equal to 200 percent of the federal poverty level (FPL). Qualified disabled working individuals do not receive regular Medicaid benefits. The Texas Medicaid program only pays the Medicare Part A premiums.

Quality Assessment and Performance Improvement (QAPI) Projects

Federal regulations require Medicaid health plans to develop, maintain and operate quality assessment and performance improvement (QAPI) projects. QAPI projects are ongoing for all services that the managed care organization (MCO) provides.

Quality Incentive Payment Program (QIPP)

The Quality Incentive Payment Program (QIPP) seeks to improve quality and innovation in the provision of nursing facility services. Both public and private nursing facilities can participate in the program. Payments are made quarterly by the STAR+PLUS managed care organizations (MCOs) to the nursing facilities, based on their completion of required quality improvement activities and their performance on the agreed-upon quality measures.

***Readiness Reviews***

The Health and Human Services Commission (HHSC) conducts readiness reviews to determine if a managed care organization (MCO) can provide the services that it is being contracted to provide. Readiness reviews include all activities the MCO must complete prior to the operational start date of a contract.

Research and Demonstration 1115 Waivers

Research and Demonstration 1115 waivers allow flexibility for states to test new ideas for operating their Medicaid programs — including implementing statewide health system reforms, providing services not typically covered by Medicaid, or allowing innovative service delivery systems to improve care, increase efficiencies and reduce costs.

Reserved Capacity Group

A portion of 1915(c) waiver program slots is set aside for people in reserve capacity groups. When a waiver program slot opens, a person from a reserve capacity group bypasses the general interest list and is given an eligibility determination for the program. Examples of reserved capacity groups are Crisis Diversion and the Promoting Independence (PI) Initiative. See Crisis Diversion and Promoting Independence (PI) Initiative.

Resource-Based Fee (RBF)

Under the fee-for-service (FFS) delivery model, resource-based fees (RBFs) are a type of Medicaid rate used to pay physicians and other practitioners for services. Rates are uniform statewide and are categorized as either RBFs or access-based fees (ABFs).

RBFs are calculated based on the actual resources required by an economically efficient provider to deliver a service. Reimbursement rates for physician and other practitioner services are evaluated at least once every two years as a part of a biennial fee review process. See Access-Based Fee (ABF) and Fee-for-Service (FFS) Rates.

Retrospective Drug Utilization Review (DUR)

Retrospective Drug Utilization Reviews (DURs) examine prescribing practices, a person's medication use and pharmacy dispensing practices, after a person has received the medication. The Health and Human Services Commission (HHSC) and managed care organizations (MCOs) conduct multiple reviews each calendar year on topics such as identifying patterns of drug misuse, medically unnecessary prescribing or inappropriate prescribing. Intervention letters are sent to physicians to help better manage a person's drug therapy. The Texas Drug Utilization Review (DUR) Board also reviews and approves the retrospective DURs for fee-for-service (FFS). See Prospective Drug Utilization Review (DUR) and Texas Drug Utilization Review (DUR).

Risk Margin

In capitation rate setting, a risk margin is added in case of possible fluctuations in predicted claims cost. This margin is calculated as a percentage of the initial capitation rate. To the extent that a managed care organization (MCO) successfully manages member care and keeps medical costs and administrative costs on target, the risk margin may result in profit for the health plan. When the Health and Human Services Commission (HHSC) reduces the risk margin, it can potentially translate to additional profit limits. See Capitation Rates and Profit Limits.

Rural Health Clinics (RHC)

A rural health clinic (RHC) is a clinic located in a rural area, designated by the U.S. Health Resources and Services Administration as a “shortage area.” To qualify as an RHC, the clinic must be located in a non-urbanized and medically underserved area and have a nurse practitioner or physician assistant in the clinic 50 percent of the time. An RHC may not exist as a rehabilitation agency or serve primarily as a treatment facility for mental diseases.

Rural Hospital

A rural hospital is typically categorized by its location outside of a city or metropolitan area and by its size and caseload, which are usually considerably smaller than urban hospitals.

***School Health and Related Services (SHARS)***

The School Health and Related Services (SHARS) program reimburses independent school districts, including public charter schools, for providing Medicaid services to children with disabilities. This program covers certain health-related services documented in a student’s Individualized Education Program or, for audiology services only, a student’s 504 plan (pending CMS approval). Services include: audiology services; counseling; physician and nursing services; physical, speech, and occupational therapies; personal care services; and psychological services.

Service Area (SA)

Service areas (SAs) are the defined geographic regions within the state where services are delivered by certain health plans and their providers. SAs may also be referred to as Service Delivery Areas (SDAs).

Service Comparability

In general, the state is required to provide the same level of services to all members, except where federal law specifically requires a broader range of services or allows a reduced package of services.

Service Coordination

Managed care organizations (MCOs) must provide service coordination to all MMP, STAR Health, STAR Kids and STAR+PLUS members, and to a subset of STAR and CHIP members who meet the eligibility criteria. Service coordination, as well as eligibility for the service, may be defined differently by program. See A Closer Look, page 50.

Service Responsibility Option

Service Responsibility Option (SRO) is available in managed care programs, Community Attendant Services (CAS) and Primary Home Care (PHC). SRO is a hybrid of the agency option and Consumer Directed Services (CDS) option in which a person, the provider agency and, when applicable, the managed care organization (MCO) work together to give the person increased control over the delivery of their own services. In managed care, services with the CDS option also have SRO. See Agency Option and Consumer Directed Services (CDS) Option.

Single State Agency

As required by federal law, a single agency must be designated by each state to administer and supervise the administration of the Medicaid State Plan. In Texas, the Health and Human Services Commission (HHSC) fulfills this function. See Health and Human Services Commission (HHSC).

STAR

STAR is a statewide managed care program primarily for pregnant women, low-income children and their caretakers. Most people in Texas Medicaid get their coverage through STAR.

STAR Health

STAR Health is a statewide managed care program that provides coordinated health care services to children and young adults in foster care or kinship care, including those receiving benefits under the Medically Dependent Children Program (MDCP) waiver.

STAR Kids

STAR Kids is a statewide managed care program for children and youth age 20 and younger with disabilities, including children and youth receiving benefits under the Medically Dependent Children Program (MDCP) waiver.

STAR+PLUS

STAR+PLUS is a statewide managed care program for adults with disabilities and those age 65 and older.

STAR+PLUS Home and Community-based Services (STAR+PLUS HCBS) Program

STAR+PLUS Home and Community Based Services (STAR+PLUS HCBS) is a program delivered through managed care that provides a cost-effective alternative to living in a nursing facility for people who are elderly or have disabilities. STAR+PLUS HCBS is authorized through the 1115 Transformation Waiver. Eligibility determinations for STAR+PLUS HCBS are based on medical necessity as outlined in the Texas Administrative Code and STAR+PLUS contract. Managed care organizations (MCOs) provide medical necessity assessments, which are used by the Health and Human Services Commission (HHSC) to decide whether a person qualifies for STAR+PLUS HCBS. STAR+PLUS HCBS has an interest list for applicants who do not have Medicaid. See Appendix B, page 161, for a full list of STAR+PLUS HCBS services. See STAR+PLUS.

State-Owned Teaching Hospital

Teaching hospitals provide medical education, including post-graduate residency training programs, which incur higher expenses than hospitals without these programs. The portion of these costs attributable to serving Medicaid patients may be covered by Graduate Medical Education (GME) payments. See Graduate Medical Education (GME) Payments.

State Plan

State plans describe the nature and scope of the Medicaid program, including administration, member eligibility, benefits and provider reimbursement. The Centers for Medicare & Medicaid Services (CMS) must approve all state plans. The state plan in Texas gives the Health and Human Services Commission (HHSC), as the single state agency, the authority to administer the Medicaid program. See Single State Agency.

State Supported Living Center (SSLC)

State Supported Living Centers (SSLCs) are certified intermediate care facilities for individuals with an intellectual disability or related condition (ICFs/IID) that serve people with an intellectual disability who have medical or behavioral health needs. SSLCs provide 24-hour residential services, comprehensive behavioral treatment, health care and other services in a campus setting.

Statewide Availability

In general, a state must offer the same benefits to everyone throughout the state. Exceptions to this requirement are possible through Medicaid waiver programs and special contracting options.

Substance Use Disorder (SUD)

The problematic use of alcohol, prescription drugs, illegal drugs or other substances that lead to significant impairment or distress. The pattern must meet the diagnostic criteria set forth in the latest edition of the American Psychiatric Association's Diagnostic and Statistical Manual of Mental Disorders (DSM). SUD Medicaid benefits include assessment/evaluation, individual and group counseling, medication assisted therapy (MAT), and residential and withdrawal management services. Except for MAT, SUD services must be provided by a licensed chemical dependency treatment facility (CDTF). See Medication Assisted Therapy (MAT).

Sufficient Coverage

Federal law allows states to determine what constitutes "reasonably sufficient" coverage in terms of the amount, duration and scope of services. Each state defines these parameters; thus, state Medicaid plans vary in what and how much they cover.

Supplemental Hospital Funding

The Health and Human Services Commission (HHSC) administers supplemental hospital payment programs that help cover the cost of uncompensated care, incentivize improvements to care quality and fund graduate medical education. Examples include the Uncompensated Care (UC) and Delivery System Reform Incentive Payment (DSRIP) pools created under the 1115 Transformation Waiver. See Delivery System Reform Incentive Payment (DSRIP) Pool, Uncompensated Care Pool, and the 1115 Transformation Waiver.

Supplemental Nutrition Assistance Program (SNAP)

The Supplemental Nutrition Assistance Program (SNAP), also known as food stamps, helps people buy the food they need for good health.

Supplemental Security Income (SSI)

Supplemental Security Income is a federal cash assistance program for low-income people with disabilities and those age 65 and older. In Texas, all people eligible for SSI are automatically eligible for Medicaid.

Suspension of Default Enrollment

Suspension of default enrollment is a contractual non-compliance remedy that temporarily stops a managed care organization from receiving new members through the default enrollment process. See Default Enrollment.



1115 Transformation Waiver

The Health and Human Services Commission (HHSC) submitted — and the Centers for Medicare & Medicaid Services (CMS) approved — a proposal for a 10-year Section 1115 Demonstration waiver through Sept. 30, 2030. The 1115 Transformation Waiver, also known as the Texas Healthcare Transformation and Quality Improvement Program (THTQIP), provides the federal authority for operations of most of the state’s managed care programs, including STAR, STAR+PLUS, STAR Kids and the Children’s Dental Program, while preserving supplemental funding for providers. The waiver contains several directed payment programs (DPPs) and supplemental payment programs including the Public Health Provider-Charity Care (PHP-CCP) and Uncompensated Care (UC). See Directed Payment Program (DPP), Public Health Provider-Charity Care Program and Uncompensated Care Pool.

Targeted Review

The Health and Human Services Commission (HHSC) conducts a targeted review of a managed care organization (MCO) when a significant or recurring problem occurs. The scope, entity and focus of targeted reviews vary based on complaints received and past instances of non-compliance.

Teledentistry

Teledentistry refers to dentistry services delivered by a dentist or dental hygienist using audio-visual telecommunications.

Telehealth

Non-medical, non-dental health services, like behavioral health services, delivered by a licensed professional using audio-visual or audio-only telecommunications.

Telemedicine

Health care delivered by a medical professional, such as a doctor, using audio-visual telecommunications.

Teleservices

Teleservices is an umbrella term that encompasses telehealth, telemedicine, teledentistry and home telemonitoring.

Temporary Assistance for Needy Families (TANF)

The Temporary Assistance for Needy Families (TANF) program is a cash-assistance program that helps families pay for basic living needs such as food, housing, utilities, childcare and other expenses.

Texas Department of Family and Protective Services (DFPS)

The Texas Department of Family and Protective Services (DFPS) is responsible for investigating charges of abuse, neglect or exploitation of children, elderly adults and adults with disabilities. DFPS also manages children in state conservatorship or foster care.

Texas Department of Insurance (TDI)

The Texas Department of Insurance (TDI) regulates both commercial and state-funded health insurance plans because managed care organizations (MCOs) are licensed as Health Maintenance Organizations (HMOs) in Texas. TDI requires MCOs to file a network adequacy report once a year. See Network Adequacy.

Texas Drug Utilization Review (DUR) Board

The Texas Drug Utilization Review (DUR) Board is a Health and Human Services Commission (HHSC) advisory board whose members are appointed by the HHSC Executive Commissioner. The duties of the Texas DUR Board include developing and submitting recommendations to HHSC for the Preferred Drug List (PDL), suggesting restrictions or prior authorizations for certain prescription drugs, developing and reviewing educational interventions for Medicaid providers and reviewing drug utilization across the Medicaid program. See Clinical Prior Authorization and Preferred Drug List (PDL).

Texas Formulary

The Texas Formulary is a list of all the drugs covered by Texas Medicaid and the Children's Health Insurance Program (CHIP).

Texas Healthcare Learning Collaborative (THLC)

Texas Healthcare Learning Collaborative (THLC) is a website that serves as a public reporting platform, contract oversight tool, and a tool for managed care organization (MCO) quality improvement efforts. It was developed for use by the Health and Human Services Commission (HHSC), MCOs, providers and the public to obtain up-to-date MCO and hospital performance data on key quality-of-care measures — including potentially preventable events (PPEs), Healthcare Effectiveness Data and Information Set (HEDIS) data and other quality-of-care information.

Texas Health Steps

Texas Health Steps is a required program, known in federal law as the Early and Periodic Screening, Diagnosis and Treatment (EPSDT) program. Texas Health Steps provides medical and dental prevention and treatment services for children of low-income families who are age 20 and younger and are enrolled in Medicaid. The program offers comprehensive and periodic evaluation of a child's health — including growth and development, nutritional status, and vision, dental and hearing care. See Comprehensive Care Program (CCP) and Early and Periodic Screening, Diagnosis and Treatment (EPSDT) Program.

Texas Home Living (TxHmL)

Texas Home Living (TxHmL) is a 1915(c) waiver program that provides community-based services to current Medicaid members with intellectual disabilities or related conditions, as an alternative to an intermediate care facility for individuals with an intellectual disability or related condition (ICF/IID). Eligibility determinations for TxHmL are based on level of care (LOC) criteria outlined in the Texas Administrative Code. TxHmL has an interest list. See Appendix B, page 162, for a full list of TxHmL services.

Third Party Liability (TPL)

Third-party liability (TPL) is a requirement of federal law for Medicaid to be the payer of last resort. This means that other sources of coverage a member or Medicaid-eligible person may have, such as private health insurance, may be required to pay claims before Medicaid. Medicaid is also required to take reasonable measures to identify liable third parties and process claims accordingly.

Time and Distance Standards

Time and distance standards are metrics used to measure whether managed care organization (MCO) or dental maintenance organization (DMO) provider networks provide members sufficient access to medically necessary services. HHSC uses time or distance standards as part of its MCO and DMO monitoring and oversight process. Time refers to how many days a member must wait for an appointment, and distance refers to how many miles they must travel for care. Standards vary by type of provider. See Network Adequacy.

***Uncompensated Care (UC) Pool***

The Uncompensated Care (UC) pool is a hospital funding pool under the Texas Healthcare Transformation and Quality Improvement Program also known as the 1115 Transformation Waiver. UC payments are cost-based and help offset the costs of uncompensated care provided by hospitals and other health care providers. Though previously defined as either unreimbursed costs for Medicaid or uninsured patients incurred by hospitals, uncompensated care costs are currently defined as unreimbursed charity care costs. The pool has historically been smaller than the reported total uncompensated care costs. See the 1115 Transformation Waiver.

Uniform Hospital Rate Increase Program (UHRIP)

The Uniform Hospital Rate Increase Program (UHRIP) is designed to reduce hospitals' uncompensated care costs through enhanced payments for medically necessary covered hospital services provided to managed care members.

Uniform Managed Care Contract

The uniform managed care contract (UMCC) is a legal document that establishes the requirements that all managed care organizations (MCOs) must follow.

Urban Hospital

An urban hospital is categorized by its location within a city or metropolitan area and its size and caseload, which are usually considerably larger than rural hospitals.

***Value-Based Enrollment***

Value-based enrollment is the mechanism the Health and Human Services Commission (HHSC) uses for determining which managed care organization (MCOs) a new member will enroll with if they do not choose their MCO. New members who do not select an MCO are default enrolled. Value-based enrollment scores MCOs to determine each MCO's monthly auto-enrollment allocation. HHSC has created a single value score based on key performance metrics. Scores are calculated by service area and used, along with member choice data, to determine an MCO's monthly target auto-enrollment allocation. See Default Enrollment.

Vendor Drug Program

The Vendor Drug Program (VDP) provides outpatient drug coverage for individuals enrolled in fee-for-service (FFS). In addition, the VDP maintains control of certain aspects of the pharmacy administration for both FFS and managed care—including managing federal and supplemental drug rebates, the Preferred Drug List (PDL), and clinical prior authorizations. See Clinical Prior Authorization, Fee-for-Service (FFS) and Preferred Drug List (PDL).



Waivers

Federal law allows states to apply to the Centers for Medicare & Medicaid Services (CMS) for permission to depart from certain Medicaid requirements. Federal law allows three types of waivers: Research and Demonstration 1115 waivers, Freedom of Choice 1915(b) waivers, and Home and Community-Based Services (HCBS) 1915(c) waivers.

Waste

Waste refers to any practice that a reasonably prudent person would deem careless or that would allow inefficient use of resources, items or services.



Your Texas Benefits Website

A self-service website (YourTexasBenefits.com) where people can apply for Medicaid and other Texas Health and Human Services Commission (HHSC) programs. Members of Medicaid and the Children's Health Insurance Program (CHIP) can also sign on to: view benefit information, edit and manage personal information, select their health plan, print temporary Medicaid cards or order replacement cards, set up and view their Texas Health Steps alerts, and view services provided by Medicaid.

Youth Empowerment Services (YES)

The Youth Empowerment Services (YES) is a 1915(c) waiver program that provides community-based services to children and adolescents age 3 through age 18 who have severe mental, emotional or behavioral disturbances, and their families.

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- All MCS areas
- Access and Eligibility Services
- Actuarial Analysis
- Behavioral Health Services
- Budget Management
- Forecasting
- Office of Data Analytics and Performance
- Office of the Inspector General
- Provider Finance
- Regulatory Services

Every attempt has been made to ensure the accuracy of the material reported in this book at the time of publication in December 2024.

